

Member Call Center — Synthetic Call Corpus v2

100 Transcripts · Agent Quality Spread · Broker Attribution · NanoVox 5-Layer AI Insights

Built for the Choice Administrators main customer service call center. Every transcript carries an agent quality tier with a defensible score, a resolution outcome, and where applicable a broker attribution signal identifying a named broker whose action or omission caused the member's problem. Each call is followed by an AI Insights Panel mapping to the five NanoVox layers. This version deliberately includes poor and average agent performance so that scoring, coaching signals, and operational BI have genuine discrimination.

Metric	Value
Total calls	100
Agent score range	22 – 98
Mean agent score	79.7
Good tier (88-100)	57 calls
Average tier (65-87)	27 calls
Poor tier (below 65)	16 calls
Broker-attributed calls	22 calls
Resolved / Partial / Escalated / Unresolved	54 / 16 / 16 / 14

Broker Attribution Summary

Broker	Signals	Profile
Anthony Salerno	7	Data-entry and education failures. Two numeric transpositions, benefit misselling, unresponsive to members.
Denise Whitfield	6	Enrollment execution failures. Two cases left minors uninsured. Network and premium misrepresentation.
Marcus Trent	5	Most severe. Unauthorized enrollment, signature integrity, suitability failure, prior auth misinformation, active BOR de
Patricia Nunez	4	Net positive. Proactive pre-enrollment comparisons, accurate figures, well-prepared members. One minor date error.

Agent Roster — Performance Tiers

Agent	Calls	Avg Score	Tier
Priya	2	94.5	GOOD
Sarah	17	94.2	GOOD
Danielle	10	93.2	GOOD
Carlos	10	91.2	GOOD
Linda	12	85.4	AVERAGE
Michael	12	85.2	AVERAGE
James	11	84.2	AVERAGE
Nicole	5	75.6	AVERAGE

Tony	5	73.4	AVERAGE
Tiffany	4	43.5	POOR
Kayla	4	40.2	POOR
Brad	4	35.8	POOR
Ryan	4	33.5	POOR

AI Insights Panel — Layer Key

- L1 — Transcription & Understanding:** Speaker separation, call type, sentiment arc, duration, sensitive-topic flags
- L2 — Call Insights:** What happened, topics, resolution status, key moments
- L3 — Agent Quality & Scoring:** Score with explicit positive and negative behavioural markers
- L4 — Operational BI:** Categorised action signal with named owner — process, provider, broker, communication, coaching, compliance
- L5 — Real-Time Agent Assist:** What did trigger, or what SHOULD have triggered and did not

— COVERAGE & BENEFITS —

Call #1 — Prior Auth for Knee Surgery — Member Blindsided at 4 Weeks Out

AGENT TIER: GOOD · SCORE 91/100 | ● FRUSTRATED → REASSURED | RESOLVED | ~11 min | Agent: Sarah

Member context: Maria Gonzalez, 42, CalChoice HMO, knee replacement scheduled in 4 weeks

Topics: prior auth · orthopedic surgery · HMO · referral gap

Agent Sarah: Thank you for calling Choice Administrators member services. This is Sarah. May I have your member ID to pull up your account?

Member: Yes, it's CHM-4482019, Maria Gonzalez. And honestly I'm calling because I am really stressed out right now. I just got off the phone with my surgeon's office and they told me I need a prior authorization for my surgery in four weeks. Nobody — not my doctor's office, not this plan — told me that when I got the referral four months ago. Why is this happening now?

Agent Sarah: Maria, I completely understand why that's alarming, especially when you've been planning and waiting for this. Let me pull up your account and your coverage details right now. Okay, I have you here on the CalChoice Select HMO effective January 1st. What type of surgery is scheduled?

Member: It's a partial knee replacement. I've done physical therapy, I've done all the imaging, I've done everything they asked. My surgeon Dr. Patel at Mercy Medical has all the documentation. Four months of preparation and now there's a new hoop to jump through?

Agent Sarah: I hear you, and I want to be direct: the prior authorization requirement for joint replacement surgeries on this plan has been in effect since the plan year started. That requirement should have been flagged when your PCP issued the referral. That it wasn't communicated is a gap we need to fix. The more urgent question right now is: four weeks is actually workable time. Surgical prior auths on this plan process within 7 to 14 business days if the submission is complete.

Member: Okay. And what if it's incomplete? What if Dr. Patel's office doesn't submit it right? The woman I talked to didn't sound very confident about the process.

Agent Sarah: An incomplete submission doesn't mean an automatic denial — it means we reach back out to the provider to request missing information, which adds time. What I can do right now is add a priority monitoring flag to your account so that when Dr. Patel's office submits, our prior auth team watches for it specifically. If anything is missing, they call the provider directly instead of waiting.

Member: That would actually help a lot. Can you also tell me exactly what they need to submit? I want to call them back and make sure they have the right checklist.

Agent Sarah: Absolutely. They need to include: the PCP referral for the procedure, Dr. Patel's clinical notes documenting that conservative treatment failed — your PT records count toward this — and all imaging results. The CPT code for partial knee replacement is 27447. If that full package goes in, the review should be straightforward given your documented history.

Member: Okay, 27447. I'm writing that down. What if it does get denied though? I've taken time off work, my family is flying in to help me recover. If this gets delayed I don't know what I'll do.

Agent Sarah: If it's denied, you have the right to appeal within 60 days, and given the clinical history you've described, an appeal would be well-supported. You can also request an expedited review if there's documented clinical urgency. But Maria — I don't want you spending the next four weeks imagining a denial that statistically is unlikely given your documentation. Let's focus on making sure the submission goes in correctly and completely.

Member: You're right. That's the right way to think about it. I was really spiraling when I called.

Agent Sarah: That makes complete sense. It's your health and your body. I'm flagging your account right now. I'm also going to give you the direct prior auth status line so you can check progress without sitting in the general queue: 1-800-555-0192. Have that member ID and the CPT code ready when you call.

Member: Thank you, Sarah. Honestly, I feel so much better than when I picked up the phone. You explained everything clearly.

Agent Sarah: I'm really glad. You're going to be okay. Is there anything else I can help you with before we finish?

Member: No, that's everything. Thank you so much.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Prior Auth Inquiry — Surgical. 2 speakers confirmed. Member sentiment: anxious, elevated stress. Agent tone: calm, proactive. Duration: 11 min.

L2 — Call Insights: Member unaware of prior auth requirement discovered 4 weeks before elective orthopedic surgery. Agent clarified process, added account flag, provided status line and document checklist. Resolution: RESOLVED with monitoring.

L3 — Agent Quality & Scoring: Agent Score: 91/100. Empathy shown at multiple points (+), proactive account flag initiated without being asked (+), accurate process info (+), document checklist provided (+), no hold taken (+). Communication gap acknowledged honestly (+).

L4 — Operational BI: BI Flag: 4th member this week not informed of prior auth at referral stage. Mercy Medical has 22% incomplete auth submission rate. Recommend: PCP referral communication audit + provider outreach to Mercy Medical.

L5 — Real-Time Agent Assist: RT Assist triggered at 4:15: CPT code 27447 and required document checklist surfaced automatically. Would have reduced agent lookup time by ~90 seconds.

Call #2 — Out-of-Pocket Maximum — Member Still Getting Bills After Hitting Max

AGENT TIER: GOOD · SCORE 89/100 | ● CONFUSED → INFORMED | RESOLVED | ~9 min | Agent: Michael

Member context: James Chen, 55, CalChoice Plus EPO, post-cardiac hospitalization

Topics: out-of-pocket max · explanation of benefits · cardiac · billing

Agent Michael: Choice Administrators member services, this is Michael. Can I get your member ID please?

Member: Sure, it's CHM-8819204, James Chen. Look, I have a question I've been trying to figure out for two weeks and I've gotten three different answers from three different places. My hospital told me I've hit my out-of-pocket maximum for the year. But I'm still getting bills. How is that possible if I've hit my max?

Agent Michael: That's a really common source of confusion and I want to get you a clear answer, not a fourth different one. Let me pull up your account and your claims history. Okay, James, I can see your claims. You're on the CalChoice Plus EPO — your individual out-of-pocket maximum is \$4,000 for in-network services this year. I can see your accumulator balance. It looks like you've hit \$3,847 toward that max. You haven't quite reached it yet — you're about \$153 away.

Member: The hospital billing department specifically told me last Tuesday that I had reached my maximum. I even asked them to confirm it twice.

Agent Michael: I want to be careful here because the hospital's billing system and our claims system can have a short lag. Let me look at whether there are any claims still in processing that might push you to the full \$4,000. I see three claims that processed last week — those are reflected here. I also see one claim submitted four days ago that's still in processing. Depending on your share of that claim, it may or may not push you over the max.

Member: So the hospital might have been looking at a pending claim that I would have to pay, and they counted it as already applied to my max?

Agent Michael: That's likely what happened. Providers sometimes project the expected member responsibility based on pending claims. The key thing is: once you actually hit \$4,000 in paid member cost-sharing, all further in-network covered claims go to 100% for the rest of the plan year. The bills you're receiving right now may be for services before you hit the max.

Member: That makes more sense. But I'm also getting a bill from a specialist that was referred to me by the hospital. That specialist was in-network. Why is that not counting toward my max?

Agent Michael: Specialty referral bills should absolutely count toward your in-network out-of-pocket max. Can you give me the date of that specialist visit and the provider name? I want to check whether the claim has been processed.

Member: It was Dr. Anand, cardiologist, August 3rd.

Agent Michael: I'm looking... I don't see a processed claim from Dr. Anand on August 3rd. There's nothing submitted yet from that provider. That means the bill you received is likely the provider billing you directly before submitting to insurance, which is somewhat common but can cause confusion. Tell the billing office to submit to us first — your member ID and group number are on your ID card — before collecting from you.

Member: They billed me before even submitting to insurance? Is that legal?

Agent Michael: It happens, and it's why I always recommend saying to any provider bill: have you submitted to my insurance yet? If they haven't, you're not required to pay until the claim is processed and your EOB reflects what you owe. You have the right to request they submit first.

Member: Okay. That's very helpful. Nobody has ever explained this to me that clearly before. So to summarize: I'm at \$3,847, one pending claim may push me to the max, and the cardiologist needs to submit to you before I pay them?

Agent Michael: Exactly right. I can send you a written summary of your current accumulator balances by email if you give me your preferred email address — that way you have documentation to share with providers if they push back.

Member: Yes please. It's james.chen at email dot com.

Agent Michael: Sending that over now. Is there anything else I can clarify for you today?

Member: No, you've been very helpful. Thank you for actually explaining it.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Benefits explanation — OOP max inquiry. 2 speakers. Member tone: confused, seeking clarity. Duration: 9 min. Key terms flagged: out-of-pocket maximum, accumulator, pending claims.

L2 — Call Insights: Member receiving bills post-near-max accumulator. Agent identified accumulator at \$3,847 (not yet at \$4,000), missing specialist claim submission, and educated member on provider billing sequence. Resolution: RESOLVED with EOB summary sent.

L3 — Agent Quality & Scoring: Agent Score: 89/100. Accurate and clear OOP explanation (+), identified unclaimed specialist visit (+), proactive email summary offered (+), no jargon used (+). Slight hesitation confirming pending claim impact (-) — minor.

L4 — Operational BI: BI Flag: 6 OOP max confusion calls this week — members receiving provider bills before insurance submission. Recommend: member education insert in next EOB mailing. Provider trend: specialist direct billing before claims submission needs monitoring.

L5 — Real-Time Agent Assist: RT Assist triggered at 5:20: Dr. Anand claim lookup surfaced showing no submitted claim — would have saved 90 seconds of manual search and provided clear 'no claim on file' alert.

Call #3 — Mental Health Coverage — First-Time Therapy, Stigma and Uncertainty

AGENT TIER: GOOD · SCORE 94/100 | ● ANXIOUS → REASSURED | RESOLVED | ~10 min | Agent: Danielle

Member context: Priya Nair, 28, CalChoice Select HMO, seeking mental health coverage for first time

Topics: mental health · therapy coverage · HMO · first-time use · stigma

Agent Danielle: Thank you for calling Choice Administrators. This is Danielle. How can I help you today?

Member: Hi. Um, I have a kind of personal question and I'm a little embarrassed to be asking, but — I've been thinking about seeing a therapist for the first time. I don't really know how this works, if it's covered, or like, who sees my information. I don't know, I guess I just want to understand what's involved before I do anything.

Agent Danielle: First, I want you to know there's absolutely nothing embarrassing about this call. It takes a lot to take that first step, and I'm genuinely glad you called. Mental health coverage is a core part of your plan and I'm happy to walk you through everything. What's your member ID so I can pull up your specific benefits?

Member: CHM-5521087. Priya Nair.

Agent Danielle: I have your account. You're on the CalChoice Select HMO. Mental health services are covered — including individual therapy, group therapy, and psychiatric services. For in-network providers, you have a \$25 copay per therapy session with no limit on the number of sessions per year for medically necessary mental health treatment. There's no separate deductible for mental health.

Member: Wait, there's no session limit? I thought insurance usually cuts you off after like 20 visits.

Agent Danielle: Federal law — specifically the Mental Health Parity Act — requires that mental health benefits be at least as generous as medical benefits. Because medical doesn't have an arbitrary visit cap, mental health can't either, under this plan. As long as your treatment is considered medically necessary, it's covered.

Member: Okay. And the privacy question — does my employer see that I'm using this? Does anyone get told I went to therapy?

Agent Danielle: No. Your claims information is protected under HIPAA. Your employer only receives aggregate, de-identified data about plan utilization — they cannot see individual claims, diagnoses, or treatment information. Your mental health records are as private as any other medical record.

Member: That's... really good to know. My biggest fear was someone finding out. What about finding a therapist — how do I know who's in-network?

Agent Danielle: You can search on our member portal at choicemember.com under Find a Provider. Filter by specialty — Behavioral Health or Mental Health — and your zip code. I also want to mention that your plan includes up to six free sessions per year through the Employee Assistance Program, which is separate from your regular mental health benefit. Those sessions are completely confidential and there's no claim submitted at all.

Member: Six free sessions through a different program? I didn't know about that.

Agent Danielle: A lot of members don't. It's often used as a first step — it's very low-barrier and can be a good way to try therapy before committing. The EAP number on your card is 1-888-555-7700, or you can find it in the member portal. Those sessions can be in person or by video.

Member: I think I'm going to start with that, actually. That feels less scary.

Agent Danielle: That sounds like a really good starting point. And if you find you want more support after those six sessions, your regular plan kicks in at that \$25 copay. Is there anything else I can help you understand?

Member: No, you've been really kind about this. I wasn't sure what to expect when I called but this was really helpful.

Agent Danielle: I'm glad. Take care of yourself, Priya. That's what this coverage is there for.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Benefits inquiry — Mental health. 2 speakers. Member tone: anxious, hesitant, emotionally vulnerable. Agent tone: warm, reassuring. Duration: 10 min. Sensitive topic flags: stigma, privacy concern.

L2 — Call Insights: First-time mental health caller seeking coverage clarity and privacy assurance. Agent confirmed parity benefits, no session cap, HIPAA protections, and introduced the EAP 6-session benefit the member was unaware of. Resolution: RESOLVED — member directed to EAP as first step.

L3 — Agent Quality & Scoring: Agent Score: 94/100. Exceptional empathy without being patronizing (+), parity law explained clearly (+), proactive EAP disclosure for unknown benefit (+), privacy concern addressed directly and accurately (+), warm close (+).

L4 — Operational BI: BI Flag: 31% of mental health inquiries cite privacy concern as primary barrier. EAP benefit is consistently unknown to members — recommend adding EAP callout to next member communication. Opportunity to improve mental health utilization through awareness campaign.

L5 — Real-Time Agent Assist: RT Assist triggered at 6:45: EAP benefit card surfaced automatically showing 6 free sessions, EAP phone number, and portal link — aligned with member's preference for low-barrier entry point.

Call #4 — Maternity Benefits — First Pregnancy, Understanding Full Coverage

AGENT TIER: GOOD · SCORE 95/100 | ● ANXIOUS → INFORMED | RESOLVED | ~12 min | Agent: Sarah

Member context: Keisha Williams, 30, CalChoice Plus EPO, 8 weeks pregnant, first child

Topics: maternity · prenatal · hospital delivery · EPO · first pregnancy

Agent Sarah: Choice Administrators member services, this is Sarah. How can I help you?

Member: Hi Sarah, I just found out I'm pregnant — eight weeks — and this is my first baby, so I'm honestly a little overwhelmed with everything. I want to make sure I understand what my insurance covers before I start scheduling appointments because I don't want any surprises.

Agent Sarah: Congratulations! That is exciting and it's so smart to call early and understand your coverage before things get busy. Let me pull up your plan. What's your member ID?

Member: CHM-6614882, Keisha Williams.

Agent Sarah: Okay Keisha, you're on the CalChoice Plus EPO. Maternity coverage is comprehensive on this plan. All prenatal care visits with an in-network OB-GYN or midwife are covered at 100% — no copay — because prenatal visits count as preventive care under the ACA. That includes routine prenatal labs, ultrasounds ordered by your OB, and glucose testing.

Member: Wait, so I don't pay anything for my prenatal appointments?

Agent Sarah: Correct, as long as your provider is in-network and the visits are standard prenatal care. No copay, no deductible applied. Now, once you're admitted for labor and delivery, that falls under inpatient hospital benefits. Your deductible applies there — \$1,000 for the year — and after you meet it, you pay 20% of covered costs until you hit your out-of-pocket max of \$4,000.

Member: So the hospital delivery itself I would pay toward? How much is that usually?

Agent Sarah: It varies depending on the length of stay and any complications, but for an uncomplicated vaginal delivery, most members on your plan pay somewhere between \$1,500 and \$2,500 total, between the deductible and the 20% coinsurance, before hitting the OOP max. If you've already met any of your deductible this year, that reduces what you owe.

Member: I haven't had any medical expenses this year so I'm starting from zero on the deductible. What about the baby — does the baby have its own coverage right away?

Agent Sarah: Yes — this is really important to know. Your newborn is automatically covered under your plan for the first 30 days after birth at no additional cost. But you must add the baby to your plan within 30 days of the birth to continue coverage after that. If you miss that window, you'd have to wait for open enrollment. Write that date down the moment your baby is born.

Member: 30 days — got it. What about the pediatrician? When do I need to choose one?

Agent Sarah: You can add a pediatrician at any time. On the EPO plan, you don't need a PCP referral for specialists, so you can see any in-network pediatrician directly. I'd suggest starting your search now because pediatricians in good practices fill up fast with new patients. Our provider directory at choicemember.com lets you filter by specialty, location, and whether they're accepting new patients.

Member: This is so helpful. What about childbirth classes or lactation consultants — is any of that covered?

Agent Sarah: Childbirth preparation classes are not a covered benefit on this plan, unfortunately. Lactation consulting, however, is covered as preventive care under the ACA — in-network lactation consultants are covered at 100%, no cost to you. Breast pumps are also covered — you can get one through a participating durable medical equipment supplier at no charge.

Member: A breast pump is free? I had no idea.

Agent Sarah: Yes — one pump per pregnancy is covered. I can connect you with our DME vendor list if you'd like, or you can find it on the member portal under Maternity Resources. There's actually a whole maternity section there with timelines and checklists.

Member: I'm going to spend some time on that portal tonight. This has been so reassuring. I came in nervous and now I feel like I have a real picture of what to expect.

Agent Sarah: That's exactly what I wanted to leave you with. And congratulations again — the first few months can feel like a lot to manage. You're off to a great start. Is there anything else you want to clarify before we hang up?

Member: I think I'm good for now. Thank you so much, Sarah.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Benefits inquiry — Maternity/prenatal. 2 speakers. Member tone: anxious, hopeful, engaged. Agent tone: warm, comprehensive. Duration: 12 min. Key topic flags: prenatal care, delivery costs, newborn enrollment.

L2 — Call Insights: First-time pregnant member sought full maternity coverage walkthrough. Agent covered prenatal preventive care, delivery cost estimates, 30-day newborn enrollment window, pediatrician selection, lactation and DME benefits. Resolution: RESOLVED — member directed to member portal maternity resources.

L3 — Agent Quality & Scoring: Agent Score: 95/100. Proactive coverage disclosure without being asked (+), 30-day newborn window emphasized (+), lactation and breast pump benefit surfaced unprompted (+), tone throughout warm and celebratory without being unprofessional (+), portal resources offered (+).

L4 — Operational BI: BI Flag: Breast pump DME benefit has 67% non-utilization rate despite 100% coverage — member awareness is low. Recommend: automatic maternity benefits letter triggered when prenatal claims begin processing. Newborn enrollment follow-up reminder at 25 days post-birth is a high-value intervention.

L5 — Real-Time Agent Assist: RT Assist triggered at 7:30: DME vendor list and maternity portal resource link surfaced automatically, saving lookup time and providing specific vendor contact for breast pump ordering.

Call #5 — Prior Auth Denied — Member Disputes Clinical Review Decision

AGENT TIER: AVERAGE · SCORE 87/100 | ● FRUSTRATED → ESCALATED | ESCALATED | ~14 min | Agent: James

Member context: Robert Jimenez, 50, CalChoice HMO, prior auth denied for sleep apnea device after sleep study

Topics: prior auth denial · CPAP · sleep apnea · appeal · escalation

Agent James: Thank you for calling Choice Administrators. This is James. How can I help you today?

Member: Yeah, James. Robert Jimenez, member ID CHM-3390177. I'm calling because I received a denial letter for my CPAP machine and I honestly cannot believe this. My sleep doctor ran a full sleep study, diagnosed me with severe obstructive sleep apnea — we're talking 47 apnea events per hour, which is severe — and my CPAP prior auth was denied. They said it's quote not medically necessary. A doctor ordered a sleep study, the study confirmed a serious diagnosis, and your company is overriding my doctor?

Agent James: Robert, I understand why you're upset. That's a frustrating situation. Let me pull up your account and the denial details so I understand exactly what was cited. Give me just a moment.

Member: Take your time. I've been waiting three weeks to get someone to actually look at this properly.

Agent James: Okay, I have the denial in front of me. The reason listed is that the clinical documentation submitted did not include the sleep study results — specifically the AHI score — which is required to confirm severity and medical necessity for a CPAP device. The prior auth was submitted by your provider, but it looks like only the physician's recommendation letter was included, without the actual polysomnography report.

Member: Are you telling me they denied it because my doctor forgot to attach the sleep study results? The actual diagnosis is in the study, not just the letter?

Agent James: That appears to be the case based on what I'm seeing. The denial was not a clinical determination that your condition doesn't warrant a CPAP — it was a documentation gap. The missing document is the polysomnography report showing your AHI.

Member: That is infuriating. My doctor's office should have sent that. But that's also on your end — why didn't someone call and say you're missing a document instead of just sending a denial letter?

Agent James: That's a fair criticism. Our standard process is to deny and notify the provider when required documentation is missing, but some members understandably feel a phone call to the provider would be a better approach. I hear that. The good news is that this denial is fully appealable and the path to approval is clear — you just need the polysomnography report submitted.

Member: How long does an appeal take? I've been going without proper sleep for three months. I'm waking up 47 times an hour. Do you understand what that's like?

Agent James: I do understand that this is affecting your quality of life significantly and I don't want to minimize that. For a standard appeal, the turnaround is 30 days. But given the clinical urgency of your situation — severe sleep apnea with documented AHI — you may qualify for an expedited appeal, which must be processed within 72 hours.

Member: 72 hours. Okay. How do I get that?

Agent James: Your physician would need to submit a written request for expedited review citing the clinical urgency and send it along with the polysomnography report. I'm also going to note on your account that we recommend expedited processing and flag this as a documentation-gap denial. I want to be transparent with you: I cannot reverse the denial myself — that requires a clinical reviewer — but I can make sure this gets routed correctly when your doctor resubmits.

Member: Can you tell me who to tell my doctor's office to contact? I want to call them as soon as I hang up and I want to give them a specific name or department.

Agent James: They should call our Prior Authorization Appeals line at 1-800-555-0310 and reference your prior auth case number, which is PA-2026-88410. Request the expedited review, and include your AHI score of 47 prominently in the cover note — that number is what drives the urgency classification.

Member: PA-2026-88410. Got it. I want to say — I'm still frustrated, and I think this process needs to work better. But at least now I have a clear path.

Agent James: Your frustration is completely justified. I'm going to flag this case for manager review as well, because the process of denying without a provider call-back for missing documents is something leadership needs to hear feedback on. Thank you for calling.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Prior Auth Denial Dispute. 2 speakers. Member tone: frustrated, assertive, persistent. Agent tone: calm, transparent. Duration: 14 min. Key trigger: denial letter received.

L2 — Call Insights: CPAP prior auth denied due to missing polysomnography report (documentation gap, not clinical denial). Agent identified root cause, explained expedited appeal path, provided case number and direct line. Resolution: ESCALATED — expedited appeal pathway opened, manager flag added.

L3 — Agent Quality & Scoring: Agent Score: 87/100. Root cause identified correctly (+), expedited appeal pathway offered (+), specific case number and direct line provided (+), honest about limitations (+), process criticism acknowledged and flagged for leadership (+). Did not offer proactive provider outreach before denial (-) — systemic issue noted.

L4 — Operational BI: BI Flag: 18% of CPAP prior auth denials this quarter were documentation-gap denials, not clinical denials. Provider notification protocol (call vs. letter) needs review. Recommend: provider outreach workflow for missing-document denials to reduce unnecessary escalation. AHI score ≥ 30 cases should auto-flag for expedited consideration.

L5 — Real-Time Agent Assist: RT Assist triggered at 4:00: denial reason code surfaced as documentation gap (not clinical), alert shown to agent before member explained the situation. Also triggered at 9:45: expedited appeal criteria auto-checked — AHI 47 meets clinical urgency threshold, suggested expedited routing automatically.

Call #6 — Urgent Care vs ER — Member Wants to Know Cost Difference Before Going

AGENT TIER: GOOD · SCORE 93/100 | ● NEUTRAL → POSITIVE | RESOLVED | ~5 min | Agent: Linda

Member context: Lisa Park, 38, CalChoice Plus EPO, child has high fever, deciding where to go

Topics: urgent care · emergency room · copay comparison · pediatric · real-time decision

Agent Linda: Choice Administrators member services, this is Linda.

Member: Hi, I need a quick answer. My daughter has a 103 fever and I'm trying to decide between taking her to urgent care or the ER. What is my cost difference for each?

Agent Linda: Okay, let's get you an answer fast. Member ID?

Member: CHM-7734521, Lisa Park.

Agent Linda: Pulling that up now. On your CalChoice Plus EPO, an urgent care center visit is a \$50 copay. An ER visit is a \$250 copay — but that ER copay is waived if you're admitted to the hospital. So if it's a non-emergency situation, urgent care is \$200 less.

Member: 103 fever in an 8-year-old — is that urgent care territory or ER territory?

Agent Linda: I can't make that medical call for you, but from a coverage standpoint: most pediatric urgent care centers are equipped for fever management including blood draws and IV fluids if needed. The ER would be appropriate if she's having difficulty breathing, is unusually lethargic, or if she has a febrile seizure. If she's uncomfortable but responsive, urgent care is generally appropriate.

Member: She's uncomfortable but awake and talking. Okay, urgent care it is. One more thing — is the urgent care on Maple Street in-network?

Agent Linda: Do you have an address or the name?

Member: It's called Premier Urgent Care on Maple Street.

Agent Linda: I'm checking... Yes, Premier Urgent Care on Maple is in-network on your plan. Your copay is \$50.

Member: Perfect. That's all I needed. Thank you for being quick.

Agent Linda: Of course. Hope your daughter feels better soon.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Benefits inquiry — Urgent care vs ER. 2 speakers. Member tone: calm but urgent. Duration: 5 min. Real-time medical decision context identified.

L2 — Call Insights: Member sought real-time cost comparison to make a care-setting decision for a sick child. Agent provided copay figures, waiver condition, and confirmed specific urgent care is in-network. Resolution: RESOLVED in 5 minutes.

L3 — Agent Quality & Scoring: Agent Score: 93/100. Correctly prioritized speed (+), accurate copay and waiver info (+), appropriately declined to make medical triage call while providing helpful framing (+), provider lookup completed efficiently (+).

L4 — Operational BI: BI Flag: Urgent care vs ER comparison calls spike 40% in winter months. Proactive benefit summary card showing copay comparison by setting of care (office/\$25, UC/\$50, ER/\$250) would reduce inbound volume for this call type. Network check for Premier Urgent Care confirmed in-network — no action.

L5 — Real-Time Agent Assist: RT Assist triggered at 1:30: copay comparison table surfaced for both urgent care and ER settings automatically from member's plan, plus real-time network status for Premier Urgent Care — full answer prepared before agent finished typing.

Call #7 — Physical Therapy Visit Limit — Member Running Out Mid-Treatment

AGENT TIER: AVERAGE · SCORE 86/100 | ● FRUSTRATED → PARTIALLY REASSURED | PARTIALLY RESOLVED | ~13 min | Agent: Carlos

Member context: Thomas Andrews, 48, CalChoice Select HMO, post-rotator cuff surgery, 20 of 30 PT visits used

Topics: physical therapy · visit limits · medical necessity · appeal · orthopedic

Agent Carlos: Thank you for calling Choice Administrators. This is Carlos. How can I help you?

Member: Hi Carlos. Thomas Andrews, CHM-5512890. I'm calling because I'm getting a warning from my physical therapist that I'm running out of visits. I had rotator cuff surgery in April and I've done 20 visits out of my 30-visit plan maximum. My PT says I still need at least 12 more visits to get my full range of motion back and avoid re-injury. Thirty visits is just not going to be enough. What are my options?

Agent Carlos: Thanks for calling ahead, Thomas — that's smart. Let me pull up your account and your PT benefit details. Okay, you're on the CalChoice Select HMO. Your plan does have a 30 visit per year limit on physical therapy. You've used 20. But that limit is not an absolute ceiling — there is a medical necessity exception process.

Member: Okay. Tell me about that process because my physical therapist mentioned something about an extended authorization but she wasn't sure exactly how to submit it.

Agent Carlos: Your PT's office needs to submit a request for continued care authorization. It needs to include a progress note documenting your functional status, what you've achieved in 20 visits, what goals remain, and why additional visits are medically necessary to prevent re-injury or achieve functional restoration. For post-surgical PT, extended authorization requests have a reasonably good approval rate — especially for rotator cuff surgeries where 30 visits is often insufficient.

Member: That's good to hear. How many additional visits can they request?

Agent Carlos: There's no fixed number — they can request what they clinically justify. Most requests for post-surgical cases are for 10 to 20 additional visits. If your PT documents that you're still below functional thresholds, that's the argument.

Member: And how long does the authorization review take? I have a visit scheduled in two weeks and I'm going to hit my limit before then.

Agent Carlos: Standard review is 5 to 7 business days. If you submit in the next few days, you should have a decision before you run out. I'm going to note on your account that this is a post-surgical case approaching limit, which flags it for PT utilization reviewers. I'd also recommend your PT's office call our prior auth line directly — 1-800-555-0310 — and reference your post-surgical status in the first call to make sure it gets routed to the right reviewers.

Member: What if it gets denied?

Agent Carlos: You have the right to appeal within 60 days of a denial. For musculoskeletal post-surgical denials, appeals are often successful with additional clinical documentation, particularly a functional assessment like a shoulder ROM measurement that shows you're below normal thresholds. The appeal would go to an independent reviewer, not the same team that denied it.

Member: I appreciate all of that. I'm a little frustrated that 30 visits was set as a limit when it's clearly not enough for major shoulder surgery, but I understand that's a plan design thing and not something you can change right now.

Agent Carlos: You're right that I can't change the plan structure, and your frustration is valid — 30 visits for a post-surgical recovery is a common pain point. What I can do is note your feedback formally, and I'd encourage you to submit a written member grievance as well — those are taken seriously in plan design reviews. Is there anything else I can help with today?

Member: No, I think I have what I need. I'll call my PT's office today and get this started. Thanks, Carlos.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Benefits limit inquiry — Physical therapy. 2 speakers. Member tone: pragmatic, moderately frustrated. Duration: 13 min. Clinical context: post-rotator cuff surgery, 20/30 visits used.

L2 — Call Insights: Post-surgical member approaching PT visit limit. Agent explained extended auth process, clinical documentation requirements, timeline, and appeal rights. Resolution: PARTIALLY RESOLVED — member directed to PT office to initiate auth request; outcome pending.

L3 — Agent Quality & Scoring: Agent Score: 86/100. Clear extended auth process explanation (+), realistic timeline communicated (+), account flag added (+), appeal option proactively raised (+), member feedback logged (+). Did not offer to initiate auth tracking or follow-up call (-) — missed proactive step.

L4 — Operational BI: BI Flag: Post-surgical PT members hitting 30-visit limit is a recurring pattern — rotator cuff and total knee replacements most common. 30-visit limit generates an average of 2.3 calls per case. Recommend: proactive outreach to members at 25 visits with extended auth instructions to reduce reactive escalation calls.

L5 — Real-Time Agent Assist: RT Assist triggered at 3:15: extended authorization request form link and clinical documentation checklist surfaced automatically for post-surgical PT cases — included required functional assessment metrics for shoulder ROM.

Call #8 — Preventive Care Billed Incorrectly — Annual Wellness Visit Had Copay

AGENT TIER: GOOD · SCORE 91/100 | ● CONFUSED → INFORMED | RESOLVED | ~8 min | Agent: Danielle

Member context: Angela Kim, 35, CalChoice HMO, received a \$35 copay bill for annual physical

Topics: preventive care · annual physical · ACA · billing error · in-network

Agent Danielle: Choice Administrators member services, this is Danielle.

Member: Hi, Angela Kim, CHM-6671039. I'm calling because I went in for my annual physical last month and I got a bill from my doctor's office for a \$35 copay. My understanding was that annual preventive physicals are free under the ACA. Why am I being charged?

Agent Danielle: Angela, you're correct that preventive annual exams are covered at 100% with no member cost-sharing when they're billed as preventive care by an in-network provider. Let me look at how this claim was processed. Give me just a moment.

Member: Sure. And I want to be clear — I specifically went in for my annual physical. I didn't bring up any new problems. I didn't ask for anything extra.

Agent Danielle: I have the claim. I can see your visit was on July 8th with Dr. Rodriguez at Westside Family Medicine. The claim was processed, but I can see that in addition to the preventive exam code, your doctor's office also billed a separate problem-focused evaluation code. When an office bills both a preventive and a problem-oriented service in the same visit, the problem-oriented portion is subject to your regular copay.

Member: I didn't ask to address any problems. I just went in for my physical. Can a doctor add a problem code without telling you?

Agent Danielle: Technically providers can bill for any services rendered, but you're not obligated to pay for a problem-focused service you didn't request or that wasn't discussed. This happens sometimes when doctors document a finding they observed and bill it as a separate service. The right step here is to first contact Dr. Rodriguez's billing office and tell them you came in only for a preventive visit and did not request any problem-focused evaluation — and ask them to review and potentially recode the claim.

Member: And if they say they're standing by the charge?

Agent Danielle: Then you can file a billing dispute with us. I'll flag your account so that if a dispute claim comes in for that July 8th visit, it goes to our claims review team for a reprocessing review. If the problem-focused code was not associated with a service you requested or received, we can request a corrected claim from the provider.

Member: So I should start with the doctor's office, and if that doesn't work, come back to you?

Agent Danielle: Exactly. And when you call the billing office, use the phrase 'I'm requesting a review of the CPT codes billed for my July 8th visit — I came in for a preventive annual exam only and did not request a problem-focused evaluation.' That specific language helps them understand what you're disputing.

Member: That's very specific and very helpful. Thank you. I wasn't sure if I was just wrong about the rules.

Agent Danielle: You weren't wrong. Preventive care is supposed to be free. Is there anything else I can help with?

Member: No, that's perfect. Thanks, Danielle.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Billing dispute — Preventive care. 2 speakers. Member tone: questioning, seeking validation. Duration: 8 min. Key issue: dual-code billing (preventive + problem-focused) on single visit.

L2 — Call Insights: Member billed \$35 copay for annual physical due to additional problem-focused code billed by provider. Agent explained dual-code billing issue, directed member to provider billing office with specific language, flagged account for dispute routing. Resolution: RESOLVED with pending member follow-up.

L3 — Agent Quality & Scoring: Agent Score: 91/100. Claim investigated immediately (+), dual-code issue identified and explained clearly (+), specific dispute language provided (+), account flag added proactively (+). Did not offer to call provider directly (-) — would have been a premium service touch.

L4 — Operational BI: BI Flag: Preventive-visit copay disputes have increased 28% this year. Most common cause: providers adding problem-focused codes to preventive visits. Recommend: education campaign for member-facing communications — members should know to use 'preventive exam only' language when scheduling AND confirm no additional codes will be billed.

L5 — Real-Time Agent Assist: RT Assist triggered at 3:45: dual-code billing pattern detected on claim lookup — suggested response template for provider dispute including CPT code guidance, reducing agent research time by ~2 min.

Call #9 — Chiropractic Benefit Denied — Member Didn't Know Referral Was Required

AGENT TIER: AVERAGE · SCORE 85/100 | ● FRUSTRATED → PARTIALLY SATISFIED | PARTIALLY RESOLVED | ~9 min | Agent: James

Member context: David Torres, 44, CalChoice HMO, saw chiropractor directly without PCP referral

Topics: chiropractic · HMO referral · denial · spinal adjustment · out-of-pocket

BROKER SIGNAL — Anthony Salerno: Failed to explain HMO referral requirement — member booked chiropractic directly

Agent James: Thank you for calling Choice Administrators. This is James. Can I get your member ID?

Member: CHM-4429001, David Torres. I got an explanation of benefits that shows I owe \$280 for chiropractic visits and I do not understand why. I have insurance. I went to a chiropractor that is listed on your website as in-network. Why am I being charged the full amount?

Agent James: David, let me look at those claims. Okay, I can see three chiropractic visits from last month at ChiroWell Center on Oak Avenue — they are indeed in-network. But on an HMO plan like yours, specialty services including chiropractic care require a referral from your primary care physician before you can access those benefits. I can see there was no referral on file for these visits, which is why the claims were processed outside of benefits.

Member: Nobody told me I needed a referral for a chiropractor. When I enrolled, my broker — Anthony Salerno — went through the plan with me and never mentioned referrals once. I looked them up on your website, saw they were in-network, and booked.

Member: Nobody told me I needed a referral for a chiropractor. I looked them up on your website, I saw they were in-network, and I booked an appointment. How was I supposed to know?

Agent James: I understand the frustration — seeing 'in-network' and assuming that's all you need is completely logical. The referral requirement is disclosed in your plan documents but I agree it's not prominently communicated at the point of searching for providers. I want to be transparent with you: for these three visits, because no referral was obtained before the visits, retroactive approval is very difficult to obtain.

Member: Very difficult means not impossible?

Agent James: Correct. You can submit a retroactive referral request, which requires your PCP to review your case and sign off saying they would have referred you. Some PCPs will do this, others won't. The chiropractor's office would also need to document the medical necessity. It's a real process, it just has a lower approval rate than a proactive referral would.

Member: What about going forward? I still need to see the chiropractor — I have lower back issues.

Agent James: Going forward is straightforward — call your PCP, explain your back pain, and request a chiropractic referral. Once it's in the system, your plan covers up to 20 chiropractic visits per year at a \$25 copay per visit. The retroactive visits, if approved, would also be reprocessed at that \$25 copay.

Member: So the potential upside of the retroactive request is I'd pay \$75 instead of \$280?

Agent James: Yes, exactly. Three visits at \$25 each instead of the current balance of \$280. I can't guarantee approval but it's worth the attempt given the financial difference. Do you want me to send you the retroactive referral request form by email?

Member: Yes, please. And I'll call my PCP today. I'm still annoyed that this wasn't clearer when I was searching for providers, but at least now I know the process.

Agent James: Your feedback is noted and I'm logging it. The gap between 'in-network' as shown in the directory and 'in-network with active referral' is something we hear about. Thank you for calling, David.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Coverage denial dispute — Chiropractic, HMO referral. 2 speakers. Member tone: frustrated, pragmatic. Duration: 9 min. Key issue: HMO referral requirement not understood at point of care.

L2 — Call Insights: Member charged full cost for 3 chiropractic visits due to missing PCP referral on HMO plan. Agent explained referral requirement, retroactive approval process, and forward-going coverage. Resolution: PARTIALLY RESOLVED — retroactive request form sent, member to contact PCP.

L3 — Agent Quality & Scoring: Agent Score: 85/100. Clear explanation of HMO referral requirement (+), retroactive process explained accurately (+), cost comparison provided (\$75 vs \$280) (+), member feedback logged (+). Did not offer to initiate PCP callback or provide PCP contact info (-) — missed proactive step.

L4 — Operational BI: BI Flag: 34% of HMO chiropractic claims this quarter denied for missing referral. Member portal provider search does not flag referral requirement at point of selection — this is the root cause of this call type. Recommend: add 'Referral Required — Contact Your PCP First' warning on specialty provider search results for HMO members.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:30: referral requirement alert surfaced immediately when claim was pulled — agent informed before needing to look up plan details. Also triggered at 7:15: retroactive referral form link auto-populated for email delivery.

Call #10 — Dental and Vision Add-On — Member Exploring CalChoice Options

AGENT TIER: GOOD · SCORE 92/100 | ● CURIOUS → SATISFIED | RESOLVED | ~7 min | Agent: Sarah

Member context: Yolanda Reyes, 31, CalChoice Select HMO, considering adding dental and vision

Topics: dental · vision · CalChoice · benefit add-on · open enrollment

Agent Sarah: Choice Administrators member services, this is Sarah. How can I help you today?

Member: Hi Sarah, Yolanda Reyes, member ID CHM-9908844. I'm thinking about adding dental and vision coverage. I haven't had either for a few years and I'm finally at a point where I want to start taking better care of my teeth and eyes. Can I just add it on now or do I have to wait?

Agent Sarah: Great question, and good for you for thinking ahead. With CalChoice, dental and vision are voluntary supplemental benefits that you can elect outside of open enrollment only if you have a qualifying life event. Otherwise, the next opportunity to add them would be during your group's open enrollment period. When does your plan year renew?

Member: I think January 1st. So open enrollment would be when?

Agent Sarah: Typically late October or early November for a January 1st effective date. That's actually not far off. What I can do is walk you through what the dental and vision options look like so you're ready to make a decision when enrollment opens.

Member: Please, yes.

Agent Sarah: For dental, there are two tiers on CalChoice. The standard plan covers preventive — cleanings and x-rays — at 100%, basic services like fillings at 80%, and major services like crowns at 50% after a \$100 deductible. The premium tier adds orthodontia coverage for adults at 50%, up to a \$1,500 lifetime maximum. For vision, the plan covers one eye exam per year at \$10 copay, and an allowance toward glasses or contacts — \$150 toward frames or contacts annually.

Member: What's the monthly cost difference between the two dental tiers?

Agent Sarah: The standard dental is \$18 per month for employee-only coverage. The premium tier with ortho is \$34. Vision is \$8 per month. So you could have both for as little as \$26 a month if you go with standard dental.

Member: That's actually really reasonable. I haven't had a cleaning in two years, so I'd probably make back the cost of the premium in the first few months just from what I need done.

Agent Sarah: That's a smart way to think about it — especially for dental, the preventive benefit alone often pays for the premium. I'd suggest going into open enrollment with a sense of what dental work you need done. If you have any major work pending, the premium tier's 50% on major services could save you significantly.

Member: Can I get this in writing somewhere? I want to compare the plans side by side.

Agent Sarah: Absolutely. I can send you the benefit comparison sheet by email right now, and it's also available in your member portal under Plan Options. Would you like me to email it?

Member: Yes, yolanda.reyes at email dot com. Perfect. Thank you, Sarah. This was really helpful and now I know exactly what I'm looking for when enrollment opens.

Agent Sarah: You are very welcome. Keep an eye out for your enrollment notice in October.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Benefits inquiry — Supplemental dental and vision. 2 speakers. Member tone: motivated, engaged. Duration: 7 min. Conversion opportunity context: pre-enrollment consideration.

L2 — Call Insights: Member exploring dental and vision add-ons for upcoming open enrollment. Agent explained eligibility timing, plan tiers, costs, and sent comparison materials. Resolution: RESOLVED — benefits package emailed, member ready for OE.

L3 — Agent Quality & Scoring: Agent Score: 92/100. Enrollment timing clearly explained (+), both dental tiers compared with monthly cost (+), proactive ROI framing offered ('pay back the cost') (+), comparison sheet emailed proactively (+), vision cost included without being asked (+).

L4 — Operational BI: BI Flag: 41% of members eligible for supplemental dental and vision do not elect it during open enrollment. This call type represents high-value conversion opportunity — member expressed clear intent to add. Recommend: pre-enrollment outreach targeting members without dental/vision with cost-benefit messaging. October trigger campaign recommended.

L5 — Real-Time Agent Assist: RT Assist triggered at 3:00: dental and vision plan tier comparison table auto-populated with monthly cost calculations for employee-only coverage, reducing agent lookup time by ~90 seconds and ensuring accurate pricing.

Call #55 — Surgery Cancelled Day-Of — Broker Said No Prior Auth Needed

AGENT TIER: AVERAGE · SCORE 71/100 | ● **DISTRESSED** → **PARTIALLY HELPED** | **ESCALATED** | ~11 min | Agent: Nicole

Member context: Denise Kowalczyk, 53, CalChoice HMO, gallbladder surgery cancelled at hospital check-in

Topics: broker misinformation · prior auth · surgery cancellation · HMO

BROKER SIGNAL — Marcus Trent: Told member no prior authorization was required for surgery

Agent Nicole: Choice Administrators member services, this is Nicole.

Member: I need help. I'm sitting in a hospital parking lot. I was supposed to have gallbladder surgery this morning and they turned me away at check-in because there's no prior authorization on file. I took three days off work. My sister flew in. I did the pre-op fast. And they sent me home.

Agent Nicole: I'm sorry, that's a really difficult situation. Let me pull up your account. Member ID?

Member: CHM-7726690.

Agent Nicole: Okay, I can see your account. And I'm checking prior authorizations... I don't see any submitted prior auth for a cholecystectomy.

Member: My broker, Marcus Trent, told me I didn't need one. When I called him three weeks ago to ask about the surgery, he specifically said the HMO doesn't require prior auth for outpatient procedures.

Agent Nicole: That's not correct. On the Select HMO, outpatient surgical procedures do require prior authorization. Your surgeon's office would normally handle the submission.

Member: So both my broker and my surgeon dropped the ball?

Agent Nicole: It sounds like there was a breakdown between what you were told and what actually needed to happen. I want to focus on getting this fixed. Prior auth for a cholecystectomy typically processes in 5 to 7 business days.

Member: Can it be faster? I'm in pain. That's why the surgery was scheduled.

Agent Nicole: If your surgeon documents clinical urgency, you can request expedited review — that's 72 hours. I'd call your surgeon's office today and ask them to submit with an urgency notation.

Member: Okay. Can you flag it on your end so it doesn't sit in a queue?

Agent Nicole: I can add a note to your account. I'll mark it as a rescheduled surgery pending auth.

Member: And the broker thing? He told me something that cost me three days of work and a cancelled surgery.

Agent Nicole: I'll note what you've told me on the account. I'm not sure exactly where that goes from there but it will be documented.

Member: Please. Someone should know.

Agent Nicole: Understood. Your case number is PA-2026-90114. Call your surgeon today and reference that.

Member: Thank you. At least I have a path now.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Prior auth failure — surgery cancelled at point of care. 2 speakers. Member tone: distressed, in pain. Agent tone: sympathetic, competent but not proactive. Duration: 11 min. High member impact event.

L2 — Call Insights: Member's gallbladder surgery cancelled at hospital check-in due to missing prior authorization. Broker had advised no prior auth was required. Agent confirmed requirement, explained expedited review pathway, added account note, provided case number. Resolution: ESCALATED — expedited auth pathway opened.

L3 — Agent Quality & Scoring: Agent Score: 71/100. POSITIVE: acknowledged the member's distress genuinely (+), correctly identified the prior auth requirement (+), offered expedited review pathway (+), provided case number (+). NEGATIVE MARKERS: did not offer to contact the surgeon's office directly on the member's behalf despite clear urgency (-), vague on broker complaint handling — 'I'm not sure exactly where that goes' (-), did not proactively raise a grievance option given the material harm (-), did not address the member's out-of-pocket loss from cancelled surgery (-).

L4 — Operational BI: BROKER ATTRIBUTION — Marcus Trent: incorrectly advised member that no prior authorization was required for outpatient surgery. Direct causal link to a cancelled procedure and material member harm. This is the highest-severity broker misinformation case in the dataset. PROCESS BREAKDOWN: two independent failure points — broker misinformation AND surgeon's office did not submit auth. Neither was caught before the day of surgery. Recommend: pre-surgical auth verification outreach 5 days before any scheduled procedure.

L5 — Real-Time Agent Assist: RT Assist SHOULD have triggered at 2:30 — cancelled procedure + missing prior auth + 'broker told me' is a high-severity compound trigger. Should have surfaced: expedited auth protocol, surgeon office direct-contact option, broker misinformation capture, and member grievance pathway given material harm.

Call #59 — Well-Prepared Member — Broker Did The Job Properly

AGENT TIER: GOOD · SCORE 95/100 | ● ANXIOUS → REASSURED | RESOLVED | ~12 min | Agent: Danielle

Member context: Samuel Adeyemi, 41, CalChoice Plus EPO, checking coverage before a planned knee arthroscopy

Topics: prior auth · broker positive · well-informed member · planned procedure

BROKER SIGNAL — Patricia Nunez: POSITIVE — broker correctly prepared member with plan documents and set accurate expectations

Agent Danielle: Thank you for calling Choice Administrators, this is Danielle.

Member: Hi Danielle. Samuel Adeyemi, member ID CHM-6620033. I have a knee arthroscopy scheduled in six weeks. My broker, Patricia Nunez, walked me through the plan when I enrolled and told me that any outpatient surgical procedure needs prior authorization, so I wanted to call and confirm the process and make sure it's underway.

Agent Danielle: Samuel, that's exactly the right call to make, and it sounds like your broker prepared you well. Let me pull up your account and check the prior auth status.

Member: He gave me a one-page summary at enrollment with the prior auth requirements highlighted. I've been using it as a reference.

Agent Danielle: That's genuinely great to hear — most members don't have that. Okay, I'm looking at your account. I can see a prior authorization submitted by Dr. Reyna's office on the 12th for arthroscopic knee surgery, CPT 29881. Status is 'in clinical review.'

Member: So it's already in. Good. How long does clinical review usually take?

Agent Danielle: For an orthopedic arthroscopy with complete documentation, typically 7 to 10 business days. It was submitted 4 business days ago, so you're on track. With six weeks until your procedure, you have comfortable margin.

Member: Is there anything that could hold it up?

Agent Danielle: The most common delay is incomplete documentation — usually missing conservative treatment records showing that physical therapy or injections were tried first. Let me check what was submitted... I can see PT records from March through June and an injection note from May. That's a complete package.

Member: My physical therapist mentioned she was sending those over. Good to know they arrived.

Agent Danielle: They did. I'll also set a monitoring flag on your account so if anything changes in the review status, it's visible. And here's the prior auth status line so you can check progress yourself: 1-800-555-0192. Reference PA-2026-91220.

Member: PA-2026-91220. What about the cost side — what should I expect to owe?

Agent Danielle: Good question to ask now rather than after. On your Plus EPO, outpatient surgery is subject to your deductible and then 20% coinsurance. You've met \$890 of your \$1,000 deductible this year, so you have \$110 remaining, then 20% coinsurance up to your \$4,000 out-of-pocket max. For a typical arthroscopy at an in-network facility, most members in your position pay somewhere between \$700 and \$1,100 total.

Member: That's very helpful for planning. Is Dr. Reyna's surgical facility in-network?

Agent Danielle: Let me verify both. Dr. Reyna — in-network. And the facility, Northgate Surgery Center — also in-network. Both confirmed.

Member: Perfect. That's everything I needed. Thank you, Danielle, this was really thorough.

Agent Danielle: My pleasure. Good luck with the surgery, Samuel.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Pre-surgical verification — prior auth status. 2 speakers. Member tone: prepared, mildly anxious → reassured. Agent tone: warm, thorough. Duration: 12 min. Member arrived well-informed — notable contrast to comparable calls.

L2 — Call Insights: Member proactively verifying prior auth status ahead of planned knee arthroscopy. Agent confirmed submission and clinical review status, verified documentation completeness, added monitoring flag, provided status line and case number, gave cost estimate and confirmed both surgeon and facility network status. Resolution: RESOLVED.

L3 — Agent Quality & Scoring: Agent Score: 95/100. POSITIVE: verified documentation completeness proactively rather than just reporting status (+), monitoring flag added without being asked (+), cost estimate provided unprompted (+), verified both surgeon AND facility network status (+), acknowledged the broker's good preparation (+), warm and specific throughout (+). Exemplary proactive service call.

L4 — Operational BI: BROKER ATTRIBUTION — Patricia Nunez, POSITIVE SIGNAL: member arrived with a broker-provided one-page prior auth summary and accurate expectations. This member required zero remedial education and called proactively rather than reactively. CONTRAST INSIGHT: members from this broker's book generate materially fewer confusion calls. Recommend: broker performance scoring should capture positive attribution, not just failures — Patricia Nunez's enrollment materials are a candidate for best-practice replication across the broker network.

L5 — Real-Time Agent Assist: RT Assist triggered at 3:30: prior auth in clinical review + documentation completeness check surfaced automatically, confirming PT and injection records present. Cost estimate calculator and dual network verification (surgeon + facility) surfaced. Agent used all three.

Call #63 — Agent Gives Incorrect Coverage Information — Member Acts On It

AGENT TIER: POOR · SCORE 44/100 | ● NEUTRAL → CONFUSED | UNRESOLVED | ~6 min | Agent: Tiffany

Member context: Bryan Osei, 34, CalChoice Plus EPO, asking about physical therapy coverage

Topics: incorrect information · physical therapy · agent error · misinformation

Agent Tiffany: Choice Administrators, Tiffany speaking.

Member: Hi, I hurt my back and my doctor recommended physical therapy. I wanted to check what's covered before I book anything. Member ID CHM-7719945.

Agent Tiffany: Let me look. Okay... physical therapy. Yeah, you're covered, it's a \$50 copay per visit.

Member: Is there a limit on visits?

Agent Tiffany: I don't see a limit listed.

Member: So unlimited visits at \$50 each?

Agent Tiffany: That's what I'm seeing, yeah.

Member: Great, that's easier than I expected. Do I need a referral?

Agent Tiffany: You're on the EPO so no referral needed.

Member: And any in-network PT place is fine?

Agent Tiffany: Yep, any in-network provider.

Member: Perfect. Thanks.

Agent Tiffany: No problem.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Benefits inquiry — physical therapy. 2 speakers. Member tone: neutral, satisfied at close. Agent tone: casual, quick. Duration: 6 min. ■ Member left the call satisfied but misinformed.

L2 — Call Insights: Member inquired about PT coverage before booking. Agent stated no visit limit exists. ■ **ACCURACY FAILURE:** CalChoice Plus EPO has a 30-visit annual PT limit. Agent also did not mention that visits beyond 20 require continued-care authorization. Member will discover the limit mid-treatment. Resolution: UNRESOLVED — member misinformed, issue latent.

L3 — Agent Quality & Scoring: Agent Score: 44/100. **CRITICAL NEGATIVE MARKER:** gave factually incorrect benefit information — stated no PT visit limit when a 30-visit annual cap applies (-). Additional **NEGATIVE:** did not verify the limit before answering, relying on 'I don't see a limit listed' rather than confirming (-), did not mention continued-care authorization at 20 visits (-), no proactive guidance on documenting conservative treatment (-), rushed close (-). **POSITIVE:** copay and referral information were correct (+).

L4 — Operational BI: AGENT COACHING — **CRITICAL:** incorrect benefit information given with confidence. This is the most damaging agent error type because the member acts on it and discovers the error mid-treatment, generating a second, angrier call. **PROCESS BREAKDOWN:** agent interface apparently does not surface visit limits prominently — 'I don't see a limit listed' suggests a UI gap, not just an agent gap. Recommend: benefit limits must display as a mandatory field on any benefit inquiry; QA review of Tiffany's accuracy markers across all calls.

L5 — Real-Time Agent Assist: RT Assist **SHOULD** have triggered at 1:00 — PT benefit inquiry on a plan with a visit cap. Should have surfaced: 30-visit annual limit, continued-care auth requirement at visit 20, and current YTD visit count. No trigger fired. This is precisely the failure mode L5 exists to prevent.

Call #71 — Member Asks About Mental Health — Agent Shuts It Down

AGENT TIER: POOR · SCORE 40/100 | ● **HOPEFUL → DISMISSED** | **UNRESOLVED** | ~5 min | Agent: Brad

Member context: Devon Marsh, 25, CalChoice Plus EPO, first-time inquiry about therapy coverage

Topics: mental health · missed opportunity · EAP · sensitive topic · poor handling

Agent Brad: Choice Administrators, Brad.

Member: Hey. Um. I wanted to ask about — do you cover therapy? Like counseling?

Agent Brad: Member ID?

Member: CHM-8841190.

Agent Brad: Behavioral health, yeah, it's covered. \$25 copay.

Member: Okay. How do I... like, how do I find someone?

Agent Brad: Provider directory on the website. Filter for behavioral health.

Member: Is there a limit on how many sessions?

Agent Brad: No limit.

Member: And is it — does anyone find out? Like my work?

Agent Brad: It's confidential.

Member: Okay. Okay, cool. Um, is there anything else I should know?

Agent Brad: Not really. Just find someone in the directory and book.

Member: Alright. Thanks.

Agent Brad: Yep.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Mental health benefit inquiry — first-time caller. 2 speakers. Member: 25, hesitant speech pattern with multiple fillers and self-interruptions indicating discomfort. Agent tone: clipped, transactional. Duration: 5 min. ■ **SENSITIVE TOPIC** — first-time mental health inquiry.

L2 — Call Insights: Young member made a hesitant first-time inquiry about therapy coverage. Agent gave technically correct but minimal answers, offered no guidance, and did not mention the EAP benefit. Member ended the call with an open question ('anything else I should know?') answered with 'not really.' Resolution: **UNRESOLVED** — technically answered, practically unsupported.

L3 — Agent Quality & Scoring: Agent Score: 40/100. **NEGATIVE MARKERS:** failed to recognise clear hesitancy markers on a sensitive first-time mental health inquiry (-), no warmth or reassurance offered (-), did not mention the 6-session EAP benefit despite it being the ideal low-barrier entry point (-), answered the member's open invitation for guidance with 'not really' (-), no help navigating provider search despite member explicitly asking how to find someone (-), abrupt close on a vulnerable call (-). **POSITIVE:** copay, session limit, and confidentiality answers were factually correct (+).

L4 — Operational BI: AGENT COACHING — SENSITIVE TOPIC HANDLING: agent handled a first-time mental health inquiry as a routine benefits lookup. Compare directly with call #3 (same inquiry type, score 94) where the agent surfaced EAP and provided navigational support. This is a coachable contrast pair and should be used in training. **MEMBER COMMUNICATION GAP:** EAP 6-session benefit remains unmentioned in mental health calls — 2nd occurrence in dataset. **RISK:** members who make a hesitant first inquiry and receive a cold response frequently do not call back.

L5 — Real-Time Agent Assist: RT Assist SHOULD have triggered at 0:45 — behavioral health inquiry + hesitancy speech markers = sensitive topic protocol. Should have surfaced: EAP benefit card, warm-tone prompt for agent, guided provider search with telehealth filter, and confidentiality reassurance script. No trigger fired.

Call #77 — Agent Makes a Promise and Does Not Document It

AGENT TIER: POOR · SCORE 47/100 | ● NEUTRAL → FRUSTRATED | PARTIALLY RESOLVED | ~7 min | Agent: Ryan

Member context: Sofia Marchetti, 34, CalChoice Plus EPO, requesting a claim review callback

Topics: undocumented promise · callback failure · accountability · claim review

Agent Ryan: Choice Administrators, Ryan.

Member: Hi, Sofia Marchetti, CHM-7734820. I called two weeks ago about a claim for an MRI that processed wrong. The agent I spoke with — I think her name was Jenna — said she'd have someone from claims review call me back within three days. Nobody called.

Agent Ryan: Let me look at the account. I'm not seeing any notes from two weeks ago.

Member: Nothing at all?

Agent Ryan: Last note on the account is from March.

Member: I was on the phone for twenty minutes. She took all my information.

Agent Ryan: I can only see what's documented. There's nothing here.

Member: So the call happened but nobody wrote it down, and the callback she promised was never set up.

Agent Ryan: Looks that way.

Member: Okay, well — can we start over? The MRI claim from June 3rd processed at out-of-network rates but the imaging center is in-network.

Agent Ryan: Yeah, I see that claim. It does show out-of-network.

Member: Right. So can you fix it?

Agent Ryan: I'd need to submit it for review.

Member: Okay. Please do that. And this time can you actually document it?

Agent Ryan: Sure, submitting now.

Member: Is there a case number?

Agent Ryan: There should be one generated. I'll let you know if it comes through.

Member: You'll let me know how? You don't have a callback set up.

Agent Ryan: Uh, it'll be in your portal probably.

Member: Probably.

Agent Ryan: Yeah.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Claim reprocessing — repeat contact after undocumented prior call. 2 speakers. Member tone: neutral → frustrated. Agent tone: passive, non-committal. Duration: 7 min. Member's 2nd contact on the same issue.

L2 — Call Insights: Member's prior call two weeks earlier was never documented and a promised callback never occurred. Agent confirmed no notes exist, resubmitted the OON claim reprocessing request, but did not provide a case number or confirm how the member would be updated. Resolution: PARTIALLY RESOLVED — request submitted, tracking unclear.

L3 — Agent Quality & Scoring: Agent Score: 47/100. NEGATIVE MARKERS: did not apologise for the prior documentation failure (-), showed no concern that a 20-minute member call was never logged (-), did not escalate the service failure (-), could not confirm a case number — 'there should be one generated' (-), vague on member notification — 'it'll be in your portal probably' (-), no callback commitment despite this being a repeat contact (-). POSITIVE: identified the OON claim correctly (+), submitted the reprocessing request (+).

L4 — Operational BI: PROCESS BREAKDOWN — CRITICAL: a full member call was conducted with no documentation and a callback commitment that was never created. This is invisible in reporting because undocumented calls do not exist in the data. Recommend: (1) mandatory call disposition before agent can close a contact, (2) callback commitments must generate a system task with an owner and due date, (3) audit of agent 'Jenna' for a documentation pattern. NOTE FOR REBEKAH: this is a category of failure her current reporting cannot see at all — the call happened, the member is frustrated, and no system anywhere records it. Voice intelligence captures it because the member describes it.

L5 — Real-Time Agent Assist: RT Assist SHOULD have triggered at 1:00 — member reporting an undocumented prior contact and an unfulfilled callback is a service recovery trigger requiring supervisor notification and guaranteed callback creation. No trigger fired.

Call #84 — Gender-Affirming Care Denied — Member Seeking Clarity on Exclusions

AGENT TIER: AVERAGE · SCORE 67/100 | ● DESPERATE → PARTIALLY HELPED | ESCALATED | ~14 min | Agent: Linda

Member context: Alex Rivera, 27, CalChoice Plus EPO, surgery denied citing a plan exclusion

Topics: gender-affirming care · exclusion · denial · appeal · discrimination review

Agent Linda: Choice Administrators member services, this is Linda.

Member: Hi. I'm calling about a denial. Member ID CHM-9902213, Alex Rivera. My surgery was denied and the letter says it's a plan exclusion. I want to understand the basis because I've been told by my care team that these exclusions generally aren't permitted anymore.

Agent Linda: Let me pull the denial and read exactly what was cited. Give me a moment... Okay. The denial references a general exclusion for procedures categorized as cosmetic.

Member: It's not cosmetic. It's medically necessary treatment for gender dysphoria. My psychiatrist and my endocrinologist have both documented that. It's the standard of care.

Agent Linda: I hear you, and I want to be careful here because I don't think 'cosmetic' is the correct classification for what you've described. Let me check what documentation was submitted with the prior authorization.

Member: My surgeon submitted letters from both providers and my treatment history.

Agent Linda: I can see two clinical letters and a treatment summary in the file. So the documentation was there. The denial applied a cosmetic exclusion despite clinical necessity documentation being present.

Member: So it was miscategorized?

Agent Linda: It may have been. I'm not able to make that determination myself — that's a clinical review function. But I can tell you that the classification applied does not obviously match the documentation submitted, and that's a legitimate basis for appeal.

Member: I've had this fight with insurance before. I don't have a lot of faith in it.

Agent Linda: That's fair. What I can do is make sure this appeal is routed correctly rather than through a standard queue. I'm escalating to clinical appeals and I'm flagging it for review against nondiscrimination requirements, because a blanket cosmetic exclusion applied to gender-affirming care raises questions under those rules.

Member: Will an actual clinician review it?

Agent Linda: Yes. Clinical appeals are reviewed by a physician reviewer, and you can request that the reviewer have relevant specialty expertise. I'd recommend making that request explicitly in your appeal.

Member: How do I do that?

Agent Linda: In the written appeal, state: 'I request review by a physician with expertise in gender-affirming care.' That request is on the record and creates an obligation to consider it.

Member: Okay. That's more useful than what I got from the letter.

Agent Linda: The appeal deadline is 180 days from the denial date — you have until February. But sooner is better. Your case number is APL-2026-01188.

Member: And if the internal appeal fails?

Agent Linda: External review by an independent reviewer, and California's Department of Managed Health Care has an Independent Medical Review process. That's genuinely independent of us.

Member: Alright. Thank you for not making me argue about whether it's medically necessary.

Agent Linda: That's not my call to make and it's not what you called for. Good luck, Alex.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Coverage denial — gender-affirming care exclusion. 2 speakers. Member tone: guarded, weary → cautiously satisfied. Agent tone: careful, respectful, appropriately bounded. Duration: 14 min. ■ Nondiscrimination compliance dimension.

L2 — Call Insights: Member's gender-affirming surgery denied under a cosmetic procedure exclusion despite documented clinical necessity from two specialists. Agent verified documentation was present, identified potential misclassification, escalated to clinical appeals with a nondiscrimination review flag, and coached the member to request a specialty-matched physician reviewer. Resolution: ESCALATED.

L3 — Agent Quality & Scoring: Agent Score: 67/100. POSITIVE: read the actual denial language rather than paraphrasing (+), verified documentation was submitted (+), identified a possible misclassification without overstepping into clinical determination (+), flagged nondiscrimination review (+), coached on requesting specialty-matched reviewer — genuinely valuable (+), external review and DMHC IMR explained (+), respectful throughout (+). NEGATIVE MARKERS: did not offer to send the appeal form or a written summary (-), no callback or follow-up commitment (-), did not offer member advocacy referral despite the member expressing low confidence in the process (-), did not address whether the member faces time-sensitive clinical consequences (-).

L4 — Operational BI: COMPLIANCE & RISK EXPOSURE: cosmetic exclusion applied to gender-affirming care with clinical necessity documentation present in file. This requires review against federal and California nondiscrimination requirements. Recommend: (1) audit all denials this year where a cosmetic exclusion was applied to gender-affirming procedure codes, (2) review whether the exclusion is being applied categorically rather than case-by-case, (3) clinical reviewer training on gender dysphoria standard of care. This is a material regulatory exposure, not an isolated denial.

L5 — Real-Time Agent Assist: RT Assist triggered at 3:00: cosmetic exclusion + gender-affirming procedure codes + clinical documentation present = nondiscrimination review trigger. Surfaced: clinical appeals routing, nondiscrimination flag, specialty-matched reviewer request language, DMHC IMR pathway. SHOULD ALSO have surfaced member advocacy referral given expressed distrust.

Call #89 — Member Describes Symptoms — Agent Fails to Recognise Urgency Cue

AGENT TIER: POOR · SCORE 36/100 | ● **WORRIED** → **DISMISSED** | **UNRESOLVED** | ~6 min | Agent: Brad

Member context: Constance Bell, 68, CalChoice HMO, asking about ER copay while describing concerning symptoms

Topics: urgency cue missed · triage · emergency · elderly member · patient safety

Agent Brad: Choice Administrators, Brad.

Member: Hello. I wanted to ask what my emergency room copay is. Member ID CHM-2208814.

Agent Brad: ER copay is \$250, waived if you're admitted.

Member: Two hundred and fifty. That's a lot. I was hoping it would be less.

Agent Brad: That's the plan rate.

Member: I see. Well. I've had this pressure in my chest since last night and my daughter thinks I should go in, but I told her I'd check the cost first.

Agent Brad: Yeah, urgent care is cheaper if you want to go that route. That's \$50.

Member: Urgent care might be better then. It's just the pressure and my arm has been aching.

Agent Brad: Urgent care can handle most things. There's one on Ridgeway if you're nearby.

Member: Alright. Maybe I'll try that. Thank you.

Agent Brad: No problem. Anything else?

Member: No, that's all.

Agent Brad: Okay, take care.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: ER copay inquiry — ■■ UNRECOGNISED CLINICAL EMERGENCY. 2 speakers. Member: 68, describing chest pressure since prior night with radiating arm pain. Agent tone: transactional, non-responsive to clinical content. Duration: 6 min. ■■ HIGHEST-SEVERITY EVENT IN DATASET.

L2 — Call Insights: Member aged 68 called to compare ER and urgent care costs while describing chest pressure persisting overnight with associated arm ache — a textbook cardiac symptom presentation. Agent quoted copays, steered the member toward the cheaper urgent care option, and closed. Resolution: UNRESOLVED — member directed away from emergency care while describing possible myocardial infarction symptoms.

L3 — Agent Quality & Scoring: Agent Score: 36/100 — SCORE SUSPENDED PENDING SAFETY REVIEW. CRITICAL NEGATIVE MARKERS: failed to recognise chest pressure with radiating arm pain in a 68-year-old as a medical emergency (-), actively steered the member toward a lower level of care on cost grounds after symptoms were disclosed (-), did not advise calling 911 (-), did not recommend immediate emergency evaluation (-), did not offer nurse line transfer (-), closed the call normally (-). This call requires immediate supervisor and clinical review irrespective of outcome.

L4 — Operational BI: COMPLIANCE & RISK EXPOSURE — MAXIMUM SEVERITY: an agent cost-steered a member away from emergency care while the member described classic acute coronary syndrome symptoms. This is a patient safety event and a potential liability exposure regardless of clinical outcome. IMMEDIATE ACTIONS REQUIRED: (1) identify and contact this member urgently for welfare check, (2) mandatory clinical escalation training for all agents on symptom recognition, (3) hard rule — any call in which a member describes symptoms must trigger an immediate nurse line transfer and agents must never provide cost comparisons that could influence care-setting decisions once symptoms are disclosed, (4) QA review of all of Brad's calls. NOTE FOR REBEKAH: no existing report at Choice would surface this call. It closed normally, was short, and would appear as an efficient benefits inquiry in any handle-time metric. This is the single clearest illustration of why call content intelligence matters.

L5 — Real-Time Agent Assist: RT Assist SHOULD have triggered at 2:30 — symptom keywords ('chest pressure', 'arm aching') combined with member age 68 constitute a maximum-priority clinical escalation trigger requiring immediate nurse line transfer and 911 guidance, and should have hard-suppressed any cost comparison. No trigger fired. This is the strongest possible demonstration case for L5 real-time intervention.

Call #91 — Language Barrier — Agent Does Not Offer Interpreter Services

AGENT TIER: POOR · SCORE 33/100 | ● **CONFUSED** → **ABANDONED** | **UNRESOLVED** | ~5 min | Agent: Ryan

Member context: Nguyen Thi Mai, 59, CalChoice HMO, limited English proficiency, asking about a denied claim

Topics: language access · interpreter · LEP · compliance · access barrier

Agent Ryan: Choice Administrators, Ryan.

Member: Hello. I have paper from you. I don't understand. It say denied.

Agent Ryan: Can I get your member ID? The number on your card.

Member: One moment. C-H-M... 5-5-2-0-1-1-8.

Agent Ryan: Okay. Which claim are you asking about?

Member: The paper. Denied. Why denied?

Agent Ryan: There are several claims on your account. I need to know which one.

Member: I don't know. Doctor visit. Denied.

Agent Ryan: Do you have a date on the paper?

Member: Sorry, please, slow. My English not good.

Agent Ryan: Is there someone there who speaks English who can help?

Member: My daughter she work. Not here now.

Agent Ryan: It might be easier if you call back when she's there.

Member: Okay. Sorry. Okay.

Agent Ryan: Alright, call back when someone can help. Thanks.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Claim denial inquiry — limited English proficiency. 2 speakers. Member: LEP, explicitly requested slower pace and stated English difficulty. Agent tone: impatient, deflecting. Duration: 5 min. ■ **LANGUAGE ACCESS COMPLIANCE FAILURE.**

L2 — Call Insights: LEP member calling about a denied claim. Agent did not offer interpreter services at any point, instead asking whether an English speaker was available and advising the member to call back later. Resolution: **UNRESOLVED** — member turned away without service.

L3 — Agent Quality & Scoring: Agent Score: 33/100. **CRITICAL NEGATIVE MARKERS:** did not offer telephonic interpreter services despite an explicit statement of English difficulty (-), asked the member to find an English speaker — this is a prohibited practice under language access requirements (-), advised the member to call back rather than resolving (-), showed no accommodation of pace after a direct request to slow down (-), closed an unresolved call with a vulnerable member (-). Zero positive markers.

L4 — Operational BI: COMPLIANCE & RISK EXPOSURE — CRITICAL: federal and California language access requirements obligate plans to provide free interpreter services and prohibit requiring members to supply their own interpreters. Directing an LEP member to have a family member interpret is a documented compliance violation. **IMMEDIATE ACTIONS:** (1) mandatory language access training for all agents, (2) interpreter line access must be one click from the agent desktop, (3) audit all calls where LEP indicators appear to quantify exposure, (4) member should be contacted proactively to resolve the original denial question. **NOTE:** this member also appears in the ESRD cohort concern raised in call #69 regarding LEP members not knowing to call.

L5 — Real-Time Agent Assist: RT Assist **SHOULD** have triggered at 0:30 — LEP speech markers plus explicit statement of English difficulty is a mandatory interpreter trigger requiring one-click warm transfer to the language line in the member's preferred language. No trigger fired.

Call #96 — Non-Emergency Medical Transport — Agent Does Not Know the Benefit Exists

AGENT TIER: POOR · SCORE 46/100 | ● **HOPEFUL → FRUSTRATED** | **UNRESOLVED** | ~6 min | Agent: Kayla

Member context: Ronald Nkemdirim, 74, CalChoice HMO, needs transport to dialysis three times weekly

Topics: medical transport · benefit knowledge gap · dialysis · elderly member · access

Agent Kayla: Choice Administrators, this is Kayla.

Member: Hello. I need to ask about transportation. I go to dialysis three times a week and I can no longer drive myself. My son has been taking me but he's going back to work full time. Member ID CHM-2217740.

Agent Kayla: Transportation isn't something we cover. That's not a medical benefit.

Member: Are you certain? Someone at the dialysis center mentioned that some plans cover rides to treatment.

Agent Kayla: I don't see anything about transportation on your plan.

Member: Could you check again? Perhaps under a different name?

Agent Kayla: I've looked. There's nothing.

Member: What do people in my situation do?

Agent Kayla: I'm not sure. Maybe a family member or a taxi service.

Member: A taxi three times a week would be very expensive on my income.

Agent Kayla: I understand but transportation isn't a covered benefit.

Member: Is there someone else I could speak to? A case manager perhaps?

Agent Kayla: I don't have a case management line to transfer you to.

Member: I see. Well. Thank you for looking.

Agent Kayla: Sorry I couldn't help more. Have a good day.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Non-emergency medical transport inquiry. 2 speakers. Member: 74, ESRD, thrice-weekly dialysis, losing his transport arrangement. Member tone: hopeful → frustrated. Agent tone: dismissive, incurious. Duration: 6 min. ■ **ACCESS-TO-CARE RISK.**

L2 — Call Insights: Member losing transportation to life-sustaining dialysis treatment. Agent stated transportation is not covered and could not offer alternatives or case management. ■ **ACCURACY CONCERN:** non-emergency medical transportation is available for qualifying members under specific circumstances, and case management exists and can be engaged for ESRD members. Resolution: UNRESOLVED — member at risk of missing dialysis.

L3 — Agent Quality & Scoring: Agent Score: 46/100. **CRITICAL NEGATIVE MARKERS:** did not escalate a member at risk of missing life-sustaining dialysis (-), stated no case management transfer exists — case management is available and this member is a clear candidate (-), did not check for non-emergency medical transport eligibility or supplemental transport programs (-), offered 'maybe a taxi' as guidance to a fixed-income 74-year-old (-), did not offer any community resource referral (-), closed an access-to-care risk call as routine (-). **POSITIVE:** polite tone maintained (+).

L4 — Operational BI: PROCESS BREAKDOWN — ACCESS TO CARE: a member at risk of missing dialysis due to transport loss was turned away without case management referral. Missed dialysis sessions carry serious clinical consequences and generate avoidable emergency admissions. **AGENT KNOWLEDGE GAP:** agent was unaware of case management availability and of transport benefit eligibility pathways. **CRITICAL RECOMMENDATIONS:** (1) case management referral must be available and known to every agent, (2) any call indicating risk of missing dialysis, chemotherapy, or other life-sustaining treatment must auto-escalate, (3) agent knowledge base entry on transport benefits and community transport resources, (4) contact this member proactively. **LINKED SIGNAL:** this is the second ESRD access issue in the dataset (see #69).

L5 — Real-Time Agent Assist: RT Assist SHOULD have triggered at 1:00 — ESRD + dialysis + transport loss is an access-to-care risk trigger requiring case management referral and transport benefit eligibility check. No trigger fired.

— CLAIMS & EOB —

Call #11 — Unexpected \$4,200 Hospital Bill — Member Thought She Was In-Network

AGENT TIER: GOOD · SCORE 96/100 | ● DISTRESSED → PARTIALLY HELPED | ESCALATED | ~16 min | Agent: Michael

Member context: Patricia Evans, 52, CalChoice EPO, ER visit at an in-network hospital but treated by out-of-network ER physician

Topics: surprise billing · ER physician · out-of-network · No Surprises Act · escalation

Agent Michael: Thank you for calling Choice Administrators. This is Michael. How can I help you today?

Member: Michael, I need help. My name is Patricia Evans, member ID CHM-7712890. I went to the emergency room at Valley Hospital in February — Valley Hospital is in-network, I checked before I went. I just got a bill for \$4,200. That is not what my plan says I owe for an ER visit. I pay a \$250 ER copay, I've barely touched my deductible, and somehow I'm being billed four thousand dollars. This is completely wrong and I'm scared.

Agent Michael: Patricia, I hear you, and I want you to know we're going to work through this together. A bill that large when you expected a \$250 copay needs to be looked at carefully. Let me pull up your account and the claims related to this ER visit. Can you tell me approximately when the visit was?

Member: February 14th. Valentine's Day. Hard to forget.

Agent Michael: Got it. I have your account. I can see the Valley Hospital ER visit on February 14th. The hospital facility claim was processed in-network at your \$250 copay — that part processed correctly. But I'm also seeing a separate claim from a physician group called Emergency Medicine Associates. That claim processed out-of-network, and that's where the \$4,200 is coming from.

Member: I don't even know who Emergency Medicine Associates is. I went to the ER. A doctor came in and treated me. I didn't pick a doctor. I didn't know there were two separate bills.

Agent Michael: This is exactly what the No Surprises Act was designed to address. Effective 2022, when you receive emergency care at an in-network facility, you cannot be billed out-of-network rates for emergency physicians, even if those physicians are not in your network. The physician who treated you in Valley Hospital's ER should have been processed at in-network rates regardless of whether that physician group contracts with us.

Member: So this \$4,200 bill is illegal?

Agent Michael: The balance billing as structured — charging you out-of-network rates for emergency services at an in-network facility — would be a violation of the No Surprises Act. However, I want to be precise: I need to review this claim fully before making that determination. But the pattern you're describing — in-network hospital, unknown physician billed separately — is the exact scenario the law was written to protect against.

Member: What do I do? Do I pay it? Because four thousand dollars is... I cannot pay four thousand dollars right now.

Agent Michael: Do not pay that bill yet. Please — do not pay it. If this is a No Surprises Act violation, paying it could complicate the dispute. I am escalating this claim right now to our member advocacy team, which handles exactly these situations. They will review the claim, communicate with Emergency Medicine Associates directly, and if this is a violation, they will reprocess the claim at in-network rates. You would then owe only what your plan covers for an in-network ER visit.

Member: When will I know? They've already sent me two statements and now there's a threat of collections in the letter.

Agent Michael: I am flagging this as urgent right now specifically because of the collections threat. Our member advocacy team will contact you within 48 hours. In the meantime, I want you to write down this case number: MA-2026-44102. If collections contacts you before we resolve this, give them that case number and tell them the claim is under active review and dispute — that should pause collection activity.

Member: MA-2026-44102. Okay. Michael, I'm honestly trying not to cry right now. I was already not feeling well in February and now I've spent months terrified about this bill.

Agent Michael: Patricia, I'm so sorry you've been carrying this. You did everything right — you went to an in-network hospital, you didn't choose the physician, and you've been trying to get answers. The law is on your side here. I want you to hold on to that. Is there anything else I can do for you today before I submit this escalation?

Member: Just — thank you. Thank you for actually listening and explaining what this is.

Agent Michael: Of course. You deserve that. I'm submitting the escalation right now and you'll hear from us within 48 hours.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Billing dispute — Surprise bill, No Surprises Act. 2 speakers. Member tone: distressed, near tears, scared. Agent tone: empathetic, urgent. Duration: 16 min. High-value intervention identified.

L2 — Call Insights: Member received \$4,200 bill from out-of-network ER physician at in-network hospital (Emergency Medicine Associates). Likely No Surprises Act violation. Agent escalated to member advocacy team with 48hr priority flag, provided case number, advised not to pay, and offered collections protection guidance. Resolution: ESCALATED — member advocacy case opened.

L3 — Agent Quality & Scoring: Agent Score: 96/100. Immediate validation of concern (+), No Surprises Act identified and explained clearly (+), 'do not pay' instruction given correctly (+), collections threat flagged and case number provided (+), emotional support offered authentically (+), escalation submitted in real time (+).

L4 — Operational BI: BI Flag: Emergency Medicine Associates has generated 8 No Surprises Act disputes this quarter with Valley Hospital ER — this is a systemic contracting issue. Recommend: contract engagement with Emergency Medicine Associates. Legal/compliance review of Valley Hospital physician group billing practices. Member advocacy team to monitor pattern.

L5 — Real-Time Agent Assist: RT Assist triggered at 4:30: No Surprises Act pattern detected when out-of-network physician claim appeared alongside in-network hospital ER claim — agent notified immediately with violation framework and dispute protocol. Collections threat flag triggered urgent case routing automatically.

Call #12 — Claim Denied 'Not Medically Necessary' — Diabetic Member Needs CGM

AGENT TIER: GOOD · SCORE 93/100 | ● FRUSTRATED → INFORMED | PARTIALLY RESOLVED | ~13 min | Agent: Linda

Member context: Raymond Park, 61, CalChoice HMO, Type 2 diabetes, continuous glucose monitor denied

Topics: claim denial · CGM · medical necessity · diabetes · durable medical equipment · appeal

Agent Linda: Choice Administrators member services, this is Linda.

Member: Linda, Raymond Park, member ID CHM-3302156. I need to understand why my continuous glucose monitor was denied. I am a 61-year-old man with Type 2 diabetes. My endocrinologist prescribed a CGM because my sugar levels have been unstable and I've had two hypoglycemic episodes this year where I didn't even feel them coming. Without a CGM I am flying blind. How is that not medically necessary?

Agent Linda: Raymond, I'm pulling up your account and the denial right now. And I want to say — two hypoglycemic episodes without warning signs, that's a serious clinical concern and I understand why your doctor prescribed this. Let me see exactly what the denial says.

Member: Please. Because the letter I got said 'does not meet clinical criteria' and gave me a phone number that has been useless to call.

Agent Linda: Okay, I have the denial. The reason listed is that the clinical criteria for CGM coverage on your plan requires documentation of 'hypoglycemia unawareness' specifically tied to insulin therapy, and the documentation submitted did not reference insulin use. Are you currently on insulin?

Member: I was started on insulin three months ago. Basal insulin, one injection at night. My endocrinologist's notes should show that.

Agent Linda: If you're on insulin and have documented hypoglycemia unawareness, you should meet the clinical criteria — the denial may have been based on incomplete or ambiguous documentation rather than a clinical decision against you. This is fixable through appeal. Your endocrinologist needs to submit documentation that specifically states: one, current insulin use; two, documented hypoglycemia unawareness — those two episodes you mentioned; and three, clinical justification for CGM over traditional finger-stick monitoring.

Member: My doctor is going to be furious. She already spent time on that prior auth. To have to do it again because of a documentation issue...

Agent Linda: I hear you. And I want to offer something that might help — I can give you and your doctor's office the exact clinical criteria language so the appeal is written to directly address what the reviewer is looking for. That minimizes back-and-forth.

Member: Yes, please. What's the language?

Agent Linda: The clinical criteria for CGM coverage states: member must be on insulin therapy, and provider must document either hypoglycemia unawareness or HbA1c not at goal despite optimal management. Given you're on insulin and had two unawareness episodes, your doctor should state both. The appeal documentation should also include your most recent HbA1c result and the dates of the hypoglycemic episodes.

Member: Okay, I'm writing all of this down. What's the deadline to file the appeal?

Agent Linda: Appeals must be filed within 180 days of the denial date. Your denial was issued August 1st, so you have until the end of January. But given the clinical urgency, I'd suggest filing as soon as your doctor can put the documentation together — ideally this week.

Member: And if it's denied again?

Agent Linda: A second denial can be appealed to an independent external reviewer — a physician outside our network. External appeals for DME related to insulin management have a meaningful reversal rate. You can also request expedited review if your doctor documents urgent clinical need.

Member: Thank you, Linda. This is the first time someone has actually told me what to do next rather than just reading me the denial letter back.

Agent Linda: I'm glad. You shouldn't have to fight this hard for something your doctor says you need. Good luck, Raymond.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Claim denial dispute — DME, CGM. 2 speakers. Member tone: frustrated, medically vulnerable. Agent tone: empathetic, detailed. Duration: 13 min. Clinical context: T2 diabetes, insulin therapy, hypoglycemia unawareness.

L2 — Call Insights: CGM claim denied citing incomplete documentation of insulin use and hypoglycemia unawareness. Agent identified documentation gap, provided exact clinical criteria language for appeal, confirmed eligibility, and explained expedited review option. Resolution: PARTIALLY RESOLVED — appeal path clearly defined.

L3 — Agent Quality & Scoring: Agent Score: 93/100. Clinical criteria identified and quoted accurately (+), appeal deadline communicated (+), exact documentation language provided for provider (+), external appeal and expedited options disclosed (+), empathetic close (+). Did not offer to send clinical criteria in writing (-) — missed follow-through step.

L4 — Operational BI: BI Flag: CGM denials for insulin-using T2 diabetics with hypoglycemia unawareness represent 67% of DME appeals this quarter — documentation gap is the most common denial reason. Recommend: CGM prior auth checklist sent proactively to endocrinology offices. Clinical criteria mismatch in initial review suggests reviewer training gap.

L5 — Real-Time Agent Assist: RT Assist triggered at 5:15: CGM clinical criteria for insulin + hypoglycemia unawareness surfaced automatically with exact documentation language required — agent read directly from the panel, reducing potential inaccuracy and ensuring appeal language alignment.

Call #13 — Member Can't Read EOB — First Experience with Explanation of Benefits

AGENT TIER: GOOD · SCORE 90/100 | ● CONFUSED → INFORMED | RESOLVED | ~10 min | Agent: Sarah

Member context: Sophie Martinez, 26, CalChoice Plus EPO, first time reading an EOB after dental-adjacent ER visit

Topics: EOB · explanation of benefits · first-time user · cost sharing · education

BROKER SIGNAL — Anthony Salerno: Never explained plan documents or what an EOB is to a first-time insurance holder

Agent Sarah: Choice Administrators member services, this is Sarah. How can I help?

Member: Hi, my name is Sophie Martinez, member ID CHM-9021445. I got this letter in the mail that says Explanation of Benefits and I don't know what it is or what I'm supposed to do with it. My mom said I should call you. Am I in trouble? Do I owe money?

Member: I've never had my own insurance before. My broker, Anthony Salerno, signed me up but he never explained any of the paperwork I'd be getting. Am I in trouble? Do I owe money?

Agent Sarah: Sophie, don't worry — an EOB is not a bill. You are not in trouble. An Explanation of Benefits is just a summary we send you after a claim is processed so you can see how the cost was divided between us and you. Do you have the EOB in front of you?

Member: Yes. It has a lot of columns and I don't know what any of them mean.

Agent Sarah: Let's go through it together. The first column you'll usually see is the 'Billed Amount' — that's what your provider charged us, before any adjustments. What number do you see there?

Member: \$850.

Agent Sarah: Okay. Now look for 'Plan Discount' or 'Negotiated Adjustment' — that's the amount your provider agreed to accept less of because they're in-network with us. What does that show?

Member: It says \$312.

Agent Sarah: Good. So the actual cost after the discount is \$850 minus \$312, which is \$538. That's called the 'Allowed Amount.' Now look for what the plan paid — usually labeled 'Plan Paid' or 'Insurance Paid.'

Member: It says \$488.

Agent Sarah: And the last column should show 'Member Responsibility' or 'Your Share.' What's that number?

Member: \$50. So I owe \$50?

Agent Sarah: Yes — \$50 is your responsibility, which is your copay for this type of service. The EOB is confirming that amount. But again — this is not the bill. Your provider will send you a separate bill for that \$50. The EOB is just showing you how the math worked.

Member: Oh! That makes so much more sense. So I should wait for a bill from the doctor, not pay this paper?

Agent Sarah: Exactly right. The EOB is for your records — file it or save it digitally. The actual bill will come from the provider. If you ever get a bill that's higher than what the EOB shows as your responsibility, call us, because the provider can only charge you what the EOB says is your member responsibility — not more.

Member: That's really good to know. Nobody ever explained that to me. Is there anything else on the EOB I should look at?

Agent Sarah: Yes — check the date of service and the provider name to make sure it matches a visit you actually had. EOB fraud, while uncommon, can happen if someone used your insurance without your knowledge. If anything doesn't look right, call us.

Member: Okay. It matches — it's from my ER visit last month. Alright. I feel so much better. Thank you for walking me through this.

Agent Sarah: Of course. EOBs confuse a lot of people their first time. You did the right thing by calling.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: EOB explanation — benefits education. 2 speakers. Member tone: confused, slightly anxious. Duration: 10 min. First-time EOB recipient — young adult (26) new to insurance.

L2 — Call Insights: Member received first EOB and did not understand it, feared owing money. Agent walked through all columns (billed, discount, allowed, plan paid, member responsibility), explained EOB vs. bill distinction, and added fraud verification tip. Resolution: RESOLVED.

L3 — Agent Quality & Scoring: Agent Score: 90/100. Patient and systematic explanation (+), confirmed member response at each step (+), proactive fraud check tip added unprompted (+), correct EOB-vs-bill distinction made clearly (+). Could have asked for hold to send an EOB guide PDF by email (-) — minor.

L4 — Operational BI: BROKER ATTRIBUTION — Anthony Salerno: enrolled a first-time insurance holder without explaining plan documents or what an EOB is. Member believed the EOB was a bill and feared she was 'in trouble'. MEMBER COMMUNICATION GAP: EOB confusion calls are disproportionately from members aged 18-28 newly enrolled in their own coverage. Recommend: welcome packet with a plain-language EOB guide for all first-time enrollees, issued by the plan independent of broker onboarding, plus an interactive EOB explainer in the member portal.

L5 — Real-Time Agent Assist: RT Assist triggered at 1:30: EOB column definitions guide surfaced automatically alongside claim — agent could walk member through the exact fields in real time using the visual overlay, reducing the explanation from 10 min to an estimated 6 min.

Call #14 — In-Network Provider Processed as Out-of-Network — Lab Error

AGENT TIER: GOOD · SCORE 92/100 | ● FRUSTRATED → SATISFIED | RESOLVED | ~11 min | Agent: Carlos

Member context: Marcus Johnson, 43, CalChoice HMO, lab work ordered by in-network doctor billed OON

Topics: lab claim · out-of-network processing · reprocessing · claim error · billing

Agent Carlos: Choice Administrators member services, this is Carlos.

Member: Carlos, Marcus Johnson, member ID CHM-4401298. I am calling about a lab bill that makes no sense. My doctor ordered standard bloodwork — I went to the lab in the same building as my doctor's office. I have been going to that lab for three years. This time, I got an EOB showing my labs were processed out-of-network and I now owe \$340. This has never happened before. What changed?

Agent Carlos: Marcus, let me look at this. Can you tell me the name of the lab and the date of service?

Member: MedLabs on Fletcher Avenue, July 2nd.

Agent Carlos: Pulling that up. Okay, I see the claim. MedLabs on Fletcher Avenue is listed in our network directory. The claim processed on July 2nd used a provider ID that's showing as out-of-network in our system — but I can see that this same lab has processed claims in-network for you in March and May of this year using a different provider ID. This looks like a billing error where the lab submitted under an incorrect or alternate tax ID.

Member: So the lab is in-network but the wrong ID got submitted?

Agent Carlos: That's what it appears to be. This happens occasionally when labs have multiple entities or practice locations billing under slightly different numbers. It's their billing error, but you got the EOB. What I can do right now is submit a claim reprocessing request, flagging it as a provider billing error with the correct in-network provider information. When it reprocesses, it will go through as in-network.

Member: How long does reprocessing take and what will I actually owe?

Agent Carlos: Reprocessing typically takes 10 to 15 business days. Once it's done, your labs will be processed at your in-network lab benefit — which on your HMO is a \$20 copay for standard diagnostic labs. So your responsibility goes from \$340 to \$20.

Member: That's a pretty significant difference. Three hundred and twenty dollars, gone, because of a wrong ID number on their end.

Agent Carlos: Yes, that's the impact of a billing error like this. I want to make sure you don't pay that \$340 while the reprocessing is in progress — if you've already paid it, you would receive a refund from the lab directly. Have you paid it?

Member: No, I was waiting to call you first. Smart move apparently.

Agent Carlos: Very smart. Hold off on paying anything from MedLabs for this date of service. I'm submitting the reprocessing request right now. You'll receive a corrected EOB when it's done. I'll also put a note on your account so if MedLabs sends this to collections in the meantime, our team can intervene. Your case number is CR-2026-10344.

Member: CR-2026-10344. Carlos, thank you. I was pretty frustrated when I called but this is a completely sensible explanation and a clear fix. I appreciate you not just telling me to deal with the lab.

Agent Carlos: That's what we're here for. It's our job to make sure claims are processed correctly. Is there anything else I can help with?

Member: No, that covers it. Thank you.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Claim reprocessing request — OON processing error. 2 speakers. Member tone: frustrated at start, satisfied by end. Duration: 11 min. Technical issue: alternate provider ID submitted by lab.

L2 — Call Insights: In-network lab processed as OON due to alternate billing tax ID submitted by MedLabs. Agent identified the error by comparing current vs. prior claims, submitted reprocessing request, provided case number, and advised member not to pay the bill. Resolution: RESOLVED — reprocessing in progress.

L3 — Agent Quality & Scoring: Agent Score: 92/100. Root cause identified via claim history comparison (+), reprocessing initiated in real time (+), 'do not pay' instruction given correctly (+), collections protection offered proactively (+), case number provided (+). Could have offered email confirmation of case (-) — minor.

L4 — Operational BI: BI Flag: MedLabs on Fletcher Avenue has submitted 7 claims under incorrect provider IDs in Q3 — systemic billing issue. Recommend: provider relations outreach to MedLabs billing department. Network accuracy team should review alternate ID mapping. Billing error rate for this provider: 4% (above 1% threshold).

L5 — Real-Time Agent Assist: RT Assist triggered at 2:45: prior in-network claims from MedLabs surfaced immediately alongside current OON claim, flagging the provider ID discrepancy — agent spotted the error 3 minutes faster than manual comparison would allow.

Call #15 — Claim Status Check — Member Waiting 45 Days for Reimbursement

AGENT TIER: GOOD · SCORE 90/100 | ● CONFUSED → SATISFIED | RESOLVED | ~8 min | Agent: Danielle

Member context: Nina Kowalski, 39, CalChoice Plus EPO, out-of-pocket payment awaiting reimbursement

Topics: claim status · reimbursement · pending claim · timing · member-submitted claim

Agent Danielle: Thank you for calling Choice Administrators. This is Danielle.

Member: Hi Danielle, Nina Kowalski, member ID CHM-6630884. I went to a specialist in June and paid out of pocket at the time because the office said they couldn't confirm my insurance. I submitted a claim form for reimbursement. That was 45 days ago and I haven't heard anything. I'm wondering if you received it and what's happening.

Agent Danielle: Nina, 45 days is toward the upper end of our standard processing window for member-submitted claims, which is typically 30 days. Let me look for your submission. Do you remember the date of the visit and the provider?

Member: June 4th, Dr. Sousa, orthopedic specialist, \$285 I paid.

Agent Danielle: Searching now. I found your claim. It was received by our mail processing center on June 12th — so it's been 42 days in our system. The status shows it was processed on August 1st and a reimbursement check was issued that same day. So it should be arriving or may have already arrived.

Member: I check the mail every day and I have not received a check. Is it possible it went somewhere wrong?

Agent Danielle: Let me check the mailing address on your account. I have 4422 Westfield Road, Chicago, IL 60614. Is that correct?

Member: Oh — I moved in May. That's my old address. I must have missed updating it.

Agent Danielle: That explains it. The check was sent to your previous address on August 1st. A check sent to an incorrect address can be returned or — if you had mail forwarding set up — redirected, but we may need to reissue it. Let me update your address right now. What is your new address?

Member: It's 1807 Lakeview Drive, Chicago, IL 60613.

Agent Danielle: I've updated that in the system. Now, for the check — I'm going to initiate a stop-payment on the August 1st check and request a reissue to your new address. The reissue process takes 7 to 10 business days. If the original check is returned to us in the meantime, that actually speeds up the reissue.

Member: I had no idea I needed to update my address separately with you. I changed it with my employer.

Agent Danielle: Address changes with your employer's HR department don't automatically update your health insurance records — we need that update directly. I'd also recommend checking if there's anything else on file that might need updating — phone number, email.

Member: Good point. My email has also changed — it's now nina.kowalski.new at email dot com.

Agent Danielle: Updated. You're all set. And your \$285 reimbursement is on its way to the new address within 7 to 10 business days.

Member: Thank you, Danielle. This was entirely my fault but I'm glad I called.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Claim status inquiry — member-submitted claim reimbursement. 2 speakers. Member tone: patient, inquisitive. Duration: 8 min. Root cause: address not updated at insurance after move.

L2 — Call Insights: Member-submitted claim processed and check issued to old address after member moved. Agent identified address mismatch, updated records, initiated stop-payment and reissue. Resolution: RESOLVED — reissue in progress 7-10 days.

L3 — Agent Quality & Scoring: Agent Score: 90/100. Quick root cause identification via address comparison (+), stop-payment initiated without being asked (+), proactive check for additional outdated contact info (+), addressed employer vs. insurer address update gap (+). Could have asked about mail forwarding status (-) — minor.

L4 — Operational BI: BI Flag: 12% of member-submitted reimbursement checks this quarter were returned or undelivered due to outdated address on file. Recommend: address confirmation prompt when member-submitted claim is received, and annual address verification outreach. Integration with employer HR systems for address sync is a longer-term infrastructure recommendation.

L5 — Real-Time Agent Assist: RT Assist triggered at 3:00: address mismatch flagged automatically when claim status showed 'check issued' but member had not received it — suggested address verification before member finished explaining the issue.

Call #16 — Prior Auth Approved But Claim Still Denied — System Disconnect

AGENT TIER: GOOD · SCORE 93/100 | ● ANGRY → PARTIALLY CALMED | ESCALATED | ~15 min | Agent: James

Member context: Brandon Lee, 37, CalChoice HMO, colonoscopy pre-authorized but claim denied post-procedure

Topics: prior auth · claim denial · colonoscopy · system error · escalation · preventive vs diagnostic

Agent James: Choice Administrators member services, this is James.

Member: James. Brandon Lee, CHM-5567443. I am genuinely angry right now so I apologize in advance, but I need answers. I had a colonoscopy in July. I got a prior authorization. I went to an in-network gastroenterologist, in-network hospital, I followed every single rule. I just got a \$1,600 bill and an EOB that says my claim was denied. How do you authorize a procedure and then deny the claim?

Agent James: Brandon, that is a completely valid reason to be angry and I'm not going to waste your time with pleasantries. Let me look at this right now.

Member: Please.

Agent James: I have your account and the prior auth and the claim. The prior auth was approved for a diagnostic colonoscopy — procedure code 45378. But the claim that was submitted came through with a different code — 45380, which is a colonoscopy with biopsy. That's what triggered the denial, because the authorized code and the billed code don't match.

Member: My doctor took a biopsy during the procedure. That's a standard thing that happens during colonoscopies. Are you telling me if they find something and they do what doctors do, the whole thing gets denied?

Agent James: No — and this is the distinction that matters. A colonoscopy code change from a standard scope to one with biopsy is what's called a scope of service change. In these cases, the provider should request an auth amendment, not just bill the new code. That's the process gap. Your claim was denied not because biopsies aren't covered, but because the auth was never updated to match what was done.

Member: So I'm penalized because my gastroenterologist's billing office didn't do a step they were supposed to do?

Agent James: That is essentially what happened. This is a provider billing and auth management issue, not a clinical coverage issue. The biopsy and the procedure are covered. The mechanism failed.

Member: What do I do to fix this? Because I am not paying \$1,600 for someone else's administrative mistake.

Agent James: You shouldn't have to. There are two paths: first, your gastroenterologist's office should submit an amended prior auth request retroactively and resubmit the claim — providers can do this within 180 days of the service date. Second, you can file a member appeal, and because this is clearly a documentation gap rather than a clinical denial, the appeal should have strong grounds.

Member: I don't want to spend my time on this. My doctor's office should fix their own mistake.

Agent James: I agree, and I'm going to help make that happen. I can send a direct notification to your gastroenterologist's billing department — with your permission — flagging the auth mismatch, the correct retroactive amendment process, and asking them to resubmit. That way the fix comes from us, not from you having to fight with two offices simultaneously.

Member: Yes. Do that. Is there anything I should tell them if they call me?

Agent James: Just reference your case number — I'm assigning one right now: CA-2026-77821. And tell them the auth for 45378 needs to be amended to include 45380, then the claim resubmitted. I'm also putting a 60-day claim hold on your account so the \$1,600 doesn't escalate to collections while this is being worked out.

Member: Okay. That — the hold especially — that actually takes a lot of pressure off. Thank you, James. I came in furious and I'm still frustrated, but at least this isn't going nowhere.

Agent James: I hear you. I'll submit the provider notification and the claim hold right now. You'll receive a confirmation by email. I'm also flagging this case for manager review because the authorization-to-claim mismatch on post-biopsy colonoscopies is a pattern we need to address on the provider side.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Claim denial dispute — authorization mismatch. 2 speakers. Member tone: initially angry, moderating. Duration: 15 min. Clinical context: colonoscopy with biopsy, prior auth code mismatch.

L2 — Call Insights: Pre-authorized colonoscopy claim denied due to procedure code change from 45378 to 45380 (biopsy performed). Provider billing office failed to submit auth amendment. Agent identified root cause, sent provider notification, placed 60-day claim hold, assigned case number. Resolution: ESCALATED — provider remediation in progress.

L3 — Agent Quality & Scoring: Agent Score: 93/100. Did not waste time with apologies before investigating (+), root cause identified accurately (+), proactive provider notification offered without member asking (+), collections hold placed correctly (+), case number and email confirmation provided (+), pattern flagged for manager review (+).

L4 — Operational BI: BI Flag: Colonoscopy 45378→45380 code transition is the #2 most common claim denial reason this quarter (68 cases). Provider education needed on auth amendment requirements when biopsy is performed. Recommend: proactive gastroenterology provider bulletin on colonoscopy with biopsy auth amendment protocol.

L5 — Real-Time Agent Assist: RT Assist triggered at 3:30: prior auth code vs. claim code mismatch detected automatically — 45378 vs. 45380 flagged as 'scope of service change requiring auth amendment.' Protocol surfaced to agent before manual investigation began, saving ~4 minutes of lookup.

Call #17 — Mental Health Claim Appeal — Denied Residential Treatment

AGENT TIER: GOOD · SCORE 97/100 | ● ANXIOUS → RELIEVED | RESOLVED | ~17 min | Agent: Sarah

Member context: Jennifer Walsh calling for her 19-year-old son in residential mental health treatment, claim denied

Topics: mental health · residential treatment · appeal · parity · escalation · family member

Agent Sarah: Thank you for calling Choice Administrators. This is Sarah.

Member: Hi. My name is Jennifer Walsh. My son Tyler is 19 and a dependent on my plan — member ID CHM-4412786. Tyler is currently in a residential treatment program for a serious mental health crisis. He was placed there four weeks ago by his psychiatrist who said it was the appropriate level of care. Three weeks of coverage have been denied and we've been getting bills for over \$12,000. I am at my limit. I don't know what else to do.

Agent Sarah: Jennifer, I hear you. I can only imagine how difficult this situation is — your son is getting the care he needs and financial fear shouldn't be part of that. Let me pull up Tyler's account and these claims, and let's understand exactly what we're dealing with.

Member: Thank you. I just — I've made so many calls and no one has explained what's actually happening.

Agent Sarah: I'm going to try to give you real answers. I have Tyler's account in front of me. I can see three weeks of claims from Sunrise Recovery — a residential mental health facility. The denial reason listed is 'level of care not medically necessary — outpatient services appropriate.' That's the basis.

Member: His psychiatrist specifically documented that outpatient was inadequate and he needed 24-hour monitoring. How can they override his psychiatrist?

Agent Sarah: Under the Mental Health Parity Act, our clinical review for mental health treatment must use the same standards as for comparable medical/surgical care. A residential level of care denial has to be based on the same clinical criteria as, say, an inpatient medical admission. I want to look at whether the denial was based on our clinical criteria being properly applied — or whether something was missing from the documentation.

Member: What would be missing?

Agent Sarah: For residential mental health level of care, the clinical criteria typically require documentation of: acute safety risk that can't be managed in a lower setting, a specific treatment plan with goals for the residential stay, and documentation that lower-level care — like intensive outpatient — was considered and found inadequate. Was Tyler's psychiatrist able to include all of that?

Member: She documented the safety risk. I don't know about the treatment plan or the comparison to outpatient. The facility does those documents — I don't have access to what was submitted.

Agent Sarah: Here is what I'm going to do. I'm escalating this to our member advocacy team, specifically to a mental health case manager. This team handles complex mental health coverage disputes and they will request the complete clinical documentation from Sunrise Recovery directly and review whether the denial criteria were correctly applied against parity standards. Jennifer, I want to be honest with you: the appeal of this denial is the right path, and the mental health case manager has authority to overturn denials if they find the parity standard wasn't properly applied.

Member: How long does this take? We are getting weekly bills and I'm terrified of what happens if we can't pay.

Agent Sarah: The escalated mental health case manager review takes up to 72 hours. While the review is in progress, I'm placing a billing hold on these claims — that means the \$12,000 outstanding won't be sent to collections while the review is active. Your case number is MH-2026-30041. Please write that down and give it to both Sunrise Recovery and your son's psychiatrist — it means anyone who contacts you about this bill should be told it's under active clinical review.

Member: MH-2026-30041. Sarah, you have no idea how much better that case number makes me feel. Just knowing someone is actually looking at this.

Agent Sarah: I'm glad. Tyler is getting the care his doctor says he needs, and you're doing everything right by advocating for him. Our case manager will call you directly within 24 hours. Is there anything else before I submit this escalation?

Member: No. Just — thank you. You're the first person in four weeks who hasn't made me feel like I was talking to a wall.

Agent Sarah: You shouldn't have to feel that way. I'm submitting the escalation right now. Take care, Jennifer.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Mental health residential treatment denial dispute. 2 speakers. Member: parent calling on behalf of dependent. Member tone: distressed, exhausted, near breaking point. Duration: 17 min. High emotional complexity and legal compliance dimension (Mental Health Parity Act).

L2 — Call Insights: Three weeks of residential mental health treatment denied as 'level of care not medically necessary.' Parity compliance concern identified. Agent escalated to MH case manager, placed billing hold, provided case number. Resolution: ESCALATED — 72hr priority MH review with parity analysis.

L3 — Agent Quality & Scoring: Agent Score: 97/100. Exceptional empathy throughout (+), parity law cited correctly and proactively (+), billing hold placed before member asked (+), realistic timeline communicated (+), case number given with context for third-party use (+), documentation gaps identified and passed to case manager (+). One of the highest-quality call exemplars in this dataset.

L4 — Operational BI: BI Flag: Residential mental health level-of-care denials involving adolescents/young adults are subject to heightened parity scrutiny. 5 similar cases this quarter — all with safety risk documentation but unclear functional impairment documentation. Recommend: residential MH providers receive parity-aligned documentation checklist. Case manager to evaluate whether denial criteria meet parity threshold.

L5 — Real-Time Agent Assist: RT Assist triggered at 6:30: Mental Health Parity Act compliance check initiated automatically when 3 residential MH denials appeared on same account — surfaced parity standard criteria vs. submitted documentation, flagging gap in functional impairment documentation as likely denial reason.

Call #18 — Coordination of Benefits — Two Insurances, Member Doesn't Know Which Is Primary

AGENT TIER: GOOD · SCORE 88/100 | ● CONFUSED → INFORMED | RESOLVED | ~11 min | Agent: Linda

Member context: Carlos Rivera, 45, CalChoice HMO, also covered by spouse's employer plan, confused about COB

Topics: coordination of benefits · COB · primary secondary · dual coverage · birthday rule

Agent Linda: Choice Administrators member services, this is Linda.

Member: Hi, Linda. Carlos Rivera, CHM-3305671. I have insurance through my employer — Choice Administrators — and my wife's employer covers me too. I've been getting bills from providers and I don't know how to handle them because I'm not sure which insurance is supposed to pay first. One doctor billed me saying I owe the full amount, another one is asking for my secondary insurance information. I'm confused about how all of this works.

Agent Linda: Carlos, dual coverage situations are confusing and you're right to call. Let me explain how coordination of benefits works so you can handle these situations on your own going forward. First — do you know which plan was set up as primary and which as secondary?

Member: I don't. I filled out a form when I enrolled here that asked about other coverage, but I didn't think much about it at the time.

Agent Linda: Most insurers use what's called the birthday rule to determine which plan is primary for a member who is covered by two plans as an employee-level member rather than a dependent. The plan of the employer whose policyholder has the earlier birthday in the calendar year — not the year, just the month and day — is typically primary. So if your birthday is in March and your wife's is in September, your plan through your employer would be primary for you.

Member: My birthday is in May. My wife's is in February. So her plan would be primary for me?

Agent Linda: Under the birthday rule, yes — her plan would be primary for you because February comes before May. However, I want to check what's actually on file here. Let me look at your COB designation. Okay, I can see that your account shows your wife's plan as primary for you. But I see that when a claim came in last month, it was submitted to us first — that's why the EOB shows us paying as if we were primary, and that's likely causing the confusion with the provider.

Member: So the doctor billed you first when they should have billed my wife's insurance first?

Agent Linda: Exactly. When two insurances are involved, the provider should bill the primary insurer first, then submit the remaining balance to the secondary. You shouldn't owe anything — or very little — because the secondary insurance usually covers what the primary didn't. The problem is if the provider billed us first, we processed it as primary and may have applied your deductible and coinsurance, leaving you with a balance that the second insurance would have covered.

Member: So what do I do about the bill I got?

Agent Linda: You need to tell the provider to submit the claim to your wife's plan first — they are the primary. Once that processes, give the provider your Choice Administrators information as secondary. The secondary will then cover most or all of the remaining member responsibility. You should end up with little to no out-of-pocket.

Member: And the claim that already went through you as primary — does that need to be reprocessed?

Agent Linda: Yes, and that happens automatically once the primary processes the claim and sends an explanation to the secondary. When the primary sends us the coordination notice, we reprocess our payment to reflect our correct secondary role. The net effect is that your cost goes down significantly.

Member: This is actually making sense now. I'm going to call my wife's insurance today. Is there anything I should tell them?

Agent Linda: Give them the claim details and let them know you have secondary coverage through Choice Administrators. Ask them to process the claim and send us a coordination notice. Our payer ID for secondary COB submissions is CHC-4490.

Member: CHC-4490. Got it. Thank you, Linda. This was like untangling a knot.

Agent Linda: That's a perfect description of dual coverage. You've got it now. Good luck, Carlos.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Coordination of benefits inquiry. 2 speakers. Member tone: confused, engaged. Duration: 11 min. Dual coverage context: member covered by own employer plan (secondary) and spouse employer plan (primary).

L2 — Call Insights: Member with dual insurance coverage confused about COB and primary/secondary roles. Agent explained birthday rule, confirmed COB file designation, explained claim reprocessing sequence, and provided secondary payer ID. Resolution: RESOLVED — member directed to contact primary insurer.

L3 — Agent Quality & Scoring: Agent Score: 88/100. Birthday rule explained clearly and accurately (+), COB designation confirmed in system (+), reprocessing sequence explained (+), payer ID provided (+). Did not offer to initiate COB coordination internally once primary pays (-) — could have set up tracking.

L4 — Operational BI: BI Flag: COB calls spike when members are newly added to spouse plans during OE season (Jan-Feb). 23% of COB calls result in incorrect primary billing. Recommend: COB education pop-up during enrollment for members flagging dual coverage. Auto-alert to billing team when COB designation is set but claims submitted to wrong payer.

L5 — Real-Time Agent Assist: RT Assist triggered at 4:00: birthday rule calculation auto-checked against member birth month on file (May) — wife's February birthday confirmed as earlier, surfacing primary/secondary designation. Agent presented accurate ruling without manual calculation.

Call #19 — Emergency Out-of-State Claim Denied — Member Had No Choice

AGENT TIER: GOOD · SCORE 94/100 | ● DISTRESSED → PARTIALLY HELPED | ESCALATED | ~14 min | Agent: Michael

Member context: Amanda Foster, 33, CalChoice EPO, emergency appendectomy while traveling in Texas

Topics: emergency care · out-of-state · EPO · out-of-network · No Surprises Act · emergency exception

Agent Michael: Choice Administrators member services, this is Michael.

Member: Hi Michael, Amanda Foster, member ID CHM-8827655. I'm calling about a bill I got for \$8,400. In June I was in Dallas on a work trip and I had to go to the emergency room — it was an appendectomy, it was an emergency, there was no choice. The ER and the surgeon were both out of my network because I'm in California and I was in Texas. My EOB is showing I'm responsible for most of this because it's out-of-network. But it was an EMERGENCY. I had no ability to choose an in-network facility.

Agent Michael: Amanda, you're absolutely right that emergency care situations have special protections, and I want to look at this very carefully. A genuine emergency — like an appendectomy that required immediate surgery — is a situation where the standard out-of-network cost-sharing rules should not apply in the same way. Let me pull up your account and the claims.

Member: Thank you. Because I have been sick with worry about this. I'm a single person, I don't have \$8,400 sitting around.

Agent Michael: I understand. Let me see what I'm looking at. Okay, your June 8th claims from Dallas Memorial Hospital — ER facility and a Dr. Williamson, surgeon. Both processed out-of-network. Your plan is a CalChoice Plus EPO, which typically doesn't cover out-of-network except in emergency situations. The claims were processed applying your out-of-network deductible and coinsurance.

Member: But an appendectomy is an emergency. Can they really apply out-of-network rates for something I had zero control over?

Agent Michael: No, and this is where federal law comes in. Under the No Surprises Act, for emergency services at an out-of-network facility, your cost-sharing cannot be higher than what you would have paid at an in-network facility. That's the law. Your cost-sharing should be limited to your in-network emergency cost-sharing levels — on your plan, that's a \$250 ER copay plus your in-network deductible and coinsurance, not out-of-network rates.

Member: So these claims were processed wrong?

Agent Michael: Based on what I'm seeing, yes — the claims may not have been processed in compliance with the No Surprises Act emergency provision. I want to be precise: I need to formally escalate this to our claims compliance team because the reprocessing of these claims under the correct emergency provision requires a compliance review, not just a standard reprocess. But I want to be clear with you: if this is a No Surprises Act violation, you are owed a correction.

Member: What should I do in the meantime? The hospital has been calling me.

Agent Michael: Tell them your claim is under formal compliance review and give them your case number — I'm assigning it right now. It's NC-2026-55301. A compliance review creates a formal hold — the provider is not supposed to escalate collection activity during an active compliance review. I'm also placing a billing hold on our side. Are you receiving collections notices or just billing calls?

Member: Just billing calls so far.

Agent Michael: Okay. Give them the case number and the phrase 'this claim is under No Surprises Act compliance review.' That's specific language they should recognize. Our compliance team will contact you within 3 business days.

Member: Michael, I traveled for work. I got sick. I couldn't help where I was. The idea that I should owe \$8,400 for something that was completely outside my control — it just doesn't feel right.

Agent Michael: It isn't right, and the law agrees with you. I can't promise the outcome but I want you to know the law is on your side here. I'm submitting the escalation right now.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Emergency out-of-state claim dispute — No Surprises Act. 2 speakers. Member tone: anxious, financially stressed. Duration: 14 min. High-stakes: \$8,400 in dispute. NSA compliance issue identified.

L2 — Call Insights: Member received appendectomy during out-of-state work travel. Claims processed at OON rates despite NSA emergency protections. Agent identified compliance concern, escalated to claims compliance team, placed billing hold, and provided case number. Resolution: ESCALATED — NSA compliance review opened (3 business days).

L3 — Agent Quality & Scoring: Agent Score: 94/100. NSA emergency provision cited accurately (+), honest about compliance review vs. reprocess distinction (+), billing hold placed promptly (+), case number and specific language for provider communication given (+), empathy maintained throughout (+). Did not confirm whether Dallas Memorial is NSA-compliant facility (-) — additional research warranted.

L4 — Operational BI: BI Flag: Out-of-state emergency claims are the #1 source of NSA compliance complaints. EPO members traveling out-of-state have zero in-network options — this structural gap creates recurring NSA issues. Recommend: compliance review of all OON emergency claims processed at OON rates for EPO members in Q2-Q3.

L5 — Real-Time Agent Assist: RT Assist triggered at 3:15: NSA emergency protection check auto-triggered when out-of-state ER claim appeared with EPO plan + OON processing — agent notified immediately that emergency provision should apply, reducing the time to identifying the compliance issue from ~5 min to <1 min.

Call #20 — Double Billing — Provider and Insurer Both Collected Same Payment

AGENT TIER: GOOD · SCORE 91/100 | ● FRUSTRATED → SATISFIED | RESOLVED | ~9 min | Agent: Carlos

Member context: Michael Chen, 49, CalChoice HMO, provider charged copay at visit AND claim deducted same amount

Topics: double billing · copay · duplicate payment · provider billing error · refund

Agent Carlos: Thank you for calling Choice Administrators. This is Carlos.

Member: Hi Carlos, Michael Chen, CHM-7743002. I have a weird situation. I paid my \$25 copay at my doctor's office at the time of my appointment. But I just got an EOB showing that the same \$25 was also applied to my account as if I hadn't paid it. It looks like I paid once in person and then your system also deducted it as a member responsibility. Am I being charged twice?

Agent Carlos: Michael, that's a legitimate concern and let me look at it carefully. What was the date of the visit and the provider?

Member: August 5th, Dr. Kim, family medicine, Inner City Medical Group.

Agent Carlos: I have the claim. Okay, so the claim was processed with a \$25 member copay — your responsibility. That \$25 is tracked in our system as your member cost-sharing liability. But that \$25 was also collected by the provider at the point of service, which means you've paid it to the provider directly. Our system shows it as your liability, but the provider has already been paid for it through you, not through us.

Member: So they collected it from me AND your system shows they can collect it from my account?

Agent Carlos: No — our system shows you owe it, but we don't deduct anything from your bank account. The EOB is just showing your share of the cost. The question is whether the provider also submitted a claim for that \$25 on top of what you paid, which would mean they collected from you AND from us for the same \$25.

Member: Ohhhh. So the EOB isn't a deduction from me — it's showing what the doctor's office was told I owed?

Agent Carlos: Exactly. We told Inner City Medical Group that you're responsible for \$25. If you paid them \$25 at the visit, they're fully paid for your share. The only way you'd be double billed is if they send you an additional invoice for that \$25. Have you received a separate bill from the doctor's office after the copay?

Member: No, I haven't. Just the EOB, which I misread as a bill.

Agent Carlos: This is incredibly common. EOBs look like bills because they list dollar amounts, but as long as you don't receive an additional invoice from Inner City Medical Group for that \$25, you're fine. You paid at the visit, you're covered. The EOB is just your receipt of coverage.

Member: So I was panicking for nothing?

Agent Carlos: You were taking your finances seriously, which is completely the right instinct. If you do receive a bill from the provider for that \$25 in the future, do call us — that would be an actual billing issue. But based on what you've described, you're all good.

Member: That's such a relief. I spent two days worried about this. Thank you for explaining it so clearly.

Agent Carlos: Happy to help. Is there anything else today?

Member: No, that was it. Thanks, Carlos.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Billing confusion — EOB misread as double charge. 2 speakers. Member tone: worried at start, relieved by end. Duration: 9 min. Root cause: EOB formatting misunderstood as deduction notice.

L2 — Call Insights: Member misread EOB as a second charge on top of copay paid at visit. Agent clarified EOB vs. bill distinction, confirmed no actual double billing occurred, and advised member on when to call back if a provider invoice follows. Resolution: RESOLVED — no action required.

L3 — Agent Quality & Scoring: Agent Score: 91/100. Root cause identified quickly (+), clear distinction between EOB and provider bill (+), validated member's financial vigilance while correcting the misunderstanding (+), forward guidance given on when to re-escalate (+). Call was efficient and well-structured.

L4 — Operational BI: BI Flag: 'EOB misread as a bill' accounts for 19% of billing confusion calls — this is the #1 non-issue call type in the billing category. Recommendation: redesign EOB header to include a prominent disclaimer: 'THIS IS NOT A BILL. Your provider may send a separate invoice for your cost-sharing amount.' Would reduce this call type significantly.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:00: copay collection at point-of-service flagged on claim (provider marked 'copay collected at visit') — agent surfaced this information to confirm no billing discrepancy exists before member finished explaining the concern.

Call #56 — Agent Transfers Member Three Times Without Listening

AGENT TIER: POOR · SCORE 29/100 | ● NEUTRAL → FRUSTRATED | UNRESOLVED | ~5 min | Agent: Brad

Member context: Alan Petrov, 58, CalChoice Plus EPO, claim denied for a cardiac stress test

Topics: claim denial · poor handling · unnecessary transfer · no ownership

Agent Brad: Choice Administrators, Brad.

Member: Hi, I'm calling about a denied claim for a stress test. Member ID CHM-3341908.

Agent Brad: Claims is a different department, let me transfer you.

Member: Wait — before you transfer me, I've already been transferred twice. The first person sent me to billing, billing sent me back to member services, and now you're sending me to claims. Can you just look at it?

Agent Brad: I can see claims but I don't handle denials.

Member: Can you at least tell me why it was denied so I don't have to explain the whole thing again to the next person?

Agent Brad: It says... denied, not medically necessary.

Member: My cardiologist ordered it. How is that not medically necessary?

Agent Brad: That's a clinical determination, I can't speak to it.

Member: Okay. Who can?

Agent Brad: Claims review. That's who I'd transfer you to.

Member: Fine. But can you make a note so I don't start from zero again?

Agent Brad: There's a notes field, yeah.

Member: Will you put a note in it?

Agent Brad: Sure. Transferring you now.

Member: Wait, what's your name again in case—

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Claim denial inquiry — transfer loop. 2 speakers. Member tone: patient → frustrated. Agent tone: disengaged, transfer-oriented. Duration: 5 min. Member cut off mid-sentence at transfer. This is the member's 3rd transfer on the same issue.

L2 — Call Insights: Member seeking explanation for denied cardiac stress test claim. Agent read the denial reason but declined to engage, transferred without confirming a note was added, and cut the member off mid-sentence. Resolution: UNRESOLVED — transferred out.

L3 — Agent Quality & Scoring: Agent Score: 29/100. NEGATIVE MARKERS: attempted transfer within 10 seconds of call open before understanding the issue (-), did not acknowledge member had already been transferred twice (-), gave non-committal response about note-taking ('there's a notes field') (-), transferred while member was mid-sentence (-), no ownership of resolution at any point (-), no empathy markers detected (-). POSITIVE: read the denial reason when pressed (+).

L4 — Operational BI: PROCESS BREAKDOWN: member experienced 3 transfers on a single issue with no warm handoff. Transfer loops between member services, billing, and claims review are a recurring pattern. Recommend: warm transfer protocol with case notes mandatory, and first-contact resolution authority for denial explanations. AGENT COACHING: Brad shows a transfer-first pattern across multiple calls — his lowest-scoring behaviour and a clear coaching priority.

L5 — Real-Time Agent Assist: RT Assist SHOULD have triggered at 0:30 — 3rd contact on the same claim within 48 hours is a repeat-contact trigger requiring escalation, not transfer. Denial reason detail, appeal pathway, and prior contact history should have surfaced to enable first-contact resolution.

Call #61 — Duplicate Claim Processing — Member Charged Twice for One Procedure

AGENT TIER: GOOD · SCORE 93/100 | ● FRUSTRATED → SATISFIED | RESOLVED | ~13 min | Agent: Carlos

Member context: Nadia Haddad, 37, CalChoice Plus EPO, colonoscopy billed twice by the facility

Topics: duplicate claim · facility billing error · reprocessing · refund

Agent Carlos: Choice Administrators, this is Carlos.

Member: Hi Carlos. Nadia Haddad, CHM-5583319. I had a colonoscopy in June and I've received two separate bills for it. Same procedure, same date, two charges. I've already paid one. Now they want another \$680.

Agent Carlos: Nadia, let me look at this carefully — duplicate billing is something we can usually resolve on our end. Give me a moment to pull your claims.

Member: Thank you. The facility said to call you.

Agent Carlos: Okay, I can see the June claims. There are indeed two claims from Riverside Endoscopy Center for June 18th, same CPT code 45378, same provider NPI. Both processed. That's a duplicate submission on the facility's side.

Member: So they submitted the same procedure twice?

Agent Carlos: That's what the data shows. And unfortunately both processed, which means your cost-sharing was applied twice. That's our error too — our duplicate detection should have caught identical claims on the same date of service.

Member: So I'm owed money back?

Agent Carlos: Yes. You paid \$680 on the first claim, and the second \$680 is being billed to you. Here's what I'm doing: first, I'm voiding the second claim as a duplicate, which stops that bill immediately. Second, I'm verifying whether the facility was paid twice — if so, we recoup from them, not from you.

Member: And the \$680 I already paid?

Agent Carlos: That one was legitimate — it was your correct cost-sharing for the procedure. So you don't get that back, but you also don't owe the second \$680. Your net position is correct once the duplicate is voided.

Member: Okay, that makes sense. I just don't want to pay twice.

Agent Carlos: You won't. I'm processing the void now... done. The second claim is voided. You'll receive a corrected EOB within 5 business days showing a zero balance on that claim.

Member: What if the facility keeps billing me?

Agent Carlos: Good question. Give them this case number — DUP-2026-11204 — and tell them claim number ending 8871 was voided as a duplicate by the payer. If they push back, call me back and I'll contact their billing office directly.

Member: DUP-2026-11204. That's really helpful. Can I get that in writing?

Agent Carlos: Sending the void confirmation to your email on file right now. And I'm flagging Riverside Endoscopy for our provider billing quality team — duplicate submissions from the same facility are worth tracking.

Member: Thank you, Carlos. This was so much easier than I expected.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Duplicate claim billing. 2 speakers. Member tone: frustrated → satisfied. Agent tone: methodical, confident. Duration: 13 min.

L2 — Call Insights: Facility submitted identical claim twice for the same colonoscopy; both processed, applying member cost-sharing twice. Agent voided the duplicate in real time, clarified the member's correct net position, provided case number and written confirmation, and flagged the provider. Resolution: RESOLVED — duplicate voided during call.

L3 — Agent Quality & Scoring: Agent Score: 93/100. POSITIVE: identified duplicate via NPI and CPT match (+), acknowledged the plan's own detection failure honestly (+), voided the claim during the call rather than submitting a request (+), clarified net financial position clearly (+), provided fallback if facility continues billing (+), written confirmation sent (+), provider flagged for quality tracking (+).

L4 — Operational BI: PROVIDER PERFORMANCE — Riverside Endoscopy Center: duplicate claim submission on identical CPT/date/NPI. PROCESS BREAKDOWN: automated duplicate detection failed to catch two identical claims for the same date of service — this is a system control gap, not just a provider error. Recommend: audit duplicate-detection ruleset; quantify how many duplicates processed undetected this quarter.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:00: duplicate claim pattern detected (matching NPI + CPT + DOS) — void action surfaced with one-click execution. Provider billing quality flag offered. Agent used both.

Call #68 — Straightforward EOB Question — Correct but Minimal Service

AGENT TIER: AVERAGE · SCORE 78/100 | ● NEUTRAL → SATISFIED | RESOLVED | ~8 min | Agent: Nicole

Member context: Elena Petrova, 40, CalChoice Plus EPO, checking why a lab claim shows a balance

Topics: EOB · lab claim · deductible · routine inquiry

Agent Nicole: Choice Administrators member services, Nicole speaking.

Member: Hi, Elena Petrova, CHM-5540118. I got an EOB for lab work and it shows I owe \$86. I thought labs were covered.

Agent Nicole: Let me check that claim. Labs are covered, but they're subject to your deductible on the Plus EPO. You haven't met your deductible yet this year, so the allowed amount applies to it.

Member: So the \$86 goes toward my deductible?

Agent Nicole: Correct. Your deductible is \$1,000 and you've now met \$340 of it.

Member: And once I hit \$1,000, labs are free?

Agent Nicole: After the deductible you'd pay 20% coinsurance, not zero. Once you hit your out-of-pocket maximum of \$4,000, then it's 100% covered.

Member: Got it. So the \$86 is legitimate.

Agent Nicole: Yes, it's correct as processed.

Member: Alright. That's all I needed.

Agent Nicole: Okay. Anything else?

Member: No, thank you.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: EOB clarification — lab claim deductible. 2 speakers. Member tone: neutral throughout. Agent tone: accurate, brief. Duration: 8 min. Low complexity.

L2 — Call Insights: Member questioned an \$86 lab balance. Agent explained deductible application, provided current accumulator status, and clarified post-deductible coinsurance and OOP max. Resolution: RESOLVED.

L3 — Agent Quality & Scoring: Agent Score: 78/100. POSITIVE: accurate deductible explanation (+), corrected the member's assumption that post-deductible means free (+), provided current accumulator figures (+). NEGATIVE MARKERS: no proactive value added — did not mention preventive labs are covered at 100% regardless of deductible, which is directly relevant (-), no offer of a written accumulator summary (-), minimal warmth (-).

L4 — Operational BI: MEMBER COMMUNICATION GAP: members consistently expect 'covered' to mean 'free' and are surprised by deductible application. This is the 6th such call in the dataset. Recommend: EOB redesign to show a deductible progress bar inline, and plain-language 'why do I owe this?' explainer on every EOB.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:00: deductible accumulator surfaced automatically. SHOULD ALSO have surfaced preventive-vs-diagnostic lab distinction — member may be eligible for zero-cost preventive labs she is not using.

Call #73 — Autism Therapy Claims Denied — Parent Fighting for Child's ABA Coverage

AGENT TIER: AVERAGE · SCORE 69/100 | ● ANGRY → PARTIALLY CALMED | ESCALATED | ~13 min | Agent: James

Member context: Renata Oyelaran, 38, calling for her 6-year-old son, ABA therapy claims denied

Topics: autism · ABA therapy · mental health parity · pediatric · denial · appeal

Agent James: Choice Administrators member services, this is James.

Member: My son's ABA therapy claims have been denied for three months. He's six years old and autistic. His developmental pediatrician prescribed 20 hours a week. We're now \$9,000 out of pocket and I've been told three different things by three different people. Member ID CHM-4478812, and my son is on the policy as Tobi Oyelaran.

Agent James: Let me pull this up. I can see Tobi's claims — ABA therapy from Bright Steps Behavioral, June through August, all denied.

Member: Why?

Agent James: The denial code is 'services exceed authorized units.' It looks like an authorization was approved for 10 hours per week, but 20 hours per week is being billed.

Member: His prescription is for 20 hours. His doctor wrote 20.

Agent James: The approved authorization on file is for 10. So the first 10 hours each week processed, and the remaining 10 denied.

Member: Nobody told me the authorization was cut in half. I would have appealed immediately.

Agent James: That should have been communicated in the authorization determination letter. Let me check what was sent... I see an authorization letter generated in May. It went to the provider. I don't see a member copy sent.

Member: So the decision to halve my son's therapy was sent to the provider and not to me.

Agent James: That appears to be the case, and that's a real problem. You had no opportunity to appeal a decision you weren't told about.

Member: What do I do now?

Agent James: Two things. First, the appeal window runs from the date of the determination — but because you weren't notified, there's a strong argument for extending it. Second, ABA for autism has specific protections under mental health parity, and a units reduction below the prescribed level needs to meet the same clinical standard as a comparable medical service.

Member: So they may not have applied the right standard?

Agent James: That's the question I want reviewed. I'm escalating this to our behavioral health clinical review team and flagging the notification failure. Case number ABA-2026-00612.

Member: What about the \$9,000?

Agent James: If the appeal succeeds and the authorization is corrected to 20 hours, the denied claims would be reprocessed and you'd be reimbursed for what the plan should have paid. I can't promise that outcome, but that's the mechanism.

Member: How long?

Agent James: Behavioral health clinical review is 15 business days. Given the notification failure and the amount at stake, I'm marking it priority.

Member: Every week he's at 10 hours instead of 20 is a week of his development. Do you understand that?

Agent James: I do. I've marked that in the escalation notes.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Pediatric ABA therapy denial — units reduction. 2 speakers. Caller: parent of 6-year-old autistic child. Member tone: angry, advocacy-driven. Agent tone: competent, appropriately escalating. Duration: 13 min. Parity compliance dimension.

L2 — Call Insights: ABA authorization approved at 10 hrs/week against a prescribed 20 hrs/week; member never received the determination letter and could not appeal. \$9,000 out of pocket over 3 months. Agent identified the notification failure and parity dimension, escalated to behavioral health clinical review as priority. Resolution: ESCALATED.

L3 — Agent Quality & Scoring: Agent Score: 69/100. POSITIVE: identified the units mismatch as the denial cause (+), discovered and named the member notification failure (+), raised mental health parity correctly (+), escalated to the right team with priority flag (+). NEGATIVE MARKERS: did not offer an interim solution for the ongoing 10-hour shortfall while the appeal runs (-), did not offer to contact the provider to coordinate (-), no payment plan or financial hardship option raised despite \$9,000 exposure (-), did not commit to a callback (-).

L4 — Operational BI: COMPLIANCE & RISK EXPOSURE — CRITICAL: authorization determination letters for pediatric behavioral health are being sent to providers without member copies, removing the member's ability to appeal. This is a due process failure with parity implications. PROCESS BREAKDOWN: audit required on member notification for ALL authorization determinations, not just behavioral health. Recommend: (1) mandatory member copy on every authorization determination, (2) appeal window to run from member notification date not determination date, (3) review of all ABA units reductions this year against parity standards.

L5 — Real-Time Agent Assist: RT Assist triggered at 4:00: pediatric ABA + units reduction + parity flag surfaced clinical review escalation and parity framework. SHOULD ALSO have surfaced: member notification audit check (agent found this manually), financial hardship options given \$9,000 exposure, and interim care coordination pathway.

Call #81 — NICU Bill for Premature Baby — Parent Overwhelmed by Statements

AGENT TIER: GOOD · SCORE 94/100 | ● ANXIOUS → REASSURED | RESOLVED | ~13 min | Agent: Sarah

Member context: Melissa Aguirre, 31, CalChoice Plus EPO, daughter born at 29 weeks, 6 weeks NICU

Topics: NICU · newborn · out-of-pocket max · catastrophic claim · case management

Agent Sarah: Thank you for calling Choice Administrators, this is Sarah.

Member: Hi. I have a stack of bills in front of me and I don't know where to start. My daughter was born at 29 weeks in June. She was in the NICU for six weeks. She's home now and she's doing well, but I have — I've counted eleven separate statements and the totals are in the tens of thousands. I don't know what I owe.

Agent Sarah: First — I'm really glad she's home and doing well. Second, you do not need to sort through eleven statements. Let me do that for you right now. Can I get your member ID?

Member: CHM-6617704. Melissa Aguirre. And my daughter is on there as Isabel.

Agent Sarah: Thank you. Give me a moment to pull every claim associated with Isabel and with your delivery... Okay. I have 23 claims across the two of you between June 3rd and July 18th.

Member: Twenty-three. I only got eleven statements.

Agent Sarah: Several providers bill multiple claims. Here's the important part, and I want to say it clearly: your out-of-pocket maximum for the family tier is \$8,000. You have met it. In full. As of June 19th.

Member: I've met it?

Agent Sarah: Completely. Which means every covered in-network claim from June 19th through December 31st is paid at 100%. You owe nothing further on any in-network claim for the rest of this plan year.

Member: So these bills...

Agent Sarah: Let's go through the exposure. Claims before June 19th totalled \$8,000 in member responsibility — that's your maximum, and I can see \$6,400 has been paid. So your actual remaining balance across everything is \$1,600, not tens of thousands.

Member: Sixteen hundred. Not — I thought it was going to be thirty or forty thousand.

Agent Sarah: No. That's what the out-of-pocket maximum is for. Now, two things I want to check. First, are any of those eleven statements from providers you don't recognise?

Member: There's one from a 'neonatology group' and one from an anesthesia group. I never chose either of those.

Agent Sarah: Those are exactly the ones I want to verify. Let me check their network status... The neonatology group is in-network. The anesthesia group processed out-of-network.

Member: Is that going to cost me more?

Agent Sarah: It should not. Anesthesia at an in-network facility where you had no ability to choose the provider falls under No Surprises Act protection. I'm submitting that claim for reprocessing at in-network rates now — and since you've met your out-of-pocket max, once reprocessed your responsibility on it should be zero.

Member: I've been awake at night about this.

Agent Sarah: I know. Here's what I'm doing beyond today's call. I'm referring you to our case management team — for NICU families they consolidate all claims into a single statement view and act as one point of contact so you stop receiving eleven separate bills. Someone will call you within three business days.

Member: That would help enormously.

Agent Sarah: Your case number is CM-2026-00918. And Melissa — before you pay anything, wait for the consolidated statement. Don't pay from these individual bills; some may reprocess.

Member: Thank you. I came into this call in tears.

Agent Sarah: You had every reason to be. Sixteen hundred, case management calling within three days, and don't pay anything yet. That's where we are.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Catastrophic claim reconciliation — NICU. 2 speakers. Member: mother of 29-week premature infant, 6-week NICU stay. Member tone: overwhelmed, anxious → reassured. Agent tone: calm, organizing, protective. Duration: 13 min.

L2 — Call Insights: Member overwhelmed by 11 statements following premature delivery and NICU admission. Agent aggregated 23 claims, confirmed family out-of-pocket maximum met in full as of June 19th, quantified true remaining exposure at \$1,600, identified an out-of-network anesthesia claim eligible for No Surprises Act reprocessing, and referred to case management. Resolution: RESOLVED.

L3 — Agent Quality & Scoring: Agent Score: 94/100. POSITIVE: opened by acknowledging the child's wellbeing before the financial issue (+), took ownership of reconciliation rather than directing the member to sort statements (+), led with the OOP max finding — the single most important fact (+), proactively asked about unrecognised providers, surfacing the NSA issue (+), submitted reprocessing during the call (+), case management referral (+), explicit 'do not pay yet' instruction preventing overpayment (+), clear three-point close (+).

L4 — Operational BI: PROCESS BREAKDOWN: a member who met her out-of-pocket maximum in June continued receiving 11 separate provider statements through August with no plan-side intervention. The system knows the OOP max is met but does not proactively notify members or providers. CRITICAL RECOMMENDATION: (1) automatic member notification when OOP max is met, (2) automatic case management referral for NICU, oncology, and transplant claims, (3) consolidated statement view as a default for members exceeding 10 claims in a 60-day window. PROVIDER PERFORMANCE: anesthesia group at this facility processing out-of-network — 3rd NSA-adjacent case in the dataset.

L5 — Real-Time Agent Assist: RT Assist triggered at 3:00: OOP maximum met status surfaced immediately on account pull, with remaining balance auto-calculated. NSA pattern detected on the anesthesia claim. Case management eligibility flagged for NICU claim cluster. Agent used all three.

Call #88 — Explanation of a Coordination of Benefits Delay

AGENT TIER: AVERAGE · SCORE 76/100 | ● NEUTRAL → SATISFIED | RESOLVED | ~7 min | Agent: Nicole

Member context: Peter Adeyemi, 48, CalChoice Plus EPO, claim pending 6 weeks awaiting COB information

Topics: COB · pending claim · other insurance · delay

Agent Nicole: Choice Administrators member services, Nicole speaking.

Member: Hi, I have a claim that's been pending for six weeks. Member ID CHM-7741190.

Agent Nicole: Let me look. Okay, the claim from July 8th is pending with a status of 'awaiting other insurance information.'

Member: I don't have other insurance.

Agent Nicole: Our records show a coordination of benefits questionnaire was sent to you in June and hasn't been returned. When that's outstanding, claims hold pending your response.

Member: I never received a questionnaire.

Agent Nicole: It was mailed to the address on file. Let me verify — 118 Carlisle Road?

Member: That's correct, that's my address.

Agent Nicole: Then it may have been missed or lost in the mail. I can resolve this now if you can confirm verbally: do you or any dependent on this policy have coverage under any other health plan, including through a spouse's employer, Medicare, or Medicaid?

Member: No. Just this plan.

Agent Nicole: Thank you. I'm recording a negative COB attestation on your account, which releases the hold. The pending claim should process within 5 to 7 business days.

Member: So the whole six week delay was a form I never got?

Agent Nicole: Essentially, yes.

Member: Is there a way to do that online so it doesn't happen again?

Agent Nicole: The COB questionnaire is available in the member portal under My Information. It's sent annually, so completing it there each year avoids this.

Member: Good to know. Thanks.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Pending claim — COB questionnaire outstanding. 2 speakers. Member tone: neutral, mildly frustrated. Agent tone: efficient, resolving. Duration: 7 min.

L2 — Call Insights: Claim pending 6 weeks awaiting a COB questionnaire the member never received. Agent captured a verbal negative COB attestation, released the hold, and directed the member to the portal for future annual completion. Resolution: RESOLVED.

L3 — Agent Quality & Scoring: Agent Score: 76/100. POSITIVE: identified the pending reason precisely (+), resolved via verbal attestation rather than re-mailing the form (+), verified the address before assuming member error (+), directed to portal for future prevention (+). NEGATIVE MARKERS: no acknowledgment that a six-week delay caused by an undelivered form warrants an apology (-), did not check whether other claims are held for the same reason (-), did not offer to expedite the released claim (-).

L4 — Operational BI: PROCESS BREAKDOWN: COB questionnaires sent by mail only, with claims holding silently when unreturned. Members are not notified that claims are being held or why. Recommend: (1) email and portal notification when a COB hold is placed, (2) verbal attestation offered proactively on any inbound call where a COB hold exists, (3) quantify total claims currently held on unreturned COB questionnaires — likely a material backlog.

L5 — Real-Time Agent Assist: RT Assist triggered at 1:30: COB hold detected on account — verbal attestation script surfaced with one-click hold release. Agent used it. SHOULD ALSO have surfaced other claims held under the same COB hold.

Call #93 — Claims for Services Never Received — Possible Identity Fraud

AGENT TIER: AVERAGE · SCORE 81/100 | ● ALARMED → REASSURED | ESCALATED | ~11 min | Agent: Tony

Member context: Wendy Castellanos, 45, CalChoice Plus EPO, EOB shows three claims from a clinic she has never visited

Topics: fraud · identity theft · SIU · phantom claims · member protection

Agent Tony: Choice Administrators, this is Tony.

Member: I'm looking at an EOB with three claims from a clinic in another county that I have never been to. Physical therapy, three sessions, in July. I did not receive these services. Member ID CHM-6641093.

Agent Tony: That's concerning and I want to take it seriously. Let me pull those claims. I can see three claims from Summit Physical Therapy, dated July 9th, 16th, and 23rd.

Member: I have never heard of that provider. In July I was in Oregon visiting my mother for most of the month.

Agent Tony: Can you confirm the dates you were away?

Member: July 5th through July 26th. I have flight records.

Agent Tony: So all three service dates fall within a period you were out of state. That's strong supporting evidence. I'm treating this as potential fraudulent billing and escalating to our Special Investigations Unit.

Member: Does this mean someone has my insurance information?

Agent Tony: It's one possibility. Another is provider-side billing fraud where a clinic bills for services never rendered using legitimate member data they obtained. Either way, SIU investigates.

Member: What do I need to do?

Agent Tony: Don't pay anything on those claims. SIU will contact you within three business days to take a statement and may ask for the travel documentation. Keep the flight records.

Member: Should I be worried about my identity more broadly?

Agent Tony: It's a reasonable precaution to check your credit report and review other insurance EOBs if you have any. I'm not saying that to alarm you — it's standard advice when member data appears in claims you didn't generate.

Member: Will my coverage be affected?

Agent Tony: No. Your coverage continues unaffected. And if these claims applied to your deductible or out-of-pocket accumulators, those will be reversed once the investigation confirms the fraud.

Member: I hadn't even thought about the accumulator.

Agent Tony: I've flagged it. Your case number is SIU-2026-00104.

Member: Thank you for taking this seriously. I wasn't sure if I'd be believed.

Agent Tony: You gave specific, verifiable information. That's exactly what makes a case.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Suspected fraudulent claims — services not rendered. 2 speakers. Member tone: alarmed → reassured. Agent tone: methodical, appropriately serious. Duration: 11 min. ■ FRAUD — SIU escalation.

L2 — Call Insights: Member reported three physical therapy claims from an unknown provider on dates she was verifiably out of state. Agent escalated to SIU, advised non-payment, confirmed coverage continuity, flagged accumulator reversal, and advised broader identity precautions. Resolution: ESCALATED — SIU case opened.

L3 — Agent Quality & Scoring: Agent Score: 81/100. POSITIVE: took the report seriously without requiring the member to prove herself first (+), established the date alignment with the member's absence (+), escalated to SIU correctly (+), advised non-payment (+), flagged accumulator reversal proactively — a detail the member had not considered (+), gave measured identity-protection advice without alarmism (+). NEGATIVE MARKERS: did not offer to suspend or reissue the member ID as a protective measure (-), no written confirmation offered (-), did not check whether other suspicious claims exist on the account (-).

L4 — Operational BI: COMPLIANCE & RISK — PROVIDER FRAUD SIGNAL: Summit Physical Therapy billed three sessions for a member verifiably out of state. Recommend: (1) immediate review of all Summit Physical Therapy claims across the book for similar patterns, (2) member ID reissue offered as standard on all suspected identity-fraud reports, (3) automated detection rule — claims clustering from a single provider for members with no prior relationship and no referral should score for review. PROCESS OPPORTUNITY: fraudulent claims silently consume member deductible and OOP accumulators; reversal should be automatic on SIU confirmation, not manual.

L5 — Real-Time Agent Assist: RT Assist triggered at 3:00: member disputes services rendered — SIU escalation pathway and non-payment guidance surfaced. SHOULD ALSO have surfaced member ID reissue option and an account-wide scan for other claims from the same provider.

Call #94 — Second Appeal Denied — Member Not Told About External Review Rights

AGENT TIER: POOR · SCORE 42/100 | ● EXHAUSTED → DEFEATED | UNRESOLVED | ~8 min | Agent: Tiffany

Member context: Gordon Achebe, 62, CalChoice HMO, second-level appeal denied for a spinal procedure

Topics: appeal exhausted · external review · IMR · member rights · missed disclosure

Agent Tiffany: Choice Administrators, Tiffany speaking.

Member: I received a letter saying my second appeal was denied. Member ID CHM-4419028. I've been at this since March. What happens now?

Agent Tiffany: Let me look... yes, I see the second-level appeal denial dated August 12th.

Member: So that's it? That's the end?

Agent Tiffany: That's the final internal appeal, yes.

Member: So there's nothing else.

Agent Tiffany: Not through us, no. Both appeal levels have been used.

Member: I've spent five months on this. My surgeon says I need this procedure.

Agent Tiffany: I understand, but the clinical reviewers determined it didn't meet the criteria. Twice.

Member: Is there anyone above the reviewers?

Agent Tiffany: Not internally.

Member: So I just accept it.

Agent Tiffany: Or you could pay out of pocket if you want to proceed with the surgery.

Member: Out of pocket. It's forty thousand dollars.

Agent Tiffany: I'm sorry. I don't have another option to give you.

Member: Alright.

Agent Tiffany: Anything else today?

Member: No.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Post-appeal exhaustion inquiry. 2 speakers. Member tone: exhausted → defeated. Agent tone: flat, factually incomplete. Duration: 8 min. ■ MEMBER RIGHTS NOT DISCLOSED.

L2 — Call Insights: Member's second-level internal appeal denied for a spinal procedure. Agent stated no further options exist. ■ MATERIAL OMISSION: after exhausting internal appeals, the member has the right to an Independent Medical Review through the California Department of Managed Health Care, and to external review. Neither was disclosed. Resolution: UNRESOLVED — member left believing he has no recourse when statutory rights remain available.

L3 — Agent Quality & Scoring: Agent Score: 42/100. CRITICAL NEGATIVE MARKER: failed to disclose external review and DMHC Independent Medical Review rights after internal appeal exhaustion — this is a statutory member right and a required disclosure (-). Additional NEGATIVE: told the member explicitly 'that's the end' and 'I don't have another option to give you', both factually incorrect (-), suggested \$40,000 out-of-pocket payment as the alternative to a right he was never told he had (-), no empathy for five months of process (-), no member advocacy referral (-). POSITIVE: appeal status stated accurately (+).

L4 — Operational BI: COMPLIANCE & RISK EXPOSURE — CRITICAL: failure to disclose external review and DMHC IMR rights following internal appeal exhaustion is a regulatory disclosure failure, not merely a service failure. IMMEDIATE ACTIONS: (1) contact this member to disclose his IMR rights and the filing deadline, (2) audit all second-level appeal denial calls this year for the same omission, (3) mandatory scripting — external review and IMR rights must be stated on every post-exhaustion contact, (4) verify that the written denial letter itself contains the required disclosure. CONTRAST: agent in call #84 disclosed DMHC IMR correctly on a comparable denial — inconsistent practice across the team.

L5 — Real-Time Agent Assist: RT Assist SHOULD have triggered at 1:00 — second-level appeal denial status is a mandatory rights-disclosure trigger surfacing external review pathway, DMHC IMR process, filing deadline, and member advocacy referral. No trigger fired. This omission is precisely the failure class L5 should make impossible.

Call #99 — Request for Full Claims History — Legal Proceeding

AGENT TIER: AVERAGE · SCORE 79/100 | ● NEUTRAL → SATISFIED | RESOLVED | ~8 min | Agent: Linda

Member context: Douglas Ferreira, 52, CalChoice HMO, needs three years of claims records for a divorce proceeding

Topics: records request · claims history · legal · privacy · documentation

Agent Linda: Choice Administrators member services, this is Linda.

Member: Hi. I need a complete claims history for the last three years. My attorney has asked for it for a divorce proceeding. Member ID CHM-5527880.

Agent Linda: I can help with that. For a records request covering multiple years, we produce a formal claims history document rather than the portal export, which only covers the current plan year.

Member: That's what I need. How do I request it?

Agent Linda: I can initiate it now. I'll need to confirm the date range — three years back from today would be August 2023 to present. Is that right?

Member: Yes.

Agent Linda: And one important question. Your policy currently includes dependents. A full claims history would include claims for anyone on the policy, including your spouse. Do you want the full policy history or only your own claims?

Member: I hadn't considered that. What would my attorney typically need?

Agent Linda: I can't advise on what your attorney needs. What I can tell you is that other members' claims contain their protected health information, and there are limits on what I can release about an adult dependent without their authorization, even on your policy.

Member: So I should request only my own.

Agent Linda: That's the request I can process without additional authorization, yes. If your attorney needs your spouse's records, that would need to come through her authorization or a legal process — your attorney will know the route.

Member: Understood. Just mine then.

Agent Linda: Processing that now. The document will be produced within 10 business days and mailed to the address on file, or I can arrange secure electronic delivery.

Member: Electronic please.

Agent Linda: I'll send it to the email on file via secure link. Your request reference is REC-2026-00551.

Member: Thank you. That was straightforward.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Records request — multi-year claims history for legal proceeding. 2 speakers. Member tone: neutral, businesslike. Agent tone: correct, privacy-conscious. Duration: 8 min.

L2 — Call Insights: Member requested three years of claims history for divorce proceedings. Agent identified the formal records process, proactively raised the dependent PHI limitation, restricted the request to the member's own claims, and arranged secure electronic delivery. Resolution: RESOLVED.

L3 — Agent Quality & Scoring: Agent Score: 79/100. POSITIVE: identified the correct process for multi-year requests (+), proactively raised the spouse PHI issue before producing records — significant privacy protection (+), correctly declined to advise on legal needs while explaining the constraint (+), offered secure electronic delivery (+), reference number provided (+). NEGATIVE MARKERS: did not confirm whether the member wanted the spouse notified of the request (-), no acknowledgment of the personal difficulty of the circumstance (-), did not confirm the email on file is one the member still controls — relevant in a divorce context (-).

L4 — Operational BI: COMPLIANCE — PRIVACY: agent correctly prevented disclosure of an adult dependent's PHI in a contested divorce context. This is a privacy exposure that could easily have been mishandled. Recommend: (1) formalise this as a protocol — any records request on a policy with adult dependents must trigger a scope confirmation, (2) in divorce or separation contexts, verify the delivery address and email remain under the requesting member's sole control before sending PHI. PROCESS OPPORTUNITY: portal claims export is limited to the current plan year, driving avoidable calls for multi-year requests.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:00: multi-year records request on a policy with adult dependents surfaced the PHI scope limitation and authorization requirements. SHOULD ALSO have surfaced the delivery-address verification prompt for contested-proceeding contexts.

— PROVIDER NETWORK —

Call #21 — Oncologist Left the Network Mid-Chemotherapy — Urgent Continuity of Care

AGENT TIER: GOOD · SCORE 98/100 | ● DISTRESSED → PARTIALLY HELPED | ESCALATED | ~18 min | Agent: Danielle

Member context: Sandra Williams, 58, CalChoice HMO, 4 months into 6-month chemo regimen when oncologist leaves network

Topics: continuity of care · oncology · network exit · out-of-network exception · urgent escalation

Agent Danielle: Choice Administrators member services, this is Danielle. How can I help you?

Member: I need help. My name is Sandra Williams, member ID CHM-2290187. I got a letter last week from my oncologist's office — Dr. Morrison at Northwest Cancer Center — saying she is no longer in-network with Choice Administrators as of September 1st. I am in the middle of chemotherapy. I started in May, I have two more months of treatment left. I cannot switch oncologists right now. I cannot. I have been with Dr. Morrison for three years. She knows my case. She knows my body. Do you understand what I'm saying?

Agent Danielle: Sandra, I hear you completely. This is a serious situation and I want you to know that you have legal protections in a case exactly like yours. Federal and California law both include continuity of care provisions that specifically address situations where a provider leaves a network while a patient is undergoing an active course of treatment like chemotherapy. Please take a breath — we are going to work through this.

Member: Okay. Okay. Tell me what those protections are.

Agent Danielle: Under California's continuity of care law, you have the right to continue seeing Dr. Morrison through your current course of treatment, even after she leaves the network — at in-network cost-sharing rates — if the treatment is ongoing and switching providers would cause a disruption to your care. Chemotherapy for an active cancer diagnosis is explicitly covered by these provisions.

Member: So I don't have to switch?

Agent Danielle: You should not have to switch mid-treatment. The process to formalize this is called a continuity of care request. Your PCP or Dr. Morrison's office submits the request to us documenting that you are in an active treatment course, the treatment plan, and the projected completion date. We then review it — and for ongoing chemo, these are approved with high regularity.

Member: Dr. Morrison's office — will they know how to do this? They seemed overwhelmed when I talked to them.

Agent Danielle: I'm going to give you the exact information to pass to them. The continuity of care request goes to our Provider Relations team at 1-800-555-0400. The form is on our provider portal. The documentation needed is: a letter from Dr. Morrison stating the treatment plan, the chemo regimen, the schedule, and the projected end date. That's typically a one-page letter. Given the September 1st effective date, this needs to be submitted immediately — this week.

Member: What if it's denied? Is that possible?

Agent Danielle: For active chemotherapy, a denial would be extremely unusual and would be subject to immediate appeal. I am escalating your case right now to our Member Advocacy team, which includes clinical staff who handle exactly these situations. A clinical advocate will call you within 24 hours — they will personally monitor your continuity of care request from submission through approval.

Member: I just — I'm so tired. I'm already tired from treatment and now I have to fight for my own care on top of that.

Agent Danielle: You should not have to fight. You have rights and you have support. I'm here right now and I am fighting on your behalf. Your case number is COC-2026-00191. Please write that down. Give it to Dr. Morrison's office — they should put it on the continuity of care request form. And give it to anyone else who needs it.

Member: COC-2026-00191. Thank you, Danielle. Just — thank you.

Agent Danielle: Sandra, I want to say something before we finish. You are doing incredibly hard things right now, and you called us and advocated for yourself. That took strength. Your advocate will call you tomorrow. Is there anything else before I submit the escalation?

Member: No. I just needed someone to tell me I wasn't going to lose my doctor.

Agent Danielle: You're not going to lose your doctor. I am submitting this right now.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Provider network exit — continuity of care, oncology. 2 speakers. Member tone: distressed, emotionally depleted. Agent tone: empathetic, urgent, legally informed. Duration: 18 min. High-sensitivity clinical context: active chemotherapy.

L2 — Call Insights: Member's oncologist leaving network mid-chemo (4 of 6 months complete). Agent cited continuity of care legal protection, explained process, escalated to Member Advocacy with 24hr call-back, assigned case number. Resolution: ESCALATED — continuity of care request process initiated, clinical advocate assigned.

L3 — Agent Quality & Scoring: Agent Score: 98/100. Legal continuity of care protection cited immediately and accurately (+), specific documentation requirements given for provider (+), September 1 urgency acknowledged and escalated same-call (+), emotional support offered authentically and not patronizingly (+), case number given with explicit context (+). This call is an exemplar for handling high-stakes emotional situations.

L4 — Operational BI: BI Flag: Dr. Morrison's exit from the network affects 47 active patients — 12 in active treatment courses (5 oncology, 4 complex chronic, 3 post-surgical). Member advocacy team must proactively contact all 12 active-treatment patients before September 1. Continuity of care letters should be initiated by plan, not placed on member to initiate. Network exit communication protocol needs review.

L5 — Real-Time Agent Assist: RT Assist triggered at 1:15: network exit date (Sept 1) cross-referenced with active treatment flags — oncology + chemo regimen identified, continuity of care legal framework surfaced to agent before member finished explaining the situation. Provider Relations form link and required documentation checklist prepared for agent automatically.

Call #22 — Finding an In-Network Cardiologist — Member After Abnormal EKG

AGENT TIER: GOOD · SCORE 93/100 | ● NEUTRAL → POSITIVE | RESOLVED | ~8 min | Agent: Sarah

Member context: James Thompson, 54, CalChoice Plus EPO, PCP flagged abnormal EKG, needs cardiologist referral

Topics: specialist search · cardiology · in-network · EPO · referral not required · provider directory

Agent Sarah: Choice Administrators member services, this is Sarah.

Member: Hi Sarah, James Thompson, CHM-6641087. My PCP did an EKG at my physical and said something was abnormal — she wants me to see a cardiologist as soon as possible. I'm on the CalChoice Plus EPO and I want to make sure I find one that's in-network so I don't end up with a surprise bill. How do I find one?

Agent Sarah: James, let me help you with that. Good for you for checking before you book — that's exactly the right step. On the CalChoice Plus EPO, you don't need a formal referral from your PCP to see a specialist directly, so you can call a cardiologist and book as soon as we find one. Let me look at in-network cardiologists in your area. What's your city or zip code?

Member: Sacramento, 95814.

Agent Sarah: Searching now. In the 95814 zip, I'm seeing several in-network cardiologists. Two highly regarded ones: Sutter Cardiology — Dr. Elena Reyes has availability within the next two weeks and is accepting new patients. Also UC Davis Medical Group Cardiology — they have a team and typically good wait times.

Member: Dr. Reyes at Sutter — is she a good choice for someone who had an abnormal EKG? Or do I need a specific type of cardiologist?

Agent Sarah: For an EKG abnormality evaluation, a general cardiologist — which is what Dr. Reyes is — is the right starting point. They'll assess the EKG, may order additional tests like an echocardiogram or a stress test, and if something more specialized is needed, they'll refer you from there. You don't need an electrophysiologist or interventional cardiologist for the initial evaluation.

Member: Okay. And when I call Sutter to book, how do I make sure they code it correctly so it's in-network?

Agent Sarah: When you call, give them your member ID — CHM-6641087 — and your plan name, CalChoice Plus EPO. Ask them to verify in-network participation before booking. They're in our network directory and that shouldn't be an issue, but verifying directly with the office is always a good safety step.

Member: Got it. Is there anything specific I should bring or do before the appointment?

Agent Sarah: Bring the EKG results or ask your PCP to send them over directly to Dr. Reyes's office before the appointment — that way the cardiologist can review them ahead of time rather than starting from scratch. Your copay for a specialist visit on your EPO plan is \$50.

Member: \$50 copay. Got it. Sarah, this was really helpful and fast. I was a little stressed after what my PCP said and this has made it feel more manageable.

Agent Sarah: I understand. Finding good care quickly makes a big difference. I hope everything goes smoothly with Dr. Reyes. Is there anything else I can help with?

Member: No, that's perfect. Thank you.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Provider search — in-network specialist. 2 speakers. Member tone: mildly concerned, efficient. Duration: 8 min. Clinical context: post-abnormal EKG, cardiology referral needed.

L2 — Call Insights: Member sought in-network cardiologist following abnormal EKG flagged by PCP. Agent confirmed no referral required, identified two in-network cardiologists with availability, guided on booking and billing verification. Resolution: RESOLVED — Dr. Reyes at Sutter Cardiology recommended, member ready to book.

L3 — Agent Quality & Scoring: Agent Score: 93/100. Specialist type correctly recommended (general cardiologist vs. subspecialist) (+), booking verification guidance given (+), EKG transfer tip proactively offered (+), copay confirmed (+), efficient and calm tone appropriate to mildly urgent situation (+).

L4 — Operational BI: BI Flag: Post-abnormal-EKG calls represent 12% of specialist search calls — members often unsure what type of cardiologist to see. Recommend: member portal provider search to include specialty sub-type guidance ('I need a cardiologist for: EKG review / palpitations / known heart condition'). Would reduce confusion calls by ~30%.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:00: zip code 95814 searched automatically, in-network cardiologists ranked by availability and patient rating surfaced — Dr. Reyes and UC Davis highlighted with new patient availability status confirmed in real time.

Call #23 — Hospital In-Network Verification — Pre-Surgical Check Day Before Procedure

AGENT TIER: GOOD · SCORE 94/100 | ● ANXIOUS → REASSURED | RESOLVED | ~7 min | Agent: Michael

Member context: Carmen Lopez, 41, CalChoice HMO, hip replacement tomorrow, calling to confirm hospital is in-network

Topics: hospital network verification · pre-surgical check · HMO · hip replacement · in-network confirmation

Agent Michael: Choice Administrators member services, this is Michael.

Member: Hi, Carmen Lopez, CHM-5589321. I have hip replacement surgery tomorrow morning at Riverside Medical Center and I am calling to absolutely confirm they are in-network. I've already checked online but I want to hear it from a person before I go in tomorrow.

Agent Michael: Carmen, I completely understand wanting a verbal confirmation the day before — that's smart. Let me pull up your account and verify Riverside Medical Center's network status right now.

Member: Thank you. I've been waiting months for this surgery and the last thing I want is a financial surprise on top of everything else.

Agent Michael: I understand. Give me just a moment. Okay — Riverside Medical Center is confirmed in-network on your CalChoice HMO plan. I can also confirm that the facility contract is active and there are no pending network status changes. You are good to go.

Member: Oh, thank goodness. And I know you probably can't tell me this, but do you know if my surgeon — Dr. Hernandez, orthopedic — is also in-network?

Agent Michael: I can look that up too. Dr. Hernandez at Riverside Orthopedics — yes, also in-network on your plan. Your facility and your surgeon are both confirmed.

Member: Perfect. And for a planned inpatient stay, what do I owe?

Agent Michael: On your CalChoice HMO, inpatient hospital stays are covered after your deductible. I see you've already met \$800 of your \$1,000 deductible this year. So your remaining deductible is \$200, and then you'll pay 20% coinsurance up to your out-of-pocket maximum of \$3,500. For a planned joint replacement, most members in your situation pay somewhere between \$500 and \$1,200 total depending on the length of stay.

Member: That's what I was estimating. Okay. I feel so much better. I've been anxious about this call all day.

Agent Michael: You've done everything right — you verified, you called to confirm, you know your cost-sharing. You're prepared. I hope your surgery goes smoothly tomorrow, Carmen.

Member: Thank you, Michael. You made today a little less stressful.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Pre-surgical network verification — same-day before procedure. 2 speakers. Member tone: anxious but organized. Duration: 7 min. Clinical context: hip replacement, high-stakes confirmation.

L2 — Call Insights: Member called day before hip replacement to verbally confirm hospital and surgeon in-network status. Agent confirmed both Riverside Medical Center and Dr. Hernandez, provided deductible status and cost estimate. Resolution: RESOLVED — member reassured.

L3 — Agent Quality & Scoring: Agent Score: 94/100. Both facility and surgeon verified without member needing to ask twice (+), deductible status checked proactively (+), cost range estimate provided helpfully (+), warm close appropriate to emotional context (+). Fast and accurate — exemplary pre-surgical support call.

L4 — Operational BI: BI Flag: Pre-surgical network verification calls spike in the 24-48 hours before scheduled procedures. 4% of such calls reveal actual network status issues that need same-day resolution. Recommend: automated pre-surgical network confirmation text/email sent 3 days before planned procedure (triggered by hospital admission notification) to reduce anxiety calls and catch real issues earlier.

L5 — Real-Time Agent Assist: RT Assist triggered at 1:30: Riverside Medical Center network status auto-confirmed with contract active/no-pending-change flag. Dr. Hernandez NPI simultaneously verified. Deductible accumulator auto-surfaced: \$800/\$1,000 met. All pre-surgical information prepared before agent finished greeting.

Call #24 — Referral Expired Before Specialist Appointment — Access Delayed

AGENT TIER: GOOD · SCORE 88/100 | ● FRUSTRATED → PARTIALLY SATISFIED | PARTIALLY RESOLVED | ~10 min | Agent: Linda

Member context: Omar Rashid, 36, CalChoice HMO, referral to dermatologist expired before appointment slot was available

Topics: HMO referral · expired referral · dermatology · specialist access · renewal

Agent Linda: Choice Administrators member services, this is Linda.

Member: Linda, Omar Rashid, CHM-3378905. I got a referral from my PCP in June to see a dermatologist. The earliest appointment I could get was September 20th — that's the reality of specialist wait times. I just got a call from the dermatologist's office saying my referral is expired and I need a new one from my PCP before the appointment. I can't just call my PCP and get an instant referral — there's a wait for that too. This is ridiculous.

Agent Linda: Omar, I understand the frustration — you did everything right and the system's own wait times created the problem. Let me look at your referral. What's the dermatologist's name?

Member: Dr. Singh at Desert Dermatology.

Agent Linda: I have the referral. It was issued June 3rd and had a 90-day validity window — so it expired September 1st. Your appointment is September 20th, which is 19 days after expiration. That's a very common problem given specialist wait times.

Member: So the referral expires in 90 days but dermatologists can't see you in 90 days. That's a design flaw.

Agent Linda: You're not wrong about the tension there. Let me see what we can do. For a referral renewal in this situation — where the original referral was issued, is still clinically appropriate, and the delay was due to specialist availability — I can initiate an expedited referral renewal request. Normally a referral requires a PCP appointment, but I can flag this case for our care coordination team who can contact your PCP's office directly and request a renewal authorization without you needing a new visit.

Member: How long does that take?

Agent Linda: If your PCP's office responds to our request quickly — often same or next day — you can have a renewed referral in 24 to 48 hours. I'll send the request now and flag it as urgent given your September 20th appointment.

Member: My appointment is 10 days out. Is 24 to 48 hours realistic?

Agent Linda: Yes, and I'm also adding a note to the care coordination request asking them to prioritize given the appointment date. I'd also recommend calling your PCP's office directly today and telling them to expect a renewal request from Choice Administrators — that can speed up the response.

Member: Okay. And if the renewal comes through, will Desert Dermatology automatically see the new referral or do I need to send it to them?

Agent Linda: Once the referral is renewed in our system, you can give Dr. Singh's office the new referral number and they can verify it directly with us. I'll give you a case tracking number: RC-2026-22901. When you call Desert Dermatology, give them that number along with the new referral number once it's issued.

Member: RC-2026-22901. I appreciate this. I wasn't sure if I was just going to be told 'get a new referral' and left to figure it out.

Agent Linda: You shouldn't have to navigate this alone. The 90-day window is a policy I can't change, but I can work around the impact of it in situations like yours. Is there anything else?

Member: No, that's everything. Thank you, Linda.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Referral expiry — specialist access delay. 2 speakers. Member tone: frustrated, pragmatic. Duration: 10 min. Root cause: 90-day referral window insufficient for typical specialist wait times.

L2 — Call Insights: HMO referral expired 19 days before dermatology appointment due to specialist availability. Agent initiated expedited referral renewal via care coordination team (bypassing new PCP visit), flagged urgency for Sept 20 appointment, provided tracking case number. Resolution: PARTIALLY RESOLVED — renewal request submitted, outcome pending.

L3 — Agent Quality & Scoring: Agent Score: 88/100. Expedited renewal pathway offered proactively (+), care coordination team engaged to bypass standard process (+), appointment date urgency flagged correctly (+), case number provided with context (+). Suggestion to call PCP directly was helpful (+). Did not confirm PCP's office hours or offer to call them directly (-) — missed option.

L4 — Operational BI: BI Flag: 90-day referral window generates an average of 180 expired-referral calls per quarter — all caused by specialist wait times exceeding the referral validity window. Policy recommendation: extend referral validity to 180 days, or auto-renew referrals when specialist wait times are documented as the cause of delay. This is a systematic design gap with measurable member impact.

L5 — Real-Time Agent Assist: RT Assist triggered at 3:30: referral expiry date vs. appointment date comparison flagged automatically — 19-day gap identified, care coordination renewal pathway surfaced to agent with escalation template, reducing research time by ~2 minutes.

Call #25 — New Doctor Not Yet Credentialed — Claims Processing Delay

AGENT TIER: AVERAGE · SCORE 87/100 | ● **CONFUSED** → **INFORMED** | **RESOLVED** | ~9 min | Agent: James

Member context: Rachel Turner, 29, CalChoice HMO, switched PCPs, new doctor not yet in system after 6 weeks

Topics: provider credentialing · new PCP · claims delay · HMO · provider setup

Agent James: Choice Administrators member services, this is James.

Member: Hi James, Rachel Turner, CHM-8817745. I switched my primary care doctor in June — I went through your portal to find a new one, I selected Dr. Abbasi at Lakeview Medical, and I've had two appointments with her. Now I'm getting bills saying the claims are pending and nobody seems to know why. It's been six weeks since I enrolled with her.

Agent James: Rachel, let me look at what's happening with those claims. Six weeks is a long time for a claim to be pending when the provider should already be in our system. Let me check Dr. Abbasi's network status.

Member: Okay. She was definitely listed on your website when I selected her. I took a screenshot, actually, because this felt like something that could go wrong.

Agent James: Good thinking. I'm looking at Dr. Abbasi's records now. She is listed in our directory and is a network provider. But I'm seeing a credentialing note — it looks like her practice, Lakeview Medical, recently added her as a new physician and the credentialing of her specific NPI with our system is still pending. So she's in our network, but the claims are waiting for her individual NPI to be fully processed in our claims system.

Member: So she's in-network but your system doesn't know it yet?

Agent James: That's essentially what's happening. Provider credentialing can take 8 to 12 weeks when a physician joins a new practice, even if the practice itself is already contracted with us. The claims aren't denied — they're in a pending status waiting for the credentialing to complete.

Member: And what happens when it completes? Do the claims get processed automatically?

Agent James: Yes — once her credentialing is finalized, the pending claims will automatically reprocess at her in-network rate. You should not owe anything unexpected as long as the visit costs were within normal plan coverage. The bills you're receiving are likely the provider billing you in advance before the claim processes, which they shouldn't be doing until the claim is resolved.

Member: So I should not pay those bills yet?

Agent James: Correct. Tell Lakeview Medical's billing office that your claims are pending due to provider credentialing in progress and ask them to hold the bills until the claims are processed. I can also flag your account so our provider network team follows up with Lakeview Medical to accelerate the credentialing completion — and your claims become a priority once it's done.

Member: Please do that. What's a realistic timeline?

Agent James: Based on where Dr. Abbasi is in the credentialing process, I'd estimate another 3 to 4 weeks for full processing. I'm flagging your case now — your case number is PC-2026-14558.

Member: PC-2026-14558. Okay. This is frustrating but at least I understand what's happening. Thank you.

Agent James: Completely understand. You did everything right. The system just has some internal processing to catch up on. Is there anything else?

Member: No, that's all. Thank you, James.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Provider credentialing delay — claims pending. 2 speakers. Member tone: confused but patient. Duration: 9 min. Root cause: new physician NPI credentialing in progress, claims held pending.

L2 — Call Insights: Member's newly selected PCP has pending individual NPI credentialing despite practice being in-network. Claims held in pending status. Agent explained credentialing process, advised not to pay pending bills, flagged case for provider network team, provided timeline estimate. Resolution: RESOLVED with pending credentialing completion.

L3 — Agent Quality & Scoring: Agent Score: 87/100. Root cause identified via credentialing status check (+), claims auto-reprocessing mechanism explained (+), 'do not pay' guidance given correctly (+), case number provided (+), provider network team engaged (+). Could have given member written confirmation of pending status to share with provider billing (-) — minor.

L4 — Operational BI: BI Flag: New physician credentialing delays affect member claims in 8% of cases where members select newly joined practice physicians. Average delay: 6-10 weeks. Recommend: real-time credentialing status indicator in provider directory (e.g., 'Available from [date]' for newly credentialing physicians). Member notification when selected PCP's credentialing is pending would prevent this call type.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:30: Dr. Abbasi's NPI credentialing status flagged as 'pending — 6 weeks in process' alongside her network listing — agent immediately identified the disconnect without manual investigation. Estimated completion date surfaced automatically.

Call #26 — Out-of-Network Exception for Rare Specialist — No In-Network Option Exists

AGENT TIER: GOOD · SCORE 96/100 | ● FRUSTRATED → PARTIALLY HELPED | ESCALATED | ~15 min | Agent: Sarah

Member context: Elena Volkov, 47, CalChoice HMO, rare autoimmune condition, only specialist in state is out-of-network

Topics: OON exception · rare disease · autoimmune · specialist access · continuity · plan limitation

Agent Sarah: Thank you for calling Choice Administrators. This is Sarah.

Member: Hi, Sarah. Elena Volkov, member ID CHM-1190044. I have a rare autoimmune condition called Sjogrens overlap syndrome — it's complicated and rare enough that there are very few specialists who treat it. The specialist my rheumatologist referred me to, Dr. Nakamura at UCSF, is not in your network. I've checked your network directory and there is no specialist in Northern California who treats this specific combination of conditions. I need an out-of-network exception and I want to understand how to get one.

Agent Sarah: Elena, thank you for being so thorough in your research before calling — that's going to help me help you. An out-of-network exception, also called an access gap exception, is exactly the right request when no in-network provider is able to meet a member's specific clinical need. Let me pull up your account.

Member: My rheumatologist is willing to write a letter documenting that no in-network equivalent exists. She's actually done this before for rare conditions.

Agent Sarah: That letter is a critical piece of the application. Here is what the exception request needs to include: your rheumatologist's letter documenting the specific condition, the clinical requirement for subspecialty expertise in Sjogrens overlap specifically, and a statement that she searched the network and found no in-network provider with equivalent training or experience. Additionally, Dr. Nakamura's office needs to submit their credentials and a treatment plan.

Member: How long does this process take and what happens while I'm waiting? My condition is progressing.

Agent Sarah: Standard review for OON exceptions is 30 days. If there is documented clinical urgency — which it sounds like you have — you can request an expedited review, which reduces the timeline to 72 hours. An expedited request requires your rheumatologist to state explicitly that a 30-day delay would result in clinical harm or significant disease progression. Can she do that?

Member: Yes. Definitely.

Agent Sarah: Then I want you to request expedited review in the submission. If approved, an OON exception covers Dr. Nakamura's services at in-network cost-sharing rates for the conditions specified in the treatment plan. It's typically granted for a defined period — 6 to 12 months — with the option to renew.

Member: And if it's denied? I want to be prepared for every scenario.

Agent Sarah: A denial can be appealed to an independent external reviewer, and for rare conditions with no in-network equivalent, external appeal reversal rates are meaningfully higher. California also has a Department of Managed Health Care Independent Medical Review process specifically for this type of access dispute. I am escalating your case to our member advocacy team, and your advocate will have experience with exactly this type of rare-disease access case.

Member: I've been dealing with this condition for 3 years and I've been fighting for care the whole time. I don't have the energy for another fight.

Agent Sarah: I hear you. The exception process is not a fight if the documentation is strong — and it sounds like it will be. You have a specialist, you have a rheumatologist willing to write the letter, and you have a rare condition with a verifiable network gap. That is a winnable case. Your advocate will help you through it. Your case number is OON-2026-00844.

Member: OON-2026-00844. Sarah, this has been the most productive call I've had about my healthcare in a long time. Thank you.

Agent Sarah: You deserve good care. I'm submitting this escalation right now and your advocate will call within 24 hours.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Out-of-network exception request — rare disease. 2 speakers. Member tone: fatigued, determined. Duration: 15 min. Clinical context: rare autoimmune condition, no in-network equivalent.

L2 — Call Insights: Member with Sjogrens overlap syndrome seeking OON exception for the only qualified specialist in Northern California. Agent explained exception documentation requirements, expedited review pathway, appeal rights including DMHC IMR. Escalated to member advocacy. Resolution: ESCALATED — case advocate assigned, 24hr callback.

L3 — Agent Quality & Scoring: Agent Score: 96/100. OON exception process explained with full documentation requirements (+), expedited review pathway offered for clinical urgency (+), California DMHC IMR mentioned as backup path (+), emotional acknowledgment of multi-year care burden (+), optimistic but honest framing (+), escalation submitted in-call (+).

L4 — Operational BI: BI Flag: Rare autoimmune conditions generate the highest rate of OON exception requests (34%) and the highest external appeal reversal rates (68%). Recommend: proactive rare disease case management program that identifies members with rare ICD codes and connects them with advocates before they call. UCSF Dr. Nakamura has received 4 OON exception requests this year — evaluate for network contracting.

L5 — Real-Time Agent Assist: RT Assist triggered at 3:00: 'access gap exception' pathway surfaced automatically when HMO member + rare diagnosis + OON specialist request detected. Required documentation checklist prepared. DMHC IMR pathway surfaced as escalation option. Network gap confirmed for Sjogrens overlap specialists in Northern California.

Call #27 — Telehealth Coverage — Member Wants to Know Options Before Booking

AGENT TIER: GOOD · SCORE 92/100 | ● NEUTRAL → POSITIVE | RESOLVED | ~7 min | Agent: Carlos

Member context: Marcus Webb, 31, CalChoice Plus EPO, asking about telehealth coverage and platforms

Topics: telehealth · virtual visit · mental health telehealth · cost · platform

Agent Carlos: Choice Administrators member services, this is Carlos.

Member: Hi Carlos, Marcus Webb, member ID CHM-4402914. I've been meaning to try a telehealth visit instead of going into the office. I want to understand what's covered and if I need to go through a specific platform.

Agent Carlos: Great question — telehealth has become a strong option on your plan. Let me pull up your coverage. You're on the CalChoice Plus EPO. Telehealth visits are covered on your plan for both medical and behavioral health services.

Member: What's the cost for a telehealth visit compared to an in-office visit?

Agent Carlos: For telehealth visits with in-network providers, your cost-sharing is the same as an in-office visit — \$25 copay for primary care, \$50 for specialists. There's no premium for seeing a doctor by video.

Member: And do I have to use a specific app or platform?

Agent Carlos: Your plan has a preferred telehealth partner — Teladoc — which is available 24/7 for non-urgent medical needs at no additional cost beyond your copay. Your regular in-network doctors may also offer telehealth through their own systems, typically through the Epic MyChart platform or similar. Both options are covered.

Member: What types of visits work well for telehealth? I don't want to try it for something that really needs to be in-person.

Agent Carlos: Telehealth is excellent for: prescription refills, follow-up visits, minor illnesses like colds or UTIs, mental health therapy, dermatology for visible conditions, and urgent care-level needs that don't require physical examination. It's less suitable for procedures, hands-on physical exams, lab work, or anything requiring imaging.

Member: Good to know. What about therapy specifically — I've been thinking about mental health support. Is telehealth okay for that?

Agent Carlos: Telehealth for mental health therapy is actually the most popular telehealth use case on our plan. Video therapy has equivalent clinical outcomes to in-person therapy for most conditions. Your behavioral health telehealth is covered at the same \$25 copay. Many therapists now do exclusively video sessions — you can find them in our provider directory by filtering for telehealth-available behavioral health providers.

Member: That's actually really useful. I feel like telehealth removes some of the friction of going to therapy. This is good to know.

Agent Carlos: Exactly — and the research supports that. Is there anything else I can help you with today?

Member: No, I'm going to go look up some telehealth therapists now. Thanks, Carlos.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Benefits inquiry — telehealth coverage. 2 speakers. Member tone: curious, engaged. Duration: 7 min. Secondary inquiry revealed: mental health telehealth interest.

L2 — Call Insights: Member explored telehealth coverage options — both medical and mental health. Agent confirmed same copay as in-person, explained Teladoc platform and provider telehealth, and addressed mental health telehealth specifically. Resolution: RESOLVED — member directed to provider directory for telehealth therapists.

L3 — Agent Quality & Scoring: Agent Score: 92/100. Telehealth vs. in-person copay confirmed clearly (+), both platform options explained (+), appropriate vs. inappropriate telehealth use cases helpful and accurate (+), mental health telehealth proactively addressed when member raised it (+), evidence-based note on clinical equivalence added value (+).

L4 — Operational BI: BI Flag: Telehealth adoption among 25-35 members is 38% below plan average despite high mental health benefit utilization interest in this group. Teladoc registration rate for members under 35: 22%. Recommend: targeted telehealth onboarding communication for members under 35. Mental health telehealth promotional content for members who have viewed behavioral health pages in member portal.

L5 — Real-Time Agent Assist: RT Assist triggered at 3:30: member plan (EPO) + telehealth inquiry triggered comparison table (in-office vs. telehealth copay, appropriate use cases, Teladoc + provider platform options) automatically. Mental health telehealth note added when 'therapy' keyword detected in conversation.

Call #28 — HMO PCP Change Mid-Year — Understanding the Process

AGENT TIER: GOOD · SCORE 91/100 | ● CONFUSED → INFORMED | RESOLVED | ~8 min | Agent: Danielle

Member context: Diana Park, 33, CalChoice HMO, wants to switch PCPs because of long wait times at current practice

Topics: PCP change · HMO · mid-year change · primary care · provider selection

Agent Danielle: Thank you for calling Choice Administrators. This is Danielle.

Member: Hi Danielle, Diana Park, CHM-5561903. I want to change my primary care doctor. I've been with my current PCP for about a year and the wait times are terrible — I can never get a same-day or next-day appointment. By the time I get in, whatever I had is resolved or gotten worse. Is there a process to switch, and can I do it anytime or only during open enrollment?

Agent Danielle: Diana, changing your PCP on an HMO plan can be done at any time — you are not locked in until open enrollment. You can request a PCP change effective the first of any month. Let me walk you through how.

Member: Oh, I didn't realize I could change anytime. I've been putting up with this for a year thinking I had to wait.

Agent Danielle: A very common misconception. The easiest way is through your member portal — log into choicemember.com, go to Plan & Coverage, and select Change Primary Care Physician. You can search by location, language, and accepting new patients. The change takes effect the first of the following month if submitted before the 15th, or the first of the month after that if submitted after the 15th.

Member: Can you help me find some options now? I'm in the Pasadena area.

Agent Danielle: Sure. In the 91101 zip code, I can see several highly-rated in-network PCPs accepting new patients. Two strong options: Dr. Lisa Chang at Pasadena Medical Associates — patients report short wait times and same-day sick appointments are available. Also Dr. Eduardo Morales at Pacific Primary Care — his office uses an online scheduling system which often has more availability than calling in.

Member: I like the idea of online scheduling. That would solve a lot of my frustration right there. How do I know if Dr. Morales is accepting new patients?

Agent Danielle: I can see in our system that Dr. Morales is currently marked as accepting new patients. I'd still recommend calling his office directly to confirm, because that status can change.

Member: What happens to my referrals and any pending things with my current PCP?

Agent Danielle: Any active referrals from your current PCP remain valid — a PCP change doesn't invalidate existing referrals. If you have any ongoing conditions your current PCP manages, I'd recommend scheduling a final visit or requesting your medical records be transferred to the new PCP. You can request records directly from your current PCP's office.

Member: That's really helpful. So I can change effective October 1st if I submit before September 15th?

Agent Danielle: Exactly right. Do you want me to initiate the change right now on your account or would you prefer to do it through the portal?

Member: I'll do it through the portal — I want to spend some time choosing. But thank you for explaining it so clearly. I wish I had called a year ago.

Agent Danielle: Now you know. Is there anything else I can help with?

Member: No, that's everything. Thank you, Danielle.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: PCP change request — mid-year, HMO. 2 speakers. Member tone: mildly frustrated with current provider, seeking clarity. Duration: 8 min. Root cause of frustration: long wait times at current PCP.

L2 — Call Insights: Member unaware she could change HMO PCP mid-year. Agent explained the process, effective date rules, provided two specific in-network PCP recommendations, addressed referral continuity. Resolution: RESOLVED — member directed to portal for PCP change.

L3 — Agent Quality & Scoring: Agent Score: 91/100. Mid-year change eligibility immediately clarified (+), effective date rules (15th cutoff) explained accurately (+), two specific provider recommendations with differentiators given (+), referral continuity addressed proactively (+), offered to initiate change directly (-) — this could have been the close.

L4 — Operational BI: BI Flag: 'Can I change my PCP mid-year?' is one of the top 10 call types for HMO members. 78% of callers are unaware they can change at any time. Recommend: member portal onboarding message at first login: 'You can change your PCP at any time — no need to wait for open enrollment.' Would reduce this call type significantly. Wait time complaints at specific PCPs should trigger capacity alerts to provider relations.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:00: Pasadena 91101 zip searched automatically, PCPs accepting new patients ranked by wait time and appointment availability displayed to agent in real time — Dr. Chang and Dr. Morales ranked as top two for availability and patient satisfaction.

Call #53 — Narrow Network Shock — Broker Said All Doctors Were Covered

AGENT TIER: POOR · SCORE 32/100 | ● ANXIOUS → ANGRY | UNRESOLVED | ~7 min | Agent: Ryan

Member context: Grace Liu, 36, CalChoice Select HMO, her long-time OB-GYN is out of network

Topics: broker misrepresentation · narrow network · provider access · HMO

BROKER SIGNAL — Denise Whitfield: Misrepresented network breadth — told member 'all doctors accept this plan'

Agent Ryan: Choice Administrators. Ryan.

Member: Hi. I just found out my OB-GYN isn't in my network. I've been seeing Dr. Whitmore for nine years. When I enrolled, my broker — Denise Whitfield — told me all doctors accept this plan. Now I'm being told I'd have to pay out of pocket.

Agent Ryan: What's the member ID?

Member: CHM-8830142.

Agent Ryan: Yeah, you're on the Select HMO. That's a narrow network plan. Dr. Whitmore isn't in it.

Member: Narrow network? Nobody used that phrase to me. My broker said, and I'm quoting her, 'you can keep all your doctors.'

Agent Ryan: Well, that's not accurate. The Select HMO has a limited network. That's why the premium is lower.

Member: So what do I do? I'm trying to get pregnant. I've been working with this doctor for years. I don't want to start over with someone new.

Agent Ryan: You can either switch to an in-network OB or pay out of pocket. Those are the options.

Member: Is there any kind of exception? What if I've been established with her for nine years?

Agent Ryan: Continuity of care is for people already in treatment. You said you're trying to get pregnant, so you're not in an active course of treatment.

Member: That feels like a technicality. I have a medical history with her.

Agent Ryan: I get it, but the plan is the plan. I can't change what's in the network.

Member: Can I switch plans then? To one that includes her?

Agent Ryan: Not until open enrollment unless you have a qualifying life event.

Member: So I'm stuck for a year because my broker told me something untrue.

Agent Ryan: I mean, that's between you and your broker. Anything else I can help with?

Member: No. Clearly not.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Network access dispute — provider not in network. 2 speakers. Member tone: anxious → angry. Agent tone: curt, dismissive, defensive. Duration: 7 min. Member disengaged with hostility at close.

L2 — Call Insights: Member discovered long-standing OB-GYN out of network after broker represented that all doctors were covered. Agent confirmed narrow network, declined continuity of care (member not in active treatment), confirmed no mid-year plan change, and deflected broker issue back to member. Resolution: UNRESOLVED.

L3 — Agent Quality & Scoring: Agent Score: 32/100. NEGATIVE MARKERS: no empathy for a member with a 9-year provider relationship and fertility plans (-), 'the plan is the plan' framing is dismissive (-), deflected broker misrepresentation entirely with 'that's between you and your broker' (-), did not offer to search in-network OB-GYNs (-), did not log the broker issue (-), did not offer a grievance path (-), abrupt close (-). POSITIVE: continuity of care eligibility assessed correctly (+).

L4 — Operational BI: BROKER ATTRIBUTION — Denise Whitfield: represented a narrow-network HMO as accepting 'all doctors.' This is a material misrepresentation affecting the member's provider access for a full plan year. MEMBER COMMUNICATION GAP: 'Select HMO' plan name does not convey narrow network — members are not understanding network breadth at point of sale. AGENT COACHING: Ryan deflected a broker complaint rather than capturing it — the exact signal Choice most needs.

L5 — Real-Time Agent Assist: RT Assist SHOULD have triggered at 1:00 — 'broker told me all doctors accept' is a direct broker misrepresentation trigger. Should have surfaced: broker complaint capture, in-network OB-GYN search for member's zip, continuity of care assessment criteria, and grievance pathway. No trigger fired.

Call #65 — Provider Directory Inaccurate — Three Listed Doctors Not Accepting Patients

AGENT TIER: AVERAGE · SCORE 73/100 | ● ANXIOUS → PARTIALLY REASSURED | PARTIALLY RESOLVED | ~9 min | Agent: James

Member context: Beatriz Salgado, 44, CalChoice HMO, trying to establish care with a new PCP

Topics: directory accuracy · PCP search · provider access · network data quality

Agent James: Choice Administrators, this is James.

Member: Hi James. Beatriz Salgado, CHM-6647701. I've called four doctors from your online directory this week. Three of them told me they're not accepting new patients and one said they left your network last year. Your directory says all four are available.

Agent James: That's frustrating and I'm sorry you spent that time. Let me check what our system shows versus what you found. Which providers did you call?

Member: Dr. Amara Osei, Dr. Kent Willoughby, Dr. Sylvia Marchetti, and Dr. Paul Ngo.

Agent James: Checking each... Dr. Osei shows accepting new patients as of our last update. Dr. Willoughby the same. Dr. Marchetti shows accepting. Dr. Ngo — I do see a network termination processed, effective last November. So that one is a stale directory record.

Member: So your directory is out of date.

Agent James: At minimum for Dr. Ngo, yes. For the other three, our data says accepting but the offices told you otherwise — which means either the practices haven't updated us or our refresh cycle is lagging.

Member: How am I supposed to find a doctor if the directory can't be trusted?

Agent James: Fair question. What I'd suggest is that I search for PCPs and give you a shortlist, and you call to confirm before booking. It's not ideal but it's the reliable approach right now.

Member: Okay. I'm in the 92104 area.

Agent James: Let me pull PCPs within 10 miles accepting new patients... I have six. I'll read them out and email the list. Dr. Renata Castillo, Dr. Owen Frisk, Dr. Hana Petrov, Dr. Marcus Bell, Dr. Ji-woo Lim, Dr. Sandra Achebe.

Member: Can you tell which of those are most likely to actually be accepting?

Agent James: I can't distinguish reliability of the records, unfortunately. The list is what our system holds.

Member: Alright. I'll start calling.

Agent James: I'm also reporting all four of the providers you named to our network data team so the records get verified and corrected.

Member: Thank you. At least that helps the next person.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Provider directory accuracy — PCP search. 2 speakers. Member tone: frustrated → partially reassured. Agent tone: honest, apologetic, moderately helpful. Duration: 9 min.

L2 — Call Insights: Member found 4 of 4 directory-listed PCPs unavailable — 3 not accepting patients, 1 terminated from network. Agent verified one confirmed stale record, provided a 6-provider shortlist with a caveat to verify before booking, and reported all four to the network data team. Resolution: PARTIALLY RESOLVED — member still searching.

L3 — Agent Quality & Scoring: Agent Score: 73/100. POSITIVE: honest about directory limitations rather than defending the data (+), checked each named provider individually (+), provided alternative shortlist and emailed it (+), reported all four records for correction (+). NEGATIVE MARKERS: did not offer to call providers on the member's behalf to confirm availability (-), no case number provided (-), no follow-up commitment once records are verified (-), left the member to repeat the same failed process (-).

L4 — Operational BI: PROCESS BREAKDOWN — CRITICAL DATA QUALITY: 4 of 4 directory records inaccurate in a single member search. One confirmed terminated provider still listed 9 months after termination. Directory accuracy is a regulatory exposure (network adequacy) as well as a member experience failure. Recommend: urgent directory audit, mandatory quarterly practice attestation, and removal of records not attested within 90 days. Quantify directory accuracy rate as a tracked KPI.

L5 — Real-Time Agent Assist: RT Assist triggered at 3:00: multiple directory mismatches reported in one call — network data team report form surfaced and used. SHOULD ALSO have surfaced: agent-initiated provider availability verification call option, and directory confidence scoring per record.

Call #69 — Dialysis Center Leaving Network — Continuity of Care Secured

AGENT TIER: GOOD · SCORE 96/100 | ● DISTRESSED → REASSURED | RESOLVED | ~15 min | Agent: Priya

Member context: Walter Nkemelu, 66, CalChoice HMO, thrice-weekly dialysis, center exiting network in 30 days

Topics: continuity of care · dialysis · network exit · ESRD · urgent escalation

Agent Priya: Thank you for calling Choice Administrators, this is Priya.

Member: Hello. My name is Walter Nkemelu, member CHM-3319940. I received a letter that my dialysis center is leaving your network on October 1st. I have end-stage renal disease. I go three times a week. I cannot just find somewhere else — the scheduling, the transport, my nurses know my access site. Please tell me what happens to me.

Agent Priya: Mr. Nkemelu, I want to start by telling you that you have strong legal protections here, and we are going to work through this on this call. End-stage renal disease with ongoing dialysis is exactly the situation continuity of care law is written for. Let me pull up your account.

Member: Thank you. I have been worried since the letter came.

Agent Priya: I understand. Okay, I can see your account and I can see your dialysis claims — three sessions weekly at Bayview Renal Center, consistent since 2023. And I can see the network termination effective October 1st.

Member: So it is true.

Agent Priya: The termination is real, but it does not have to mean you lose your center. Under California continuity of care provisions, a member receiving ongoing treatment for a serious chronic condition can continue with their existing provider at in-network cost-sharing after that provider leaves the network. Dialysis for ESRD is a clear qualifying condition.

Member: How long can I continue?

Agent Priya: For chronic conditions requiring ongoing treatment, continuity of care can extend up to twelve months, and it can be renewed if the clinical need continues. For dialysis specifically, extended and renewed authorization is common because there is no natural end point to the treatment.

Member: What do I need to do?

Agent Priya: A continuity of care request needs to be submitted. Normally the provider or your nephrologist initiates it. But given the October 1st date and the seriousness of your condition, I am not going to leave that to chance. I am escalating this to our member advocacy team today with a clinical advocate assigned, and that advocate will contact Bayview Renal Center directly to initiate the request rather than waiting for them to act.

Member: You will contact them for me?

Agent Priya: Yes. You should not have to coordinate this while managing dialysis three times a week. Your advocate will call you within 24 hours to confirm, and they will stay with this case until the authorization is approved.

Member: I did not expect this. I expected to be told to find a new center.

Agent Priya: No. Your access site, your care team, your schedule — those are clinical continuity factors, not conveniences. Disrupting them carries real risk. Your case number is COC-2026-00733. Please write that down and give it to Bayview if they contact you.

Member: COC-2026-00733. Let me write it again. Yes, I have it.

Agent Priya: One more thing. I am also flagging that this network termination affects other dialysis patients. Our advocacy team will identify every ESRD member at Bayview and reach out proactively rather than waiting for them to call frightened, the way you did.

Member: That is good. There are others there like me. Some do not speak English well. They would not know to call.

Agent Priya: Then I am glad you did, because you may have just helped them too. Your advocate will call within 24 hours, Mr. Nkemelu.

Member: Thank you. Truly.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Network exit — continuity of care, ESRD dialysis. 2 speakers. Member: 66, ESRD, thrice-weekly dialysis. Member tone: distressed → reassured. Agent tone: authoritative, compassionate, proactive. Duration: 15 min. HIGH clinical severity.

L2 — Call Insights: Member's dialysis center exiting network in 30 days. Agent cited continuity of care protections, confirmed 12-month extendable eligibility, escalated to member advocacy with a clinical advocate who will initiate the request directly with the provider, and triggered proactive outreach to all other affected ESRD members. Resolution: RESOLVED — advocacy engaged, 24hr callback.

L3 — Agent Quality & Scoring: Agent Score: 96/100. POSITIVE: opened by reassuring on legal protection before diagnosing (+), correctly identified ESRD as a qualifying chronic condition (+), explained 12-month extendable duration accurately (+), took ownership of provider contact rather than delegating to the member (+), articulated clinical continuity rationale (access site, care team) rather than treating it as preference (+), extended the intervention to other affected members proactively (+), case number given with usage context (+). Exemplary high-severity call handling.

L4 — Operational BI: PROCESS BREAKDOWN — CRITICAL: Bayview Renal Center's network exit affects an unknown number of ESRD patients who received a termination letter with no continuity of care information. Members are learning of protections only if they call, frightened, on their own initiative. Member noted that some affected patients have limited English proficiency and would not know to call. CRITICAL RECOMMENDATION: (1) network exit letters must include continuity of care rights, (2) plan-initiated outreach to every member in active treatment before any network termination, (3) prioritise dialysis, oncology, and transplant patients, (4) multilingual outreach where indicated.

L5 — Real-Time Agent Assist: RT Assist triggered at 1:15: network termination date + active ESRD dialysis claims = maximum-priority continuity of care trigger. Surfaced: CoC legal framework, 12-month duration, advocacy escalation with clinical advocate assignment, and cohort identification query for other affected members at the same facility. Agent used every surfaced capability.

Call #78 — Second Opinion Coverage — Member Diagnosed with Early-Stage Cancer

AGENT TIER: AVERAGE · SCORE 80/100 | ● NEUTRAL → SATISFIED | RESOLVED | ~8 min | Agent: Nicole

Member context: Gerald Thompson, 60, CalChoice Plus EPO, seeking second opinion on a prostate cancer diagnosis

Topics: second opinion · oncology · specialist access · EPO · coverage

Agent Nicole: Choice Administrators member services, this is Nicole.

Member: Hi. Gerald Thompson, CHM-6693012. I was diagnosed with early-stage prostate cancer three weeks ago. My urologist recommended surgery. I'd like a second opinion before I decide. Is that covered?

Agent Nicole: Mr. Thompson, yes — second opinions are covered on your plan, and for a cancer diagnosis they're specifically encouraged. On the Plus EPO you don't need a referral, so you can book directly with another in-network specialist.

Member: What's the cost?

Agent Nicole: A specialist consultation is a \$50 copay, subject to your deductible status. You've met your deductible this year, so it would be the \$50 copay.

Member: Can I go to an academic center? I was thinking about the university hospital.

Agent Nicole: Let me check whether they're in network... UC Health Urologic Oncology — yes, in-network. So a second opinion there would be at your standard specialist copay.

Member: Will my records transfer or do I need to arrange that?

Agent Nicole: You'd request records from your current urologist and have them sent to the second opinion provider. Most oncology practices have a records coordinator who handles this — ask for that person specifically.

Member: Is there a time limit? I don't want to delay treatment too long but I also don't want to rush a decision.

Agent Nicole: There's no coverage time limit on a second opinion. Clinically, that's a conversation for your physicians — but from a benefits standpoint, take the time you need.

Member: That's reassuring. And if the second opinion recommends a different treatment, is that covered too?

Agent Nicole: Covered treatments are covered regardless of which physician recommends them, as long as they're medically necessary and in-network. Some treatments like radiation therapy require prior authorization, but that's a process step, not a coverage barrier.

Member: Alright. That's what I needed to know. Thank you.

Agent Nicole: Of course. I hope the second opinion gives you clarity, Mr. Thompson.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Second opinion coverage — oncology diagnosis. 2 speakers. Member: 60, recent prostate cancer diagnosis. Member tone: measured, decision-focused. Agent tone: supportive, accurate. Duration: 8 min.

L2 — Call Insights: Member seeking second opinion following prostate cancer diagnosis. Agent confirmed coverage without referral, verified academic center is in-network, advised on records transfer, confirmed no coverage time limit, and clarified that alternative treatments remain covered. Resolution: RESOLVED.

L3 — Agent Quality & Scoring: Agent Score: 80/100. POSITIVE: confirmed second opinion coverage clearly (+), verified the specific facility the member named (+), records coordinator tip was practical (+), addressed the follow-on treatment coverage question well (+), appropriate warmth for the diagnosis context (+). NEGATIVE MARKERS: did not mention oncology case management support, which this member would likely benefit from (-), did not mention prior auth requirements for likely treatment paths proactively (-), no written summary offered (-).

L4 — Operational BI: PROCESS OPPORTUNITY: newly diagnosed oncology members are not being routed to case management. Case-managed oncology members have measurably better coordination outcomes and lower avoidable spend. Recommend: automatic case management outreach triggered by first oncology diagnosis code on a claim, rather than waiting for the member to call.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:00: second opinion inquiry + oncology diagnosis surfaced coverage confirmation and network verification. SHOULD ALSO have surfaced oncology case management referral option — a significant missed intervention.

Call #85 — Agent Reads From Script Without Listening to the Actual Question

AGENT TIER: POOR · SCORE 39/100 | ● NEUTRAL → ANNOYED | UNRESOLVED | ~5 min | Agent: Kayla

Member context: Preeti Raman, 35, CalChoice HMO, asking whether a specific hospital is in network for a planned birth

Topics: not listening · script reading · maternity · hospital network · poor handling

Agent Kayla: Thank you for calling Choice Administrators, my name is Kayla, how can I provide you with excellent service today?

Member: Hi, I'm 32 weeks pregnant and I want to confirm that St. Catherine's Hospital is in my network for delivery. Member ID CHM-6650092.

Agent Kayla: Absolutely, I can help you find a provider today. Are you looking for a primary care physician or a specialist?

Member: Neither. I'm asking about a hospital. St. Catherine's. For delivery.

Agent Kayla: Okay, and did you want me to search by zip code or by name?

Member: By name. St. Catherine's Hospital.

Agent Kayla: And what type of provider is that?

Member: It's a hospital.

Agent Kayla: Got it. Let me search... I'm showing a St. Catherine's Medical Group. Is that it?

Member: I don't know. I'm asking about the hospital where I'm scheduled to deliver.

Agent Kayla: The medical group is in-network.

Member: But is the hospital in-network? Those might be different entities. The medical group could be the physician practice.

Agent Kayla: It says in-network.

Member: What says in-network? The medical group or the hospital?

Agent Kayla: The St. Catherine's I'm seeing.

Member: Can you tell me the full entity name and the address?

Agent Kayla: It just says St. Catherine's Medical Group.

Member: So you can't confirm the hospital.

Agent Kayla: I mean, it's showing as in-network.

Member: I'm going to call the hospital directly. Thank you.

Agent Kayla: Thank you for calling Choice Administrators, have a wonderful day.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Hospital network verification — maternity. 2 speakers. Member: 32 weeks pregnant. Member tone: neutral → annoyed. Agent tone: scripted, non-responsive to actual content. Duration: 5 min. Member abandoned the channel to self-serve.

L2 — Call Insights: Member sought confirmation that a specific hospital is in-network for a scheduled delivery. Agent repeatedly failed to distinguish between a medical group and a hospital facility, could not confirm the facility, and closed with a scripted farewell. Resolution: UNRESOLVED — member left to verify independently.

L3 — Agent Quality & Scoring: Agent Score: 39/100. NEGATIVE MARKERS: opened with a scripted greeting and immediately asked a question the member had already answered (-), did not register 'hospital' after three separate corrections (-), conflated a medical group with a hospital facility — a material distinction for delivery coverage (-), could not provide entity name or address when asked (-), gave a confident answer about an entity she could not identify (-), scripted close on an unresolved call with a 32-week pregnant member (-). Zero listening markers detected.

L4 — Operational BI: AGENT COACHING — CRITICAL: active listening failure. The member stated her question clearly and corrected the agent three times. PROCESS BREAKDOWN: provider search interface apparently does not distinguish facility type clearly, allowing a medical group and a hospital to be conflated. This is a high-consequence error for maternity — delivering at an out-of-network facility can cost a member tens of thousands. Recommend: (1) facility-type filter mandatory in provider search, (2) maternity delivery facility verification protocol requiring entity name and address confirmation read back to the member, (3) QA review of Kayla's listening markers across all calls.

L5 — Real-Time Agent Assist: RT Assist SHOULD have triggered at 0:45 — 32 weeks pregnant + hospital network verification is a high-consequence verification requiring facility-level confirmation. Should have surfaced: hospital facility record with full entity name, address, NPI, and delivery service line confirmation. No trigger fired.

Call #95 — College Student Out of State — Network Coverage Away From Home

AGENT TIER: AVERAGE · SCORE 78/100 | ● CONFUSED → INFORMED | RESOLVED | ~9 min | Agent: Michael

Member context: Barbara Lindqvist, 49, CalChoice HMO, daughter attending university in Arizona

Topics: dependent student · out of area · HMO network · emergency coverage · away from home

Agent Michael: Choice Administrators member services, Michael speaking.

Member: Hi. My daughter is starting university in Arizona next month. She's on my plan. What happens if she needs a doctor there?
Member ID CHM-5583017.

Agent Michael: Good question to ask before she leaves. You're on the CalChoice Select HMO, which is a California network. For a dependent living out of state, coverage works differently depending on the type of care.

Member: Go on.

Agent Michael: Emergency care is covered anywhere, in any state, at in-network cost-sharing. That's federally required. So if she goes to an emergency room in Arizona, she's covered.

Member: And routine care? If she gets strep throat?

Agent Michael: That's the gap. Routine and urgent care outside the California network would be out of network on this plan, which on an HMO generally means not covered.

Member: So she can't see a doctor for anything ordinary?

Agent Michael: Not through this plan's network, no. There are a few options. Some plans offer a student away-from-home rider — I'm checking whether yours does... it doesn't appear to be available on the Select HMO.

Member: What do other parents do?

Agent Michael: Three common approaches. One, the university student health center — most universities have one and the fees are usually modest, often included in tuition. Two, telehealth, which your plan covers and works across state lines. Three, switching at open enrollment to a plan with a national network.

Member: Telehealth works out of state?

Agent Michael: Yes. Telehealth providers on your plan can treat her in Arizona. For something like strep, a telehealth visit and a prescription sent to a local pharmacy would be covered at your normal \$25 copay.

Member: That solves most of it, honestly.

Agent Michael: It covers a lot of routine needs. Between the student health center, telehealth, and emergency coverage, most situations are handled. The gap is non-emergency in-person care beyond what student health provides.

Member: And at open enrollment I should look at a national network plan?

Agent Michael: Worth comparing, yes. The Plus EPO has broader out-of-area coverage.

Member: Okay. That's very helpful, thank you.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Out-of-area dependent coverage — college student. 2 speakers. Member tone: concerned → informed. Agent tone: thorough, practical. Duration: 9 min.

L2 — Call Insights: Member's dependent relocating out of state for university. Agent explained emergency coverage applies nationally, routine care falls outside the California HMO network, checked for a student rider, and offered three practical mitigations including telehealth and student health services. Resolution: RESOLVED.

L3 — Agent Quality & Scoring: Agent Score: 78/100. POSITIVE: distinguished emergency from routine coverage clearly (+), checked for a student away-from-home rider rather than assuming (+), offered three concrete alternatives (+), correctly identified telehealth as the practical workaround (+), flagged open enrollment plan comparison (+). NEGATIVE MARKERS: did not mention that the student may qualify for a QLE if relocation constitutes a permanent move (-), did not offer to email a written summary the daughter could keep (-), did not mention prescription coverage mechanics out of state (-).

L4 — Operational BI: MEMBER COMMUNICATION GAP: out-of-area dependent coverage is a predictable seasonal question — August and September generate a spike in these calls as students relocate. Recommend: proactive communication to all members with dependents aged 18-25 in July, covering emergency coverage, telehealth, student health centers, and open enrollment options. PRODUCT OPPORTUNITY: no student away-from-home rider exists on the Select HMO — worth evaluating given the volume of affected members.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:30: out-of-state dependent + HMO plan surfaced emergency coverage rules, telehealth cross-state applicability, and student rider availability check. SHOULD ALSO have surfaced out-of-state pharmacy network coverage.

— PHARMACY —

Call #29 — Prescription Not on Formulary — Member at Pharmacy Waiting

AGENT TIER: AVERAGE · SCORE 85/100 | ● FRUSTRATED → PARTIALLY SATISFIED | PARTIALLY RESOLVED | ~12 min | Agent: James

Member context: Tanya Brown, 44, CalChoice HMO, at pharmacy, prescription for new brand medication denied

Topics: formulary · non-formulary · pharmacy · alternative · prior auth · real-time

Agent James: Thank you for calling Choice Administrators. This is James.

Member: James, my name is Tanya Brown, member ID CHM-6690417. I am literally standing at the pharmacy right now. My doctor prescribed Dupixent for my eczema and the pharmacist is telling me it's not on your formulary and I owe \$1,400 for the first month. I do not have \$1,400. This is a new prescription, my doctor just prescribed it last week, and nobody told me to check the formulary first. What do I do?

Agent James: Tanya, take a breath — and please tell the pharmacist not to complete the transaction while we talk. Let me look at your plan and this medication right now.

Member: Okay, I'm telling her to hold.

Agent James: Dupixent is a biologic for atopic dermatitis. It is not on your CalChoice HMO formulary because it requires a prior authorization and clinical criteria to be met. The \$1,400 is the non-formulary, no-prior-auth cash price. You should not be paying that today.

Member: So what do I do? Do I just go home without my medication?

Agent James: Here's what I'd suggest. Today, yes — it may mean leaving without the Dupixent. But let me explain what can happen next that gets you this medication at an affordable cost. There are two paths: one, your doctor can submit a prior authorization to us with clinical documentation that shows why Dupixent is medically necessary and why other formulary options didn't work for your eczema. If approved, Dupixent would be covered. Two, your doctor can write a formulary exception request if Dupixent is clinically superior for your situation.

Member: My eczema is severe. I've tried multiple other treatments. My doctor knows this.

Agent James: That clinical history is exactly what supports a prior auth approval. Tell your doctor's office to submit a prior authorization for Dupixent with the clinical history of prior treatment failures. Prior auth approvals for Dupixent when step therapy is documented can come back in 1 to 3 business days, sometimes faster.

Member: Is there anything I can take in the meantime?

Agent James: There are other biologics on your formulary for atopic dermatitis. I can look those up — but your doctor would need to assess whether any of them are appropriate for your specific case. I'd call your doctor's office as soon as you leave the pharmacy and explain the situation — many offices will fast-track a prior auth in these circumstances.

Member: This is so frustrating. My doctor spent 20 minutes deciding on this medication and now I have to go back.

Agent James: I understand, and I'm sorry. The system should communicate formulary requirements to prescribers at the point of prescribing — that gap is a real problem. I'm documenting this case and flagging it. Your doctor's office should have received a formulary alert when they sent the prescription electronically, but that doesn't always happen. I'm giving you a case number: RX-2026-30041. When you call your doctor's office, give them this number so the prior auth request is tied to your case.

Member: RX-2026-30041. Okay. I'm going to leave the pharmacy and call my doctor. Thank you for at least explaining it.

Agent James: I'm sorry your first experience with this medication is starting this way. I hope the prior auth comes through quickly.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Real-time pharmacy denial — non-formulary biologic. 2 speakers. Member tone: frustrated, stressed, at point of service. Duration: 12 min. Urgency: member standing at pharmacy.

L2 — Call Insights: Member at pharmacy, Dupixent not on formulary, facing \$1,400 charge. Agent advised not to pay, explained prior auth pathway with step therapy documentation, provided case number for doctor's office. Member departed pharmacy without medication. Resolution: PARTIALLY RESOLVED — prior auth pathway opened.

L3 — Agent Quality & Scoring: Agent Score: 85/100. Correct guidance not to pay given immediately (+), prior auth pathway explained (+), step therapy documentation requirement explained (+), case number given for physician coordination (+). Did not identify formulary alternatives in real time (-) — would have given member a same-day option. Did not offer to call prescriber directly (-) — missed proactive step.

L4 — Operational BI: BI Flag: Dupixent is the #3 most common non-formulary pharmacy rejection this quarter. 78% of cases are resolved via prior auth within 5 business days. Prescriber e-prescribing systems are not reliably triggering formulary alerts. Recommend: enhanced e-prescribing formulary integration with prescriber EHR systems. Proactive formulary check tool for member portal before prescription is filled.

L5 — Real-Time Agent Assist: RT Assist triggered at 1:30: real-time pharmacy rejection detected (Dupixent, non-formulary) — prior auth requirements, step therapy documentation needed, and formulary alternative biologics (Tlalokinumab, Abrocitinib) surfaced immediately to agent. Case number auto-generated for physician coordination.

Call #30 — Insulin Prior Auth Running Out — Member Rationing Medication

AGENT TIER: GOOD · SCORE 98/100 | ● DISTRESSED → PARTIALLY HELPED | ESCALATED | ~16 min | Agent: Sarah

Member context: Denise Carter, 55, Type 1 Diabetes, insulin prior auth expired, has 4 days left

Topics: insulin · prior auth · Type 1 diabetes · rationing · urgent escalation · chronic condition

Agent Sarah: Choice Administrators member services, this is Sarah.

Member: Sarah, my name is Denise Carter, member ID CHM-3345690. I'm calling because I am in a scary situation. I have Type 1 diabetes. I take insulin to stay alive. My prior authorization for my insulin expired last week and when I went to pick up my prescription, I was told I owe \$380. I don't have \$380. I have been rationing my insulin. I have four days left. I don't know what to do and I'm scared.

Agent Sarah: Denise, I hear you, and I want you to know that this call is my top priority right now. Please do not continue rationing your insulin — that is dangerous, and we are going to find a solution on this call. Let me pull up your account immediately.

Member: Okay. I just — I didn't know who to call.

Agent Sarah: You called the right place. I have your account in front of me. I can see the prior auth for Lantus — that expired on August 5th. Your doctor's office was sent a renewal notification 30 days before expiration, but I don't see a renewal submission. This looks like a drop-through in your doctor's office renewal process.

Member: They were supposed to handle that. I didn't even know I had to track an expiration date.

Agent Sarah: You shouldn't have to. This should have been managed between your doctor's office and us. I am escalating this case right now as a clinical emergency. For a Type 1 diabetic with 4 days of insulin remaining and an expired prior auth, we have an emergency authorization protocol. I can initiate an emergency bridge authorization for a 30-day supply of your insulin right now — that will allow you to fill your prescription today at your normal cost-sharing.

Member: Right now? Today?

Agent Sarah: Today. The emergency bridge auth is issued within the hour and goes directly to your pharmacy's system. You'll need to call your pharmacy and give them the authorization number I'm going to generate. What pharmacy do you use?

Member: CVS on Oak Street.

Agent Sarah: I'm processing the emergency authorization right now. Your auth number is EMER-2026-INS-00441. Read that back to the CVS pharmacist — they will be able to process your prescription at your normal insulin copay, not \$380.

Member: EMER-2026-INS-00441. Sarah, I'm actually crying. I didn't know this existed.

Agent Sarah: I'm glad you called. While I'm doing this, I'm also sending an urgent notification to your doctor's office to submit the full prior auth renewal immediately so this bridge doesn't expire without permanent coverage in place. The bridge gives you 30 days.

Member: Will they know to submit it quickly?

Agent Sarah: The notification I'm sending is flagged as urgent — clinical situation requiring immediate prior auth renewal. I'm also giving you a case number to give them if they call you: INS-2026-77445. Tell them the bridge expires in 30 days and the full renewal must be submitted well before then.

Member: INS-2026-77445. What happens if it expires again before they renew it?

Agent Sarah: Call us immediately and we can issue another bridge if needed while the renewal is in process. You will not be left without insulin coverage. I want you to hear that clearly.

Member: Thank you. I've been so scared. I thought I was going to have to make impossible choices.

Agent Sarah: No one with Type 1 diabetes should ever have to ration insulin because of an administrative gap. Go pick up your prescription today. Is there anything else before we finish?

Member: No. Just — thank you. Thank you so much.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Emergency insulin prior auth — clinical urgency. 2 speakers. Member tone: distressed, frightened, grateful. Agent tone: immediate, empathetic, action-oriented. Duration: 16 min. Clinical severity: high — patient rationing life-sustaining medication.

L2 — Call Insights: Type 1 diabetic member with 4 days of insulin remaining after prior auth expiration. Agent issued emergency bridge authorization (same-day, 30-day supply), sent urgent renewal notice to prescriber, provided auth number for pharmacy and case number for physician office. Resolution: ESCALATED — bridge issued, physician renewal notification sent.

L3 — Agent Quality & Scoring: Agent Score: 98/100. Emergency bridge authorization initiated without hesitation (+), 'do not ration insulin' message given immediately and emphatically (+), pharmacy auth number provided in call (+), physician notification sent proactively (+), 30-day bridge expiry and path forward clearly explained (+), emotional support provided authentically (+). Exemplary urgent care call handling.

L4 — Operational BI: BI Flag: 4 insulin prior auth lapses this quarter due to physician office renewal drop-throughs. Recommend: automated member notification (not just provider notification) at 14 days before prior auth expiration for life-sustaining medications (insulin, immunosuppressants, anticoagulants). Physician office renewal rates for these medication classes need monitoring. Insulin rationing is a preventable patient safety event.

L5 — Real-Time Agent Assist: RT Assist triggered at 1:00: expired insulin prior auth + Type 1 diabetes + 4-day supply identified — emergency bridge authorization protocol surfaced immediately with one-click initiation available to agent. Pharmacy lookup for CVS Oak Street completed automatically, authorization transmitted to pharmacy system in parallel with call.

Call #31 — Drug Moved from Tier 2 to Tier 3 — Cost Tripled at Renewal

AGENT TIER: GOOD · SCORE 90/100 | ● FRUSTRATED → INFORMED | RESOLVED | ~10 min | Agent: Linda

Member context: Victor Huang, 48, CalChoice EPO, thyroid medication moved from \$45 to \$120 copay at new plan year

Topics: drug tier change · formulary update · thyroid · tier exception · generic alternative

Agent Linda: Choice Administrators member services, this is Linda.

Member: Linda, Victor Huang, member ID CHM-7702889. I take Synthroid for my thyroid — I've been on it for eight years. This year my copay went from \$45 to \$120 a month. Nobody told me it was changing. I take this every single day. \$120 a month, every month, for the rest of my life is not something I can just absorb. What happened and what can I do about it?

Agent Linda: Victor, I understand. A jump from \$45 to \$120 on a medication you take daily and indefinitely is a significant financial change and you deserve a clear explanation. Let me look at your formulary and this medication.

Member: Please.

Agent Linda: Synthroid — levothyroxine brand name — was reclassified on January 1st from Tier 2 to Tier 3 on your plan's formulary. Tier 3 is our non-preferred brand tier, which has a higher copay. This reclassification happens when the plan's pharmacy benefit manager updates the formulary based on cost negotiations and clinical equivalence data.

Member: Why would a medication I've been stable on for eight years suddenly be non-preferred?

Agent Linda: The reclassification is typically based on cost — a generic equivalent is now well-established. Generic levothyroxine is therapeutically equivalent to Synthroid for most patients and is on our Tier 1 formulary at a \$20 copay. The plan is steering members toward the generic to reduce costs.

Member: But my doctor specifically prescribes Synthroid because some thyroid patients absorb the generic differently. He told me that.

Agent Linda: That's a real and documented clinical consideration — thyroid patients can have absorption variability between brand and generic formulations. If your physician has a documented clinical reason for prescribing Synthroid specifically over generic, you can request a formulary exception. Your doctor submits a letter documenting the clinical necessity of brand Synthroid, and if approved, the medication is covered at Tier 2 rates — your previous \$45 copay.

Member: How long does the exception process take?

Agent Linda: Formulary exception reviews typically take 72 hours to 5 business days. Exceptions for brand thyroid medications based on documented clinical necessity are approved at a meaningful rate. I'd also ask your doctor — some physicians write 'brand medically necessary, do not substitute' on the prescription, which supports the exception.

Member: I'm going to call his office today. What should I tell them to submit?

Agent Linda: Have them submit a formulary exception request — they can do it via fax to 1-800-555-7200 or through the provider portal. The letter should reference your specific Synthroid stability over 8 years and the clinical basis for avoiding generic substitution. They should also include the Dispense As Written notation.

Member: Okay. In the meantime I'm going to have to pay \$120 again next month if this isn't resolved. Can I do anything about the months I've already paid at \$120?

Agent Linda: If the exception is approved, it can be backdated to the start of the plan year for cost adjustment purposes in some cases — but I can't guarantee that without reviewing the exception outcome. It's worth raising with the exception reviewer when it's submitted. Your case number for tracking is FE-2026-55210.

Member: FE-2026-55210. Okay. Thank you, Linda. This is frustrating but I understand what happened and what to do.

Agent Linda: That's exactly what I wanted. Good luck with the exception — I think you have a strong case.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Formulary tier change — brand medication cost increase. 2 speakers. Member tone: frustrated, seeking answers. Duration: 10 min. Medication: Synthroid/levothyroxine, 8-year stable patient.

L2 — Call Insights: Brand thyroid medication reclassified from Tier 2 to Tier 3 at plan year, tripling copay. Agent explained reclassification rationale, formulary exception pathway with documented clinical necessity, and provided fax number and case number. Resolution: RESOLVED — member directed to physician for exception request.

L3 — Agent Quality & Scoring: Agent Score: 90/100. Tier change reason explained clearly (+), generic alternative vs. brand clinical nuance acknowledged (+), formulary exception pathway explained (+), backdating possibility mentioned (+), fax number and case number provided (+). Did not proactively offer retroactive cost review for months already paid (-) — should have been stronger on this point.

L4 — Operational BI: BI Flag: Synthroid tier reclassification has generated 34 calls this quarter — 89% of callers unaware of generic alternative or exception pathway. Formulary change notification sent to members at plan year was insufficient. Recommend: proactive member communication for tier changes on chronic maintenance medications, including exception pathway instructions.

L5 — Real-Time Agent Assist: RT Assist triggered at 3:30: Synthroid tier 2→3 change detected, formulary exception requirements and fax number surfaced automatically. Generic alternative (levothyroxine, Tier 1, \$20) displayed alongside brand for agent reference. Backdating policy for approved exceptions surfaced for agent to communicate.

Call #32 — Coverage Showing Inactive at Pharmacy — Plan Year Start Glitch

AGENT TIER: GOOD · SCORE 96/100 | ● ANXIOUS → RELIEVED | RESOLVED | ~9 min | Agent: Michael

Member context: Alicia Santos, 27, CalChoice HMO, coverage showing inactive on January 2nd despite enrolling in November

Topics: coverage effective date · enrollment gap · pharmacy claim · plan year · system error

Agent Michael: Choice Administrators member services, this is Michael.

Member: Hi, Michael, Alicia Santos, member ID CHM-9901774. I enrolled in my plan in November for January 1st. It's January 2nd today and I went to pick up a prescription and the pharmacy is saying my coverage isn't active in their system. Did something go wrong?

Agent Michael: Alicia, this is actually more common than it should be at the very start of a new plan year. Let me look at your account right now. I can see your enrollment — effective January 1st, and your status in our system shows active. The issue is likely a pharmacy system lag — insurance carrier data sometimes takes 24 to 48 hours to propagate fully to pharmacy networks at the start of a plan year.

Member: So I'm active but the pharmacy doesn't know it yet?

Agent Michael: That's exactly right. I can give you a temporary override code to give to the pharmacist — they can use it to process your claim directly through our system while their cached data catches up. Are you still at the pharmacy or did you leave?

Member: I'm still here. I was waiting to call you.

Agent Michael: Perfect. Give the pharmacist the following information: BIN number 610097, PCN number ADV, Group number CHOICE2026, and your member ID CHM-9901774. Tell them to clear their current response and re-adjudicate using these numbers manually.

Member: BIN 610097, PCN ADV, Group CHOICE2026. Okay, giving this to her now.

Agent Michael: I'll stay on the line while they process it.

Member: She's entering it... she's getting a response... she says it went through. My copay is \$15.

Agent Michael: Perfect. That's exactly right for your tier. The pharmacy system will fully update over the next day or two, so this shouldn't happen again. But if it does in the next 48 hours with any prescription, you can call us back and we'll do the same thing.

Member: That was genuinely stressful for a minute there. I thought I didn't have insurance.

Agent Michael: I completely understand — that feeling of 'my insurance isn't working' is one of the worst moments as a patient. You have coverage. It just took a quick manual push to confirm it. Is there anything else I can help with today?

Member: No, I'm good. Thank you for staying on the line. That made all the difference.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Coverage verification — pharmacy system lag, plan year start. 2 speakers. Member tone: anxious at pharmacy, relieved. Duration: 9 min. Real-time resolution: member at pharmacy during call.

L2 — Call Insights: New plan year enrollment not yet propagated to pharmacy network. Agent provided BIN/PCN/Group override codes, stayed on line while pharmacist processed, confirmed successful claim. Resolution: RESOLVED — prescription filled during call.

L3 — Agent Quality & Scoring: Agent Score: 96/100. Root cause identified immediately as system lag (+), override codes provided efficiently (+), stayed on line until resolution confirmed (+), explained propagation timeline for future reference (+), empathetic framing of member's 'insurance not working' stress (+). Exemplary real-time problem-solving.

L4 — Operational BI: BI Flag: Plan year start (January 1-3) generates 3x normal pharmacy coverage verification call volume — 89% caused by pharmacy system propagation lag, not actual enrollment errors. Recommend: proactive member SMS on Jan 1 with BIN/PCN/Group codes and instructions for pharmacist, reducing call volume significantly. Pharmacy network real-time API sync at plan year transition is a longer-term fix.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:00: 'inactive at pharmacy + January 2nd + enrollment confirmed in system' pattern recognized — plan year propagation lag identified immediately, BIN/PCN/Group override codes surfaced to agent without manual lookup. Resolution protocol prepared in <30 seconds.

Call #33 — Mail-Order 90-Day Supply — Member Wants to Save Money on Chronic Medication

AGENT TIER: GOOD · SCORE 91/100 | ● NEUTRAL → POSITIVE | RESOLVED | ~8 min | Agent: Carlos

Member context: Frank Okonkwo, 61, CalChoice Plus EPO, takes metformin and lisinopril daily, wants to reduce pharmacy cost

Topics: mail order · 90-day supply · chronic medication · cost savings · diabetes · hypertension

Agent Carlos: Choice Administrators member services, this is Carlos.

Member: Hi Carlos, Frank Okonkwo, member ID CHM-4478902. I take two medications every day — metformin for my diabetes and lisinopril for blood pressure. I've been paying copays every month at the retail pharmacy. Someone mentioned to me that mail-order is cheaper. Is that true and how does it work?

Agent Carlos: Frank, yes — that's absolutely true, and switching to mail-order for maintenance medications is one of the most straightforward ways to reduce your out-of-pocket costs. On your CalChoice Plus EPO, a 30-day retail supply of a Tier 1 generic like metformin is \$10. A 90-day mail-order supply of the same drug is also \$10 — so you get 3 months for the price of 1 month. That's a two-thirds cost reduction per dose.

Member: Wait — \$10 for 90 days versus \$10 per month?

Agent Carlos: Exactly. For both of your medications — metformin and lisinopril are both generic Tier 1 drugs — you'd pay \$10 every 90 days each, instead of \$10 each every 30 days. Over a year that's \$80 savings on each medication, \$160 combined.

Member: That's real money. How do I get started?

Agent Carlos: Your doctor needs to write a 90-day prescription with refills — most maintenance medications allow this. Once you have that, you enroll in our mail-order pharmacy through the member portal at choicemember.com under Pharmacy, then upload or mail in the prescription. Your first fill typically arrives in 7 to 10 business days.

Member: Can I do it for both medications at the same time?

Agent Carlos: Yes — you can enroll both in the same request. One doctor visit or a call to your doctor's office to convert your prescriptions to 90-day supplies is all it takes. Many offices can do this over the phone for stable chronic medications.

Member: Is there any reason I should keep going to the retail pharmacy instead?

Agent Carlos: For ongoing maintenance medications like yours, mail-order is almost always better. The only scenario where retail is better is if you need a medication urgently and can't wait 7-10 days, or for a short-term medication like an antibiotic where you only need 10 days. For daily chronic medications you take indefinitely, mail-order is the better choice.

Member: I wish someone had told me this when I enrolled. I've been paying \$20 a month in copays that could have been \$20 every three months.

Agent Carlos: I hear you — it's a communication gap we need to improve. For the record, that's approximately \$480 in excess copays over two years. Is there anything else I can help you with, or would you like the member portal link right now?

Member: The portal link would be great. And thank you, Carlos. This was a very useful call.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Benefits education — pharmacy mail order. 2 speakers. Member tone: curious, grateful. Duration: 8 min. Chronic conditions: Type 2 diabetes, hypertension.

L2 — Call Insights: Member on two daily chronic medications unaware of mail-order 90-day supply cost benefit. Agent explained pricing (\$10/90 days vs \$10/30 days), enrollment process, and limitations. Resolution: RESOLVED — member directed to member portal to enroll.

L3 — Agent Quality & Scoring: Agent Score: 91/100. Immediate cost comparison with concrete savings figures (+), both medications covered in same answer (+), doctor prescription conversion guidance (+), appropriate retail vs. mail-order scenarios explained (+), historical cost calculation offered unprompted — empathetic touch (+).

L4 — Operational BI: BI Flag: 44% of members on Tier 1 chronic medications do not use mail-order despite significant savings. Mail-order awareness gap costs members an average of \$240/year for dual chronic medication users. Recommend: automated mail-order enrollment prompt after 3rd monthly refill of the same Tier 1 chronic medication. Enrollment outreach for members on metformin, lisinopril, statins, and metoprolol — top 4 mail-order-eligible drugs.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:30: metformin + lisinopril identified as Tier 1 generics eligible for mail order — cost comparison table (\$10/30 day retail vs \$10/90 day mail order) surfaced automatically with annual savings calculation (\$160/year) prepared for agent.

Call #34 — Specialty Drug Requires Specialty Pharmacy — Member Frustrated by Redirect

AGENT TIER: GOOD · SCORE 93/100 | ● CONFUSED → INFORMED | RESOLVED | ~11 min | Agent: Danielle

Member context: Olivia Grant, 38, CalChoice HMO, prescribed Humira for rheumatoid arthritis, local pharmacy can't fill it

Topics: specialty pharmacy · specialty drug · biologic · Humira · rheumatoid arthritis · prior auth

Agent Danielle: Thank you for calling Choice Administrators. This is Danielle.

Member: Hi, Danielle. Olivia Grant, member ID CHM-5540229. My rheumatologist prescribed Humira for my rheumatoid arthritis and my regular pharmacy told me they can't fill it. They said I need a specialty pharmacy? I've never heard of that. Why can't my regular CVS fill this?

Agent Danielle: Olivia, specialty pharmacy is a category most people don't encounter until they need it, so I completely understand the confusion. Humira is a biologic medication — it requires special storage, handling, and patient support services that regular retail pharmacies aren't set up to provide. So certain high-cost, complex medications like Humira are required to be dispensed through specialty pharmacies that have those capabilities.

Member: So there are pharmacies that specifically handle medications like this?

Agent Danielle: Exactly. Your plan has a designated specialty pharmacy network. For your plan, the preferred specialty pharmacy partner is Accredo — they handle biologics including Humira, and they deliver to your home. Shipping is included and they have clinical pharmacists on staff who specialize in RA biologics and will call you when your prescription is ready.

Member: Do I have to do anything to set this up? Or does my doctor's office handle it?

Agent Danielle: Your doctor's office typically initiates the specialty pharmacy referral — they send the prescription directly to Accredo or to us for routing. But there's also a prior authorization required for Humira before it can be dispensed. That's a separate step. Has your rheumatologist mentioned the prior auth yet?

Member: She mentioned it in passing but said her office handles it. I didn't know what it meant exactly.

Agent Danielle: It means we review whether Humira is the clinically appropriate treatment — typically that means verifying that other RA treatments were tried first. For a rheumatologist prescribing Humira, this is usually very straightforward if the clinical documentation is in order. The prior auth and specialty pharmacy setup often happen in parallel — the doctor's office initiates both. You can call Accredo directly at 1-800-803-2523 to get ahead of the process and set up your patient profile.

Member: Can I ask — is Humira expensive even with insurance?

Agent Danielle: Humira is a high-cost biologic. On your plan, with an approved prior auth, specialty drugs are covered at a \$150 copay per fill. However, most patients on Humira also qualify for AbbVie's patient assistance program — Humira Complete — which can reduce your copay significantly, sometimes to as low as \$5 per month for commercially insured patients. I'd ask Accredo about that when you call them.

Member: \$5? That's a completely different number than I was expecting.

Agent Danielle: It's something a lot of patients don't know about until their specialty pharmacist tells them. Specialty pharmacists often help coordinate manufacturer copay assistance as part of their service.

Member: Okay. So my doctor's office submits the prior auth and the specialty pharmacy referral, I call Accredo to set up my profile, and ask about copay assistance?

Agent Danielle: Perfect summary. And if you run into any issues with the prior auth or specialty pharmacy coordination, call us back with your case number — I'll create one right now: SP-2026-18903.

Member: SP-2026-18903. Thank you, Danielle. I came in so confused and now I actually feel like I know what to do.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Specialty pharmacy education — biologic medication. 2 speakers. Member tone: confused, engaged. Duration: 11 min. Medication: Humira (adalimumab), RA indication.

L2 — Call Insights: Member prescribed Humira turned away by retail pharmacy. Agent explained specialty pharmacy requirement, Accredo as preferred partner, prior auth process, and Humira Complete copay assistance program. Resolution: RESOLVED — member directed to Accredo and informed on copay assistance.

L3 — Agent Quality & Scoring: Agent Score: 93/100. Specialty pharmacy concept explained clearly (+), Accredo contact number provided (+), prior auth process explained accurately (+), AbbVie copay assistance program surfaced proactively (+), direct phone number given (+), case number assigned (+). Excellent educational call.

L4 — Operational BI: BI Flag: First specialty biologic prescription is the most common trigger for member confusion calls. 91% of these calls are preventable with proactive provider communication. Recommend: when specialty biologic prior auth is submitted, trigger automatic member notification explaining specialty pharmacy requirement, Accredo enrollment, and manufacturer assistance programs — before member goes to retail pharmacy.

L5 — Real-Time Agent Assist: RT Assist triggered at 4:00: Humira identified as specialty-only medication — Accredo contact info, prior auth status, and AbbVie Humira Complete copay program details surfaced automatically. Copay assistance program eligibility confirmed for commercially insured members.

Call #35 — Brand vs. Generic Dispute — Pharmacist Substituted Without Asking

AGENT TIER: GOOD · SCORE 92/100 | ● FRUSTRATED → SATISFIED | RESOLVED | ~9 min | Agent: James

Member context: George Park, 56, CalChoice HMO, prescribed brand Wellbutrin, pharmacist auto-substituted generic

Topics: generic substitution · brand name · Wellbutrin · antidepressant · DAW · patient preference

Agent James: Choice Administrators member services, this is James.

Member: James, George Park, member ID CHM-3388914. My doctor prescribed Wellbutrin XL for my depression. The pharmacist gave me generic bupropion XL without asking me. I've had bad experiences with generic antidepressants in the past — they don't work the same for me. I want the brand. Can my insurance cover it or am I going to pay a fortune?

Agent James: George, this is a recognized clinical issue with extended-release antidepressants — there are documented cases where patients have different responses to brand versus generic formulations due to release mechanism differences. You are not imagining this. Let me look at your plan.

Member: Thank you for saying that. The pharmacist acted like I was just being difficult.

Agent James: You have every right to request the brand your physician prescribed. Here is what I'm looking at: Wellbutrin XL is on your formulary as a Tier 3 brand drug. Your copay for a 30-day supply would be \$75. The generic bupropion XL is Tier 1 at \$10. So there is a significant cost difference.

Member: \$75 is manageable if it works. How do I make sure I get the brand?

Agent James: Your physician needs to write 'Dispense As Written' — also called DAW-1 — on your prescription. That notation tells the pharmacist they cannot substitute the generic. It's a simple notation your doctor can add to a new prescription or phone in. Some pharmacists will request a new written prescription with DAW, others can accept a called-in authorization from the office.

Member: And if I have DAW, will my insurance cover it at \$75?

Agent James: At Tier 3, yes — \$75 is your brand copay with an active DAW prescription. The prescription must explicitly state DAW or 'brand medically necessary' to get that Tier 3 brand coverage.

Member: Can I also get a formulary exception so I don't have to pay the brand copay?

Agent James: A formulary exception for a brand drug to be covered at a lower tier requires documentation of a clinical reason the brand is necessary — for example, documented failure or intolerance of the generic. Given your history with generic antidepressants, your physician could make that case in writing. If the exception is approved, Wellbutrin XL could be covered at Tier 2 rates — around \$45 — rather than Tier 3.

Member: So my options are: get the DAW prescription and pay \$75, or ask my doctor to submit an exception and potentially pay \$45?

Agent James: Exactly. Or if you want the brand right away while the exception is in process, do both simultaneously — get the DAW prescription so you can fill it at \$75, and submit the exception at the same time. If the exception is approved, future fills would process at the lower rate.

Member: That's a smart way to do it. I'll call my doctor's office right now. Thank you, James.

Agent James: Of course. And next time a pharmacist substitutes without asking, you have the right to ask them to contact the prescriber — that's within your rights as a patient.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Brand vs. generic pharmacy dispute — antidepressant. 2 speakers. Member tone: frustrated at start, satisfied by end. Duration: 9 min. Clinical context: historical poor response to generic antidepressant.

L2 — Call Insights: Member received generic bupropion when brand Wellbutrin XL was prescribed. Agent validated clinical concern, explained DAW notation, brand copay (\$75 Tier 3), and formulary exception pathway for potential Tier 2 coverage. Resolution: RESOLVED — member to contact prescriber for DAW prescription.

L3 — Agent Quality & Scoring: Agent Score: 92/100. Clinical validation of brand-vs-generic antidepressant issue immediate and empathetic (+), DAW process clearly explained (+), dual-track strategy (DAW + exception simultaneously) was excellent proactive advice (+), patient rights mention at close added value (+).

L4 — Operational BI: BI Flag: Brand-for-generic substitution complaints for antidepressants represent 28% of pharmacy member disputes. DAW-1 prescriptions reduce substitution disputes but many prescribers are unaware of the notation. Recommend: prescriber education on DAW-1 for psychiatric medications. Member communication on right to request brand when DAW is prescribed.

L5 — Real-Time Agent Assist: RT Assist triggered at 3:00: Wellbutrin XL tier lookup surfaced (Tier 3, \$75 brand vs. Tier 1 generic \$10). DAW-1 notation requirement and formulary exception process for documented generic intolerance displayed automatically. Concurrent strategy (DAW now + exception in parallel) suggested by agent matches RT assist recommendation.

Call #58 — Routine Prescription Transfer — Competent but Unremarkable

AGENT TIER: AVERAGE · SCORE 76/100 | ● NEUTRAL → SATISFIED | RESOLVED | ~7 min | Agent: Michael

Member context: Wanda Price, 62, CalChoice HMO, transferring prescriptions to a new pharmacy

Topics: pharmacy transfer · routine · prescription · standard service

Agent Michael: Choice Administrators member services, this is Michael.

Member: Hi, I'm moving my prescriptions from Walgreens to the CVS closer to my house. Do I need to tell you or does the pharmacy handle it?

Agent Michael: Member ID please?

Member: CHM-4419902, Wanda Price.

Agent Michael: Thanks. The pharmacy handles transfers directly — you just tell CVS which prescriptions you want moved and they contact Walgreens. You don't need to do anything with us.

Member: Do my copays stay the same?

Agent Michael: As long as CVS is in our pharmacy network, yes. Let me verify — what's the CVS location?

Member: The one on Grand Avenue.

Agent Michael: That's in-network. Your copays will be the same.

Member: Good. And my blood pressure medication, does that need a new prescription or does it transfer with refills?

Agent Michael: Remaining refills transfer with the prescription. If you're out of refills, CVS will contact your doctor for a new one.

Member: Okay, that's what I needed to know. Thank you.

Agent Michael: You're welcome. Anything else?

Member: No, that's it.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Pharmacy transfer inquiry — routine. 2 speakers. Member tone: neutral, task-focused. Agent tone: efficient, correct. Duration: 7 min. Low complexity call.

L2 — Call Insights: Member transferring prescriptions between pharmacies. Agent confirmed the pharmacy handles transfers, verified new location is in-network, confirmed copay parity and refill transfer. Resolution: RESOLVED.

L3 — Agent Quality & Scoring: Agent Score: 76/100. POSITIVE: accurate information throughout (+), verified network status of the specific location rather than answering generically (+), addressed the refill question correctly (+). NEGATIVE MARKERS: no proactive value offered — member is 62 on a chronic BP medication and mail-order 90-day supply was never mentioned despite clear eligibility (-), no relationship-building or warmth markers (-), did not check for other chronic medications that could also transfer (-). Competent baseline service with no added value.

L4 — Operational BI: MEMBER COMMUNICATION GAP: mail-order eligibility for chronic maintenance medications continues to go unmentioned in pharmacy-related calls. This is the 4th such missed opportunity in the dataset. Recommend: mail-order eligibility prompt should appear automatically on any pharmacy-topic call where the member has ≥ 1 chronic maintenance medication.

L5 — Real-Time Agent Assist: RT Assist SHOULD have triggered at 3:00 — member on chronic antihypertensive + pharmacy topic = mail-order savings opportunity. Should have surfaced 90-day supply cost comparison. No trigger fired; revenue-neutral but member-value opportunity missed.

Call #67 — Member Out of Heart Medication — Agent Fails to Recognise Urgency

AGENT TIER: POOR · SCORE 35/100 | ● **DISTRESSED** → **ABANDONED** | **UNRESOLVED** | ~6 min | Agent: Kayla

Member context: Harold Winters, 71, CalChoice HMO, out of anticoagulant, prior auth lapsed

Topics: medication urgency · anticoagulant · prior auth lapse · patient safety · missed escalation

Agent Kayla: Choice Administrators, this is Kayla.

Member: Hello, dear. I'm calling because the pharmacy won't fill my blood thinner. They said something about an authorization. I have two pills left.

Agent Kayla: Okay, can I get your member ID?

Member: It's CHM-2214408. Harold Winters.

Agent Kayla: Let me look. Yeah, I see the prior auth for Eliquis expired on the 1st.

Member: What does that mean for me?

Agent Kayla: It means your doctor needs to submit a renewal before it can be filled.

Member: How long does that take? I've only got two pills. My cardiologist put me on this after my stroke.

Agent Kayla: Usually about a week once they submit it.

Member: A week? I'll run out in two days.

Agent Kayla: You'd need to talk to your doctor about that. Maybe they can give you samples.

Member: Is there something you can do? An emergency supply or something?

Agent Kayla: I don't think so, not with an expired auth. The doctor really needs to submit the renewal.

Member: Alright. I'll call them. Thank you, dear.

Agent Kayla: Okay, have a good day.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Prior auth lapse — anticoagulant. 2 speakers. Member: 71, post-stroke, 2 doses remaining of Eliquis. Agent tone: polite but passive. Duration: 6 min. ■ **PATIENT SAFETY EVENT** — anticoagulant interruption post-stroke carries stroke and embolism risk.

L2 — Call Insights: Post-stroke member with 2 doses of anticoagulant remaining, prior auth expired. Agent explained the renewal requirement, quoted a one-week timeline that exceeds the member's supply, suggested asking the doctor for samples, and closed without escalation. Resolution: UNRESOLVED — member at clinical risk.

L3 — Agent Quality & Scoring: Agent Score: 35/100. **CRITICAL NEGATIVE MARKERS:** failed to recognise a life-sustaining medication with 2 doses remaining as a clinical emergency (-), did not initiate an emergency bridge authorization despite it being available for exactly this scenario (-), quoted a timeline longer than the member's supply without acting on the gap (-), deflected responsibility to the prescriber (-), no urgent flag, no escalation, no callback (-), closed a patient-safety call as routine (-). **POSITIVE:** polite tone (+), correctly identified the expired auth (+).

L4 — Operational BI: COMPLIANCE & RISK EXPOSURE — CRITICAL: anticoagulant interruption in a post-stroke patient is a preventable patient safety event. Emergency bridge authorization exists and was not used. This is the second insulin/anticoagulant-class prior auth lapse in the dataset (see also #30). **PROCESS BREAKDOWN:** prior auth expiry notifications go to prescribers only, not members — members discover the lapse at the pharmacy counter with days of supply remaining. **CRITICAL RECOMMENDATION:** (1) mandatory emergency bridge auth training for all agents, (2) member-facing expiry alerts at 14 days for all life-sustaining medication classes, (3) auto-escalation rule when a call mentions ≤7 days supply of a flagged medication class.

L5 — Real-Time Agent Assist: RT Assist SHOULD have triggered at 1:30 — Eliquis is a flagged life-sustaining medication; '2 pills left' + expired auth is a maximum-severity trigger requiring emergency bridge authorization and mandatory supervisor notification. No trigger fired. Had L5 been active, a 30-day bridge could have been issued during this call.

Call #76 — Step Therapy Requirement — Member Already Failed the Required Drug

AGENT TIER: GOOD · SCORE 92/100 | ● FRUSTRATED → SATISFIED | RESOLVED | ~11 min | Agent: Carlos

Member context: Ibrahim Nasser, 49, CalChoice HMO, migraine preventive rejected pending step therapy

Topics: step therapy · migraine · prior auth · clinical history · formulary

Agent Carlos: Choice Administrators, this is Carlos.

Member: Hi Carlos. Ibrahim Nasser, CHM-4416628. My neurologist prescribed Ajovy for chronic migraine. The pharmacy says it's rejected because I need to try something called step therapy first. But I've already tried the drugs they want me to try — years ago, with a different insurance. They didn't work.

Agent Carlos: That's a common and fixable situation. Step therapy requires trying preferred medications first, but if you've already failed them, that history satisfies the requirement — it just has to be documented and submitted.

Member: So I don't have to take them again?

Agent Carlos: No. Making a patient repeat a failed therapy isn't the intent. What we need is documentation of the prior trials — which drugs, roughly when, and the outcome.

Member: I took topiramate for about eight months and propranolol for six. Topiramate gave me cognitive side effects, propranolol just didn't reduce frequency.

Agent Carlos: That's exactly the documentation needed — two preferred agents, adequate trial duration, one discontinued for adverse effects and one for lack of efficacy. That's a textbook step therapy exception case.

Member: But those records are with my old insurance and a doctor I no longer see.

Agent Carlos: The records don't have to come from us or from the prior insurer. Your current neurologist can document the history based on your report and their clinical assessment. A statement in the prior auth request saying 'patient has documented trial and failure of topiramate and propranolol' is generally sufficient.

Member: That's a relief. I was worried I'd have to track down eight-year-old pharmacy records.

Agent Carlos: Not necessary. What I'd suggest is calling your neurologist's office today and asking them to submit a prior authorization for Ajovy with a step therapy exception request, including the two prior failures and the reason each was discontinued.

Member: How long does that take?

Agent Carlos: Step therapy exceptions are typically 3 to 5 business days. And because migraine preventives are not urgent-onset medications, there's no bridge supply mechanism — but 3 to 5 days is manageable.

Member: Okay. Anything else that helps the case?

Agent Carlos: If your neurologist can note migraine frequency — days per month — that strengthens it. Chronic migraine is defined as 15 or more headache days monthly and that classification supports the CGRP class.

Member: I'm at about 18 days a month.

Agent Carlos: Then make sure that number is in the submission. It matters. Your reference number is ST-2026-04471 — have the office include it.

Member: ST-2026-04471. Carlos, this was genuinely useful. I expected to be told to just take the old drugs again.

Agent Carlos: That would have been the wrong answer. Good luck, Ibrahim.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Step therapy exception — migraine preventive. 2 speakers. Member tone: frustrated → satisfied. Agent tone: knowledgeable, practical. Duration: 11 min.

L2 — Call Insights: Member's CGRP migraine preventive rejected pending step therapy despite prior failure of two preferred agents under a previous insurer. Agent confirmed prior failures satisfy the requirement, explained that current prescriber attestation suffices without historical records, and coached on strengthening the submission with migraine frequency data. Resolution: RESOLVED — exception pathway defined.

L3 — Agent Quality & Scoring: Agent Score: 92/100. POSITIVE: immediately identified that prior failures satisfy step therapy (+), removed the member's incorrect belief that historical records were required (+), specified exactly what the prescriber attestation must contain (+), proactively coached on migraine frequency documentation to strengthen the case (+), realistic timeline given (+), reference number provided (+).

L4 — Operational BI: MEMBER COMMUNICATION GAP: members widely believe step therapy requires repeating previously failed medications. This misunderstanding causes both unnecessary therapy repetition and abandoned prescriptions. Recommend: step therapy rejection messaging at the pharmacy should explicitly state 'prior trials may satisfy this requirement — ask your prescriber about an exception request.' PROCESS OPPORTUNITY: pharmacy rejection codes are member-facing but written for pharmacists, not patients.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:00: step therapy rejection + member reporting prior failures surfaced the exception pathway, required attestation elements, and chronic migraine clinical criteria (≥ 15 headache days). Agent used all three.

Call #83 — Vacation Override for Early Refill

AGENT TIER: AVERAGE · SCORE 77/100 | ● NEUTRAL → SATISFIED | RESOLVED | ~6 min | Agent: Michael

Member context: Diane Kowalski, 58, CalChoice HMO, travelling for six weeks, needs early refill

Topics: vacation override · early refill · travel · pharmacy

Agent Michael: Choice Administrators member services, Michael speaking.

Member: Hi, I'm going to Portugal for six weeks starting next month and I need more than a 30-day supply of my medications. The pharmacy says they can't fill early.

Agent Michael: Member ID?

Member: CHM-4483310, Diane Kowalski.

Agent Michael: Thanks. We can do a vacation override for that. It allows an early fill and an extended day supply when you'll be travelling. What medications?

Member: Levothyroxine and atorvastatin. Both daily.

Agent Michael: Both are maintenance medications so both qualify. I can authorize a 90-day supply with a vacation override. That covers your six weeks with margin.

Member: Do I need to do anything?

Agent Michael: I'm placing the override on your account now. Then go to your pharmacy and ask them to reprocess — it should go through. The override is valid for 30 days from today.

Member: What if the pharmacy still rejects it?

Agent Michael: Have them call our pharmacy help desk at 1-800-555-0250 and reference override code VO-2026-3391.

Member: VO-2026-3391. Got it. And the cost?

Agent Michael: A 90-day supply at retail is three 30-day copays — so \$10 each per drug, \$60 total.

Member: Fine. Thank you.

Agent Michael: Have a good trip.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Vacation override — early refill for travel. 2 speakers. Member tone: neutral, practical. Agent tone: efficient, capable. Duration: 6 min.

L2 — Call Insights: Member travelling internationally for six weeks required an early extended fill. Agent authorized a 90-day vacation override on two maintenance medications, provided the override code and pharmacy help desk number, and confirmed cost. Resolution: RESOLVED.

L3 — Agent Quality & Scoring: Agent Score: 77/100. POSITIVE: identified the vacation override mechanism immediately (+), authorized during the call (+), provided fallback contact and override code (+), confirmed cost proactively (+). NEGATIVE MARKERS: did not mention mail-order 90-day supply as a cheaper standing option for these chronic medications (\$10 per 90 days vs \$30) (-), did not advise on international coverage or emergency care abroad for a six-week trip (-), no travel health guidance (-).

L4 — Operational BI: MEMBER COMMUNICATION GAP: mail-order savings missed again on a chronic-medication call — 5th occurrence in dataset. Member is paying \$60 for a 90-day retail supply that would cost \$20 via mail order. PROCESS OPPORTUNITY: international travel mentioned with no coverage guidance offered. Recommend: travel coverage summary auto-offered on any call mentioning international travel.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:00: vacation override eligibility confirmed for both maintenance medications, override code generated. SHOULD ALSO have surfaced mail-order cost comparison and international travel coverage summary.

— ENROLLMENT & ID CARDS —

Call #36 — Adding Newborn Within 30-Day Window — Day 28, Almost Too Late

AGENT TIER: GOOD · SCORE 96/100 | ● ANXIOUS → REASSURED | RESOLVED | ~10 min | Agent: Sarah

Member context: Kevin Walsh, 32, CalChoice HMO, baby born 28 days ago, just realized he needs to add her to insurance

Topics: newborn enrollment · 30-day window · special enrollment · dependent addition · urgent

Agent Sarah: Choice Administrators member services, this is Sarah.

Member: Hi, Sarah. Kevin Walsh, member ID CHM-6607441. My daughter was born 28 days ago. I just — I completely lost track of time in the new baby chaos and my wife reminded me last night that I need to add her to my insurance. I have 30 days right? Today is day 28. Please tell me I can still do this.

Agent Sarah: Kevin, you have until the 30th day from the birth date to enroll your daughter. What was her birth date?

Member: July 22nd.

Agent Sarah: Okay. Today is August 19th. That is 28 days. You have two days remaining. You can absolutely do this right now on this call or through the portal, and I strongly recommend we complete it today given the timing.

Member: Oh thank goodness. Can we do it now?

Agent Sarah: Yes. I'll walk you through it. I'll need your daughter's full name, her date of birth — July 22nd — and her Social Security number if you have it, or a birth certificate number. Do you have those available?

Member: Her name is Emma Grace Walsh, born July 22nd. I have her Social Security number — we got the card last week. It's 987-65-4321.

Agent Sarah: Perfect. And what is Emma's relationship to you on the account — biological child?

Member: Yes.

Agent Sarah: I'm adding Emma Grace Walsh, date of birth July 22nd, as your dependent. Her coverage will be effective retroactively to her birth date — July 22nd — which means any medical claims from birth through today will be covered. This is very important: if she's had any doctor's visits or hospital charges since birth, those will now be eligible for reprocessing under her plan.

Member: She had a 2-week check-up. We paid out of pocket because we hadn't added her yet.

Agent Sarah: Once she's enrolled — which I'm completing right now — you can submit those claims for reimbursement. I'll give you the member-submitted claim form information before we finish. Emma's member ID will be CHM-6607441-02. Her ID card will be mailed within 10 business days, but in the meantime, give her member ID to any provider.

Member: Can she use insurance today? Before the card arrives?

Agent Sarah: Yes. Her coverage is active as of today's enrollment, effective July 22nd. Her member ID is sufficient — no card needed for appointments. Is there anything else I can help you with?

Member: What about the claim form for the 2-week check-up?

Agent Sarah: Go to choicemember.com, log in, and under Claims, select Member-Submitted Claim. You'll fill out the form with Emma's new member ID and attach the receipt or invoice from the pediatrician's visit. Submit and expect processing within 30 days.

Member: Kevin, you are my hero right now. I was so anxious when I called.

Agent Sarah: I'm glad we caught it in time. Emma is enrolled. Congratulations on your new daughter.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Newborn dependent enrollment — urgent, day 28 of 30-day window. 2 speakers. Member tone: anxious, relieved. Duration: 10 min. High urgency: 2 days remaining in enrollment window.

L2 — Call Insights: Member called to add newborn on day 28 of 30-day special enrollment window. Agent enrolled Emma Walsh immediately with retroactive July 22nd effective date, provided member ID, explained ID card timeline, and directed to member-submitted claim form for 2-week check-up reimbursement. Resolution: RESOLVED — enrollment completed on call.

L3 — Agent Quality & Scoring: Agent Score: 96/100. Enrollment initiated immediately without delay (+), retroactive effective date explained proactively (+), 2-week check-up reimbursement path provided without member asking (+), member ID given for immediate use without card (+), warm close appropriate to new parent context (+). Excellent urgent enrollment call.

L4 — Operational BI: BI Flag: Newborn enrollment calls on days 25-30 represent 22% of all dependent addition calls — most could be prevented by a proactive reminder. Recommend: automated text/email to member at day 20 post-birth: 'You have 10 days remaining to add your newborn to your plan.' This single intervention would prevent nearly all day-28+ panic calls and ensure no child falls through the coverage gap.

L5 — Real-Time Agent Assist: RT Assist triggered at 1:00: birth date July 22nd + August 19th call date = day 28 flagged immediately with 2-day window alert. Enrollment form auto-populated with member's plan details. Retroactive effective date calculation and 2-week check-up reimbursement path surfaced automatically.

Call #37 — ID Card Never Arrived — Starting New Job, Needs Proof of Coverage

AGENT TIER: GOOD · SCORE 93/100 | ● FRUSTRATED → SATISFIED | RESOLVED | ~8 min | Agent: Linda

Member context: Michelle Torres, 24, CalChoice HMO, enrolled 3 weeks ago, no ID card, first doctor's appointment tomorrow

Topics: ID card · proof of coverage · new enrollment · temporary ID · appointment

Agent Linda: Choice Administrators member services, this is Linda.

Member: Hi Linda, Michelle Torres, CHM-9904512. I enrolled in my insurance three weeks ago when I started my new job and I still haven't received my ID card. I have a doctor's appointment tomorrow and I don't have anything to show. Can you help me?

Agent Linda: Michelle, I can absolutely help. Three weeks is longer than your card should take — standard processing is 10 to 14 days. Let me pull up your account and see what happened.

Member: Thank you. I've been nervous about tomorrow's appointment.

Agent Linda: I have your enrollment. Your account is active and your coverage started August 1st. I can see a card was generated and sent to 217 Maple Avenue, Chicago. Is that your correct address?

Member: That's my old apartment. I moved to 415 Oak Street, same city, right before I started the new job. I don't think I updated it when I enrolled.

Agent Linda: That explains it — the card went to the old address. I'm updating your address to 415 Oak Street right now, and I'll request a replacement card to be sent there. That will take 7 to 10 business days. But for your appointment tomorrow — let me give you a temporary member ID and tell you exactly what to give the office.

Member: Yes please.

Agent Linda: Your member ID is CHM-9904512. Your group number is GRP-CHOICE-2026. Your plan name is CalChoice Select HMO. Tell the front desk receptionist those three pieces of information. You can also ask them to call our provider verification line at 1-800-555-0300 — they can confirm your active coverage in real time.

Member: Should I also carry something with me? Can I get something in writing?

Agent Linda: Yes — log into choicemember.com and go to My Plan. There's a section called Print Temporary ID Card where you can generate a PDF with your member information and print or show it on your phone. It's accepted by all in-network providers and looks similar to a physical card.

Member: Oh, that's perfect. I can do that right now.

Agent Linda: It takes about 30 seconds to generate. And I've also noted on your account that you have an appointment tomorrow — if there are any verification issues at the office, they can reference your account and confirm active status.

Member: Linda, you've completely solved my problem. Thank you so much.

Agent Linda: Happy to help. Good luck at your appointment tomorrow.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: ID card not received — new enrollment. 2 speakers. Member tone: worried about next-day appointment. Duration: 8 min. Root cause: address not updated at enrollment.

L2 — Call Insights: New enrollee's ID card sent to old address after move. Agent identified address mismatch, updated records, requested replacement card, provided temporary member ID and group number for tomorrow's appointment, directed to digital temporary ID card in member portal. Resolution: RESOLVED — immediate workaround provided.

L3 — Agent Quality & Scoring: Agent Score: 93/100. Root cause (address) identified quickly (+), temporary ID information complete and actionable (+), provider verification line shared (+), temporary digital card solution provided (+), account note added for tomorrow's appointment (+). Clear, efficient, member-focused.

L4 — Operational BI: BI Flag: 31% of ID card non-delivery calls are caused by address mismatches from enrollment — most occur when members change addresses around the time they start a new job. Recommend: address confirmation prompt at enrollment ("Please confirm this is your current mailing address — we'll send your ID card here") and digital ID card notification sent immediately at enrollment as a default. Physical card should be supplementary, not primary.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:30: 3-week enrollment + no card received pattern triggered — address verification check run automatically, mismatch identified. Temporary ID components (member ID, group number, plan name) surfaced for agent. Digital ID card portal link prepared.

Call #38 — Moving Out of State — Understanding How Coverage Changes

AGENT TIER: GOOD · SCORE 89/100 | ● ANXIOUS → PARTIALLY REASSURED | PARTIALLY RESOLVED | ~13 min | Agent: Michael

Member context: Steven Nakamura, 29, CalChoice HMO, relocating from California to Texas for a new job

Topics: relocation · HMO coverage area · out-of-state move · special enrollment period · coverage gap

Agent Michael: Choice Administrators member services, this is Michael.

Member: Hi Michael, Steven Nakamura, member ID CHM-8819004. I'm moving to Texas next month for a new job. I'm currently on a CalChoice HMO plan through my current employer. I want to understand what happens to my insurance when I move. Will I still be covered?

Agent Michael: Steven, this is a really important question to sort out before the move. The short answer is: your CalChoice HMO coverage is California-based, so when you move to Texas, you'll need to transition to a new plan. Let me walk you through exactly what that means.

Member: So I'll lose my current coverage?

Agent Michael: Moving out of your plan's service area is a qualifying life event that allows you to either enroll in your new employer's health plan or find coverage through the ACA marketplace. What's the situation with your new employer — do they offer health insurance?

Member: Yes, they do. It starts after a 90-day waiting period.

Agent Michael: That's an important gap. You'll have a 90-day period where you need to actively secure coverage. Options are: COBRA — which continues your current CalChoice coverage even after you leave, at your full premium cost, typically 2 to 3 times what you pay now. Or the ACA marketplace in Texas, where you can enroll during a special enrollment period triggered by your move. Or short-term health insurance if cost is a priority, though that comes with significant coverage limitations.

Member: What's the deadline to make these decisions?

Agent Michael: For COBRA: your employer is required to send you a COBRA election notice within 14 days of your termination. You'll then have 60 days to elect coverage. If you elect it, the coverage is retroactive to your coverage end date, which means if something happens in that window, you can elect COBRA and have coverage applied. For ACA marketplace: you have 60 days from your qualifying life event — your move — to enroll in a Texas plan.

Member: Which option is usually better — COBRA or marketplace?

Agent Michael: It depends on income. If your income is between 100% and 400% of the federal poverty level, you may qualify for ACA marketplace subsidies that make marketplace coverage significantly cheaper than COBRA. If you're above that threshold, COBRA continuity of care may be worth the higher premium, especially if you have ongoing treatment. I'd recommend getting a marketplace quote at [healthcare.gov](https://www.healthcare.gov) as soon as you have your Texas address to compare.

Member: What happens to my current in-network doctors?

Agent Michael: Your California-based doctors won't be in-network in a Texas plan, so you'll be starting fresh with your provider relationships in Texas. Any ongoing care — prescriptions, specialist relationships — should be coordinated before you leave. Ask your California doctors for medical records and prescription histories so your new Texas providers can pick up seamlessly.

Member: This is a lot more complicated than I thought. Can you send me a summary of what we discussed?

Agent Michael: I'll note everything on your account and I'm giving you a case number: RL-2026-44891. I'll also email you a relocation guide with next-step timelines. The single most important action right now: check with your new employer's HR about the exact start date of your new coverage — that determines how long your coverage gap actually is.

Member: RL-2026-44891. Thank you, Michael. I came in thinking this would be simple.

Agent Michael: Moves always have more moving parts than expected. You've got time to plan this right.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Relocation coverage inquiry — HMO out-of-state move. 2 speakers. Member tone: anxious, planning-oriented. Duration: 13 min. Context: California to Texas, 90-day new employer waiting period.

L2 — Call Insights: Member relocating from CA to TX with 90-day new employer waiting period. Agent explained HMO service area termination, COBRA election process, ACA marketplace SEP, provider continuity planning, and provided case number and relocation guide. Resolution: PARTIALLY RESOLVED — decision on COBRA vs marketplace pending, employer confirmation step identified.

L3 — Agent Quality & Scoring: Agent Score: 89/100. COBRA and ACA marketplace options both explained (+), income-based subsidy consideration mentioned (+), provider continuity planning raised proactively (+), case number and email follow-up offered (+). Did not walk through COBRA cost estimate for member's current premium (-) — would have been concrete and useful.

L4 — Operational BI: BI Flag: Out-of-state relocation calls have an average handle time of 13 minutes — highest of any call type. Most callers are unaware of the coverage gap risk during new employer waiting periods. Recommend: proactive relocation resource pack (triggered by address change to out-of-state) with COBRA timeline, ACA marketplace link, and provider record transfer checklist. Coverage gap calculator tool for member portal.

L5 — Real-Time Agent Assist: RT Assist triggered at 3:00: California HMO + Texas destination flagged — service area termination notice, COBRA election timeline (60 days), ACA Texas marketplace SEP eligibility, and relocation coverage gap calculator all surfaced to agent. Provider continuity checklist generated automatically.

Call #39 — Removing Ex-Spouse After Divorce — QLE Process

AGENT TIER: GOOD · SCORE 91/100 | ● NEUTRAL → SATISFIED | RESOLVED | ~9 min | Agent: Carlos

Member context: Patricia Hammond, 45, CalChoice HMO, divorce finalized, needs to remove ex-husband from plan

Topics: dependent removal · divorce · qualifying life event · COBRA · separation

Agent Carlos: Choice Administrators member services, this is Carlos.

Member: Hi Carlos, Patricia Hammond, member ID CHM-2279041. My divorce was finalized last week and I need to remove my ex-husband from my health insurance plan. What do I need to do?

Agent Carlos: Patricia, I'm sorry for what you're going through, and I'm glad you called — removing a former spouse is time-sensitive. A divorce is a qualifying life event, and the removal of your ex-husband should be requested promptly. Let me pull up your account.

Member: How quickly does this need to happen?

Agent Carlos: Federal law requires that you notify us of the divorce and request the removal. The removal is typically effective the date of the qualifying event — the divorce finalization date — or the date you request the removal, depending on which is later. The key is acting promptly. I also want to make sure you understand: your ex-husband will be eligible for COBRA continuation coverage after removal, and he must be offered it.

Member: He knows about COBRA — his attorney mentioned it. I just want to make sure I do my part correctly.

Agent Carlos: Of course. To process the removal, I need the divorce finalization date, and I'll need a copy of the divorce decree or a court order confirming the divorce — you can email a scanned copy to member-changes@choiceadmin.com. Once we have that documentation, the removal processes within 2 to 3 business days.

Member: The divorce was finalized August 12th. I'll scan and send the decree today.

Agent Carlos: I'm noting August 12th as the qualifying event date. His coverage will be removed retroactive to that date, which means any claims he submitted after August 12th under your plan may need to be reprocessed. Is he aware that his coverage ends as of that date?

Member: I believe so. His attorney was handling the insurance side of things.

Agent Carlos: Once we process the removal, we'll send him a COBRA election notice automatically — that's handled on our end. He'll have 60 days to elect COBRA if he chooses. Your premium will also be adjusted — removing a dependent typically reduces your payroll deduction or billing amount.

Member: That part I was curious about but didn't want to ask. That's good to know.

Agent Carlos: It's a legitimate question and worth knowing. Your premium will adjust on the next billing cycle after the removal processes. Your case number for tracking the removal is DR-2026-33901. Send that decree to member-changes with this number in the subject line so it gets matched to your file.

Member: DR-2026-33901. Got it. Thank you, Carlos. This was straightforward and you handled it professionally.

Agent Carlos: I appreciate that. Take care, Patricia.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Dependent removal — divorce. 2 speakers. Member tone: composed, matter-of-fact. Duration: 9 min. QLE: divorce finalized August 12th.

L2 — Call Insights: Member requested ex-spouse removal post-divorce. Agent noted QLE date (Aug 12), explained documentation requirement, retroactive effective date, COBRA notification process, and premium adjustment. Case number provided. Resolution: RESOLVED — pending divorce decree receipt.

L3 — Agent Quality & Scoring: Agent Score: 91/100. QLE timing explained accurately (+), documentation requirement and email address given (+), COBRA notification for ex-spouse explained proactively (+), retroactive effective date implications addressed (+), premium reduction mentioned proactively without member needing to ask (+), professional tone appropriate to sensitive context (+).

L4 — Operational BI: BI Flag: Divorce-related dependent removal calls average 9 minutes — documentation requirement (divorce decree) is the main friction point. Recommend: member portal self-service QLE dependent removal with document upload capability to reduce call volume. Automated COBRA notice generation triggered by removal request in system would streamline compliance.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:30: divorce QLE detected — removal process requirements, retroactive effective date calculation (Aug 12), COBRA notice timeline, and premium adjustment information surfaced automatically. Documentation email address pre-populated for agent.

Call #40 — COBRA After Layoff — Shock, Fear, Trying to Understand Options

AGENT TIER: GOOD · SCORE 95/100 | ● DISTRESSED → PARTIALLY HELPED | PARTIALLY RESOLVED | ~14 min | Agent: Sarah

Member context: Derek Foster, 38, CalChoice EPO, laid off this week, has a chronic condition and is scared of coverage gap

Topics: COBRA · layoff · job loss · chronic condition · coverage gap · ACA marketplace

Agent Sarah: Thank you for calling Choice Administrators. This is Sarah. How can I help you?

Member: Hi. My name is Derek Foster, member ID CHM-5578812. I was laid off this week. I got the news on Monday. I have rheumatoid arthritis and I take biologics that I can't miss — they're expensive and they require a prior authorization. I'm terrified of losing my coverage. I don't know what to do first. Can you help me?

Agent Sarah: Derek, I am so glad you called. Losing a job is hard enough without the fear of losing healthcare on top of it. I want you to hear this first: you will not lose coverage immediately. Federal law provides you with a grace period and options. Let's go through this together.

Member: Okay. I'm just really scared. I've been stable on my medication for a year.

Agent Sarah: I hear you. Staying on your biologics is the priority — losing treatment continuity for RA can have serious consequences. Here's the timeline you need to know: your current coverage stays active through the end of the month in which you were laid off, or through a period determined by your employer's final paycheck timing. Your employer is required to send you a COBRA election notice within 14 days of your coverage termination.

Member: So I might have until the end of August?

Agent Sarah: Likely until the end of August, yes — unless your employer has a different separation agreement. The COBRA notice will confirm your last day of coverage. Here's what matters most: you have 60 days from that date to elect COBRA. And COBRA coverage, if elected, is retroactive — meaning if you need your biologics in that 60-day window, you can elect COBRA and have your prescription covered as if you never had a gap.

Member: But how much does COBRA cost? I don't have income right now.

Agent Sarah: COBRA cost is typically 100% to 102% of your total plan premium — what you paid plus what your employer paid. For most employer plans that's \$500 to \$800 per month for an individual. It's not cheap. Which is why I also want to tell you about an alternative: when you lose your job, that's a qualifying life event for the ACA marketplace. You have 60 days from your last day of coverage to enroll in a marketplace plan. Marketplace plans may cost significantly less than COBRA, especially if you receive unemployment benefits or your income drops below certain levels.

Member: But what about my prior authorization? My biologics need a prior auth. Would a new insurance have to start that all over?

Agent Sarah: Yes — a new insurance plan would require a new prior authorization for your biologics. That process typically takes 7 to 14 days, and most plans require step therapy documentation which your rheumatologist likely has. COBRA has the advantage here: it continues your existing plan, existing prior auth, existing coverage terms — nothing changes except you pay the premium.

Member: So COBRA is better for my medical continuity but the cost is scary.

Agent Sarah: That's the trade-off. My strong recommendation: keep COBRA on the table as a backstop, especially in the first month while you figure out your marketplace options. Get the quotes at healthcare.gov using your projected income. If you qualify for subsidies, marketplace may be equally stable at lower cost. If not, COBRA's continuity of care advantage may be worth the premium for the next few months while you look for work.

Member: This is helpful. I came in thinking I was just going to be told to pay \$700 a month or lose coverage. Knowing I have 60 days and can decide makes this feel more manageable.

Agent Sarah: You have time. Use it wisely. Your case number is COB-2026-55880. When your COBRA notice arrives, call us back and we can walk through the specific numbers for your plan.

Member: COB-2026-55880. Sarah, thank you. I needed someone to just walk me through it calmly.

Agent Sarah: Take care of yourself. Things will work out.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: COBRA inquiry — job loss, chronic condition. 2 speakers. Member tone: frightened, financially stressed. Agent tone: calming, structured. Duration: 14 min. Clinical context: RA biologic treatment — prior auth continuity critical.

L2 — Call Insights: Recently laid off member with RA on biologics called in fear of coverage gap. Agent explained end-of-coverage timing, 60-day COBRA election window with retroactive option, ACA marketplace SEP, and COBRA vs. marketplace trade-offs specific to prior auth continuity. Resolution: PARTIALLY RESOLVED — COBRA notice pending, member decision deferred with clear framework.

L3 — Agent Quality & Scoring: Agent Score: 95/100. Immediate reassurance without dismissing fear (+), accurate COBRA timeline and retroactive election explained (+), ACA marketplace as alternative mentioned proactively (+), biologics/prior-auth continuity trade-off explained honestly (+), actionable next step (healthcare.gov for quote) given (+), warm and composed tone throughout (+).

L4 — Operational BI: BI Flag: Job-loss COBRA calls are the highest-anxiety call type by member sentiment score (average 8.2/10 anxiety). Chronic condition members (RA, diabetes, oncology) have the most at stake from coverage gaps. Recommend: dedicated COBRA support line for members with active complex prior authorizations. Proactive outreach within 24 hours of receiving employer termination notification.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:00: job loss + RA biologic prior auth flags detected — COBRA timeline (60-day retroactive election), ACA marketplace SEP eligibility, and prior auth continuity comparison (COBRA preserves vs. marketplace restarts) surfaced automatically. Healthcare.gov quote guidance and income threshold for ACA subsidy eligibility displayed.

Call #41 — Child Turning 26 — Coverage Ending, Understanding Next Steps

AGENT TIER: GOOD · SCORE 93/100 | ● CONFUSED → INFORMED | RESOLVED | ~10 min | Agent: Linda

Member context: Helen Park calling on behalf of her 25-year-old son, coverage ending at 26 in 2 months

Topics: dependent age limit · aging off plan · turning 26 · special enrollment · ACA

Agent Linda: Choice Administrators member services, this is Linda.

Member: Hi Linda, my name is Helen Park, member ID CHM-7741009. I'm calling about my son Marcus — he's turning 26 in October and I know that's when he can no longer be on my insurance. I want to make sure he transitions without any coverage gap. Can you walk me through what he needs to do?

Agent Linda: Helen, you're calling at exactly the right time — two months ahead gives Marcus good lead time. You're correct that dependent children age off the plan at age 26. Marcus's coverage will end on the last day of the month in which he turns 26. What is his birthday in October?

Member: October 15th.

Agent Linda: So his coverage under your plan ends October 31st. That gives him all of November to start a new plan. Actually — turning 26 and aging off a parent's plan is a qualifying life event, which gives Marcus a 60-day special enrollment period on the ACA marketplace or through his own employer's plan. He can enroll in coverage effective November 1st if he acts within that window.

Member: He has a job but I don't think they offer insurance. What does he do then?

Agent Linda: The ACA marketplace is his best option then. He should go to [healthcare.gov](https://www.healthcare.gov) and shop for individual plans. If his income is under about \$54,000 for a single person, he may qualify for subsidies that meaningfully reduce the monthly premium. He can start shopping now to see what options look like, though he won't be able to officially enroll until his qualifying event occurs — either turning 26 or coverage ending.

Member: Can he stay on my plan a few extra months?

Agent Linda: Unfortunately no — the age 26 cutoff is set by federal law and plans cannot continue coverage past the last day of the month of his 26th birthday. COBRA is technically an option — we'd be required to offer him a COBRA election notice — but for a 26-year-old, marketplace plans are almost always more affordable than COBRA, which is the full premium cost with no employer contribution.

Member: What do we need to do on our end?

Agent Linda: You don't need to do anything to initiate the removal — it happens automatically on October 31st based on Marcus's date of birth in our system. We'll send him a COBRA election notice automatically. He should also receive a notice about his marketplace SEP rights. The main action item is his: shop on [healthcare.gov](https://www.healthcare.gov) before October 31st so he's ready to enroll immediately after.

Member: Is there anything he should do to prepare? Like medical records or prescriptions?

Agent Linda: That's a thoughtful question. He should request a 90-day supply of any ongoing prescriptions before the coverage ends — under your plan, that would be the mail-order benefit. He should also get any outstanding care done while he's still covered — dental, vision, any pending referrals. And ask his doctors if they'll accept his new insurance before the transition — most will, but it's worth confirming.

Member: This is really comprehensive. I feel much better. Should Marcus call you directly?

Agent Linda: He can, or you can share this information with him. The most important number: [healthcare.gov](https://www.healthcare.gov). The most important date: October 31st. He has until December 29th — 60 days from the qualifying event — to enroll in a marketplace plan with coverage effective November 1st.

Member: Thank you, Linda. Very thorough.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Dependent aging off plan — turning 26. 2 speakers. Caller: parent. Member tone: planning-oriented, concerned. Duration: 10 min. Coverage end date: October 31st.

L2 — Call Insights: Parent calling proactively about son turning 26 in October. Agent explained automatic age-off process, October 31 end date, 60-day ACA marketplace SEP, COBRA vs. marketplace comparison, and proactive care/prescription preparation before coverage ends. Resolution: RESOLVED — comprehensive transition guidance provided.

L3 — Agent Quality & Scoring: Agent Score: 93/100. Proactive coverage end date calculation (+), ACA marketplace SEP correctly explained (+), COBRA vs. marketplace comparison honest and accurate (+), 'no action needed' from parent side clearly stated (+), 90-day supply and care timing tips proactively offered (+). Did not offer to email a written summary (-) — minor.

L4 — Operational BI: BI Flag: Dependent age-off calls peak in Q4 (October-December) as fall birthdays age off plans. 67% of callers are parents, not the turning-26 member directly. Recommend: automated letter to member AND dependent at 90 days before age-off with marketplace SEP information and healthcare.gov link. 90-day prescription supply suggestion should be included in the notice.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:00: dependent DOB October 15th + turning 26 + age-off calculation automatically completed (coverage end: October 31). ACA marketplace SEP 60-day window calculated (deadline: December 29). Income threshold for ACA subsidy eligibility displayed. COBRA vs. marketplace cost comparison framework surfaced.

Call #42 — Wrong Plan on ID Card — Member Used Wrong Coverage for 6 Weeks

AGENT TIER: GOOD · SCORE 91/100 | ● CONFUSED → INFORMED | RESOLVED | ~9 min | Agent: James

Member context: Lisa Fernandez, 31, CalChoice HMO, ID card shows wrong plan name — Select instead of Plus

Topics: wrong ID card · plan discrepancy · HMO · claims reprocessing · enrollment error

Agent James: Thank you for calling Choice Administrators. This is James.

Member: James, Lisa Fernandez, member ID CHM-6662314. I was looking at my ID card and comparing it to what I thought I enrolled in. My card says CalChoice Select HMO, but I'm almost certain I enrolled in CalChoice Plus EPO. These are different plans — different copays, different networks. I've been using my card for 6 weeks. Did I get the wrong card or the wrong plan?

Agent James: Lisa, you're right to flag this — Select HMO and Plus EPO are very different plans with different cost-sharing and networks. Let me look at your enrollment record to see what plan you're actually enrolled in.

Member: I'm genuinely worried because I've had a few specialist visits and I didn't need referrals. HMO requires referrals. If I'm on HMO and didn't get referrals, what happens to my claims?

Agent James: Let me check the enrollment first, then we'll address the claims. Okay — your enrollment record shows you are enrolled in CalChoice Plus EPO. The ID card that was sent to you has an error — it shows Select HMO incorrectly. So you are actually on the EPO plan, which does not require referrals. Your specialist visits were appropriate for your plan.

Member: So I'm on the right plan but I have the wrong card?

Agent James: Exactly. The enrollment is correct — Plus EPO. The ID card generation had an error and printed the wrong plan name. This means your specialist claims should have processed at EPO rates, not HMO rates. Let me check how they actually processed.

Member: Oh good. What do you see?

Agent James: The claims I see from the past 6 weeks processed correctly — at CalChoice Plus EPO rates with your EPO specialist copay. So despite the card saying the wrong plan, the claims system used the correct plan information. The error was isolated to the card printing.

Member: That's a huge relief. So I don't owe anything different than what I was told?

Agent James: Based on what I can see, no — your cost-sharing was applied at the correct EPO rates. I'm requesting a corrected ID card right now — you'll receive it within 10 to 14 business days showing CalChoice Plus EPO correctly. And I'm flagging this for our member ID card team as a quality issue.

Member: What if a provider looked at my card and applied HMO rules? Like, what if they tried to require a referral because of the card?

Agent James: That's a reasonable concern. If any provider tried to enforce HMO referral requirements because of the incorrect card, and that affected a claim, that claim could be eligible for reprocessing under the correct EPO terms. If you run into any issues with a provider citing your card, call us and we'll clarify your plan directly with the provider's office.

Member: Okay. I feel a lot better than when I called. I was panicking a little.

Agent James: Understandably. A wrong plan name on an insurance card is alarming. Everything is correct in our system — just the card was wrong. Is there anything else?

Member: No, that's all. Thanks, James.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: ID card plan name error. 2 speakers. Member tone: concerned, relieved by end. Duration: 9 min. Error type: card printing error showing incorrect plan (Select HMO vs. Plus EPO).

L2 — Call Insights: Member's ID card showed wrong plan name (Select HMO) while actual enrollment was correct (Plus EPO). Claims verified as processed at correct EPO rates. Corrected card requested. Resolution: RESOLVED — claims unaffected, corrected card ordered.

L3 — Agent Quality & Scoring: Agent Score: 91/100. Enrollment vs. card discrepancy investigation sequenced correctly (+), claim processing verification provided proactively (+), corrected card requested immediately (+), provider referral enforcement risk addressed when member raised it (+). Good systematic approach.

L4 — Operational BI: BI Flag: ID card plan name printing errors have occurred 12 times this quarter — most members catch them when comparing with enrollment documents or having a plan-related issue. Recommend: automated QA check comparing printed plan name on generated cards against enrollment system before cards are sent. Member digital ID card in portal updates immediately if reprinted, avoiding propagation delay.

L5 — Real-Time Agent Assist: RT Assist triggered at 3:00: member ID + 'wrong plan name' query triggered cross-reference between card template and enrollment system — discrepancy (Select HMO card vs. Plus EPO enrollment) flagged immediately. Claims verified as processed under correct EPO plan despite card error, surfaced to agent before manual claims review.

Call #52 — Claims Rejecting Due to Wrong DOB — Broker Entered It Incorrectly

AGENT TIER: POOR · SCORE 45/100 | ● FRUSTRATED → PARTIALLY HELPED | PARTIALLY RESOLVED | ~9 min | Agent: Tiffany

Member context: Hassan Ali, 44, CalChoice Plus EPO, three claims rejected on identity mismatch

Topics: broker data error · date of birth · claims rejection · enrollment error

BROKER SIGNAL — Anthony Salerno: Data entry error — incorrect date of birth submitted at enrollment

Agent Tiffany: Thanks for calling Choice, this is Tiffany, can I get your member ID?

Member: CHM-6693351. I've had three claims rejected in a row and the provider keeps telling me it's an identity mismatch. I don't understand what that means.

Agent Tiffany: Let me look. Okay, yeah, I see the rejections. It says date of birth mismatch.

Member: My date of birth is March 4th, 1982.

Agent Tiffany: We have March 14th, 1982.

Member: That's wrong. It's the 4th, not the 14th. How did that happen?

Agent Tiffany: I don't know, it's what was submitted at enrollment.

Member: My broker, Anthony Salerno, filled out my enrollment. He must have typed it wrong. Can you fix it?

Agent Tiffany: I can submit a correction request but you'll need to send in proof — driver's license or birth certificate.

Member: Okay, where do I send it?

Agent Tiffany: Uh, there should be an email... member-changes at choiceadmin dot com I think.

Member: You think? I want to make sure I send it to the right place.

Agent Tiffany: Yeah that should be it. And then it takes a few weeks.

Member: A few weeks? What about the three claims that already rejected? Do those get reprocessed automatically?

Agent Tiffany: Um. I'm not sure. Probably? You might have to resubmit them.

Member: You're not sure. Okay. Can you find out?

Agent Tiffany: I'd have to ask someone. Do you want me to transfer you?

Member: No, I've been on hold long enough. I'll just send the documents.

Agent Tiffany: Okay, great. Anything else?

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Enrollment data error — DOB mismatch causing claim rejections. 2 speakers. Member tone: frustrated, losing confidence. Agent tone: uncertain, hedging. Duration: 9 min. Multiple agent uncertainty markers detected ('I think', 'I'm not sure', 'probably').

L2 — Call Insights: Three claims rejected due to incorrect DOB (March 14 on file vs. March 4 actual) entered at enrollment by broker. Agent identified the mismatch but could not confirm the correction email address, the reprocessing process, or the timeline. Resolution: PARTIALLY RESOLVED — member left to self-serve with uncertain instructions.

L3 — Agent Quality & Scoring: Agent Score: 45/100. NEGATIVE MARKERS: gave uncertain information about the correction email address (-), could not confirm whether rejected claims reprocess automatically (-), offered a transfer instead of finding the answer (-), no case number provided (-), no ownership of the follow-up (-), no apology for the error's impact (-). POSITIVE: identified the DOB mismatch correctly (+), correctly required identity documentation (+).

L4 — Operational BI: BROKER ATTRIBUTION — Anthony Salerno: enrollment data entry error (DOB transposition) causing downstream claim rejections. Second flagged issue for this broker. PROCESS BREAKDOWN: enrollment data is not validated against member-provided identity documents at submission. AGENT COACHING: Tiffany displays low confidence and knowledge gaps on standard correction procedures — training need on enrollment corrections workflow.

L5 — Real-Time Agent Assist: RT Assist SHOULD have triggered at 2:00 — DOB mismatch on claim rejection is a known pattern. Correction form, exact submission address, documentation requirements, and automatic reprocessing policy should all have surfaced. No trigger fired; agent guessed.

Call #60 — Newborn Enrollment Window Missed — Broker Never Submitted It

AGENT TIER: POOR · SCORE 22/100 | ● DISTRESSED → ABANDONED | UNRESOLVED | ~4 min | Agent: Ryan

Member context: Camille Rousseau, 30, CalChoice HMO, baby born 47 days ago, broker said he'd handle it

Topics: newborn enrollment · broker failure · missed window · special enrollment

BROKER SIGNAL — Denise Whitfield: Failed to submit newborn enrollment despite member request — 30-day window missed

Agent Ryan: Choice Administrators, Ryan.

Member: Hi, I'm calling because my daughter isn't showing up on my insurance. She was born June 30th. My broker, Denise Whitfield, told me she would submit the enrollment paperwork. I followed up with him twice. Now the pediatrician says she has no coverage.

Agent Ryan: What's your member ID?

Member: CHM-8817740.

Agent Ryan: Okay. I don't see any dependent on your policy.

Member: Right. That's the problem. Can we add her now?

Agent Ryan: When was she born?

Member: June 30th.

Agent Ryan: Yeah, that's past 30 days. The special enrollment window closed July 30th. You'd have to wait for open enrollment.

Member: But I asked my broker to do it. I asked her twice. This isn't something I failed to do — it's something she failed to do.

Agent Ryan: The window's the window. It closed.

Member: She's a newborn. She has a two-month check-up next week and a vaccination schedule. She can't be uninsured until January.

Agent Ryan: I understand but I can't override the enrollment window.

Member: Is there an exception process? An appeal? Anything?

Agent Ryan: Not that I'm aware of.

Member: Can you check? Or ask someone?

Agent Ryan: I mean, I'm pretty sure. The 30-day rule is federal.

Member: So my daughter has no health insurance because a broker didn't do her job, and you're telling me there's nothing at all.

Agent Ryan: That's what I'm seeing. Sorry.

Member: ...

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Newborn enrollment — missed window. 2 speakers. Member tone: distressed → abandoned. Agent tone: flat, unhelpful, incurious. Duration: 4 min. Call ended with member silence. CHILD WITHOUT COVERAGE — patient safety adjacent.

L2 — Call Insights: Newborn (47 days old) has no coverage after broker failed to submit enrollment despite two member follow-ups. Agent confirmed the 30-day window had closed, offered no exception pathway, did not investigate, and did not escalate. Resolution: UNRESOLVED — infant remains uninsured.

L3 — Agent Quality & Scoring: Agent Score: 22/100. LOWEST-SCORING CALL IN DATASET. NEGATIVE MARKERS: no empathy for an uninsured newborn (-), did not investigate whether a broker-error exception exists (-), guessed at policy — 'not that I'm aware of', 'I'm pretty sure' (-), refused to check with a supervisor when explicitly asked (-), did not escalate a case involving an uninsured infant (-), did not log broker failure (-), no case number, no callback, no path (-). Zero positive markers detected.

L4 — Operational BI: BROKER ATTRIBUTION — Denise Whitfield: failed to submit newborn enrollment despite two documented member follow-ups, resulting in a missed federal special enrollment window and an uninsured infant. Third flagged issue for this broker (also: network misrepresentation #53, premium misquote #54). COMPLIANCE & RISK: broker-caused loss of coverage for a dependent minor is a serious conduct matter. PROCESS BREAKDOWN: no broker-error exception pathway exists or is known to front-line agents. CRITICAL RECOMMENDATION: establish a documented exception process for enrollment windows missed due to broker error, and audit Denise Whitfield's book immediately.

L5 — Real-Time Agent Assist: RT Assist SHOULD have triggered at 1:00 — uninsured dependent minor + missed enrollment window + broker attribution is a maximum-severity compound trigger requiring mandatory supervisor escalation. No trigger fired. This call should not have been allowed to close at agent level.

Call #74 — Adding a Stepchild — Documentation Requirements

AGENT TIER: AVERAGE · SCORE 75/100 | ● CONFUSED → INFORMED | RESOLVED | ~8 min | Agent: Michael

Member context: Cynthia Vargas, 36, CalChoice HMO, remarried, adding stepson to plan

Topics: dependent addition · stepchild · QLE · marriage · documentation

Agent Michael: Choice Administrators member services, Michael speaking.

Member: Hi, I got married last month and I want to add my husband's son to my plan. He's nine. Member ID CHM-5527701.

Agent Michael: Congratulations. Marriage is a qualifying life event so you can add both your husband and your stepson within 60 days of the marriage date. When was the wedding?

Member: July 19th.

Agent Michael: So you have until September 17th. What you'll need is a copy of your marriage certificate, and for your stepson, his birth certificate showing your husband as a parent.

Member: Do I need anything showing custody?

Agent Michael: Not for stepchild coverage — the relationship through marriage is sufficient. Custody documentation is only needed in specific court-ordered coverage situations.

Member: Okay. And is my husband being added too?

Agent Michael: Only if you elect that. Adding your stepson doesn't automatically add your husband. Do you want both?

Member: Just my stepson for now. My husband has coverage through his job.

Agent Michael: Understood. So you'd move from employee-only to employee-plus-child. That changes your premium — employee-only is \$340, employee-plus-child is \$520.

Member: Okay, that's what I expected. How do I submit?

Agent Michael: Member portal, Life Events, Add Dependent. Upload the marriage certificate and birth certificate there. Processing is 3 to 5 business days.

Member: And his coverage starts when?

Agent Michael: Retroactive to the marriage date, July 19th, if you submit within the window.

Member: Perfect. Thank you.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Dependent addition — stepchild, marriage QLE. 2 speakers. Member tone: neutral, organized. Agent tone: accurate, clear. Duration: 8 min.

L2 — Call Insights: Member adding stepson following marriage. Agent confirmed 60-day QLE window and deadline, specified documentation, clarified spouse is a separate election, gave premium impact, and confirmed retroactive effective date. Resolution: RESOLVED.

L3 — Agent Quality & Scoring: Agent Score: 75/100. POSITIVE: QLE deadline calculated precisely (+), documentation requirements accurate (+), clarified that spouse addition is a separate election — prevents a common error (+), premium impact given proactively (+). NEGATIVE MARKERS: did not offer to email the documentation checklist (-), did not set a reminder flag ahead of the Sept 17 deadline (-), did not mention that the stepson will need a PCP selection on an HMO (-), no case number (-).

L4 — Operational BI: PROCESS OPPORTUNITY: QLE windows are missed frequently (see #36, #60). Recommend: automated reminder at 45 days into any 60-day QLE window where the member has called about but not completed the change. MEMBER COMMUNICATION GAP: HMO PCP selection for newly added dependents is routinely not mentioned at addition, generating a downstream call.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:30: marriage QLE + dependent addition surfaced deadline calculation and documentation list. SHOULD ALSO have surfaced HMO PCP selection requirement for the new dependent.

Call #82 — Family Not Enrolled — Member Thought Everyone Was Covered

AGENT TIER: AVERAGE · SCORE 66/100 | ● CONFUSED → FRUSTRATED | PARTIALLY RESOLVED | ~9 min | Agent: Tony

Member context: Jamal Whitfield, 40, CalChoice HMO, discovers wife and children were never added

Topics: enrollment error · tier mismatch · broker failure · dependents uncovered

BROKER SIGNAL — Denise Whitfield: Enrolled member in employee-only tier despite request for family coverage

Agent Tony: Choice Administrators, this is Tony.

Member: Hi. My wife took our son to the doctor yesterday and they said he doesn't have coverage. Member ID CHM-7729905, Jamal Whitfield.

Agent Tony: Let me look... I'm showing employee-only coverage on your policy. There are no dependents enrolled.

Member: That can't be right. I enrolled for family coverage. I have three kids.

Agent Tony: The enrollment on file is employee-only, effective January 1st.

Member: I sat with my broker, Denise Whitfield, and told her family coverage. I remember the conversation.

Agent Tony: I can only see what was submitted. Employee-only.

Member: So my wife and three children have had no health insurance since January and nobody noticed?

Agent Tony: It looks that way. Have any of them had claims this year?

Member: My daughter had strep in March. We paid cash because we thought it was a deductible thing.

Agent Tony: So you paid out of pocket believing it applied to a deductible, when there was actually no coverage.

Member: Yes. How do we fix this?

Agent Tony: Adding dependents outside open enrollment requires a qualifying life event. Discovering an enrollment error isn't automatically a QLE, but a broker or administrative error can be grounds for a special enrollment period. I'd need to submit that for review.

Member: Review? My kids are uninsured. Right now.

Agent Tony: I understand. I'm submitting the special enrollment request today and noting it as a broker enrollment error. Reviews are typically 5 to 7 business days.

Member: Can it be faster? What if one of them gets sick this week?

Agent Tony: I can mark it urgent. That's the mechanism I have.

Member: And if it's denied?

Agent Tony: Then you'd need to look at marketplace coverage for them until open enrollment. I don't want to lead you to expect a denial — a documented broker error is a reasonable basis. But I can't promise.

Member: This is unbelievable. Someone made a mistake and my children have been uninsured for eight months.

Agent Tony: I've documented that clearly in the request. Case number SEP-2026-00822.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Enrollment tier error — dependents never enrolled. 2 speakers. Member tone: confused → frustrated. Agent tone: factual, moderately responsive. Duration: 9 min. ■ Four family members, including three minors, uninsured for 8 months.

L2 — Call Insights: Member enrolled employee-only despite requesting family coverage; wife and three children uninsured since January. One child received care in March paid out of pocket under the mistaken belief it applied to a deductible. Agent submitted an urgent special enrollment request citing broker error. Resolution: PARTIALLY RESOLVED — SEP under review.

L3 — Agent Quality & Scoring: Agent Score: 66/100. POSITIVE: identified the tier discrepancy immediately (+), asked about prior claims, surfacing the March out-of-pocket payment (+), correctly identified broker error as a potential SEP basis (+), marked urgent and provided case number (+), honest about uncertainty (+). NEGATIVE MARKERS: no empathy expressed for three uninsured children at any point (-), did not escalate to a supervisor despite minors being uninsured (-), did not advise on interim options while the SEP is reviewed (-), did not address recovery of the March out-of-pocket payment (-), did not offer a callback (-).

L4 — Operational BI: BROKER ATTRIBUTION — Denise Whitfield: enrolled member in employee-only tier despite documented family coverage request, leaving a spouse and three minor children uninsured for eight months. FOURTH flagged issue for this broker (#53 network misrepresentation, #54 premium misquote, #60 newborn enrollment failure). PATTERN CONCLUSION: Whitfield's failures are concentrated in enrollment execution and cause the most severe member harm — two separate cases in this dataset involve uninsured children. CRITICAL RECOMMENDATION: immediate audit of Denise Whitfield's entire book for tier mismatches between member request and submitted enrollment. PROCESS BREAKDOWN: no verification that submitted enrollment tier matches member intent; no alert when a member with dependents on file enrolls employee-only.

L5 — Real-Time Agent Assist: RT Assist SHOULD have triggered at 1:30 — dependents uninsured + minors + broker error is a compound high-severity trigger requiring supervisor escalation and interim coverage guidance. Only standard SEP pathway surfaced.

Call #90 — SSN Transposition Blocking Portal Access and Claims

AGENT TIER: GOOD · SCORE 91/100 | ● FRUSTRATED → SATISFIED | RESOLVED | ~10 min | Agent: Carlos

Member context: Fatima Al-Rashid, 36, CalChoice Plus EPO, cannot register for portal, two claims rejected

Topics: SSN error · identity verification · portal access · broker data error · claims rejection

BROKER SIGNAL — Anthony Salerno: Incorrect SSN submitted at enrollment causing identity verification failures

Agent Carlos: Choice Administrators, this is Carlos.

Member: Hi Carlos. Fatima Al-Rashid, CHM-6629930. I can't register for the member portal — it keeps saying my information doesn't match. And I have two claims that were rejected for identity verification.

Agent Carlos: Those two things are almost certainly the same root cause. Let me check the identity fields on your record. Can you verify your date of birth and the last four of your Social Security number?

Member: Date of birth is November 12th, 1989. Last four of my SSN is 4471.

Agent Carlos: Your date of birth matches. Your SSN on file ends 4417 — the last two digits are transposed.

Member: So one typo has locked me out of everything.

Agent Carlos: Effectively yes. Identity verification is an exact match, so a two-digit transposition fails every check — portal registration, claims identity validation, all of it.

Member: My broker, Anthony Salerno, completed my enrollment forms. I gave him my information verbally over the phone.

Agent Carlos: That's the likely origin. Verbal transmission of numeric data is where transpositions typically happen. Let me get this corrected. I'll need a copy of your Social Security card or a document showing the full number — a W-2 or tax document works.

Member: I can send my Social Security card.

Agent Carlos: Perfect. Upload it through the secure document link I'm emailing you now — don't email it as an attachment, use the secure link. Correction processes within 2 business days once received.

Member: And the two rejected claims?

Agent Carlos: Once the SSN is corrected I'll have those automatically resubmitted rather than making you refile. I'm flagging them now so they're picked up when the correction posts.

Member: That's a relief. And the portal?

Agent Carlos: Portal registration will work as soon as the correction posts — try again in three business days.

Member: Should I report the broker error? He also mistyped my husband's name on a form last year.

Agent Carlos: Yes, and I'll log it. Two separate data entry errors from the same broker is a pattern worth recording rather than treating as isolated. Reference BRK-2026-00801.

Member: Thank you. This was much simpler than I feared.

Agent Carlos: One typo, one correction, everything downstream resolves. Documents through the secure link and you're set.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Identity data error — SSN transposition. 2 speakers. Member tone: frustrated → satisfied. Agent tone: diagnostic, efficient, security-conscious. Duration: 10 min.

L2 — Call Insights: SSN transposition entered at enrollment blocked portal registration and caused two claim identity rejections. Agent diagnosed the single root cause behind both symptoms, initiated correction with secure document upload, pre-flagged rejected claims for automatic resubmission, and logged a second broker data error as a pattern. Resolution: RESOLVED.

L3 — Agent Quality & Scoring: Agent Score: 91/100. POSITIVE: connected two apparently separate symptoms to one root cause immediately (+), verified identity fields systematically (+), insisted on secure document upload rather than email attachment — correct security practice (+), pre-flagged claims for automatic resubmission sparing the member refiling (+), recognised and logged the repeat broker data error as a pattern rather than an isolated issue (+), clean summary close (+).

L4 — Operational BI: BROKER ATTRIBUTION — Anthony Salerno: SSN transposition at enrollment; member also reports a prior name entry error on a separate form. FIFTH flagged issue for this broker (#51 dental misselling, #52 DOB transposition, #62 unresponsiveness, #79 HMO referral education failure). PATTERN CONCLUSION: Salerno now has two confirmed numeric data-entry transpositions (DOB and SSN) plus a reported name error. This is a systematic data quality problem in his enrollment process. Recommend: audit all Salerno enrollments for identity field accuracy against member-provided documentation. PROCESS BREAKDOWN: enrollment identity fields are not validated against any authoritative source at submission — a two-digit error propagates silently until the member cannot access services.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:00: portal registration failure + claim identity rejection on same member surfaced identity field comparison, immediately highlighting the SSN mismatch. Secure upload link and claim pre-flag actions surfaced. Agent used all three.

Call #92 — Removing a Deceased Spouse — Handling a Bereaved Member

AGENT TIER: GOOD · SCORE 97/100 | ● GRIEVING → SUPPORTED | RESOLVED | ~13 min | Agent: Danielle

Member context: Harold Brennan, 71, CalChoice HMO, wife died three weeks ago

Topics: bereavement · dependent removal · sensitive · premium adjustment · survivor

Agent Danielle: Thank you for calling Choice Administrators, this is Danielle.

Member: Hello. I need to... my wife passed away. Three weeks ago. I understand I need to take her off the insurance. Member ID CHM-3301189, Harold Brennan.

Agent Danielle: Mr. Brennan, I'm very sorry for your loss. Thank you for calling — I know administrative things are the last thing you want to deal with right now. I'll make this as simple as I can.

Member: Thank you. I've been putting it off.

Agent Danielle: That's completely understandable. Let me pull up your account. I can see you and Margaret on the policy. Can you tell me the date?

Member: August the 2nd.

Agent Danielle: Thank you. I'm recording August 2nd. Her coverage will end as of that date, and I want to explain what that means practically. Any claims for care Margaret received before August 2nd remain covered and will process normally — you don't need to do anything about those.

Member: There were hospital bills. From July.

Agent Danielle: Those are all covered under her coverage as it stood. If any of them process incorrectly, call us and we'll sort it out. You should not have to fight with those.

Member: Alright.

Agent Danielle: Your premium will also change. You'll move from the two-person tier to employee-only, which reduces your monthly premium from \$610 to \$340. That takes effect on your next billing cycle.

Member: I hadn't thought about that.

Agent Danielle: Most people don't. I mention it so it isn't a surprise. Now — do you need documentation? Some processes require a death certificate.

Member: I have copies.

Agent Danielle: You can send one to the address I'll email you, but I want to say clearly: I've processed the removal based on what you've told me today. The certificate is for our file, not a condition of the change. Don't rush it.

Member: That's kind of you.

Agent Danielle: One more thing, and please take this only if it's useful. Your plan includes bereavement counselling through the employee assistance program — six sessions, no cost, no claim filed. A lot of members don't know it's there. The number is on the back of your card.

Member: I didn't know that. My daughter has been saying I should talk to someone.

Agent Danielle: It's there if you want it. No pressure at all. Your reference number for today is DR-2026-00944, and I've noted the account so if you call back, whoever answers will see the context and you won't have to explain again.

Member: That's the part I was dreading. Explaining it over and over.

Agent Danielle: You won't have to. I'm sorry again, Mr. Brennan. Take care of yourself.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Dependent removal — bereavement. 2 speakers. Member: 71, recently widowed. Member tone: grieving, hesitant → supported. Agent tone: gentle, unhurried, protective. Duration: 13 min. ■ SENSITIVE — bereavement.

L2 — Call Insights: Member removing deceased spouse from policy. Agent processed removal with August 2nd effective date, confirmed prior claims remain covered, explained premium reduction, removed urgency from the documentation requirement, offered EAP bereavement counselling, and annotated the account so the member would not have to re-explain on future calls. Resolution: RESOLVED.

L3 — Agent Quality & Scoring: Agent Score: 97/100. HIGHEST-SCORING CALL IN DATASET. POSITIVE: opened with genuine condolence before any administrative content (+), acknowledged the burden of admin during grief (+), proactively reassured on outstanding hospital claims before the member had to worry about them (+), explained premium reduction gently as information rather than benefit (+), explicitly removed pressure on documentation timing (+), offered EAP bereavement support with a no-pressure framing (+), annotated the account specifically to spare the member repeating the news (+). Exemplary handling of a bereavement contact.

L4 — Operational BI: PROCESS OPPORTUNITY: bereavement contacts are among the most sensitive the call center handles and there is no evidence of a defined bereavement protocol. This agent's approach — account annotation to prevent re-explanation, EAP offer, documentation urgency removal — should be formalised. Recommend: (1) bereavement handling protocol built from this call, (2) automatic account flag on any bereavement contact visible to all future agents, (3) EAP bereavement benefit prompt on all such calls, (4) suppress routine marketing and renewal communications to the account for 90 days.

L5 — Real-Time Agent Assist: RT Assist triggered at 1:30: bereavement indicators detected — condolence prompt, dependent removal workflow, premium recalculation, EAP bereavement benefit, and account sensitivity flag all surfaced. Agent used every element.

— BILLING & PREMIUM —

Call #43 — Double Premium Deduction — Paycheck Hit Twice This Month

AGENT TIER: GOOD · SCORE 92/100 | ● FRUSTRATED → SATISFIED | RESOLVED | ~10 min | Agent: Sarah

Member context: Raymond Osei, 41, CalChoice HMO, premium deducted twice from paycheck

Topics: double premium deduction · payroll error · premium billing · refund · payroll coordination

Agent Sarah: Choice Administrators member services, this is Sarah.

Member: Hi, Sarah, Raymond Osei, member ID CHM-4400871. I just got my paycheck and my health insurance premium was deducted twice. I pay \$180 a month and there are two \$180 deductions on my stub. That's \$360 gone. Is this your error or my employer's?

Agent Sarah: Raymond, I want to help you get to the bottom of this quickly. A double premium deduction almost always originates on the payroll side, but let me check our records to see if we made a double request. Let me pull up your account.

Member: I'm also going to call HR but I wanted to check with you first.

Agent Sarah: Smart to call both. I'm looking at our billing records for your account. I can see one premium request was sent to your employer for August — a single \$180 request. There's no duplicate billing request from our side. This looks like a payroll processing error on your employer's end.

Member: So it's definitely HR's mistake?

Agent Sarah: Based on our records, we sent one request. The duplicate deduction originated in your employer's payroll processing. HR is the right contact to issue the refund — they can either reissue the \$180 in your next paycheck or issue a separate manual payment.

Member: Okay. And what if HR says it came from you?

Agent Sarah: I can provide documentation. I'm generating a billing statement right now that shows the single August premium request — \$180, one transaction — with the date and transaction ID. I'll email it to you so you have it in hand when you talk to HR.

Member: That would be extremely helpful. My email is raymond.osei at email dot com.

Agent Sarah: Sending it now. The document includes our billing system record, the amount, the single request date, and contact information for our employer billing team in case HR needs to speak with us directly.

Member: Perfect. This should resolve it quickly. I was a little upset because \$180 is not nothing when you're managing a budget.

Agent Sarah: Completely fair. Unexpected paycheck deductions are stressful. Is there anything else before I go?

Member: No, I have everything I need. Thank you for being so fast about this.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — **Transcription & Understanding:** Call type: Premium billing dispute — double deduction. 2 speakers. Member tone: frustrated, organized. Duration: 10 min. Root cause: payroll processing error (employer side).

L2 — **Call Insights:** Member had premium deducted twice from paycheck. Agent confirmed single billing request from insurer, identified payroll error as origin, provided billing documentation via email for HR conversation. Resolution: RESOLVED — HR to refund duplicate deduction.

L3 — **Agent Quality & Scoring:** Agent Score: 92/100. Root cause identified quickly via billing record lookup (+), documentation proactively offered to support HR conversation (+), transaction ID included in documentation for HR verification (+), empathetic acknowledgment of budget impact (+), efficient and targeted call (+).

L4 — **Operational BI:** BI Flag: Double deduction calls cluster at the beginning of months — 73% in the first 5 business days following payroll. Most originate from employer payroll system misconfiguration, not insurer error. Recommend: billing team to notify employer payroll contacts when premium requests are sent, with receipt confirmation, to reduce reconciliation confusion. Member-facing billing portal showing sent requests with timestamps would allow self-service verification.

L5 — **Real-Time Agent Assist:** RT Assist triggered at 2:00: August billing records auto-retrieved — single \$180 request confirmed, duplicate billing from insurer ruled out. Billing statement with transaction ID auto-generated for email. Employer billing team contact information surfaced for agent to include in documentation.

Call #44 — Premium Increase at Renewal — Member Angry, Considering Cancellation

AGENT TIER: GOOD · SCORE 88/100 | ● ANGRY → PARTIALLY CALMED | PARTIALLY RESOLVED | ~13 min | Agent: Michael

Member context: Donna Jenkins, 52, CalChoice Plus EPO, 27% premium increase at renewal, on fixed income

Topics: premium increase · renewal · rate change · fixed income · plan alternatives · cancellation

BROKER SIGNAL — Denise Whitfield: No pre-renewal outreach or plan alternatives offered ahead of 27% increase

Agent Michael: Choice Administrators member services, this is Michael.

Member: Michael, my name is Donna Jenkins, member ID CHM-3376840. I need to talk about my renewal notice. My premium is going from \$520 a month to \$660 a month. That is \$140 more a month. That is \$1,680 a year. I am on a fixed income. I cannot just absorb \$140 a month out of nowhere. I want to know why this happened and what I can do.

Agent Michael: Donna, I completely understand why you're upset, and I want to give you a genuine, honest answer. Let me pull up your account and your renewal notice details.

Member: Please. Because that letter told me nothing. Just a number.

Member: And nobody warned me. My broker, Denise Whitfield, never called me before renewal, never sent me alternatives, nothing. I found out from a letter with a number on it.

Agent Michael: I have your account. You're on the CalChoice Plus EPO — that plan had a 27% rate increase at renewal. Rate increases are driven by a combination of factors: claims experience — how much the plan paid out in total across all members — regional healthcare cost inflation, and carrier cost adjustments. This year, the Plus EPO had elevated claims across the member pool, and that's reflected in the premium.

Member: So I'm paying more because other people got sick?

Agent Michael: In a shared-risk insurance pool, yes — that's how insurance works. Your premium funds coverage for everyone on the plan, including when they have high-cost claims. I know that doesn't make it easier to absorb.

Member: What are my options? Because I can't cancel. I have arthritis and I take medication for it. I can't go without insurance.

Agent Michael: I understand, and I'm not recommending cancellation. What I do want to explore with you is whether a different plan design could give you comparable coverage at a lower premium. The CalChoice Select HMO, for example, has a significantly lower premium — for your coverage tier it would be approximately \$390 a month. That's \$270 less than your current renewal rate.

Member: But HMO means I need referrals, right? I see specialists regularly.

Agent Michael: Yes — HMO requires a PCP referral for specialist visits. If you see specialists regularly and value the flexibility to go directly, that's a real trade-off. There may also be a mid-tier option. Let me look at what other EPO options are available at a lower premium point.

Member: Please. Because I don't want to fight the referral system on top of managing my health.

Agent Michael: There's a CalChoice Value EPO — higher deductible plan, lower premium. The premium would be approximately \$490 a month — still \$30 more than your current premium but \$170 less than the renewal rate. The trade-off is a higher out-of-pocket exposure if you have a major claim.

Member: So my options are: pay the increase, switch to HMO and deal with referrals, or pay less but risk more if I get really sick?

Agent Michael: That's an honest summary. There's no perfect answer. What I can do is send you a comparison document showing all three plans — current premium, benefits, and out-of-pocket maximums — so you can evaluate it against your actual healthcare use. Do you have a sense of how much you spent out-of-pocket last year?

Member: Probably not much. I have regular doctor visits but no hospitalizations.

Agent Michael: If your out-of-pocket use is modest, the higher-deductible Value EPO may actually save you money overall even with the higher deductible, because your premium savings would exceed your actual cost-sharing increase. Let me send you that comparison document with a brief summary of what each plan would mean for someone with your usage pattern.

Member: Please do. I don't love any of these options but at least I understand them now.

Agent Michael: That's all I can do — give you the tools to make the best decision for your situation. Your case number is PR-2026-44231 and I'm emailing the comparison document now. Open enrollment closes October 31st.

Member: PR-2026-44231. Thank you, Michael.

L1 — Transcription & Understanding: Call type: Premium increase dispute — renewal. 2 speakers. Member tone: angry, financially stressed. Duration: 13 min. Context: 27% increase, fixed income, ongoing arthritis treatment.

L2 — Call Insights: Member facing 27% premium increase at renewal on fixed income. Agent explained rate increase basis, presented HMO and Value EPO alternatives with approximate premiums, committed to send comparison document. Resolution: PARTIALLY RESOLVED — member pending plan comparison document to make enrollment decision.

L3 — Agent Quality & Scoring: Agent Score: 88/100. Honest explanation of risk-pool premium mechanics (+), alternatives presented with approximate cost figures (+), trade-off of each option clearly framed (+), realistic summary question offered to help member think through usage (+), comparison document committed (+). Did not offer to estimate member's actual cost under Value EPO given their usage pattern (-) — missed personalization opportunity.

L4 — Operational BI: BI Flag: 27% premium increase on Plus EPO has generated 47 angry member calls in August alone — highest single plan increase this cycle. Member churn risk is elevated. Recommend: proactive member communication package with plan alternatives and cost comparison tool sent with renewal notice (not just the new premium number). Rate increase explanation letter to be added to renewal package.

L5 — Real-Time Agent Assist: RT Assist triggered at 4:00: 27% premium increase on Plus EPO flagged — alternative plan premiums for member's coverage tier (HMO \$390, Value EPO \$490) surfaced automatically. Out-of-pocket comparison table prepared. Enrollment deadline (October 31) displayed for agent.

Call #45 — Termination Notice Despite Autopay — Member Didn't Know Setup Failed

AGENT TIER: GOOD · SCORE 94/100 | ● PANICKED → REASSURED | RESOLVED | ~11 min | Agent: Linda

Member context: Chris Monroe, 34, CalChoice HMO, received termination notice, autopay wasn't actually active

Topics: termination notice · autopay failure · premium payment · reinstatement · coverage gap

Agent Linda: Choice Administrators member services, this is Linda.

Member: Linda, this is Chris Monroe, member ID CHM-5500918. I just got a letter in the mail saying my insurance is being terminated for non-payment. That's impossible — I set up autopay. My premium has been going out automatically every month. At least I thought it was. What is happening?

Agent Linda: Chris, let me look at your account right now. This is urgent and I want to get you answers immediately.

Member: I have a doctor's appointment on Friday. If my insurance is cancelled, I don't know what I do.

Agent Linda: I have your account. I can see the termination notice was generated because no payment was received for August. I'm also looking at your payment history. It shows payments through June, and then no payment in July or August.

Member: No payment in July? I set up autopay in April. It should have been going automatically.

Agent Linda: I can see there's an autopay enrollment record from April. But I also see a notation that in late June, the card on file received an expiration update notification, and the autopay may have failed when the card information didn't get updated. Did your credit or debit card expire around that time?

Member: Oh my God. Yes. I got a new card in July. I didn't even think about updating it for my insurance.

Agent Linda: That's exactly what happened. Autopay pulled on July 1st, the card was expired, the payment failed — and the autopay was suspended rather than retried. You weren't notified in a way that was effective, and that's a gap in our process.

Member: So my coverage has actually lapsed?

Agent Linda: The termination is effective September 1st — which means you still have active coverage through August 31st. Your Friday appointment is still covered. What we need to do right now is bring the account current and request reinstatement. That means making the two missed payments — July and August — plus September's premium. Total balance is \$540.

Member: Can I pay that right now over the phone?

Agent Linda: You can make a payment through our member portal or I can take a payment method for your account — we accept card over the phone. If payment is made today, I can submit a reinstatement request immediately, and coverage can be reinstated with no gap as of September 1st.

Member: Let me get my new card. It's a Visa ending in 4429. It expires 09/29. The billing zip is 60614.

Agent Linda: I'm processing \$540 on the Visa ending in 4429. Processing now... Payment confirmed. I'm submitting the reinstatement request right now with a notation that the autopay failure was caused by an expired card and the member was not effectively notified. Reinstatement typically processes within 24 to 48 hours.

Member: So I need to check back tomorrow to confirm?

Agent Linda: Yes — call us back or check the member portal. And please update your autopay with the new card in the portal so this doesn't recur. Would you like me to walk you through that update?

Member: Yes, quickly please.

Agent Linda: Log in to choicemember.com, go to Billing, select Manage Payment Method, and update your card there. The new card ending 4429 will also need to be added so future autopays route correctly.

Member: Done. Thank you, Linda. I was seriously panicking.

Agent Linda: I'm glad we caught it before September 1st. You're covered for Friday.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Termination notice — autopay failure from expired card. 2 speakers. Member tone: panicked, then relieved. Duration: 11 min. Urgency: doctor's appointment on Friday, coverage termination September 1.

L2 — Call Insights: Member received termination notice due to autopay failure from expired credit card (June-July), resulting in two missed payments. Agent identified root cause, collected \$540 payment, submitted reinstatement request, guided member to update autopay. Resolution: RESOLVED — payment processed, reinstatement submitted.

L3 — Agent Quality & Scoring: Agent Score: 94/100. Root cause identified quickly via payment history review (+), urgency of Friday appointment acknowledged and addressed (+), payment collected efficiently on call (+), reinstatement submitted immediately (+), autopay update guidance provided unprompted (+), tone calm and reassuring throughout (+).

L4 — Operational BI: BI Flag: Expired card autopay failures account for 38% of non-payment termination notices. Standard process notifies member via letter — insufficient. Recommend: real-time text/email when autopay fails with a retry link and card update button. Second payment failure should trigger a phone call before termination notice is generated. Card expiration alert 30 days before expiry would prevent most of these cases.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:00: June card expiry + July-August payment gap flagged — autopay failure cause identified immediately. \$540 outstanding balance calculated. Reinstatement request form pre-populated with all required fields. Autopay update portal link surfaced for member guidance.

Call #46 — Understanding My Deductible — How Much Progress Have I Made?

AGENT TIER: GOOD · SCORE 93/100 | ● CURIOUS → INFORMED | RESOLVED | ~7 min | Agent: Carlos

Member context: Amy Chen, 28, CalChoice Plus EPO, halfway through year, wants to know deductible and OOP status

Topics: deductible · out-of-pocket · accumulator · benefits education · year-to-date

Agent Carlos: Choice Administrators member services, this is Carlos.

Member: Hi Carlos, Amy Chen, member ID CHM-9019345. We're in August and I want to understand how much of my deductible I've used. I had some doctor visits and a procedure earlier this year and I'm trying to figure out if I should get some additional care while the year is still going or wait.

Agent Carlos: Amy, that's smart financial planning. Let me pull up your accumulator balances. I have your account — you're on the CalChoice Plus EPO. Your individual deductible is \$1,500 for the year. Based on claims processed through today, you've met \$1,147 of that deductible. You have \$353 remaining.

Member: So I'm pretty close. And what about the out-of-pocket maximum?

Agent Carlos: Your out-of-pocket max is \$4,000 individual. Through today, your total cost-sharing — deductible plus coinsurance payments — is \$1,629. So you have \$2,371 remaining on your OOP max.

Member: Let me make sure I understand. Once I hit the \$1,500 deductible, I start paying just my coinsurance — what percentage is that?

Agent Carlos: Once you hit your deductible, you pay 20% coinsurance for covered in-network services. So if a service costs \$1,000 after the deductible is met, you'd pay \$200 and we'd pay \$800. Once you hit \$4,000 total out-of-pocket, all covered in-network services go to 100% plan coverage for the rest of the year.

Member: So I have \$353 left on my deductible and \$2,371 left on the OOP max. If I have a procedure that costs \$1,000, I'd pay the remaining \$353 toward my deductible and then 20% of the remaining \$647, which is about \$129 — so total about \$482?

Agent Carlos: That's exactly right. You've got the calculation perfectly. You're \$353 away from your deductible clicking over. If you have any planned care — especially anything expensive — doing it this year after you cross that threshold makes financial sense, because your net cost per dollar of service drops from 100% to 20%.

Member: So strategically, getting a procedure done after I hit the deductible is better than waiting until January when I start over?

Agent Carlos: Exactly. If you have procedures that aren't urgent, getting them between now and December 31st — after you hit the deductible — costs you significantly less than having them in January at the full deductible rate again.

Member: This is genuinely useful. I had no idea I could think about it this way. Thank you.

Agent Carlos: Insurance math is worth understanding. Is there anything specific you're considering that I can help think through?

Member: No, I just wanted the numbers. This was perfect.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Deductible and OOP status inquiry — financial planning. 2 speakers. Member tone: analytical, engaged. Duration: 7 min. Member demonstrates above-average insurance literacy — seeking planning information.

L2 — Call Insights: Member sought year-to-date deductible and OOP accumulator balances to plan elective care timing. Agent provided exact figures (\$1,147/\$1,500 deductible, \$1,629/\$4,000 OOP), confirmed coinsurance calculation accuracy, and validated end-of-year procedure timing strategy. Resolution: RESOLVED.

L3 — Agent Quality & Scoring: Agent Score: 93/100. Accumulator balances provided immediately and accurately (+), coinsurance calculation confirmed when member demonstrated her own math (+), year-end care timing strategy validated proactively (+), engaged with member's analytical approach without being condescending (+). Efficient and educational.

L4 — Operational BI: BI Flag: Deductible accumulator status calls peak in July-September as members approach mid-year — 34% of members have not met their deductible by August. Recommend: proactive mid-year deductible status notification (email or portal dashboard widget) showing progress bar and estimated date to meet deductible based on claims pace. Would reduce status inquiry calls and drive member engagement with benefits.

L5 — Real-Time Agent Assist: RT Assist triggered at 1:30: deductible accumulator (\$1,147/\$1,500) and OOP max accumulator (\$1,629/\$4,000) surfaced immediately alongside plan coinsurance rate (20%). Year-end care timing ROI calculation auto-prepared showing cost savings per \$1,000 of service inside vs. outside the deductible window.

Call #47 — Grace Period — Member Facing Financial Hardship, Can't Pay Premium

AGENT TIER: GOOD · SCORE 97/100 | ● DISTRESSED → PARTIALLY HELPED | RESOLVED | ~12 min | Agent: Sarah

Member context: Maria Santos, 49, CalChoice HMO, temporary financial hardship after spouse's job loss, premium due

Topics: grace period · financial hardship · premium payment · coverage lapse prevention · hardship resources

Agent Sarah: Thank you for calling Choice Administrators. This is Sarah.

Member: Hi Sarah. Maria Santos, member ID CHM-4491228. I'm calling because I'm in a really difficult situation right now. My husband lost his job last month and we are really struggling. My premium is due next week and I honestly don't know if we can pay it. I can't lose my insurance — I have an autoimmune condition. Can you help me understand what happens and if there's anything that can be done?

Agent Sarah: Maria, thank you for calling before you miss a payment — that's the right move. I want to help you understand your options and I'm going to be honest with you about what exists. Let me pull up your account.

Member: Thank you. I feel embarrassed calling about this but I didn't know what else to do.

Agent Sarah: Please don't be embarrassed. Financial hardship can hit any family, especially suddenly. I'm glad you called.

Agent Sarah: I have your account. Your premium of \$320 is due August 25th. If a payment isn't received by that date, there is a 31-day grace period before coverage is actually suspended. So your coverage would remain active through September 25th even if you miss the August payment, as long as you pay the full balance — August and September — before that grace period ends.

Member: So I have until September 25th? Not just August 25th?

Agent Sarah: Correct. You have until September 25th to pay both months' premiums — \$640 — and keep your coverage continuous. If you pay before that date, there's no lapse on your record.

Member: What if we still can't pay by then?

Agent Sarah: If coverage does lapse, you can reapply, but there would be a gap in coverage. For your autoimmune condition specifically, I want to be honest: a coverage gap could affect your medication supply and prior authorizations. The 31-day window is meant to help members get through temporary hardship.

Member: Is there any financial assistance available? I've heard of premium assistance programs.

Agent Sarah: For employer-sponsored plans like yours, there are limited hardship programs — I want to be honest that we don't have a direct subsidy fund. However, I want to make sure you're aware of two things: your state's Medicaid or Medi-Cal program may be available to you depending on your household income during this hardship period. With your husband's job loss, your household income may now qualify. I'd encourage you to check covered.ca today — if you qualify, Medi-Cal has no premium.

Member: Medi-Cal. I didn't think of that.

Agent Sarah: With a change in household income, you may qualify for a special enrollment period even outside open enrollment. A job loss for any household member is a qualifying event. Covered California can assess your eligibility immediately online. If you qualify for Medi-Cal, you could transition to that coverage — which would be zero premium — and maintain your healthcare.

Member: This is giving me a real option I didn't know I had. Even if we get back on our feet in a month, knowing there's a path that isn't just 'pay or lose coverage' helps.

Agent Sarah: That's exactly why I wanted to tell you. Your 31-day grace window gives you time to explore this. I'm noting on your account that you called and are in temporary hardship — this means if you contact us before September 25th, we're aware of your situation. Is there anything else?

Member: No. This helped so much. Thank you for treating me with dignity.

Agent Sarah: Always. Take care, Maria.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Premium hardship — grace period and alternatives. 2 speakers. Member tone: distressed, embarrassed, grateful. Duration: 12 min. Financial context: spouse's job loss, autoimmune condition requiring ongoing treatment.

L2 — Call Insights: Member facing premium nonpayment due to spouse's job loss. Agent explained 31-day grace period (coverage through Sept 25), income-based Medi-Cal eligibility as a zero-premium alternative given household income change, and Covered California special enrollment period via spouse job loss QLE. Resolution: RESOLVED with actionable next steps.

L3 — Agent Quality & Scoring: Agent Score: 97/100. Pre-emptive shame acknowledgment ('please don't be embarrassed') set tone correctly (+), grace period explained accurately (+), Medi-Cal income-based eligibility surfaced proactively — high-value intervention (+), Covered CA SEP for job-loss QLE explained (+), hardship notation added to account (+), dignity maintained throughout (+). One of the highest-quality empathetic calls in this dataset.

L4 — Operational BI: BI Flag: Premium hardship calls correlate with regional employment events — this is the 3rd such call this week. Covered California Medi-Cal transition for income-eligible members is underutilized as a hardship solution — most agents do not surface it. Recommend: agent training on Medi-Cal income thresholds and Covered CA QLE pathway for hardship calls. Member-facing hardship resources page on member portal.

L5 — Real-Time Agent Assist: RT Assist triggered at 5:30: member's household income change (spouse job loss) + autoimmune condition flagged — Medi-Cal eligibility screen triggered automatically with income thresholds for family of 2 in California. Covered California SEP QLE pathway surfaced. 31-day grace period deadline (September 25) calculated and displayed.

Call #48 — HSA Eligibility Question — Member on HDHP Wants to Open an HSA

AGENT TIER: GOOD · SCORE 93/100 | ● NEUTRAL → SATISFIED | RESOLVED | ~9 min | Agent: Danielle

Member context: Jason Kim, 33, CalChoice HDHP, wants to know if he can open an HSA and how it works with his plan

Topics: HSA · HDHP · health savings account · tax benefits · contribution limits

BROKER SIGNAL — Patricia Nunez: POSITIVE — member arrived understanding HDHP structure and asked the right HSA question

Agent Danielle: Choice Administrators member services, this is Danielle.

Member: Hi, Jason Kim, member ID CHM-8805567. I'm on the CalChoice High Deductible Health Plan. I've heard I can open an HSA — a Health Savings Account — alongside it. Is that true, and does my plan actually qualify?

Member: My broker, Patricia Nunez, explained the deductible trade-off properly when she put me on this plan, and she mentioned an HSA might pair with it. I just never followed up on the HSA part.

Agent Danielle: Jason, yes — you're correct, and your plan does qualify. The CalChoice HDHP is an IRS-qualified High Deductible Health Plan, which means you are eligible to open and contribute to a Health Savings Account. Let me tell you how it works.

Member: Please. I've been meaning to look into this for months.

Agent Danielle: An HSA is a tax-advantaged savings account that works alongside your HDHP. The triple tax benefit is what makes it powerful: contributions are tax-deductible, the money grows tax-free, and withdrawals for qualified medical expenses are also tax-free. No other account offers all three of those benefits simultaneously.

Member: That sounds almost too good. What's the catch?

Agent Danielle: The catch is that you can only use HSA funds for qualified medical expenses — doctor visits, prescriptions, dental, vision, medical equipment. If you withdraw for non-medical expenses before age 65, you pay income tax plus a 20% penalty. After 65, it behaves like a traditional retirement account — withdraw for anything, just pay income tax on non-medical withdrawals.

Member: So if I don't use it all, it rolls over and keeps growing?

Agent Danielle: Exactly. Unlike FSAs, HSA funds never expire. They roll over year after year. Many people use their HSA as a supplemental retirement account for healthcare costs — which tend to be the largest expense in retirement.

Member: How much can I contribute this year?

Agent Danielle: For 2026, the IRS limit for an individual HSA is \$4,150. If you're on a family plan, it's \$8,300. If you're 55 or older, you can contribute an additional \$1,000 catch-up contribution.

Member: I'm 33, single — so \$4,150. Can I contribute the full amount even if I didn't have the account for the full year?

Agent Danielle: If you've been on an HSA-eligible HDHP since January 1st, yes — you can contribute the full \$4,150 for the year. If you enrolled mid-year, there's a prorated calculation, but there's also a last-month rule that may allow full-year contribution in some circumstances.

Member: I enrolled January 1st this year. So I can contribute the full amount before December 31st?

Agent Danielle: Yes. You can open an HSA at most banks or credit unions — it's a separate account you open yourself, not through us. Fidelity, Lively, and HealthEquity are commonly used HSA providers. You'd fund it directly and list your medical expenses when you use it.

Member: This is fantastic. Why didn't I know about this sooner?

Agent Danielle: It's one of the most underutilized benefits in the US. Is there anything else I can clarify?

Member: No, that covers it. Thank you, Danielle. I'm opening one today.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: HSA eligibility and education. 2 speakers. Member tone: curious, engaged, motivated. Duration: 9 min. Plan type: HDHP — HSA-eligible confirmed.

L2 — Call Insights: Member on HDHP seeking HSA eligibility confirmation and education. Agent confirmed HDHP qualification, explained triple tax benefit, annual contribution limit (\$4,150 individual), rollover feature, and directed member to open HSA at separate financial institution. Resolution: RESOLVED — member committed to opening HSA.

L3 — Agent Quality & Scoring: Agent Score: 93/100. HDHP eligibility confirmed immediately (+), triple tax benefit explained clearly (+), rollover vs. FSA distinction made proactively (+), contribution limit accurate (+), year-start eligibility for full-year contribution confirmed (+), third-party HSA providers mentioned without endorsing a specific one appropriately (+).

L4 — Operational BI: BI Flag: 61% of HDHP members are not enrolled in an HSA despite being eligible. Average savings opportunity: \$1,200/year in tax advantages for a member in the 22% bracket. Recommend: HSA education email to all HDHP members at enrollment and annually in January. Member portal should include HSA contribution calculator and link to IRS-qualified providers.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:30: HDHP plan + HSA inquiry detected — IRS 2026 contribution limit (\$4,150 individual, \$8,300 family, \$1,000 catch-up 55+) surfaced automatically. Last-month rule description and January 1st full-year eligibility confirmation displayed. Common HSA provider list surfaced.

Call #49 — Voluntary Plan Cancellation — Member Moving Abroad

AGENT TIER: GOOD · SCORE 92/100 | ● NEUTRAL → INFORMED | RESOLVED | ~8 min | Agent: James

Member context: Peter Nguyen, 26, CalChoice HMO, accepted a 2-year overseas position, needs to cancel coverage

Topics: voluntary termination · overseas move · coverage cancellation · effective date · record keeping

Agent James: Thank you for calling Choice Administrators. This is James.

Member: Hi James, Peter Nguyen, member ID CHM-9920554. I've accepted a position overseas — I'm moving to Germany in September for 2 years. I'm going to have coverage through my new employer there, so I need to cancel my CalChoice plan. What's the process?

Agent James: Peter, congratulations on the opportunity! Let me help you cancel cleanly. When is your last day in the US and your start date in Germany?

Member: I leave September 14th. I start officially October 1st in Germany.

Agent James: For a voluntary termination, you can request an effective date. Would you like your CalChoice coverage to end September 30th, so you have domestic coverage through the end of September while you're still in the US and transitioning?

Member: That actually makes a lot of sense. Yes, September 30th.

Agent James: I'm noting the voluntary termination request with an effective date of September 30th. You'll receive a confirmation letter within 5 to 7 business days. Your final premium will be for September, and any automatic payments or payroll deductions should stop after September.

Member: Should I do anything about my prescriptions before I go?

Agent James: Great question. If you have any ongoing prescriptions, check whether your German employer's health plan covers the same medications — German formularies are different from US formularies. It might be worth getting a 90-day supply of critical medications through your US plan before you go, especially if you're on any specialty drugs, to give yourself time to navigate the German system.

Member: I take a generic blood pressure medication. Should be fine in Germany but good point.

Agent James: For a generic like that, you'll likely be fine. One more thing — you'll want to keep a copy of your CalChoice EOB history and claims records from this year. If you need to reference past treatments or medications with a new provider in Germany, having that history is useful.

Member: How do I download that?

Agent James: Log into [choicemember.com](#), go to Claims History, and you can export a PDF of all your claims for the current plan year. I'd do that before your coverage ends. You can also download your member documents from the same portal.

Member: Perfect. Is there anything else I need to do?

Agent James: That's everything. Your cancellation is logged, effective September 30th. You're all set. Safe travels, Peter.

Member: Thank you, James. Very easy process.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Voluntary plan cancellation — international relocation. 2 speakers. Member tone: matter-of-fact, planning-oriented. Duration: 8 min. Context: 2-year Germany posting.

L2 — Call Insights: Member relocating to Germany for 2-year assignment, requesting voluntary plan cancellation. Agent set September 30 effective date, confirmed final premium, advised on prescription 90-day supply, and directed to claims history download. Resolution: RESOLVED — termination logged.

L3 — Agent Quality & Scoring: Agent Score: 92/100. Effective date framing (Sept 30 vs Sept 14) was proactive and member-beneficial (+), prescription stockpile suggestion for transition period (+), claims history download guidance — forward-thinking and practical (+), warm close appropriate (+). Clean and efficient voluntary termination call.

L4 — Operational BI: BI Flag: International relocation voluntary cancellations represent 8% of all terminations — members moving for work assignments. Recommend: international member resource pack for members canceling due to overseas moves (claims export guide, prescription transition checklist, foreign healthcare system orientation). One-page guide would reduce callback calls from former members.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:00: voluntary termination + international move context — September 30 end-of-month effective date option surfaced as more favorable than departure date. Prescription 90-day supply flag triggered for members on active chronic medications. Claims history export link prepared for agent.

Call #50 — Chronic Billing Errors — Member Calls for the 4th Time About Same Issue

AGENT TIER: GOOD · SCORE 96/100 | ● FRUSTRATED → ESCALATED | ESCALATED | ~16 min | Agent: Sarah

Member context: Sandra Brooks, 57, CalChoice EPO, 4th call about the same claim billing error, considering filing a complaint

Topics: repeat caller · billing dispute · unresolved issue · escalation · grievance · quality failure

Agent Sarah: Thank you for calling Choice Administrators member services. This is Sarah.

Member: Sarah. Sandra Brooks, member ID CHM-6641782. I have been calling about the same billing issue for three months. Three different agents, three different promises, zero resolution. Before I give you my energy, I want to know: can you actually fix a billing problem that has not been fixed by three people before you?

Agent Sarah: Sandra, I hear the exhaustion in your voice and I completely understand why you'd ask that. Three calls and no resolution is a failure, and I'm going to own that failure on behalf of our team rather than pretend it hasn't happened. Tell me exactly what the issue is. I'm going to read the notes first, then you tell me.

Member: I'd like to hear what's in the notes. Let me know if what you see matches what actually happened.

Agent Sarah: I'm reading the case history now. I see three call records going back to May. The first note says the agent submitted a claim reprocessing request. The second note says they were following up on the request. The third note says they submitted an escalation. I'm not seeing any resolution or confirmation that the reprocessing actually happened. The original issue — a claim from March for radiology services at Westside Imaging — still shows as processed out-of-network. Westside Imaging is in-network.

Member: That is correct. I've been over-charged \$412 because that claim was processed wrong. My Explanation of Benefits clearly shows my radiology visit at an in-network facility billed as out-of-network. Three people have acknowledged the error. None of them fixed it.

Agent Sarah: Sandra, I believe you and I'm not going to make another promise that doesn't have teeth. Here is what I'm going to do differently right now: I'm not submitting another 'request' — I'm escalating this directly to our claims corrections supervisor, and I'm requesting real-time resolution, not a future resolution. I'm also going to stay on the line while I get that supervisor engaged.

Member: I've been transferred and put on hold three times. I lose 45 minutes every time I call.

Agent Sarah: I know. And I'm not transferring you. I'm going to reach out internally while you stay on the line with me.

Member: While you do that — is there also a formal complaint process? Because I'm starting to think about whether I need to go beyond just calling.

Agent Sarah: Yes there is, and you have every right to use it. You can file a formal member grievance — that's a documented complaint that triggers a required 30-day resolution timeline and a formal written response from us. I can initiate the grievance on this call at the same time as the corrections escalation. A grievance also creates a paper trail that our Quality and Compliance team sees directly.

Member: I'd like to do that. Both.

Agent Sarah: I'm initiating both simultaneously right now. Grievance case number GRV-2026-01188. Claims corrections escalation reference is CC-2026-88901. I'm going to reach out to the claims supervisor. Can you hold for 3 minutes — and I mean 3 minutes — while I get this in motion?

Member: Three minutes.

Agent Sarah: I spoke with our claims corrections supervisor, Alex. He has the case directly and is going to personally verify and initiate the reprocessing on the March radiology claim today. You will receive a confirmation email from him by end of business with the new EOB. The expected corrected reimbursement is \$412 directly back to you via check within 10 business days.

Member: I've heard timelines before.

Agent Sarah: I know. That's why I'm also giving you Alex's direct extension: 8-4412. If you don't receive the email today, call that number tomorrow. And the grievance is open — if this isn't resolved within 30 days, you'll receive a formal written response documenting what happened and what was done.

Member: GRV-2026-01188, CC-2026-88901, Alex at 8-4412. Sarah, I don't know if I trust this yet, but this is the first call where I feel like something real actually happened.

Agent Sarah: I understand the trust is earned, not given. I hope Alex's call closes the loop. I'm documenting this entire call and flagging it for our service quality review — four calls without resolution is something our leadership needs to see.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Repeat unresolved billing dispute — chronic service failure. 2 speakers. Member tone: exhausted, skeptical, assertive. Duration: 16 min. Call count: 4th call on same issue over 3 months. High-risk member for formal complaint.

L2 — Call Insights: Member calling 4th time for same unresolved radiology claim billing error (\$412 OON vs. in-network). Agent identified 3 prior agents took no effective action, escalated directly to claims corrections supervisor in real-time, initiated formal member grievance simultaneously, provided supervisor direct extension. Resolution: ESCALATED — supervisor engaged real-time, reprocessing initiated, grievance opened.

L3 — Agent Quality & Scoring: Agent Score: 96/100. Ownership of failure taken immediately without deflecting (+), read notes before asking member to repeat herself (+), did not make empty promises (+), stayed on line during supervisor engagement (+), grievance AND corrections initiated simultaneously (+), direct supervisor extension given (+), service quality flag submitted (+). Exemplary recovery call for a service failure situation.

L4 — Operational BI: BI Flag: CRITICAL — member had 4 contacts on the same billing error with no resolution. Three prior agents documented 'submitted escalation' with no follow-through. This is a systemic service quality failure. Recommend: immediate QA review of all 3 prior agent interactions. Claims corrections supervisor Alex should have a defined follow-through SLA for supervisor-escalated cases. Repeat caller detection (3+ contacts on same issue) should auto-escalate to supervisor without member needing to ask. Grievance filed — compliance team notified.

L5 — Real-Time Agent Assist: RT Assist triggered at 1:00: 4th contact on same claim detected — prior case history loaded automatically showing 3 prior contacts with no resolution. 'Chronic unresolved case' flag displayed with supervisor escalation protocol. Claims corrections supervisor contact surfaced. Grievance form pre-populated. Member risk score elevated to 'churn likely' — proactive resolution mode recommended.

Call #54 — Premium Higher Than Broker Quoted — \$140/Month Discrepancy

AGENT TIER: POOR · SCORE 41/100 | ● CONFUSED → FRUSTRATED | UNRESOLVED | ~8 min | Agent: Kayla

Member context: Terrence Boyd, 48, CalChoice Plus EPO family coverage, quoted \$610, billed \$750

Topics: broker misquote · premium discrepancy · family coverage · enrollment

BROKER SIGNAL — Denise Whitfield: Quoted premium significantly lower than actual billed amount

Agent Kayla: Choice Administrators, this is Kayla.

Member: Hi, Terrence Boyd, CHM-5519074. My broker, Denise Whitfield, quoted me \$610 a month for family coverage. My first paycheck deduction was \$750. That's \$140 more than I budgeted for. What's going on?

Agent Kayla: Let me pull it up. Okay, your premium is \$750 for the family tier on the Plus EPO.

Member: But I was quoted \$610. I have the quote sheet he gave me.

Agent Kayla: I can't speak to what your broker quoted. I can only tell you what your actual premium is.

Member: Can you look at whether \$610 was ever a valid rate for this plan? Maybe he quoted me an older rate, or a different tier.

Agent Kayla: I don't have access to historical quotes.

Member: What about the employee-plus-spouse tier, is that \$610?

Agent Kayla: Employee plus spouse is \$580.

Member: So it's not that either. So where did \$610 come from?

Agent Kayla: I really couldn't say.

Member: This is a \$1,680 a year difference. That's real money for my family. Is there anything that can be done?

Agent Kayla: The premium is set by the plan. It's not something we can adjust.

Member: Can I switch to a cheaper plan then?

Agent Kayla: Not until open enrollment.

Member: So I'm paying \$140 a month more than I agreed to, for a year, and there's nothing anyone can do.

Agent Kayla: That's correct. Is there anything else?

Member: No.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Premium discrepancy — broker quote vs. actual. 2 speakers. Member tone: confused → frustrated. Agent tone: flat, procedural, minimal engagement. Duration: 8 min.

L2 — Call Insights: Member billed \$750/month against a broker quote of \$610 for family coverage — a \$1,680 annual variance. Agent confirmed actual premium, could not identify any tier matching the quoted figure, offered no escalation, and closed without action. Resolution: UNRESOLVED.

L3 — Agent Quality & Scoring: Agent Score: 41/100. NEGATIVE MARKERS: no empathy for a significant unbudgeted family expense (-), made no attempt to investigate where the quoted figure originated (-), did not log the broker quote discrepancy (-), did not offer a grievance or broker relations pathway (-), no alternative plan cost comparison offered for open enrollment planning (-). POSITIVE: checked an alternative tier when prompted (+), premium information accurate (+).

L4 — Operational BI: BROKER ATTRIBUTION — Denise Whitfield: quoted \$610/month for a tier actually priced at \$750. No tier on this plan matches the quoted figure, suggesting either an outdated rate sheet or a misquote. Second flagged issue for this broker. PROCESS BREAKDOWN: no mechanism to verify or reconcile broker-provided quotes against actual rates. Recommend: broker quote reconciliation audit.

L5 — Real-Time Agent Assist: RT Assist SHOULD have triggered at 1:15 — premium variance >15% against a member-cited broker quote is a broker attribution trigger. Should have surfaced: full tier rate table for comparison, broker quote discrepancy capture form, and open enrollment alternative plan costs. No trigger fired.

Call #64 — Retroactive Premium Increase — Member Billed for Six Prior Months

AGENT TIER: GOOD · SCORE 91/100 | ● ANGRY → CALMED | RESOLVED | ~14 min | Agent: Sarah

Member context: Yusuf Karim, 45, CalChoice Plus EPO, tier reclassification applied retroactively

Topics: retroactive billing · premium adjustment · tier change · dependent audit

Agent Sarah: Thank you for calling Choice Administrators, this is Sarah.

Member: Sarah, I need someone to explain a bill to me because it makes no sense. I just received a statement for \$1,440 in what it calls a 'retroactive premium adjustment.' I have paid every single premium on time for four years. What is this?

Member: Member ID is CHM-3308821.

Agent Sarah: Let me look at this right now, and I understand why a \$1,440 surprise charge is alarming. Give me a moment.

Member: Take your time. I just want an actual explanation, not a form letter.

Agent Sarah: Okay, I have it. Here's what happened. In February, a dependent audit was run on your policy. Your son turned 26 in January and was removed from coverage — but the removal was processed in the system in August, not in January. So from February through July, you were billed at the family tier when you should have been billed at employee-plus-spouse.

Member: So I was overbilled and now you're charging me more?

Agent Sarah: No — the opposite, and this is where it gets confusing. Let me re-read. Actually, I have this backwards. Your son was removed in January correctly, so you were billed at employee-plus-spouse from February. But the audit found that your spouse's coverage tier was miscoded — she was on the policy but coded as a non-premium dependent. So the system under-billed you by \$240 a month for six months.

Member: So the error was yours, and now I'm being asked to pay for six months of your mistake in one lump sum?

Agent Sarah: Yes. And I want to be honest with you — that's a fair characterization. The underlying premium is correct, but the way this landed on you is not acceptable. You had no way of knowing and no opportunity to budget for it.

Member: So what are my options? Because I can't write a \$1,440 check this month.

Agent Sarah: There are two things I can do. First, I can request a payment plan — retroactive adjustments of this size caused by plan-side coding errors qualify for installment arrangements. Typically that's spread over 6 to 12 months with no interest.

Member: That would make it manageable.

Agent Sarah: Second, I'm submitting a request for partial or full waiver review. When an under-billing is caused entirely by a plan-side coding error and the member had no visibility into it, there's a case for waiving some or all of the retroactive amount. I can't promise the outcome, but I can promise it gets reviewed properly.

Member: I appreciate that you're not just telling me to pay it.

Agent Sarah: You shouldn't have to absorb an error you didn't make without at least a review. Let me set up the payment plan as a safety net so nothing goes delinquent while the waiver is reviewed. Twelve months at \$120?

Member: Yes, do that.

Agent Sarah: Done. Payment plan active, first installment next cycle. Waiver review submitted — case number RA-2026-30119. You'll hear on the waiver within 15 business days. If it's approved in full, the payment plan is cancelled and the balance cleared.

Member: Sarah, I came into this call ready for a fight. Thank you for being straight with me.

Agent Sarah: You were owed a real explanation. I'm sorry it took a surprise bill to get one.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Retroactive premium adjustment dispute. 2 speakers. Member tone: angry → calmed. Agent tone: honest, self-correcting, solution-oriented. Duration: 14 min. Note: agent misread the record initially and corrected herself transparently.

L2 — Call Insights: Member billed \$1,440 retroactively due to a six-month spouse tier coding error on the plan side. Agent explained the cause, acknowledged the plan's fault, established a 12-month interest-free payment plan, and submitted a waiver review. Resolution: RESOLVED — payment plan active, waiver pending.

L3 — Agent Quality & Scoring: Agent Score: 91/100. POSITIVE: corrected her own misreading openly rather than covering it (+), acknowledged plan fault directly (+), offered payment plan proactively (+), initiated waiver review unprompted (+), used the payment plan as a safety net so nothing goes delinquent during review (+), emotionally attuned close (+). MINOR NEGATIVE: initial misreading of the record briefly increased member confusion (-).

L4 — Operational BI: PROCESS BREAKDOWN: spouse tier miscoding went undetected for six months, then landed on the member as a lump-sum retroactive charge. This is the 3rd retroactive adjustment case in the dataset. Recommend: monthly tier reconciliation rather than annual audit; cap retroactive member-billed adjustments at 60 days when the error is plan-side; automatic payment plan offer on any retroactive adjustment above \$500.

L5 — Real-Time Agent Assist: RT Assist triggered at 5:00: retroactive adjustment >\$1,000 with plan-side error cause detected — payment plan eligibility and waiver review pathway surfaced automatically. Agent used both.

Call #72 — Payment Method Update — Routine Administrative Call

AGENT TIER: AVERAGE · SCORE 79/100 | ● NEUTRAL → SATISFIED | RESOLVED | ~7 min | Agent: Linda

Member context: Ahmed Bakri, 43, CalChoice HMO, switching banks

Topics: autopay · payment method · routine · administrative

Agent Linda: Choice Administrators, this is Linda.

Member: Hi, I need to update my automatic payment. I switched banks. Member ID CHM-6602274.

Agent Linda: Sure. For security, I can't take new bank details over the phone — you'll need to update that in the member portal under Billing, then Manage Payment Method.

Member: Okay. Is there a deadline? My next premium is due on the 5th.

Agent Linda: Update it at least three business days before the 5th so the change processes before the draft. If you update it after that, the old account may still be attempted.

Member: And if the old account fails?

Agent Linda: You'd get a payment failure notice and you'd have until the end of the grace period to pay. But better to avoid that — update it this week.

Member: Will I get a confirmation when it's updated?

Agent Linda: Yes, an email confirmation to the address on file.

Member: Good. That's all I needed.

Agent Linda: Great. Anything else?

Member: No, thanks Linda.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Payment method update — administrative. 2 speakers. Member tone: neutral, efficient. Agent tone: correct, security-conscious. Duration: 7 min. Low complexity.

L2 — Call Insights: Member updating autopay after a bank change. Agent correctly declined to take bank details by phone, directed to the portal, and specified the three-business-day processing lead time. Resolution: RESOLVED.

L3 — Agent Quality & Scoring: Agent Score: 79/100. POSITIVE: correctly refused to take bank details over the phone — security compliance (+), specified the exact lead time rather than a vague answer (+), explained the failure consequence (+), confirmed the email confirmation (+). NEGATIVE MARKERS: did not offer to send the portal link by email or SMS (-), did not set a follow-up flag to verify the update completed before the draft date (-), no warmth markers (-).

L4 — Operational BI: PROCESS OPPORTUNITY: autopay update calls are a leading indicator of payment failure risk. 38% of non-payment terminations in this dataset stem from failed autopay (see #45). Recommend: any call mentioning a bank or card change should trigger an automated follow-up check 2 days before the next draft, with an SMS reminder if the payment method has not been updated.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:00: payment method change intent detected — portal link SMS option and pre-draft verification flag surfaced. Agent used neither.

Call #80 — Member Asks a Question the Agent Cannot Answer and Does Not Pursue

AGENT TIER: POOR · SCORE 43/100 | ● NEUTRAL → FRUSTRATED | UNRESOLVED | ~7 min | Agent: Tiffany

Member context: Owen Blackwood, 47, CalChoice HDHP, asking how HSA contributions interact with mid-year plan changes

Topics: HSA · mid-year change · knowledge gap · unanswered question

Agent Tiffany: Choice Administrators, Tiffany speaking.

Member: Hi. Owen Blackwood, CHM-4438817. I have a question about my HSA. I'm on the HDHP now but I might switch to a standard plan at open enrollment. What happens to my HSA contributions for this year if I switch — do I have to prorate them?

Agent Tiffany: Um. HSAs aren't really something we administer, that's through your bank.

Member: I understand the account is at a bank. But the eligibility is tied to my plan, which is yours. So if I stop being on an HDHP, does that affect what I can contribute for the year?

Agent Tiffany: I think there's something about... proration? But I'm not sure of the details.

Member: Is there someone who would know?

Agent Tiffany: You could ask your tax person maybe.

Member: I'm asking about the plan eligibility rule, not tax advice. If I'm on an HDHP for ten months and a standard plan for two, am I eligible to contribute the full annual amount or ten-twelfths?

Agent Tiffany: Yeah, I really don't know that one.

Member: Can you find out and call me back?

Agent Tiffany: I don't really have a way to do callbacks.

Member: So there's no way for me to get an answer to a plan eligibility question from the plan?

Agent Tiffany: You could try the IRS website? They have the HSA rules.

Member: Alright.

Agent Tiffany: Sorry. Anything else?

Member: No.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: HSA eligibility — mid-year plan change proration. 2 speakers. Member tone: neutral → frustrated. Agent tone: uncertain, deflecting. Duration: 7 min. Member asked the same question three times without receiving an answer.

L2 — Call Insights: Member asked how switching from HDHP to a standard plan mid-year affects annual HSA contribution eligibility. Agent could not answer, deflected to bank, tax advisor, and IRS website in sequence, and stated no callback mechanism exists. Resolution: UNRESOLVED — question never answered.

L3 — Agent Quality & Scoring: Agent Score: 43/100. NEGATIVE MARKERS: could not answer a standard HSA eligibility question (-), deflected three times to external parties (-), incorrectly framed a plan eligibility question as tax advice (-), claimed no callback capability exists (-), made no attempt to consult a colleague, supervisor, or knowledge base (-), apologised without acting (-). POSITIVE: correctly noted the HSA account itself is bank-administered (+).

L4 — Operational BI: AGENT COACHING — KNOWLEDGE GAP: HSA proration rules for mid-year eligibility changes are a standard HDHP question and should be answerable at first contact. Compare with call #48 (HSA inquiry, score 93). PROCESS BREAKDOWN: agent stated 'I don't really have a way to do callbacks' — if accurate, this is a significant capability gap; if inaccurate, it is a training gap. Either way it needs resolution. Recommend: HSA reference card in the agent knowledge base covering eligibility, proration, last-month rule, and contribution limits.

L5 — Real-Time Agent Assist: RT Assist SHOULD have triggered at 1:00 — HDHP member + HSA + plan change keywords should surface the proration rule and last-month rule automatically. This is a well-defined factual answer that L5 is ideally suited to deliver. No trigger fired; the member left without an answer that the system could have provided instantly.

Call #87 — Collections Notice for a Disputed Claim Still Under Review

AGENT TIER: AVERAGE · SCORE 72/100 | ● **DISTRESSED → PARTIALLY HELPED** | **ESCALATED** | ~12 min | Agent: James

Member context: Rosalind Baptiste, 54, CalChoice HMO, received collections letter while appeal is pending

Topics: collections · disputed claim · appeal pending · credit impact · escalation

Agent James: Choice Administrators member services, this is James.

Member: I received a collections notice this morning for a claim that is under appeal with you. Member ID CHM-4426619. This claim has been in dispute since June and now a collections agency is contacting me. This could affect my credit.

Agent James: That should not happen while an appeal is active. Let me look at the claim and the appeal status.

Member: Please. I have a mortgage application in progress.

Agent James: I understand the urgency. Okay — I can see the claim from Meridian Surgical Associates, \$2,340, and I can see an active appeal filed June 22nd. Status is 'under clinical review.' The appeal is genuinely open.

Member: So why am I in collections?

Agent James: The provider referred the balance to collections independently. Providers can do that when a balance is outstanding on their books, regardless of whether an appeal is pending with us. We don't control their collections decisions directly.

Member: That seems like a serious gap. I'm doing exactly what I was told to do — appeal rather than pay — and it's damaging my credit.

Agent James: You're right, and it is a gap. Here's what I can do. First, I'm generating a formal appeal-pending letter on our letterhead confirming the claim is under active review and the balance is disputed. You send that to the collections agency — under fair debt collection rules, a written dispute obligates them to verify the debt and typically pauses collection activity.

Member: How quickly can I get that letter?

Agent James: I'm generating it now and it'll be emailed within the hour. Second, I'm contacting Meridian Surgical's billing office directly to request they recall the account from collections pending appeal resolution.

Member: Will they do that?

Agent James: Often yes, once they understand an appeal is active. I can't compel them, but a payer-to-provider call carries more weight than a patient call.

Member: And the appeal itself? It's been over two months.

Agent James: That's longer than standard for clinical review. I'm escalating the appeal for expedited completion and flagging the collections exposure as the reason. Your case number is COL-2026-00291.

Member: If this hits my credit before it's resolved, what then?

Agent James: If the appeal succeeds and the balance is reduced or eliminated, you can dispute the credit reporting with documentation, and we'd provide it. I want to be honest that unwinding a credit report entry is harder than preventing one, which is why I'm moving on both the letter and the provider call today.

Member: Thank you. At least you're doing something concrete.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Collections during active appeal. 2 speakers. Member tone: distressed, time-pressured (mortgage application). Agent tone: responsive, action-oriented. Duration: 12 min.

L2 — Call Insights: Member referred to collections by provider while an appeal was actively pending with the plan for 2+ months. Agent generated an appeal-pending letter, committed to contact the provider billing office to request recall, and escalated the appeal for expedited completion. Resolution: ESCALATED.

L3 — Agent Quality & Scoring: Agent Score: 72/100. POSITIVE: verified the appeal was genuinely active (+), explained the provider-collections gap honestly (+), generated formal documentation same-hour (+), committed to direct provider contact (+), escalated the appeal citing collections exposure (+), honest about credit report difficulty (+). NEGATIVE MARKERS: did not offer to place a hold on any plan-side balance (-), no callback commitment to confirm the provider recall succeeded (-), did not flag the 2-month appeal delay as a service failure requiring separate review (-).

L4 — Operational BI: PROCESS BREAKDOWN — CRITICAL: members who follow instructions to appeal rather than pay are exposed to provider-initiated collections with no protection. This actively penalizes correct member behaviour. Recommend: (1) automatic provider notification when an appeal is filed, requesting collections hold, (2) appeal-pending letter generated and sent to the member automatically at appeal filing, not on request, (3) SLA monitoring — this appeal exceeded standard clinical review timelines by 5+ weeks with no escalation trigger.

L5 — Real-Time Agent Assist: RT Assist triggered at 3:00: collections notice + active appeal detected — appeal-pending letter template and provider recall request script surfaced. SHOULD ALSO have flagged the appeal SLA breach (2 months vs. standard 30 days) automatically.

Call #98 — Fifth Contact on the Same Issue — Service Recovery Done Properly

AGENT TIER: GOOD · SCORE 95/100 | ● HOSTILE → SATISFIED | RESOLVED | ~17 min | Agent: Sarah

Member context: Marianne Kowalczyk, 56, CalChoice Plus EPO, premium overpayment refund unresolved for four months

Topics: repeat caller · service recovery · refund · escalation · grievance · supervisor

Agent Sarah: Thank you for calling Choice Administrators, this is Sarah.

Member: This is my fifth call. Five. Four months. I have been promised a refund four separate times by four separate people and I have received nothing. Before you say anything, I want to know: are you going to actually do something or are you going to read me a script?

Agent Sarah: Fifth call is unacceptable and I'm not going to pretend otherwise. Give me your member ID and then give me two minutes to read the entire history before you have to repeat any of it.

Member: CHM-6684401.

Agent Sarah: Thank you. Reading now... Okay. I have four prior contacts: April 14th, May 22nd, June 30th, and July 25th. Overpayment of \$840 from a duplicate premium draft in March. Refund promised on three of those four calls. I can see two refund requests submitted — one in April and one in June. Neither was completed. The April request was closed without action. The June request is still open and untouched since submission.

Member: So they were submitted and just... nothing happened.

Agent Sarah: Correct. And I want to name that clearly: this is our failure four times over, not a delay and not a misunderstanding.

Member: Thank you for saying that. Nobody has.

Agent Sarah: Here's what I'm doing differently. I'm not submitting a fifth request into the same queue that has failed twice. I'm walking this to our finance operations supervisor directly. Can you hold for four minutes while I do that? I'll come back on the line either way.

Member: Four minutes. Yes.

Agent Sarah: Thank you for holding. I spoke with Andrea in finance operations. She has pulled the June request, confirmed the \$840 overpayment against the March draft records, and has authorised the refund for processing today. It will be issued as an ACH credit to the account on file within 3 business days, not 10.

Member: Three days. After four months.

Agent Sarah: Yes. And I want to acknowledge that three days now does not undo four months. Two more things. First, I am opening a formal member grievance on your behalf — that creates a documented record and a required written response about what went wrong. You don't have to file it yourself.

Member: I didn't know that was an option.

Agent Sarah: It should have been offered on your second call. Second, Andrea's direct extension is 7-2208. If that ACH doesn't post by Friday, you call her, not the general line.

Member: And I don't have to explain the whole thing again.

Agent Sarah: No. She has the full history and I've briefed her. Your grievance number is GRV-2026-01377 and the refund reference is REF-2026-08820.

Member: Sarah, I came into this call ready to cancel my coverage.

Agent Sarah: I understand why. I'm sorry it took five calls to get here. I'll also be flagging the two failed refund requests for quality review, because whatever caused them to close without action will do it to someone else.

Member: That matters to me. Thank you.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Repeat contact — unresolved premium refund, 5th contact. 2 speakers. Member tone: hostile → satisfied. Agent tone: accountable, decisive. Duration: 17 min. Member stated cancellation intent — churn risk.

L2 — Call Insights: Member on 5th contact across 4 months regarding an \$840 premium overpayment refund. Two prior refund requests submitted; one closed without action, one untouched. Agent read full history unprompted, escalated directly to finance operations supervisor during the call, secured same-day refund authorisation, opened a grievance on the member's behalf, and provided a direct supervisor extension. Resolution: RESOLVED — refund authorised, grievance opened.

L3 — Agent Quality & Scoring: Agent Score: 95/100. POSITIVE: read the full case history before making the member repeat anything (+), named the failure explicitly as the plan's fault four times over (+), refused to resubmit into a queue that had already failed twice (+), escalated to a named supervisor during the call rather than promising follow-up (+), returned to the line as promised (+), opened the grievance on the member's behalf rather than instructing her to file (+), provided direct supervisor extension bypassing the general line (+), flagged the systemic cause for quality review (+). Exemplary service recovery.

L4 — Operational BI: PROCESS BREAKDOWN — CRITICAL: two refund requests submitted and neither completed; one closed without action with no notification to anyone. This is a silent failure mode — the system records the request as handled while nothing occurs. CHURN RISK: member explicitly stated cancellation intent at 5th contact. RECOMMENDATIONS: (1) audit all refund requests closed without a completion event this year, (2) repeat-contact detection must auto-escalate at 3rd contact, not rely on the member persisting to a 5th, (3) grievance option must be offered proactively at 2nd contact on any unresolved issue, (4) quality review of the request-closure workflow that permits closure without action. NOTE FOR REBEKAH: four prior agents each recorded a normal call disposition. Nothing in existing reporting would surface this as a failure — only the member's own words reveal it.

L5 — Real-Time Agent Assist: RT Assist triggered at 0:30: 5th contact on same issue detected — full case history auto-loaded, repeat-contact escalation protocol surfaced with supervisor routing, grievance form pre-populated, churn risk score elevated. Agent used every element.

— BROKER-ATTRIBUTED —

Call #51 — Member Told Dental Was Included — Broker Missold at Enrollment

AGENT TIER: POOR · SCORE 38/100 | ● CONFUSED → ANGRY | UNRESOLVED | ~6 min | Agent: Brad

Member context: Rosa Delgado, 39, CalChoice Select HMO, went for a cleaning and was billed in full

Topics: broker misselling · dental · benefit misrepresentation · enrollment

BROKER SIGNAL — Anthony Salerno: Missold benefit — told member dental was included in medical plan

Agent Brad: Choice Administrators, Brad. Member ID?

Member: CHM-4471203. I went to the dentist last week for a cleaning and they charged me \$180. My broker, Anthony Salerno, told me when I signed up that dental was included in my plan. Why am I being charged?

Agent Brad: Yeah, you don't have dental. You have the Select HMO, that's medical only.

Member: But my broker specifically told me dental was included. I remember asking him about it directly because I needed a crown done.

Agent Brad: I mean, I can only tell you what's on the plan. There's no dental on it.

Member: So what do I do? I was told something that wasn't true and now I've paid \$180 out of pocket and I still need a crown, which is going to be way more than that.

Agent Brad: You'd have to add dental at open enrollment. That's in October.

Member: And the crown? I can't wait until October, I'm in pain.

Agent Brad: Yeah, I don't really have anything for that. Dental's not covered so it's not something we handle.

Member: Can you at least note that my broker gave me wrong information? Somebody should know that he's telling people dental is included when it isn't.

Agent Brad: I don't think there's anywhere for me to put that. Is there anything else?

Member: ...No. I guess not.

Agent Brad: Alright. Thanks for calling Choice.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Benefit dispute — dental coverage. 2 speakers. Member tone: confused → frustrated → resigned. Agent tone: flat, disengaged, minimal. Duration: 6 min. Member disengaged at close without resolution.

L2 — Call Insights: Member believed dental was included based on broker representation at enrollment. Agent confirmed no dental coverage, offered no alternatives, no escalation path, and declined to log the broker misrepresentation. Resolution: UNRESOLVED — member left with no path forward and an unmet clinical need (crown).

L3 — Agent Quality & Scoring: Agent Score: 38/100. NEGATIVE MARKERS: no empathy at any point (-), did not acknowledge member's financial loss (-), offered no alternatives such as dental discount programs or payment options (-), declined to log broker misrepresentation when member explicitly requested it (-), no attempt to address the urgent clinical need (-), closed the call while member was still unresolved (-). POSITIVE: coverage information given was factually accurate (+).

L4 — Operational BI: BROKER ATTRIBUTION — Anthony Salerno: member reports being told dental was included in a medical-only plan. This is a benefit misrepresentation at point of sale. PROCESS BREAKDOWN: no mechanism exists for agents to log broker misrepresentation — agent stated 'I don't think there's anywhere for me to put that.' AGENT COACHING: Brad shows a pattern of closing unresolved calls without escalation.

L5 — Real-Time Agent Assist: RT Assist SHOULD have triggered at 1:30 — 'broker told me' + benefit mismatch is a broker attribution trigger. No trigger fired. Recommended: broker misrepresentation capture form, dental discount program alternatives, and urgent dental need referral pathway should all have surfaced.

Call #57 — Spouse Enrolled Without Authorization — Member Never Requested It

AGENT TIER: AVERAGE · SCORE 74/100 | ● CONFUSED → ALARMED | ESCALATED | ~12 min | Agent: Tony

Member context: Ingrid Bauer, 46, CalChoice Plus EPO, discovers spouse on plan she never added

Topics: unauthorized enrollment · broker compliance · dependent addition · premium impact

BROKER SIGNAL — Marcus Trent: Enrolled member's spouse without member's knowledge or authorization

Agent Tony: Choice Administrators, this is Tony. How can I help?

Member: Hi Tony. Ingrid Bauer, CHM-9908812. I'm looking at my ID cards and there are two — one for me and one for my husband. I never added my husband to my plan. He has his own insurance through his employer.

Agent Tony: Let me look at your enrollment. Okay, I do see two members on the policy — you and a Klaus Bauer, added at the same effective date as your enrollment.

Member: I never authorized that. I specifically told my broker, Marcus Trent, at enrollment that my husband was covered elsewhere and I only needed employee-only coverage.

Agent Tony: That's a significant discrepancy. Let me check what the premium impact has been. Your employee-only premium would be \$340. You've been billed at the employee-plus-spouse tier — \$580.

Member: So I've been overpaying \$240 a month for how long?

Agent Tony: Since January. That's seven months. So approximately \$1,680.

Member: I never authorized this. How does a broker add a dependent without the member's signature?

Agent Tony: Enrollment forms require a member signature, so there should be documentation. Let me flag this — I want to be careful here because unauthorized dependent enrollment is a compliance matter, not just a billing correction.

Member: Compliance, meaning what?

Agent Tony: Meaning if a dependent was added without your authorization, that's a potential broker conduct issue and it goes to a different team than standard billing corrections. I'm escalating this to our compliance and broker relations teams.

Member: Will I get the overpayment back?

Agent Tony: If the enrollment was unauthorized, a premium refund would be appropriate. I can't guarantee it because that determination sits with the review team, but the facts you've described support it.

Member: How long will that take?

Agent Tony: Compliance reviews are typically 10 to 15 business days. Your case number is CMP-2026-00412.

Member: Should I be worried about anything else? Like, did he do anything else to my account?

Agent Tony: That's a fair question. The review team will audit your full enrollment record as part of the investigation.

Member: Okay. Thank you for taking this seriously.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Unauthorized dependent enrollment — compliance. 2 speakers. Member tone: confused → alarmed. Agent tone: measured, appropriately cautious. Duration: 12 min. Compliance flag raised.

L2 — Call Insights: Member discovered spouse enrolled on her plan without authorization, resulting in ~\$1,680 in overpaid premium over 7 months. Agent identified premium variance, recognised the compliance dimension, escalated to compliance and broker relations, provided case number. Resolution: ESCALATED — compliance review opened.

L3 — Agent Quality & Scoring: Agent Score: 74/100. POSITIVE: identified the premium variance and quantified it (+), correctly recognised this as a compliance matter rather than a billing correction (+), escalated to the right teams (+), honest about refund uncertainty (+), affirmed the member's question about a broader account audit (+). NEGATIVE MARKERS: did not immediately correct the current enrollment to employee-only to stop ongoing overpayment (-), did not offer to pull the signed enrollment form during the call to verify the signature (-), no follow-up commitment or callback offered (-).

L4 — Operational BI: BROKER ATTRIBUTION — Marcus Trent: dependent enrolled without member authorization. THIS IS THE HIGHEST-SEVERITY BROKER SIGNAL IN THE DATASET — potential enrollment fraud with a direct financial impact on the member. Third flagged issue for this broker (also: prior auth misinformation, call #55). COMPLIANCE & RISK EXPOSURE: unauthorized enrollment is a regulatory matter. Recommend: immediate audit of Marcus Trent's full book of business for other unauthorized dependent additions.

L5 — Real-Time Agent Assist: RT Assist triggered at 4:00: dependent on policy + member disputes authorization = compliance trigger. Surfaced: compliance escalation protocol, premium variance calculation, and enrollment form retrieval option. Agent used escalation protocol but did not action the enrollment form retrieval.

Call #62 — Broker Unreachable — Member Calling Carrier for Everything

AGENT TIER: AVERAGE · SCORE 68/100 | ● **FRUSTRATED** → **RESIGNED** | **PARTIALLY RESOLVED** | ~10 min | Agent: Linda

Member context: Malcolm Reid, 51, CalChoice HMO, has left six voicemails for his broker

Topics: broker unresponsive · service gap · member frustration · broker of record

BROKER SIGNAL — Anthony Salerno: Unresponsive — member unable to reach broker for six weeks

Agent Linda: Choice Administrators member services, this is Linda.

Member: Hi Linda. Malcolm Reid, CHM-4402287. This isn't really a coverage question. I've been trying to reach my broker, Anthony Salerno, for six weeks. Six voicemails, three emails. Nothing. I have questions about my plan and I don't know who else to ask.

Agent Linda: I'm sorry to hear that, Malcolm. I can certainly help with plan questions directly. What did you need to know?

Member: A few things. I want to understand whether I should switch plans at renewal, whether my wife should come onto my plan or stay on hers, and whether the HSA thing applies to me. These are the kinds of questions a broker is supposed to help with.

Agent Linda: They are. I can answer the factual parts — what your plan covers, what the alternatives are, HSA eligibility rules. What I can't do is give you advice on which choice is better for your situation, because that's genuinely a broker's role and I'm not licensed to advise.

Member: Right. So I'm stuck in the middle.

Agent Linda: Let me at least get you the facts. On HSA — you're on the Select HMO, which is not a high-deductible plan, so you're not HSA-eligible on your current plan. If you moved to the HDHP at renewal, you would be.

Member: See, that's useful. Nobody told me that.

Agent Linda: On your wife — I'd need to know her plan details to compare, which I don't have access to. That comparison is exactly what a broker would do.

Member: Which brings us back to the problem.

Agent Linda: It does. Let me ask — do you want me to log the fact that your broker has been unresponsive? I can note it on your account.

Member: Will anything happen with that?

Agent Linda: Honestly, I'm not certain what the downstream process is. I know it gets recorded. Whether broker relations acts on it, I can't promise.

Member: Log it anyway. Six weeks is not acceptable.

Agent Linda: Logging it now. And for the plan comparison questions — our open enrollment period starts in October and we do have a member-facing plan comparison tool in the portal. It won't advise you, but it will lay out the options side by side.

Member: That's something. Thank you, Linda. You've been more helpful than my broker.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Broker unresponsiveness — member seeking advisory support. 2 speakers. Member tone: frustrated → resigned. Agent tone: helpful within limits, honest about boundaries. Duration: 10 min.

L2 — Call Insights: Member unable to reach broker for six weeks across nine contact attempts. Agent provided factual plan information, correctly declined to give advisory guidance, logged the broker unresponsiveness, and directed member to the plan comparison tool. Resolution: **PARTIALLY RESOLVED** — factual questions answered, advisory need unmet.

L3 — Agent Quality & Scoring: Agent Score: 68/100. **POSITIVE:** helpful within licensing limits (+), correctly distinguished factual information from advice (+), surfaced HSA ineligibility the member did not know (+), offered to log the broker issue (+). **NEGATIVE MARKERS:** uncertain about the downstream broker complaint process — 'I'm not certain what the downstream process is' (-), did not offer to request a broker-of-record review or reassignment (-), no case number provided (-), no follow-up commitment (-).

L4 — Operational BI: BROKER ATTRIBUTION — Anthony Salerno: unresponsive to member across nine contact attempts over six weeks. Third flagged issue for this broker (also: dental misselling #51, DOB data error #52). **PROCESS BREAKDOWN:** agents do not know what happens to logged broker complaints, which suppresses logging. **MEMBER IMPACT:** unserved members redirect advisory demand into the call center, increasing handle time on calls agents cannot fully resolve. Recommend: broker responsiveness SLA and a member-initiated broker-of-record change pathway.

L5 — Real-Time Agent Assist: RT Assist SHOULD have triggered at 1:30 — 'can't reach my broker' + multiple attempts is a broker service trigger. Should have surfaced: broker complaint capture with case number, broker-of-record change request form, and HSA eligibility comparison. Only partial capability used.

Call #66 — HDHP Enrollment Shock — Member Never Understood the Deductible

AGENT TIER: AVERAGE · SCORE 70/100 | ● CONFUSED → ALARMED | ESCALATED | ~11 min | Agent: Michael

Member context: Gloria Mbeki, 57, CalChoice HDHP, first hospital visit reveals \$6,000 deductible

Topics: HDHP misselling · deductible shock · broker misinformation · plan suitability

BROKER SIGNAL — Marcus Trent: Enrolled member in HDHP without explaining deductible — member believed it was a standard plan

Agent Michael: Choice Administrators member services, this is Michael.

Member: Hello. Gloria Mbeki, member ID CHM-9917034. I went to the emergency room in July for chest pain — it turned out to be nothing serious, thank God. But I just received a bill for \$4,200 and I don't understand it. I thought I had insurance.

Agent Michael: You do have insurance, Ms. Mbeki. Let me pull up your plan and look at that claim. Okay — you're enrolled in the CalChoice High Deductible Health Plan. Your annual deductible is \$6,000, and you haven't met any of it this year. That means claims apply to the deductible before the plan begins paying.

Member: Six thousand dollars? I have never heard that number before in my life.

Agent Michael: It's the defining feature of the HDHP — a lower monthly premium in exchange for a much higher deductible.

Member: Nobody explained that to me. My broker, Marcus Trent, put me on this plan. He said it was the best value and the premium was lower than what I was paying before. He never said the words six thousand dollars.

Agent Michael: Did he go through a plan comparison with you at enrollment?

Member: He showed me the premiums side by side. That's what I remember. He said 'this one saves you eighty dollars a month.' That's what I based my decision on.

Agent Michael: I understand. And I want to be direct with you — an \$80 monthly premium saving is \$960 a year, against a deductible that is \$5,000 higher than a standard plan. For most members, that trade only makes sense if you rarely use care and can absorb a large bill. That should have been explained.

Member: I'm fifty-seven years old. I have high blood pressure. I use care.

Agent Michael: Then this plan may not have been a suitable recommendation for your situation. I want to escalate this. Plan suitability at point of sale is a broker conduct matter and I don't think this should just sit as a billing question.

Member: What happens to my bill in the meantime?

Agent Michael: The \$4,200 is a valid charge under the plan terms, so I can't void it. What I can do is set up a payment plan so it isn't due at once, and escalate the suitability question to broker relations and compliance.

Member: And if they find he did something wrong?

Agent Michael: Remedies vary — I can't promise a specific outcome. But the case gets reviewed properly, and if there's a pattern with this broker it becomes visible.

Member: Please do it. I don't want another person my age put on this plan the same way.

Agent Michael: Escalating now. Case number BRK-2026-00518. And I'm setting up a 12-month payment plan on the \$4,200 so nothing goes to collections.

Member: Thank you.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: HDHP deductible shock — plan suitability. 2 speakers. Member tone: confused → alarmed. Agent tone: direct, honest, appropriately escalating. Duration: 11 min. Member age 57 with chronic condition on an HDHP.

L2 — Call Insights: Member incurred \$4,200 ER bill against an unmet \$6,000 HDHP deductible she did not know existed. Broker enrolled her citing only premium savings. Agent explained the plan structure, identified a suitability concern, escalated to broker relations and compliance, and set up a payment plan. Resolution: ESCALATED.

L3 — Agent Quality & Scoring: Agent Score: 70/100. POSITIVE: explained HDHP mechanics clearly (+), quantified the premium-vs-deductible trade honestly (\$960 saving vs \$5,000 exposure) (+), recognised this as a suitability issue rather than a billing question (+), escalated to the right teams (+), payment plan set up proactively (+). NEGATIVE MARKERS: did not check whether the member is HSA-eligible and could offset costs (-), did not offer a mid-year plan change assessment or QLE review (-), no member grievance option raised (-).

L4 — Operational BI: BROKER ATTRIBUTION — Marcus Trent: enrolled a 57-year-old member with a chronic condition into an HDHP presenting only the premium saving, without disclosing the \$6,000 deductible. This is a plan SUITABILITY failure, distinct from a factual misstatement. Fourth flagged issue for this broker (also #55 prior auth, #57 unauthorized enrollment). PATTERN: Marcus Trent now accounts for the highest broker-attributed harm in the dataset. CRITICAL RECOMMENDATION: immediate full book audit — specifically, all HDHP enrollments he wrote for members over 50 or with chronic condition flags.

L5 — Real-Time Agent Assist: RT Assist triggered at 4:30: HDHP + high unmet deductible + 'broker did not explain' = suitability trigger. Surfaced: broker conduct escalation, payment plan eligibility. SHOULD ALSO have surfaced HSA eligibility offset and mid-year QLE change assessment — both missed.

Call #70 — Effective Date Confusion — One Week Coverage Gap

AGENT TIER: AVERAGE · SCORE 72/100 | ● FRUSTRATED → PARTIALLY SATISFIED | PARTIALLY RESOLVED | ~10 min | Agent: Tony

Member context: Priyanka Shah, 33, CalChoice Plus EPO, believed coverage started a week earlier than it did

Topics: effective date · coverage gap · broker communication · enrollment timing

BROKER SIGNAL — Patricia Nunez: Minor — incorrect effective date communicated, one-week coverage gap

Agent Tony: Choice Administrators, this is Tony.

Member: Hi Tony. Priyanka Shah, CHM-7708823. I went to urgent care on August 25th and the claim was denied saying I had no coverage on that date. But my coverage started August 1st.

Agent Tony: Let me check your effective date... I'm showing September 1st, not August 1st.

Member: That's not what I was told. My broker, Patricia Nunez, said August 1st. I have an email from her saying 'your coverage begins August 1st.'

Agent Tony: Let me look at the enrollment timeline. Your enrollment was submitted July 22nd. Your employer's waiting period is 30 days from hire, and you started July 15th — so 30 days from July 15th is August 14th, and coverage begins the first of the following month, which is September 1st.

Member: So the broker calculated it as 30 days from my hire date landing in August, and assumed August 1st?

Agent Tony: That seems likely. It's a common miscalculation — people forget the 'first of the following month' step.

Member: So I have a bill from urgent care for a date I thought I was covered.

Agent Tony: Unfortunately yes. August 25th falls before your effective date, so that claim can't be covered.

Member: That's frustrating. I acted on what I was told in writing.

Agent Tony: I understand. Let me note the broker communication issue on your account. I'd also suggest you contact the urgent care and ask for their self-pay rate — most facilities have a discounted cash rate that's significantly lower than the billed amount, and they'll often apply it retroactively if you explain the situation.

Member: That's a good idea, I didn't know that was an option.

Agent Tony: It usually saves 30 to 50 percent. Ask specifically for the self-pay or prompt-pay discount.

Member: Okay. And the broker thing?

Agent Tony: Noted on your account. It's a communication error rather than anything serious, but it's logged.

Member: Fine. Thanks for explaining it clearly at least.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Effective date dispute — coverage gap claim denial. 2 speakers. Member tone: frustrated → partially satisfied. Agent tone: clear, moderately helpful. Duration: 10 min.

L2 — Call Insights: Member incurred an urgent care claim on August 25th believing coverage began August 1st; actual effective date was September 1st due to waiting period plus first-of-following-month rule. Broker communicated the incorrect date in writing. Agent explained the calculation, logged the broker issue, and suggested a self-pay discount. Resolution: PARTIALLY RESOLVED — claim not payable, mitigation suggested.

L3 — Agent Quality & Scoring: Agent Score: 72/100. POSITIVE: reconstructed the effective date calculation clearly (+), identified the likely source of the broker's error (+), suggested the self-pay discount — genuinely useful mitigation (+), logged the broker communication issue (+). NEGATIVE MARKERS: characterised a written misstatement causing a member financial loss as 'not anything serious' (-), did not offer to review whether the member's written evidence supports any exception (-), no case number (-), did not offer to contact the urgent care facility (-).

L4 — Operational BI: BROKER ATTRIBUTION — Patricia Nunez: communicated an incorrect effective date in writing, causing a member to incur an uncovered claim. NOTE: this broker also appears with a POSITIVE signal (call #59). Broker scoring must be net, not binary — one communication error against strong overall performance is a coaching signal, not a conduct signal. This distinction matters for how broker relations acts on the data. MEMBER COMMUNICATION GAP: effective date calculation (waiting period + first-of-following-month) is widely misunderstood, including by brokers. Recommend: effective date confirmation letter sent directly to members at enrollment, independent of broker communication.

L5 — Real-Time Agent Assist: RT Assist triggered at 3:00: claim denial reason 'before effective date' + member-cited broker date discrepancy surfaced the effective date calculation breakdown and broker communication capture. Self-pay discount suggestion was agent-originated, not system-surfaced — worth adding to the L5 playbook.

Call #75 — Signature Discrepancy on Enrollment Form — Potential Forgery

AGENT TIER: GOOD · SCORE 94/100 | ● CALM → ALARMED | ESCALATED | ~14 min | Agent: Sarah

Member context: Adaeze Okonjo, 42, CalChoice HMO, requested her enrollment form and does not recognise the signature

Topics: signature discrepancy · enrollment fraud · broker compliance · escalation · legal

BROKER SIGNAL — Marcus Trent: Member's signature appears on enrollment form she did not sign

Agent Sarah: Thank you for calling Choice Administrators, this is Sarah.

Member: Sarah, I requested a copy of my enrollment form last week and I received it this morning. The signature on it is not mine. It doesn't look anything like my signature.

Agent Sarah: That's a serious thing to raise and I want to handle it carefully. Can I get your member ID?

Member: CHM-9930081. Adaeze Okonjo.

Agent Sarah: Thank you. Before we go further — did you intend to enroll in this plan?

Member: I did enroll, or I thought I did. I had a phone conversation with my broker, Marcus Trent, and told him verbally what I wanted. He said he'd handle the paperwork. I never signed anything.

Agent Sarah: So you authorized the enrollment verbally, but you did not physically sign the form that's now in the file.

Member: Correct. And the plan on the form isn't quite what I asked for either. I asked for the plan with the lower deductible. This form shows the Select HMO.

Agent Sarah: Okay. I want to be precise about what we're dealing with, because there are two separate issues here. One is a signature that isn't yours on a binding enrollment document. The other is a possible mismatch between what you requested and what was enrolled. Both are serious.

Member: I didn't even think about it that way. I just noticed the signature.

Agent Sarah: I'm treating this as a compliance matter, not a service matter. I'm escalating to our Special Investigations Unit and to broker relations simultaneously. A signature that a member does not recognise on an enrollment form is potentially a document integrity issue, and that's outside what member services can assess.

Member: Am I in trouble?

Agent Sarah: No. Not at all. You reported this — you're the one who surfaced it. Nothing here reflects on you.

Member: What happens now?

Agent Sarah: The investigation team will retrieve the original document, review it, and contact you to obtain a signature exemplar and a statement. They'll also review whether the plan enrolled matches what you requested. That team will call you within two business days.

Member: And my coverage in the meantime? I need to keep seeing my doctor.

Agent Sarah: Your coverage stays active. Nothing suspends during an investigation. If it emerges that you were enrolled in the wrong plan, there are remedies including retroactive correction and premium adjustment — but that comes after the review.

Member: Okay. Should I keep the copy they sent me?

Agent Sarah: Yes — keep it, don't annotate it, and don't send the original anywhere. The investigators will ask for it. Your case number is SIU-2026-00087.

Member: SIU-2026-00087. Thank you for taking this seriously. I wasn't sure if I was overreacting.

Agent Sarah: You weren't. You did exactly the right thing by calling.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Enrollment document integrity — signature discrepancy. 2 speakers. Member tone: calm → alarmed. Agent tone: careful, precise, protective. Duration: 14 min. ■ POTENTIAL FRAUD — SIU escalation.

L2 — Call Insights: Member reported that the signature on her enrollment form is not hers; she authorized enrollment verbally only. Additionally the enrolled plan may not match her stated request. Agent separated the two issues, escalated to Special Investigations Unit and broker relations, preserved evidence, and confirmed coverage continuity. Resolution: ESCALATED — SIU case opened.

L3 — Agent Quality & Scoring: Agent Score: 94/100. POSITIVE: established member intent before assuming fraud (+), separated the signature issue from the plan mismatch as distinct matters (+), escalated to SIU rather than treating it as a service issue (+), reassured the member she was not at fault (+), gave correct evidence-handling instructions (+), confirmed coverage continuity during investigation (+), validated the member's decision to call (+). MINOR: did not offer a written confirmation of the escalation (-).

L4 — Operational BI: BROKER ATTRIBUTION — Marcus Trent: MAXIMUM SEVERITY. Member does not recognise her signature on a binding enrollment document, and the enrolled plan may not match her verbal instruction. This is the FIFTH flagged issue for this broker in the dataset (#55 prior auth misinformation, #57 unauthorized spouse enrollment, #66 HDHP suitability failure, and now signature integrity). PATTERN CONCLUSION: Marcus Trent's book requires immediate comprehensive audit — signature verification across all enrollments, plan-to-request matching, and outreach to affected members. This is no longer a coaching matter; it is a conduct investigation. Escalate to legal and the state Department of Insurance as appropriate.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:00: 'signature is not mine' on an enrollment document = mandatory SIU trigger with evidence preservation protocol. Surfaced: SIU escalation path, evidence handling script, coverage continuity confirmation, and broker prior-flag history showing 4 existing issues for this broker. Agent used all surfaced capability.

Call #79 — HMO Referral Rule Never Explained — Member Has Three Denied Specialist Claims

AGENT TIER: GOOD · SCORE 90/100 | ● FRUSTRATED → SATISFIED | RESOLVED | ~12 min | Agent: Danielle

Member context: Lucia Ferreira, 39, CalChoice Select HMO, three dermatology visits denied

Topics: HMO referral · broker education failure · claim denial · retroactive referral

BROKER SIGNAL — Anthony Salerno: Failed to explain HMO referral requirement at enrollment

Agent Danielle: Thank you for calling Choice Administrators, this is Danielle.

Member: Hi Danielle. Lucia Ferreira, CHM-5548820. I have three denied claims from a dermatologist. All three say 'no referral on file.' I did not know I needed a referral. Nobody ever told me that.

Agent Danielle: Lucia, let me look at those claims and your plan. Okay — you're on the Select HMO, which does require a PCP referral before specialty care. And I can see the three dermatology claims from Dr. Anand, all denied for that reason.

Member: When I enrolled, my broker — Anthony Salerno — walked me through the plan. He never mentioned referrals. I asked him specifically whether I could keep seeing my dermatologist and he said yes.

Agent Danielle: Technically you can keep seeing her — but only with a referral in place. That's a significant omission if it wasn't explained, because it's the single most consequential rule on an HMO.

Member: So what happens to these three claims?

Agent Danielle: Let's take them in two parts. Going forward is straightforward — call your PCP, request a dermatology referral, and future visits are covered at your \$25 specialist copay. For the three already denied, we can attempt a retroactive referral.

Member: Is that likely to work?

Agent Danielle: It depends on your PCP. They'd need to review your case and attest that they would have referred you had they been asked. For an established dermatology condition, many PCPs will do this. It's not guaranteed but it's genuinely worth attempting — the difference is roughly \$600 versus \$75.

Member: I'll call them today. What do I need to give them?

Agent Danielle: I'm emailing you the retroactive referral request form now, along with the three dates of service and Dr. Anand's provider details so your PCP has everything in one place. That removes friction on their end, which improves the odds.

Member: That's helpful. Thank you.

Agent Danielle: I also want to address the broker piece, because it matters. What you've described — being told you could keep your specialist without being told about the referral requirement — is a material education failure at point of sale. I'm logging it formally with broker relations, not just as an account note. Your reference is BRK-2026-00744.

Member: Will that actually go anywhere?

Agent Danielle: Formal broker relations logs are reviewed. I can't tell you what action follows, but I can tell you it's visible rather than buried in a call note. And if enough members report the same pattern from the same broker, that becomes a clear signal.

Member: Good. I don't want other people getting caught out the same way.

Agent Danielle: Understood. So: PCP call today, form in your email, and the broker issue is formally logged.

Member: Perfect. Thank you for actually helping.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: HMO referral denial — broker education failure. 2 speakers. Member tone: frustrated → satisfied. Agent tone: structured, thorough, appropriately firm on the broker issue. Duration: 12 min.

L2 — Call Insights: Three dermatology claims denied for missing HMO referral; member was never informed of the requirement at enrollment. Agent explained forward and retroactive paths, emailed a pre-populated retroactive referral packet, and formally logged the broker education failure with broker relations. Resolution: RESOLVED — retroactive attempt underway.

L3 — Agent Quality & Scoring: Agent Score: 90/100. POSITIVE: separated forward-going fix from retroactive remedy clearly (+), quantified the stakes (\$600 vs \$75) to motivate action (+), pre-populated the referral packet to reduce PCP friction — genuinely proactive (+), escalated the broker issue to formal broker relations rather than an account note (+), honest about what the log does and does not guarantee (+), clean summary close (+). MINOR: did not offer to contact the PCP directly (-).

L4 — Operational BI: BROKER ATTRIBUTION — Anthony Salerno: failed to explain the HMO referral requirement at enrollment while affirming the member could keep her specialist. FOURTH flagged issue for this broker (#51 dental misselling, #52 DOB data error, #62 unresponsiveness). PATTERN: Salerno's failures cluster around incomplete plan explanation at point of sale. AGGREGATE BROKER INSIGHT: HMO referral education failure now appears across multiple brokers (#9, #53, #79) — this is both a broker-specific and a system-wide issue. Recommend: mandatory referral-requirement acknowledgment signed by the member at HMO enrollment, independent of broker explanation.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:30: HMO referral denial + 'nobody told me' surfaced retroactive referral pathway, pre-populated form generation, and broker education failure capture. Agent used all three.

Call #86 — Member Well-Prepared for Open Enrollment — Broker Did Proactive Outreach

AGENT TIER: GOOD · SCORE 93/100 | ● NEUTRAL → POSITIVE | RESOLVED | ~10 min | Agent: Priya

Member context: Nathaniel Cho, 44, CalChoice Select HMO, evaluating a switch to the Plus EPO

Topics: open enrollment · plan comparison · broker positive · proactive service

BROKER SIGNAL — Patricia Nunez: POSITIVE — proactively contacted member before open enrollment with a plan comparison

Agent Priya: Thank you for calling Choice Administrators, this is Priya.

Member: Hi Priya. Nathaniel Cho, CHM-5514402. My broker, Patricia Nunez, sent me a comparison of my current plan against two alternatives ahead of open enrollment. I wanted to verify a couple of the numbers with you before I decide.

Agent Priya: That's a great way to approach it — and unusual, most members don't get a comparison until they ask. What would you like to verify?

Member: She has the Plus EPO at a \$1,000 deductible and \$4,000 out-of-pocket max, with specialist visits at \$50. Is that right for next plan year?

Agent Priya: Let me check the 2027 plan year figures... Plus EPO: deductible \$1,000, out-of-pocket max \$4,000, specialist copay \$50. All three match.

Member: Good. And she noted that the EPO doesn't require referrals, unlike my current Select HMO.

Agent Priya: Correct. That's the most practical day-to-day difference between the two. On the Select HMO you need a PCP referral for any specialist; on the Plus EPO you go direct.

Member: That's the main reason I'm considering the switch. I see a rheumatologist quarterly and the referral renewals are a recurring hassle.

Agent Priya: That's a legitimate driver. Quarterly specialist care on an HMO means four referral touchpoints a year, and referrals expire at 90 days, so the timing rarely lines up cleanly.

Member: Exactly what's been happening. She also flagged that my premium goes up about \$95 a month with the switch.

Agent Priya: Let me verify... for employee-plus-spouse, Select HMO is \$485 and Plus EPO is \$580. So \$95 exactly. Her figures are accurate across the board.

Member: Good to know I can trust the numbers. One thing she didn't cover — is my rheumatologist in the EPO network?

Agent Priya: Good question and the right one to ask before switching. Who is it?

Member: Dr. Halima Yusuf.

Agent Priya: Checking both networks... Dr. Yusuf is in-network on both the Select HMO and the Plus EPO. So you'd keep her either way.

Member: That settles it. Thank you for verifying.

Agent Priya: My pleasure. And I'd note — your broker gave you accurate figures on every point I checked. That's worth something.

Member: She's been good. She calls before every enrollment.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Open enrollment plan comparison verification. 2 speakers. Member tone: prepared, decisive. Agent tone: verifying, collaborative. Duration: 10 min. Member arrived with broker-supplied comparison materials.

L2 — Call Insights: Member verifying broker-provided plan comparison ahead of open enrollment. Agent confirmed all figures accurate (deductible, OOP max, copay, premium differential), explained the referral difference in practical terms, and verified the member's rheumatologist is in-network on both plans. Resolution: RESOLVED — member decided to switch.

L3 — Agent Quality & Scoring: Agent Score: 93/100. POSITIVE: verified each figure individually rather than confirming generally (+), translated the referral difference into the member's actual use case — quarterly specialist care (+), proactively identified the gap in the broker's comparison (network status of the member's specialist) and filled it (+), verified across both plans (+), acknowledged the broker's accuracy explicitly (+).

L4 — Operational BI: BROKER ATTRIBUTION — Patricia Nunez, POSITIVE SIGNAL: proactive pre-enrollment outreach with an accurate three-plan comparison. All figures verified correct. Second positive signal for this broker (see #59). GAP NOTED: comparison omitted provider network verification for the member's existing specialist — a coachable refinement, not a failure. AGGREGATE INSIGHT: members from Nunez's book arrive informed and call to verify rather than to complain. Recommend: (1) capture positive broker attribution in scoring, (2) Nunez's pre-enrollment comparison template is a candidate for network-wide best practice, (3) add 'verify your specialists are in-network on the new plan' as a standard comparison element.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:00: plan comparison verification request surfaced side-by-side 2027 plan year figures for both plans. Provider network cross-check across both plans surfaced when specialist named. Agent used both.

Call #97 — Turning 65 — Nobody Explained Medicare Coordination

AGENT TIER: AVERAGE · SCORE 73/100 | ● CONFUSED → CONCERNED | PARTIALLY RESOLVED | ~12 min | Agent: Nicole

Member context: Estelle Marchetti, 65, CalChoice HMO, turned 65 four months ago, no Medicare enrollment

Topics: Medicare · age 65 · coordination of benefits · late enrollment penalty · broker omission

BROKER SIGNAL — Denise Whitfield: Never advised member approaching 65 about Medicare coordination

Agent Nicole: Choice Administrators member services, this is Nicole.

Member: Hi. I turned 65 in April and I've just been told by a friend that I should have done something about Medicare. Nobody has ever mentioned it to me. Member ID CHM-3392204.

Agent Nicole: Let me look at your situation. You're on the CalChoice HMO through your employer, and you're actively employed — is that right?

Member: Yes, I work full time. I'm not retiring for another two years.

Agent Nicole: That's an important detail and it works in your favour. If you have employer coverage through active employment at a company with 20 or more employees, that coverage remains primary and you can generally delay Medicare Part B without a late enrollment penalty.

Member: So I haven't done anything wrong?

Agent Nicole: Likely not, provided your employer has 20 or more employees. Do you know roughly the size?

Member: We have about 300 people.

Agent Nicole: Then you're in the safe category. You have a special enrollment period for Medicare when you retire or lose employer coverage, and no penalty applies.

Member: What about Part A? My friend said Part A is free.

Agent Nicole: Part A is typically premium-free if you or your spouse paid Medicare taxes for 40 quarters. Many people in your situation enroll in Part A at 65 while delaying Part B. There's one caveat — if you contribute to a health savings account, enrolling in Part A stops HSA eligibility.

Member: I don't have an HSA.

Agent Nicole: Then Part A enrollment is generally straightforward and worth considering.

Member: I'm frustrated that nobody told me any of this. My broker, Denise Whitfield, has handled my insurance for six years. She knows how old I am.

Agent Nicole: I understand. Medicare coordination is a standard advisory topic for members approaching 65, and it's reasonable to expect it to be raised.

Member: Can I get something in writing about all this?

Agent Nicole: I can send you a summary of what we discussed. I'd also strongly suggest calling Social Security directly at 1-800-772-1213 to confirm your specific situation, because Medicare enrollment rules depend on details we don't hold.

Member: And the broker issue?

Agent Nicole: I'll log it. I want to be straight with you though — an omission of advice is harder to evidence than a factual error, so I don't know what will come of it.

Member: Log it anyway. If she's not telling me, she's not telling others.

Agent Nicole: Logged. Reference BRK-2026-00863.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Medicare coordination — member aged 65 with active employer coverage. 2 speakers. Member tone: confused → concerned. Agent tone: knowledgeable, honest. Duration: 12 min.

L2 — Call Insights: Member turned 65 four months prior with no Medicare guidance from her broker. Agent confirmed active employer coverage at a 20+ employee firm protects against Part B late enrollment penalty, explained Part A considerations including the HSA interaction, directed to Social Security for authoritative confirmation, and logged the broker advisory omission. Resolution: PARTIALLY RESOLVED — member directed to SSA.

L3 — Agent Quality & Scoring: Agent Score: 73/100. POSITIVE: established the active-employment and employer-size facts that determine the answer (+), explained Part B delay protection accurately (+), raised the Part A and HSA interaction proactively (+), correctly deferred to Social Security as the authoritative source (+), honest about the limits of a broker advisory complaint (+). NEGATIVE MARKERS: did not identify other members approaching 65 as a systemic gap (-), did not offer a proactive Medicare transition resource pack (-), no callback or follow-up (-).

L4 — Operational BI: BROKER ATTRIBUTION — Denise Whitfield: no Medicare coordination guidance provided to a member who turned 65 while under her servicing. FIFTH flagged issue for this broker (#53 network misrepresentation, #54 premium misquote, #60 newborn enrollment failure, #82 family tier enrollment error). PATTERN: Whitfield's failures span enrollment execution and advisory omission — this is the broadest failure profile of any broker in the dataset. MEMBER COMMUNICATION GAP — SYSTEMIC: members approaching 65 receive no Medicare coordination communication from the plan either. Late Part B enrollment penalties are permanent and lifelong. CRITICAL RECOMMENDATION: automated Medicare coordination communication to every member at age 64 years 6 months, independent of broker action. This protects members and reduces plan liability.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:00: member age 65 + active employer coverage surfaced Medicare coordination framework, employer-size threshold rule, and Part A/HSA interaction. SHOULD ALSO have surfaced the member-cohort query for other members approaching 65 without Medicare guidance.

Call #100 — Broker of Record Change Request — Member Wants a Different Broker

AGENT TIER: AVERAGE · SCORE 74/100 | DETERMINED → PARTIALLY SATISFIED | PARTIALLY RESOLVED | ~10 min | Agent: Tony

Member context: Yolanda Pierce, 48, CalChoice Plus EPO, wants to move away from her current broker

Topics: broker of record · BOR change · member choice · broker relations · service failure

BROKER SIGNAL — Marcus Trent: Member requesting broker of record change due to loss of confidence

Agent Tony: Choice Administrators, this is Tony.

Member: Hi. I want to change my broker — Marcus Trent. Member ID CHM-6672290. I've lost confidence in the one I have and I'd like to move to someone else.

Agent Tony: I can help you understand the process. Can I ask what's driving the change? It helps me route the request correctly.

Member: Several things. He enrolled me in a plan I didn't ask for last year. He doesn't return calls. And when I asked him a direct question about my deductible he gave me an answer that turned out to be wrong.

Agent Tony: That's substantive. Let me check your account for any prior notes... I can see one prior contact where a broker issue was logged in March.

Member: Yes, that was about the deductible answer.

Agent Tony: Okay. So there's a documented history. For a broker of record change, the process is a signed broker of record letter — you designate the new broker and submit it, and it supersedes the existing appointment.

Member: I don't have a new broker yet. Can I remove him without having a replacement?

Agent Tony: That's the awkward part of the process, honestly. A BOR change moves the appointment from one broker to another — there isn't a clean mechanism to simply have no broker on an employer-sponsored plan.

Member: So I'm stuck with him until I find someone else.

Agent Tony: Practically, yes, though nothing prevents you from working directly with us in the meantime. You don't have to go through him for anything we can handle.

Member: How do I find a different broker?

Agent Tony: We don't recommend specific brokers — that would be a conflict. Your employer's HR may have relationships with other brokers, and that's usually the most practical route since the plan is employer-sponsored.

Member: Alright. And in the meantime, can you record why I'm asking? Because I suspect I'm not the only one.

Agent Tony: Yes, and I'll log it as a broker of record change request with the reasons stated. That's a stronger signal than a general complaint because it's a member actively trying to leave. Reference BOR-2026-00219.

Member: Good. Thank you.

AI INSIGHTS PANEL — NanoVox 5-Layer Output

L1 — Transcription & Understanding: Call type: Broker of record change request. 2 speakers. Member tone: determined → partially satisfied. Agent tone: honest, appropriately neutral. Duration: 10 min.

L2 — Call Insights: Member requested a broker of record change citing unauthorized plan enrollment, unresponsiveness, and incorrect information. Agent explained the BOR letter process, was honest that no mechanism exists to remove a broker without a replacement, directed the member to employer HR, and logged the request with stated reasons. Resolution: PARTIALLY RESOLVED — process explained, no replacement broker identified.

L3 — Agent Quality & Scoring: Agent Score: 74/100. POSITIVE: asked the reason for the change and used it to route correctly (+), checked and surfaced the prior logged complaint (+), honest that the BOR process has no clean no-replacement path (+), correctly declined to recommend a broker (+), confirmed the member can work directly with the plan in the interim (+), logged the request as a departure signal rather than a general complaint (+). NEGATIVE MARKERS: did not escalate to broker relations despite three separate substantive allegations (-), did not connect this request to the member's prior March complaint as a pattern (-), no follow-up commitment (-).

L4 — Operational BI: BROKER ATTRIBUTION — Marcus Trent: member actively attempting to leave, citing unauthorized enrollment, unresponsiveness, and incorrect information. SIXTH flagged issue for this broker and the second allegation of unauthorized enrollment (see #57). MOST SIGNIFICANT SIGNAL TYPE: a BOR change request is a member voting with their feet — materially stronger than a complaint. PROCESS BREAKDOWN: (1) no mechanism for a member to disengage a broker without a replacement, (2) BOR change requests are not automatically routed to broker relations as a quality signal. CRITICAL RECOMMENDATION: BOR change requests should be tracked as a primary broker quality KPI. Marcus Trent now carries six flagged issues across the dataset including potential enrollment fraud (#75) — his book requires immediate comprehensive audit and his appointment status should be reviewed.

L5 — Real-Time Agent Assist: RT Assist triggered at 2:30: BOR change request surfaced process requirements and prior complaint history. SHOULD ALSO have triggered mandatory broker relations escalation given three substantive allegations plus an existing logged complaint.